

Praxia Update

Daily orthopaedic literature digest · Sunday, 20 September 2026 · 492 new articles across 10 journals



American Journal of Sports Medicine

Volume 54, Issue 11 · September 2026 · 32 new articles

Deeper Dive Into Methodology: Kaplan-Meier Curves.

Carey JL

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Adductor Canal Block and Local Anesthetic Versus Local Anesthetic Alone in ACL Reconstruction: A Double-Blind Randomized Controlled Trial.

Ojaghi R, Locke E, Elmi P, Pickell M

BACKGROUND: Effective postoperative analgesia is crucial for early recovery after anterior cruciate ligament reconstruction (ACLR). Local infiltration analgesia (LIA) and adductor canal block (ACB) are common regional techniques, but their combined efficacy remains unclear.

PURPOSE: To compare the effectiveness of LIA alone versus LIA combined with ACB in patients undergoing ACLR, with primary outcomes including postoperative opioid consumption and quadriceps function.

STUDY DESIGN: Randomized controlled trial; Level of evidence, 1.

METHODS: A double-blind randomized controlled trial enrolled 100 patients undergoing ACLR under general anesthesia. Patients were randomized into 2 groups: LIA + sham (saline injection) (n = 50) and LIA + ACB (n = 50). The primary outcome was postoperative opioid consumption in the first 24 hours. Secondary outcomes included visual analog scale (VAS) pain score, quadriceps function assessed by straight leg raise (SLR) at 3 hours, Quality of Recovery-15 (QoR-15) score, and Knee Injury and Osteoarthritis Outcome Score (KOOS) at 1 week. Statistical analysis was performed using t tests and chi-square tests with a P value <.05 considered significant.

RESULTS: There was no significant difference in 24-hour opioid consumption between the LIA + ACB and LIA-only groups (P = .109). Similarly, VAS pain scores at 24 hours postoperatively showed no significant difference [...]

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Outcomes of Suture Tape-Augmented ACL Reconstruction in Patients With High Internal Rotational Tibial Subluxation: A Minimum 3-Year Cohort Study.

Wang Y, Zhang Y, Shi W, Wang J, Wang H, Liu Y, Yuan F, Ao Y et al.

BACKGROUND: Anterior cruciate ligament reconstruction (ACLR) using hamstring tendon (HT) autografts faces persistent challenges of graft failure, especially in patients with high internal rotational tibial subluxation (IRTS).

HYPOTHESIS: Patients with high IRTS who undergo HT autograft ACLR with suture tape augmentation (STA) will have lower graft failure rates and superior clinical outcomes as compared with those without STA.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: This retrospective cohort study included patients with high IRTS-defined as lateral minus medial anterior tibial subluxation >5.8 mm based on prior studies-who underwent primary ACLR using HT autografts with STA and had a minimum follow-up of 3 years. Propensity score matching (1:1) was performed to identify a control group of patients with similarly high IRTS who underwent HT ACLR without STA. Postoperative outcomes were assessed by the International Knee Documentation Committee score, Lysholm score, and Tegner activity scale, as well as by return-to-sport status and graft failure. Clinically meaningful improvements were evaluated by the minimal clinically important difference, Patient Acceptable Symptom State, and substantial clinical benefit.

RESULTS: This study included 62 patients with high IRTS: 31 with STA and 31 matched controls. The mean IRTS was 6.6 mm (range, 5.9-8.9) in the STA gro [...]

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Primary ACL Repair and Reconstruction in Isolated ACL Ruptures: Forgotten Joint Scores and the Association Between Residual Laxity and Joint Awareness.

Altun O, Ergi Y, Altıntaş M, Özdemir E, Dağar U, Bozkurt M

BACKGROUND: Primary anterior cruciate ligament (ACL) repair has recently reemerged as a treatment option for carefully selected proximal ACL tears. However, evidence regarding patient-reported joint awareness and the relationship between postoperative laxity and joint awareness remains limited.

PURPOSE: To compare joint awareness, clinical outcomes, and postoperative laxity between primary ACL repair and hamstring tendon autograft reconstruction in carefully selected patients with isolated ACL rupture, and to explore the association between residual laxity and postoperative joint awareness.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: This retrospective cohort study included 85 patients with isolated ACL rupture treated with either primary ACL repair (n = 26) or hamstring autograft reconstruction (n = 59), with a minimum follow-up of 24 months. Clinical outcomes, including visual analog scale score, Lysholm score, International Knee Documentation Committee (IKDC) score, Tegner activity scale score, postoperative knee laxity, return to sport, and rerupture rates, were evaluated. The authors also used the Forgotten Joint Score-12 (FJS-12), a measure of joint awareness, to evaluate patients. A higher score reflects lower joint awareness, suggesting function more similar to a native knee. Multivariable regression analyses were performed to evaluate variables associ [...]

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Quadriceps Tendon Autografts Are Associated With Increased Short-term Reoperation Rates for Extension Deficit or Pain and Inferior Patient-Reported Outcomes Compared With Hamstring Tendon Autografts: An Analysis of 5653 Cases of Primary Anterior Cruciate Ligament Reconstruction.

Kekki C, Cristiani R, Ståhlman A, von Essen C

BACKGROUND: Anterior cruciate ligament reconstruction (ACLR) rates are rising, and quadriceps tendon (QT) autografts have gained popularity in recent years. Although QT autografts show objective and subjective outcomes comparable to hamstring tendon (HT) and bone-patellar tendon-bone (BPTB) autografts, nonrevision reoperations across graft types and their impact on patient-reported outcome measure (PROM) scores remain poorly studied.

PURPOSE: To compare nonrevision reoperation rates and PROM scores between primary ACLR using HT, BPTB, and QT autografts at 2 years' follow-up.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: In this retrospective cohort study, 5653 primary ACLR procedures (2015-2022) were categorized by autograft type. Reoperations for extension deficit or pain, meniscal injuries, or cartilage injuries within 2 years were identified through medical records. Revision ACLR and graft failure were not included. Patient-reported outcomes were assessed preoperatively and at 2 years postoperatively using the Knee Injury and Osteoarthritis Outcome Score (KOOS). The minimal important change, patient acceptable symptom state, and treatment failure were evaluated using the KOOS4. Subgroup analysis examined associations between graft diameter, sex, and reoperations for extension deficit or pain.

RESULTS: The QT group had a higher overall reoperation rate (19.2% [...])

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Sociodemographic Predictors of Postpresentation Delay to Pediatric ACL Reconstruction and Associated Intra-articular Injury Severity.

Hamad CD, Wu S, Joachim K, Chung E, Liu T, Silva M, Bowen R, Baghdadi S

BACKGROUND: Delayed anterior cruciate ligament reconstruction (ACLR) in pediatric patients is associated with progressive intra-articular injury. Although disparities in access to orthopaedic care have been reported, it remains unclear whether structural determinants of health influence postpresentation surgical delay and downstream functional recovery.

PURPOSE: To determine whether sociodemographic and structural determinants are associated with prolonged postpresentation delay to pediatric ACLR and with intra-articular injury severity and postoperative functional outcomes.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: The records of 443 pediatric patients who underwent ACLR (2013-2025) at an urban academic referral center with a pediatric orthopaedic urgent care were reviewed. Exposures included race/ethnicity, insurance type, and neighborhood deprivation (Childhood Opportunity Index [COI]). Delay was defined as time from first orthopaedic presentation to surgery, with prolonged delay defined as >120 days. Outcomes included high-grade cartilage injury (Outerbridge grade 3 or 4), meniscal tear presence, failure to return to sport (≥ 365 days of follow-up), hop test symmetry, isometric strength symmetry, and graft failure. Multivariable regression models were adjusted for race/ethnicity, age, sex, insurance, and national COI.

RESULTS: The median time from injury [...]

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Association of Early Knee Extension Range of Motion Deficits With Cartilage T2 Relaxation Following Anterior Cruciate Ligament Reconstruction.

Allison A, Harrington J, Weaver B, Manzer M, Sajja BR, Tao MA, Wellsandt E

BACKGROUND: Anterior cruciate ligament (ACL) injury significantly increases the risk for developing knee osteoarthritis (OA), yet the early contributors to cartilage degeneration remain poorly understood. While range of motion (ROM) deficits after ACL reconstruction (ACLR) are associated with long-term development of radiographic OA, the association between early ROM recovery and cartilage composition has not been established.

HYPOTHESIS: Knee extension ROM deficits at 2 months after ACLR would be associated with worsening tibiofemoral and patellofemoral cartilage T2 relaxation times at 6 months.

STUDY DESIGN: Cohort study; Level of evidence, 2.

METHODS: A total of 30 participants (15-35 years) were enrolled within 1 month of ACL injury and before ACLR. At 2 months post-ACLR, active knee extension ROM was measured in the supine position using a goniometer. An extension deficit was considered $>3^\circ$ less extension in the injured knee compared to the uninjured side. Quantitative magnetic resonance imaging-based T2 relaxometry of the injured knee was performed preoperatively and at 6 months post-ACLR to measure percent change in mean T2 relaxation times in predefined tibial and femoral cartilage regions. Independent t tests were used to compare changes in T2 values between participants with and without an extension deficit.

RESULTS: The mean age of the participants was 18.8 ± 3.9 [...]

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Changes in ACL T2* Metrics Over the Course of the Female Menstrual Cycle: A New Biomarker for the ACL Injury Risk?

Argentieri E, Hamilton TW, Koff MF, Koch KM, Potter HG

BACKGROUND: Sex-based disparities in the anterior cruciate ligament (ACL) injury risk may be partly explained by cyclic variations in sex hormones that drive systemic shifts in tissue osmoregulation. Ultrashort echo time (UTE) T2* magnetic resonance imaging provides a noninvasive, quantitative means of evaluating the water content and collagen orientation in tissue.

HYPOTHESIS: Eumenorrheic female participants will exhibit significant ACL T2* changes across the cycle, while anovulatory control participants will demonstrate no significant changes over time.

STUDY DESIGN: Cohort study; Level of evidence, 2.

METHODS: A total of 25 women with no prior knee injuries were enrolled: 10 premenopausal, eumenorrheic participants and 15 anovulatory control participants (6 postmenopausal and 9 premenopausal using oral contraceptives). Bilateral knee magnetic resonance imaging was performed at 4 time points evenly spaced over 1 month, with the first visit within 24 hours of menses onset. Ovulation status was confirmed using commercially available ovulation predictor kits. Three-dimensional UTE sequences (11 echoes; 0.03-25 ms) were acquired to evaluate bicomponent ACL T2* metrics. Linear mixed-effects models assessed temporal differences in long and short T2* values, and the Cohen d quantified effect size.

RESULTS: Eumenorrheic participants demonstrated significantly shorter preovulator [...]

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Longitudinal Quantitative Mapping of Meniscal Morphological and Signal Intensity Differences After ACL Surgery.

Barnes DA, Murray CJ, Movahhedi M, Beveridge JE, Kiapour AM, Murray MM, Fleming BC

BACKGROUND: Meniscal tears are frequently associated with anterior cruciate ligament (ACL) injuries. Evaluating longitudinal changes in meniscal integrity using quantitative magnetic resonance imaging after ACL surgery may provide valuable insight into joint recovery and subsequent reinjury risk.

HYPOTHESES: (1) The cross-sectional area (CSA) and normalized signal intensity (SI) profiles of the lateral and medial menisci for the surgical knees would be greater than those of the contralateral uninjured knees observed at 6, 12, and 24 months after surgery. (2) The CSA and Constructive Interference in Steady State (CISS) SI profiles obtained at 24 months would be closer to those of the contralateral limb when compared with those of the 6- and 12-month time points.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: Deidentified postsurgical CISS magnetic resonance imaging scans of patients (n = 82; median age 17.6 years; 42 females and 40 males; 57 ACL restoration and 25 ACL reconstruction) were analyzed. CSA and normalized CISS SI values were extracted from the lateral and medial menisci to create morphological profiles. Statistical parametric mapping was used to compare the CSA and normalized CISS SI profiles between the surgical and contralateral limbs to identify specific regions that were different among the menisci at 6, 12, and 24 months.

RESULTS: The CSA and nor [...]

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Meniscal Allograft Transplantation Failure Is Associated With Female Sex: A 15-Year Survival Analysis.

Park YL, Wackerle AM, Jiang C, Vieider RP, Winkler PW, Irrgang JJ, Musahl V, Hughes JD

BACKGROUND: While short- and midterm outcomes after meniscal allograft transplantation (MAT) are well documented, there is a paucity of data regarding long-term graft survival.

PURPOSE: To assess mid- to long-term graft survivorship and patient-reported outcomes (PROs) after MAT.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: All patients who underwent MAT with bone block or soft tissue fixation between 1993 and 2020 at a single institution were identified. Individuals with <5 years of follow-up and untreated grade 4 cartilage lesions were excluded. Primary outcomes included graft survivorship and long-term PROs including International Knee Documentation Committee Subjective Knee Form (IKDC SKF) score, Knee injury and Osteoarthritis Outcome Score, and Marx Activity Scale score. Graft failure was defined as the need for meniscal allograft resection or repair, revision MAT, or conversion to total knee arthroplasty. Clinical failure was defined as an IKDC SKF score <56 at the final follow-up. Statistical analyses included chi-square/Fisher exact tests for categorical variables, the Mann Whitney U or t test for continuous variables, and Kaplan-Meier survival analysis with endpoints as graft failure. Statistical significance was defined as a P value <.05.

RESULTS: This study included 160 patients (57% of the screened population; 46% female; median age at surgery, 28 [...])

Length Changes of the Lateral Knee: Fibular Collateral Ligament, Anterolateral Ligament, and Iliotibial Band Fibers.

Augustin EJ, Oppenheim ZR, Vega J, Gomez Verdejo F, Casanova FJ, Escribano G, Koluman AC, Bi AS et al.

BACKGROUND: The lateral knee is stabilized by the fibular collateral ligament (FCL), anterolateral ligament (ALL), and iliotibial band (ITB) fibers. Understanding their flexion-dependent behavior is critical for guiding graft fixation during reconstruction.

HYPOTHESIS/PURPOSE: The hypothesis was that the FCL and ALL would shorten with flexion, while ITB fibers would show more minor, transient changes. The purpose was to characterize native length changes of these structures in cadaveric knees.

STUDY DESIGN: Descriptive laboratory study.

METHODS: Ten fresh-frozen knees were dissected with preservation of lateral soft tissue attachments. Bony landmarks were marked for the attachment sites: FCL (n = 10), ALL (n = 7), and ITB (n = 10 distal Kaplan fibers, n = 9 proximal). A 3-dimensional digitizer recorded coordinates at 0°, 15°, 30°, 45°, 60°, and 90° of flexion. Attachment distances were calculated, and paired t tests or Wilcoxon tests (based on Shapiro-Wilk normality) compared 0° with each flexion angle.

RESULTS: Distances between the FCL and ALL attachment sites progressively decreased with flexion, with mean reductions of -7.59 and -6.44 mm, respectively, from 0° to 90° (both $P \leq .05$). Distances from the Gerdy tubercle to the distal and proximal Kaplan attachments of the ITB increased significantly by 2 to 6 mm between 15° and 60° before returning toward baseline at 90°: di [...]

Quadriceps and Hamstring Muscle Strength Recovery After a Multiligament Knee Injury: A Long-term Follow-up.

Trøan I, Holm I, Moatshe G, LaPrade RF, Engebretsen L, Bere T

BACKGROUND: Multiligament knee injuries (MLKIs) are serious and heterogeneous injuries that often lead to long-term functional limitations. There is limited literature regarding objective muscle strength and functional recovery after surgical treatment of MLKIs.

PURPOSE: To investigate long-term quadriceps and hamstring muscle strength and functional recovery after surgical treatment of MLKI and to compare postoperative outcomes between single cruciate MLKI and bicruciate MLKIs.

STUDY DESIGN: Cross-sectional study; Level of evidence, 3.

METHODS: Patients surgically treated for an MLKI at a single level 1 trauma center between January 2013 and December 2020 were included. Isokinetic quadriceps and hamstring muscle strength were assessed using a Biodex dynamometer, and physical function was measured by single-leg hop, triple hop, crossover hop, and 6-m timed hop tests, with the limb symmetry index (LSI) calculated.

RESULTS: Of 124 eligible patients, 90 (72%) participated at a mean follow-up time of 7 years (SD, 2.1) since surgery to evaluation. The median age at the time of testing was 49 years (range, 21-72) and 59% were male. In total, 54% of the patients had not regained full quadriceps strength (LSI <90%), and 46% had persistent hamstring deficits for peak torque/body weight (PT/BW) at 60 deg/s. Across the entire cohort, there was a significant difference between the unin [...]

Selective Epiphysiodesis of the Anterior Distal Femoral Physis Alters Trochlear Morphology in a Rabbit Model.

Wills DJ, Dan MJ, Oliver R, Caldwell B, Cance N, Dejour D, Walsh WR

BACKGROUND: Trochlear dysplasia is a leading contributor to patellofemoral instability, but the developmental role of the anterior distal femoral physis in shaping trochlear morphology remains poorly understood.

HYPOTHESIS: Selective closure of different portions of the anterior distal femoral physis would alter trochlear morphology in a predictable manner based on the region of growth arrest.

STUDY DESIGN: Controlled laboratory study.

METHODS: A total of 12 New Zealand White rabbits underwent unilateral selective epiphysiodesis of the anterior distal femoral physis at either the medial, central, or lateral region (n = 4 per group). Contralateral limbs served as internal controls. At 6 months postoperatively, gross, micro-computed tomography (μ CT), and histological assessments were performed. μ CT parameters including sulcus angle, trochlear depth, trochlear inclination, and facet symmetry were analyzed. A caliper was used to measure trochlear width as well as trochlear ridge height and length. The paired t test compared treated versus control limbs within each group.

RESULTS: Selective epiphysiodesis altered trochlear morphology in a region-specific pattern. Lateral epiphysiodesis resulted in a decreased proximal sulcus angle ($P = .005$) and an increased lateral trochlear depth proximally ($P = .021$), centrally ($P = .031$), and distally ($P = .036$), as measured on axial μ CT. Ce [...]

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Platelet-Derived Growth Factor Synergistic Enhancement of Bone Marrow Stimulation for Enhanced Labral Repair and Functional Recovery in a Rat Bankart Model.

Mendíaz F, Vaish B, Rifat S, Gray N, Hoang L, Nguyen T, Borrelli J, Tang L

BACKGROUND: Bone marrow stimulation (BMS) is used to treat intra-articular defects, such as articular cartilage defects or to facilitate healing of soft tissue repair; however, BMS yields inconsistent clinical outcomes, potentially due to the inefficient delivery of endogenous progenitor cells.

HYPOTHESIS: Combining BMS with localized placement of platelet-derived growth factor (PDGF) would enhance in situ progenitor cell responses, leading to enhanced labral tissue repair and functional recovery in a rat model.

STUDY DESIGN: Controlled laboratory study.

METHOD: A Bankart glenoid labral tear was created in the right shoulder of 50 adult rats. The animals were randomly divided into 5 groups: (1) healthy (baseline), (2) suture only (control), (3) suture + PDGF, (4) suture + BMS, and (5) suture + PDGF + BMS. Healing was assessed at 1.5 (n = 3), 3, and 8 weeks (n = 4). Baseline groups were included at 3 (n = 4) and 8 weeks (n = 5). At each timepoint, labral healing was evaluated by quantifying progenitor cell recruitment, tissue differentiation, and general morphology. Functional recovery was assessed weekly via gait analysis.

RESULTS: BMS alone failed to elicit cell recruitment to the injured labral region. However, the combined BMS + PDGF treatment resulted in a 7-fold increase in progenitor cell accumulation at the tear site compared with BMS alone (27.46 ± 11.51 vs 3.5 ± 3 . [...])

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Role of Hypoxia-Inducible Factor-1 α in Regulating Muscle Degeneration After Rotator Cuff Tears.

Zhang H, Lee A, Liu M, Diaz A, Zhang Y, Kim HT, Feeley BT, Liu X

BACKGROUND: Secondary muscle degeneration after a rotator cuff tear (RCT) critically affects clinical outcomes. Vascular compromise after a tendon injury creates a complex microenvironment that may be associated with the degeneration of rotator cuff muscle. The role of hypoxia-inducible factor-1 α (HIF-1 α), a master regulator of cellular stress responses to hypoxia, in modulating muscle abnormalities after an RCT remains undefined.

PURPOSE: To define the role of HIF-1 α in stem cell differentiation and muscle degeneration after an RCT in a murine model.

STUDY DESIGN: Controlled laboratory study.

METHODS: A supraspinatus tendon transection and suprascapular nerve transection (TTDN) model was established in C57BL/6J, platelet-derived growth factor receptor α (PDGFR α)-green fluorescent protein (GFP) reporter, and inducible cell-specific HIF-1 α knockout mice. Vascularity and HIF-1 α colocalization with fibroadipogenic progenitor (FAP) cells and satellite cells were analyzed. Fibrosis, fatty infiltration, and myofiber cross-sectional area were assessed. In vitro, HIF-1 α was modulated in isolated FAP cells via CRISPR-Cas9 or

prolyl hydroxylase domain inhibitors to evaluate FAP cell differentiation.

RESULTS: TTDN induced significant capillary density reduction (CD31+) at 1, 2, and 6 weeks after an injury. Global HIF-1 α expression decreased after TTDN compared to the sham side (1 week [...])

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Preoperative Proximal Migration of the Radial Head as an Independent Predictor of Suboptimal Outcomes After Osteochondral Autograft Transplantation for Capitellar Osteochondritis Dissecans: A Retrospective Cohort Study.

Kamijo H, Takahashi N, Matsuki K, Ishii D, Akiyama T, Sugaya H, Kawashima I

BACKGROUND: Osteochondral autograft transplantation (OAT) is widely performed for capitellar osteochondritis dissecans (OCD). However, preoperative predictors of suboptimal postoperative outcomes remain unclear.

PURPOSE/HYPOTHESIS: The authors aimed to evaluate clinical outcomes after OAT for capitellar OCD and identify preoperative risk factors associated with suboptimal outcomes. They hypothesized that radiographic indicators of disease severity, including preoperative proximal migration of the radial head, lesion size, and lateral wall disruption, would be associated with suboptimal postoperative clinical outcomes.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: The records of adolescent athletes who underwent OAT for capitellar OCD with a minimum 2-year follow-up were retrospectively reviewed. Clinical outcomes included elbow range of motion (ROM) and Timmerman-Andrews (T-A) score. A suboptimal outcome was defined as a postoperative T-A score <160. Preoperative radiographs were used to measure proximal migration of the radial head relative to the coronoid process, hypertrophy of the radial head, OCD lesion area, and a 5-grade lateral wall disruption classification; measurement reliability was assessed. Multivariate logistic regression analysis was performed to identify independent predictors of a suboptimal outcome, and receiver operating characteristic (ROC) [...]

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Higher Rates of PASS and SCB After Arthroscopic Subspine Decompression Are Associated With a Positive Diagnostic AIIS Injection: A Propensity Score-Matched Cohort Study.

Duan YP, Yang F, Liu RG, Sun H, Huang H, Xu Y, Ju XD, Wang JQ

BACKGROUND: Hip arthroscopy effectively treats femoroacetabular impingement syndrome (FAIS), but persistent pain may be related to concomitant extra-articular pathology such as subspine impingement syndrome (SSI). Standard diagnosis of SSI often relies on 3-dimensional computed tomography (3D-CT) morphology (Hetsroni type II/III), although this morphology is common in individuals who are asymptomatic and correlates poorly with symptoms.

PURPOSE: To compare minimum 2-year clinical outcomes after arthroscopic subspine decompression in patients with concurrent FAIS and type II/III anterior inferior iliac spine (AIIS) morphology, stratified by diagnostic method: 3D-CT morphology alone versus 3D-CT morphology plus a positive ultrasound-guided diagnostic injection.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: This study included patients aged 18 to 55 years with type II/III AIIS morphology who underwent primary hip arthroscopy for FAIS and SSI between January 2021 and November 2023 and had minimum 2-year follow-up. Patients diagnosed by CT morphology alone (CT classification group) were propensity score matched 1:1 to patients with a positive ultrasound-guided AIIS injection (injection group), with 57 patients per group. Matching variables were age, sex, body mass index, lateral center-edge angle, alpha angle, Tönnis grade, and Beighton score. All patients underwent [...]

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Arthroscopic Correction of Pincer-Type FAI in the Presence of Acetabular Retroversion Results in Excellent Patient-Reported Outcomes With Low Risk of Reoperation or Conversion to Arthroplasty at Minimum 5 Years.

Filan D, Mullins K, Carton P

BACKGROUND: Acetabular retroversion is a distinct morphologic variation that may result in pincer-type femoroacetabular impingement (FAI). The role of arthroscopic management within this cohort is not fully understood.

PURPOSE: To assess patient-reported outcome measures (PROMs) and survivorship at 5-year follow-up in a series of cases undergoing arthroscopic correction of pincer-type FAI with acetabular retroversion.

STUDY DESIGN: Cohort study; Level of evidence, 3.

METHODS: A single-center, prospective hip preservation registry was reviewed for all cases undergoing primary hip arthroscopy for symptomatic FAI between January 2014 and July 2020, with documented radiographic assessment of acetabular retroversion (ie, presence or absence of crossover sign, ischial spine sign, and/or posterior wall sign on a standing, standardized anteroposterior radiograph). Exclusion criteria were Tönnis grade >1, concomitant pathologies (protrusio, Perthes disease, avascular necrosis), excessive pelvic rotation or tilt on radiographs, and/or no crossover sign. Cases were assigned to 1 of 2 groups: moderate-global retroversion group (study group: crossover sign plus either or both ischial spine sign and posterior wall sign) or control group (crossover sign only). Case-control (1:1) fuzzy matching was performed based on age (± 2 years), sex, and Tönnis grade (exact). Clinical outcome evaluation [...]

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Artificial Intelligence Cannot Replace Peer Reviewers but May Help Editors Triage: A Comparative Analysis of a Large Language Model and Human Reviewer Recommendations at the American Journal of Sports Medicine.

Patel R, Shultz C, Ollivier M, Wascher DC

BACKGROUND: The peer review system faces increasing strain from rising manuscript volumes, reviewer fatigue, and well-documented interreviewer disagreement. Large language models (LLMs) have shown potential to support the peer review process, but their ability to replicate editorial decisions at high-impact medical journals and their utility as manuscript screening tools remain unknown.

PURPOSE: To compare the agreement between an LLM and the final editorial decision on manuscripts submitted to the American Journal of Sports Medicine and to evaluate the potential of LLMs as a manuscript screening tool.

STUDY DESIGN: Cross-sectional agreement study.

METHODS: Fifty-four manuscripts randomly selected from submissions to the American Journal of Sports Medicine (September 2024-October 2024) were reviewed by a locally deployed LLM (Minstral 3 14B; Mistral AI) using a standardized prompt. The artificial intelligence (AI) produced a categorical recommendation (reject, cascade, revision, or accept) and a numerical score (0-100) for each manuscript. Agreement with the final editorial decision was assessed by Cohen kappa (4-category model) for pooled human reviewers (n = 139 reviews) and the AI (n = 54). Screening performance was evaluated by positive predictive value (PPV), sensitivity, and specificity.

RESULTS: Pooled human reviewers demonstrated fair agreement with the final decis [...]

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Prevalence of Psychiatric Diagnoses and Preoperative Opioid Use Among Patients Undergoing Hip Arthroscopy: A Systematic Review and Meta-analysis.

Villegas Meza AD, Nocek M, Morales Lugo B, Mitchell BC, Philippon MJ

BACKGROUND: Psychiatric morbidity (anxiety, depression, prior opioid use) is variably reported among patients undergoing hip arthroscopy, with prevalence estimates influenced by heterogeneous case definitions and ascertainment methods. Clarifying true prevalence and method-related differences is needed to inform preoperative screening and comparative research.

PURPOSE: To quantify the pooled prevalence of psychiatric diagnoses and prior preoperative opioid use among adults undergoing hip arthroscopy and to evaluate how prevalence varies by ascertainment method.

STUDY DESIGN: Systematic review and meta-analysis; Level of evidence, 3.

METHODS: The authors performed a PRISMA-compliant search of PubMed, Embase, PsycINFO, and Web of Science from inception to June 19, 2025. Observational studies reporting extractable prevalence of any psychiatric diagnosis, anxiety, depression, or prior opioid use among adult hip arthroscopy cohorts were eligible. Two reviewers independently screened articles, extracted data, and assessed risk of bias (Joanna Briggs Institute checklist). Pooled prevalences were estimated using random effects meta-analysis on logit-transformed proportions with continuity corrections; heterogeneity was summarized with τ^2 and I^2 . Prespecified subgroup and meta-regression examined the ascertainment method as a moderator.

RESULTS: Thirty-eight studies met inclusion cr [...]

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Concomitant Intra-articular Injuries in 37,423 Primary and Revision ACL Reconstructions: A Systematic Review and Meta-analysis of Comparative Studies.

Vega TF, Thamrongskulsiri N, Brinkman JC, Pérez Lloveras GO, Casanova F, Cerezal Á, San Roman AV, Borace AF et al.

BACKGROUND: Intra-articular lesions of menisci and cartilage are common in anterior cruciate ligament reconstruction (ACLR) and may differ between primary and revision procedures. Clarifying their prevalence, morphology, and risk factors is important for prognosis and surgical planning.

PURPOSE: To perform a systematic review and meta-analysis comparing the prevalence and risk factors for intra-articular injuries in primary versus revision ACLR.

STUDY DESIGN: Systematic review and meta-analysis; Level of evidence, 4.

METHODS: Following PRISMA (Preferred Reporting Items for Systematic Reviews and Meta-Analyses) guidelines, the PubMed, MEDLINE, and Embase databases were searched, supplemented by citation review and author contact. Comparative studies reporting intraoperative meniscal and/or chondral findings in both primary and revision ACLRs were included. Pooled odds ratios with 95% confidence intervals were calculated using random- or fixed-effects models.

RESULTS: Nineteen studies consisting of 37,423 ACLRs were included. Revision ACLR was associated with higher odds of meniscal injury (OR, 1.64; 95% CI, 1.12-2.41). Meniscal treatment did not differ significantly between groups. Chondral lesions were >3-fold more prevalent in revisions (OR, 3.53; 95% CI, 2.71-4.60), especially at the trochlea (OR, 3.21; 95% CI, 2.59-3.97; $I^2 = 33\%$), medial tibial plateau (OR, 3.12; 95% CI [...])

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Statistical Robustness of Randomized Controlled Trials Comparing Biceps Tenotomy Versus Tenodesis: A Reverse Continuous Fragility Index Analysis.

Hu EY, Althoff AD, Cervantes JE, Dave U, Kessler KG, Moran TE

BACKGROUND: The reverse continuous fragility index (rCFI) is a statistical measure utilized to evaluate the relative robustness or fragility of nonstatistically significant findings of randomized controlled trials. Such studies comparing biceps tenotomy versus tenodesis during shoulder arthroscopy have generally shown similar clinical and functional outcomes between the treatments.

PURPOSE: To evaluate the statistical robustness or fragility of nonsignificant outcomes of high-quality randomized controlled studies comparing clinical outcomes of biceps tenotomy versus tenodesis during shoulder arthroscopy.

STUDY DESIGN: Systematic review and meta-analysis; Level of evidence, 1.

METHODS: The PubMed, Embase, and Cochrane Library databases were searched according to the PRISMA guidelines from 2004 to September 2025 with the following search strategy: "('biceps' OR 'biceps brachii' OR 'long head biceps') AND ('tenodesis' OR 'tenotomy') AND ('Randomized controlled trial' OR 'RCT')." Studies were included if they were randomized controlled trials that compared clinical outcomes of biceps tenodesis versus biceps tenotomy during shoulder arthroscopy. rCFI was calculated on the primary nonsignificant outcomes of all studies and averaged to obtain the mean rCFI. Loss to follow-up was recorded for each study. The reverse continuous fragility quotient was calculated to account for the stu [...]

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Return to Sport After Surgical Treatment of Capitellar Osteochondritis Dissecans Lesions in Baseball Players: A Systematic Review and Meta-analysis.

Paul RW, Anton B, Casalino G, Windsor JT, Tan X, Kohan E, Alberta FG

BACKGROUND: Because of the difficulty of baseball players with capitellar osteochondritis dissecans (OCD) lesions to return to sport (RTS), multiple operative strategies have been used for these lesions. However, there has yet to be a systematic review of RTS outcomes in specifically baseball players.

PURPOSE/HYPOTHESIS: The purpose of this study was to compare the RTS rates between surgical procedures for capitellar OCD lesions in youth baseball players, with a secondary purpose of comparing improvements in elbow range of motion (ROM) and improvements in Timmerman-Andrews scores between surgical procedures. The authors hypothesized that RTS rates would not differ based on the surgical procedure for capitellar OCD lesions in youth baseball players.

STUDY DESIGN: Systematic review; Level of evidence, 4.

METHODS: The authors performed a literature search in February 2025 to identify studies evaluating postoperative outcomes for the treatment of capitellar OCD lesions in youth baseball players. Surgical procedures were grouped into debridement/loose-body removal $\pm$ microfracture, fixation procedures, bone peg grafting, and autologous osteochondral transplantation. Preoperative and postoperative elbow flexion and extension ROM, Timmerman-Andrews scores, and RTS rate and time were collected. Meta-analysis included pooled effect estimates for improvements in flexion ROM, extension [...]

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Vitamin D and the Risk of Stress Fractures in Athletes and Military Personnel: A Systematic Review and Meta-analysis.

Moraes BF, Leitão CVFS, Rodrigues VMS, Dressler HB, Paixão JC, da Silva ABN, de Oliveira GM, Medina G et al.

BACKGROUND: Stress fractures are common overuse injuries in military personnel and athletes. Although a low vitamin D level is a proposed risk factor, the evidence is conflicting, and the specific risk in athletes has not been systematically synthesized.

PURPOSE: To assess the association between vitamin D status and the risk of stress fractures in military personnel and athletes.

STUDY DESIGN: Systematic review and meta-analysis; Level of evidence, 3.

METHODS: Major databases were systematically searched through April 16, 2025 for studies that quantitatively measured serum levels of 25-hydroxyvitamin D in military or athlete populations with a stress fracture group and a control group. Data were pooled using a random-effects model to calculate the mean difference (MD) and 95% confidence interval (CI). Subgroup analyses were conducted based on population type (military vs athlete), sex, and vitamin D status. Meta-regression was also performed to investigate sources of heterogeneity.

RESULTS: A total of 15 studies (4183 participants) were included. Overall, patients with stress fractures had significantly lower vitamin D levels compared to controls (MD, -5.82 nmol/L [95% CI, -10.35 to -1.29]). The association was also significant for military personnel (MD, -7.33 nmol/L [95% CI, -11.77 to -2.89]) and male participants (MD, -9.14 nmol/L [95% CI, -14.23 to -4.06]). Subgroup an [...]

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PASS Thresholds and Denominators After Nonoperative Management of Femoroacetabular Impingement Syndrome: Response.

Hoit G, Whelan DB, Lemieux V, Bates B, Dwyer T, Theodoropoulos J, Chahal J

[PubMed](#) · [DOI](#)

Rethinking Graft Equivalence After ACL Reconstruction With Lateral Extra-articular Tenodesis in Elite Athletes: Response.

Lynskey SJ, Ambrosanio A, Motesharei A, Jones M, Ball S, Church JS, Williams A

[PubMed](#) · [DOI](#)

Duration of Smoking Cessation Needed to Achieve Retear Rates Comparable to Those of Nonsmokers After Arthroscopic Rotator Cuff Repair: Response.

Chang HH, Chun YM, Kim SJ, Kim WM, Yoon TH

[PubMed](#) · [DOI](#)

Rethinking Graft Equivalence After ACL Reconstruction With Lateral Extra-articular Tenodesis in Elite Athletes: Letter to the Editor.

Qu X, Yi M, Tang X, Chen B

[PubMed](#) · [DOI](#)

Duration of Smoking Cessation Needed to Achieve Retear Rates Comparable to Those of Nonsmokers After Arthroscopic Rotator Cuff Repair: Letter to the Editor.

Mena L, Checchia C, Malavolta EA

[PubMed](#) · [DOI](#)

Methodological Concerns Regarding the Ankle-GO Score for Predicting Lateral Ankle Sprain in Elite Basketball Players: Letter to the Editor.

Wu Z, Liu C, Wu W

[PubMed](#) · [DOI](#)

PASS Thresholds and Denominators After Nonoperative Management of Femoroacetabular Impingement Syndrome: Letter to the Editor.

Duan YP

[PubMed](#) · [DOI](#)

Methodological Concerns Regarding the Ankle-GO Score for Predicting Lateral Ankle Sprain in Elite Basketball Players: Response.

Hardy A, Tassery F, Allaire T, Moussa MK, Valentin E, Fouchet F, Lopes R, Picot B

[PubMed](#) · [DOI](#)



The Bone & Joint Journal

Volume 108-B, Issue 9 · September 2026 · 15 new articles

Beyond alignment : expanding the dimensionality of total knee arthroplasty.

Dunbar MJ, Haddad FS

[PubMed](#) · [DOI](#)

Necrotizing fasciitis.

White TO, Ablett AD, Neo C, Clement ND, Moran CG, Morgan MS, Duckworth AD

Necrotizing fasciitis is a rapidly progressive, life-threatening infection originating in a fascial plane which requires emergency surgery. Although uncommon, with an incidence of 0.63 per 100,000 per year, it affects the limbs in 40% of cases and carries a 22% 30-day mortality rate. The diagnosis is principally clinical. Severe pain, out of proportion to any visible abnormality of the skin, is the cardinal early feature, frequently accompanied by tenderness beyond the area of erythema, swelling, and local hypoaesthesia. Crucially, the skin may initially appear normal or nearly normal while rapid fascial destruction progresses beneath. Laboratory investigations, imaging, and scoring systems lack sufficient sensitivity and specificity to confirm or exclude the diagnosis and must not delay surgical exploration. The suspected diagnosis is confirmed or excluded by a diagnostic incision to the fascial

plane. Necrotizing fasciitis is classified into four types based on the microbiology: polymicrobial (type I), Gram-positive monomicrobial (type II), Gram-negative monomicrobial (type III), and fungal (type IV). The focus of treatment is emergency radical excision of all necrotic tissue to healthy margins and broad-spectrum antibiotics. Surgery should not be delayed by medical management, imaging, or inter-hospital transfer. Failure to recognize and act within six hours of presentation [...]

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Does transposition osteotomy of the acetabulum alter the natural history of developmental dysplasia of the hip?

Tanaka S, Fujii M, Kawano S, Ueno M, Oba Y, Morimoto T

AIMS: The aim of this study was to clarify the natural history of developmental dysplasia of the hip (DDH), borderline DDH (BDDH), and normal hips, and to determine whether transposition osteotomy of the acetabulum (TOA) alters the long-term joint survival in DDH and BDDH.

METHODS: We retrospectively reviewed 683 hips treated with TOA and 353 contralateral native hips. Hips were classified by the lateral centre-edge angle (LCEA) as DDH ($< 20^\circ$), BDDH (20° to $< 25^\circ$), or normal ($\geq 25^\circ$). The median follow-up was 11 years (IQR 5.8 to 16) for contralateral native hips and 9.6 years (IQR 5.3 to 15) for TOA-treated hips. The cumulative probability of failure (progression to Tönnis grade 3, or conversion to total hip arthroplasty) was estimated using the Kaplan-Meier method. Cox proportional hazards models with patient-level cluster-robust standard errors identified predictors for failure.

RESULTS: In native hips, the 20-year survival was significantly lower in DDH than in BDDH and normal hips (69% (95% CI 55 to 80) vs 96% (95% CI 83 to 99) vs 100%, respectively; $p < 0.010$), with no significant difference between BDDH and normal hips. Increased BMI and decreased LCEA were independent predictors for failure. In DDH, TOA-treated hips had significantly increased 20-year survival compared with native hips (88% (95% CI 80 to 93) vs 69% (95% CI 55 to 80); $p < 0.001$), and native hip status, [...]

[PubMed](#) · [DOI](#)

Patient, surgeon, and institutional variation in dual-mobility component use in primary total hip arthroplasty : a retrospective cohort study using data from the National Joint Registry Study.

Zucker BE, Howard JT, Whitehouse MR, Judge A

AIMS: The aim of this study was to quantify the contribution of patient-, surgeon-, and institution-level factors to variation in dual-mobility component use in primary total hip arthroplasty (DM-THA) in England. We sought to determine whether current patterns reflect appropriate patient selection or unwarranted practice variation. We hypothesized that surgeon- and institution-level factors account for a greater proportion of variation in DM-THA use than patient-level characteristics.

METHODS: We analyzed 238,455 primary THAs recorded in the National Joint Registry which were linked with Hospital Episode Statistics for England between 1 January 2018 and 31 December 2022. Using cross-classified multilevel logistic regression modelling, we partitioned variation in DM-THA use attributable to patient, surgeon, and institutional factors. Patient risk factors were included as fixed effects, with random effects specified for surgeons and hospitals to account for clustering.

RESULTS: Of the total cohort, 7,032 underwent DM-THA. These patients were older, more frail, and more frequently treated for neck of femur fractures. Surgeon-level factors explained 32.4% and institutional factors 22.9% of variation in DM-THA use. Patient-level factors accounted for approximately 10% of the variance (marginal $R^2 = 0.10$), while the full model explained 59.8% of variation (conditional $R^2 = 0.598$). [...]

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Five-year survival and complications of more than 300 articulating high-dose antibiotic spacers in two-stage revision total hip arthroplasties for infection.

Weintraub MT, Midthun WR, Pagnano MW, Taunton MJ, Sierra RJ, Mabry TM, Perry KI, Berry DJ et al.

AIMS: Two-stage exchange arthroplasty with an antibiotic-eluting cement spacer remains the predominant strategy in the USA for managing chronic periprosthetic joint infection (PJI) following total hip arthroplasty (THA). The aim of

this study was to evaluate a commercially available, partially intraoperatively fabricated articulating high-dose antibiotic cement spacer, focusing on survival, complications, and patient-reported outcomes.

METHODS: A total of 298 patients (302 THAs) underwent two-stage exchange arthroplasty for PJI using a specific articulating antibiotic spacer between August 2005 and May 2022. Patients with a previous failed two-stage exchange were excluded, although 121 THAs (302, 40%) had undergone previous PJI-related procedures. The mean age of the patients was 65 years (SD 11), the mean BMI was 32 kg/m² (SD 7), and 116 (38%) were female. Most patients were classified as McPherson host B (158; 52%) and limb grade 2 (165; 55%), and 265 (88%) met the 2011 Musculoskeletal Infection Society criteria. The mean follow-up was six years (2 to 16). .

RESULTS: At five years after reimplantation, survival free of reinfection, aseptic rerevision, any rerevision, or any reoperation were 89% (95% CI 85.2 to 93.1), 93% (95% CI 89.3 to 96.1), 86% (95% CI 81.3 to 90.3), and 80% (95% CI 74.7 to 84.9), respectively. Dislocation accounted for most aseptic rerevisions (15/20). [...]

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A prospective double-blinded randomized controlled trial comparing conventional jig-based versus robotic arm-assisted medial unicompartmental knee arthroplasty.

Kayani B, Fontalis A, Tahmassebi J, Konan S, Plastow R, Wazir U, Shah M, Oussedik S et al.

AIMS: This study reports the interim outcomes of a randomized controlled trial comparing the accuracy of component positions, early functional outcomes, patient-reported outcome measures, and complications in conventional jig-based unicompartmental knee arthroplasty with navigational control (CO UKA) with robotic arm-assisted unicompartmental knee arthroplasty (RO UKA) for patients who have medial compartment knee osteoarthritis (OA).

METHODS: In total, 107 patients with symptomatic medial compartment OA were prospectively randomized to undergo CO UKA (n = 52) or RO UKA (n = 55), representing the currently available recruited cohort. Patients in both treatment groups had CT-based surgical planning. All procedures were completed using the medial parapatellar approach, and all patients received a standardized postoperative rehabilitation programme. Predefined study outcomes were recorded at regular intervals for two years after surgery.

RESULTS: RO UKA was associated with increased accuracy in executing the planned femoral ($p < 0.001$) and tibial ($p < 0.001$) component positions compared with CO UKA. RO UKA was associated with reduced inpatient pain scores ($p < 0.001$), decreased inpatient opioid analgesia consumption ($p = 0.008$), and shorter length of hospital stay ($p = 0.004$) compared with CO UKA. There were no differences between the treatment groups in the Oxford Knee Score (p [...])

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Impact of combined medial and lateral bone marrow lesions on clinical outcomes after medial unicompartmental knee arthroplasty : a retrospective cohort study.

Cao G, Wang A, Liu Q, Liu J, Liu Y, Huang Y

AIMS: Although the influence of medial bone marrow lesions (BMLs) on outcomes after medial unicompartmental knee arthroplasty (UKA) has been studied, the impact of concomitant lateral compartmental BMLs remains unclear. This study aims to determine whether the lateral BMLs, in addition to medial BMLs, influence clinical outcomes in patients undergoing medial UKA.

METHODS: We retrospectively reviewed patients who underwent medial UKA between January 2016 and December 2019 and had preoperative MRI. A total of 86 patients with isolated medial BMLs (BML-M) and 38 with combined medial and lateral BMLs (BML-M+L) were included. Clinical outcomes were assessed using the Numeric Rating Scale (NRS) and Oxford Knee Score (OKS). Radiological evaluation included pathological radiolucent lines, osteoarthritis progression in the lateral compartment, mechanical femorotibial angle (mFTA), and postoperative alignment zones. The proportion of patients achieving the minimal clinically important difference (MCID) for each score and a well-aligned mechanical axis was also calculated. Multivariable linear regression was employed to determine whether concomitant lateral BMLs independently influenced OKS improvement.

RESULTS: The mean follow-up was 6.4 years (SD 1.1). Both groups demonstrated significant improvements from baseline in NRS and OKS (both $p < 0.001$). No significant differences were found [...]

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Secondary patella resurfacing following primary total knee arthroplasty : patient-reported outcomes, satisfaction, and patient-acceptable symptom state.

Clement ND, Parsons T, Botfield-Gray T, Plastow R, Afzal I, Kader N, Radha S, Kader DF

AIMS: Our aims were to assess joint-specific outcome (JSO), health-related quality of life (HRQoL), and patient satisfaction following secondary patella resurfacing (SPR), and to define a patient-acceptable symptom state (PASS).

METHODS: This retrospective study was conducted over 16 years (October 2006 to December 2022) and included 182 patients undergoing SPR. Patient demographic details, American Society of Anesthesiologists (ASA) grade, preoperative and postoperative (one year: $n = 158$ and two years: $n = 143$) Oxford Knee Score (OKS), EQ-5D, and EQ-VAS scores, as well as postoperative satisfaction, were obtained from a local arthroplasty register.

RESULTS: There were clinically meaningful and statistically significant ($p < 0.001$) improvements in the OKS, with a mean change of 6.8 (95% CI 4.7 to 8.9) and 8.1 (95% CI 5.9 to 10.2) points, and in the EQ-5D of 0.174 (95% CI 0.110 to 0.238) and 0.187 (95% CI 0.119 to 0.255) at both one and two years, respectively. There were 126 (79.7%) and 120 (85.7%) satisfied patients at one and two years, respectively. Prior satisfaction with the index total knee arthroplasty (TKA) (one year only: odds ratio (OR) 10.4 (95% CI 3.10 to 34.85); $p < 0.001$) and increasing time from primary TKA to the SPR (one year: OR 1.37 (for each additional year) (95% CI 1.03 to 1.81); $p = 0.028$, and two years: OR 2.32 (95% 1.23 to 4.38); $p = 0.009$) were indep [...]

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Risk of venous thromboembolism after knee, hip, or shoulder arthroplasty : a retrospective cohort study of 476,620 procedures using linked hospital and primary care data.

Swain S, Leith N, Brown S, Wright Drakesmith C, Rees JL, Bankhead C, Powell J

AIMS: Arthroplasty is a widely used surgery for end-stage arthritis, but venous thromboembolism (VTE) remains a major postoperative complication. Most previous studies have relied solely on hospital data; no study has examined the effect of national clinical guidelines on the rates of VTE in the UK. This study aimed to assess VTE trends after arthroplasty using linked primary and secondary care data, and the impact of the National Institute for Health and Care Excellence (NICE) guidelines in the UK on VTE rates after arthroplasty.

METHODS: We used the Clinical Practice Research Datalink (CPRD) Aurum version, the largest primary care database in the UK, and linked hospital and death data for patients undergoing knee, hip, or shoulder arthroplasty between October 2007 and September 2019. The outcome was VTE event within six months of surgery. Trends in VTE were assessed using the Cox-Stuart trend test, and the impact of seasonality and clinical guidelines introduced in 2010, 2012, and 2018 via logistic regression with a polynomial function.

RESULTS: A total of 476,620 arthroplasty records were included. The overall VTE incidence was 1.5% (knee (1.6%), hip (1.4%), and shoulder arthroplasty (1%)). More than 30% of the VTEs were found through the linked data. A decline in VTE rates was observed over time for hip ($p = 0.004$) and knee arthroplasty ($p = 0.001$). Introduction of NICE g [...]

[PubMed](#) · [DOI](#)

Poor mental health in patients with symptomatic osteochondral lesions of the talus.

Pijnacker JHM, Agyeman-Prempeh NO, Sierevelt IN, Emanuel KS, Amsterdam Ankle Cartilage Team, Broekman B, Kerkhoffs GMMJ, Stufkens SAS et al.

AIMS: Osteochondral lesions of the talus (OLT) can cause pain and functional limitations, potentially affecting mental health. This study aimed to compare mental health in patients with OLT to the general Dutch population and individuals with mood or anxiety disorders, and to identify factors associated with mental health in OLT patients.

METHODS: This prospective study included patients with symptomatic OLT prior to treatment. Data for the general population and individuals with mood or anxiety disorders were obtained from the Netherlands Mental Health Survey and Incidence Study-3. Mental health was assessed using the Mental Health Inventory-5 (MHI-5). The primary outcome was the mean difference in MHI-5 between OLT patients and the general population. Multivariate linear regression examined associations between mental health and age, sex, BMI, smoking, lesion characteristics, laterality, and Foot and Ankle Outcome Score (FAOS) pain.

RESULTS: Among the 358 included OLT patients, 87% (311 of 358) scored ≤ 76 on the MHI-5. OLT patients scored significantly lower than the general population (mean difference 20.2 (95% CI 18.9 to 21.5)) and showed similar scores to individuals with a mood or anxiety disorder (mean difference 2.0 (95% CI -0.3 to 4.3)). One and two years after treatment, 79% (170/214) and 78% (134/173) of patients, respectively, scored ≤ 76 on the MHI-5. Multivaria [...]

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Total ankle arthroplasty via lateral transfibular approach with and without concomitant subtalar arthrodesis : radiological alignment, patient-reported outcomes, and range of motion at $\geq$ five-year follow-up.

Constantinescu SA, Barbero A, Carfi C, Crippa Orlandi N, Bonzano C, Indino C, Maccario C, Usuelli FG

AIMS: Concomitant subtalar pathology may require arthrodesis at the time of total ankle arthroplasty (TAA), yet direct comparisons of isolated TAA versus TAA + subtalar joint arthrodesis (STJA) via a lateral transfibular approach, using the same implant, remain limited. We compared $\geq$ five-year radiological, functional, and range of motion (ROM) outcomes.

METHODS: This retrospective single-centre cohort included 119 patients from January 2015 to December 2020 with $\geq$ five-year follow-up: 50 underwent TAA with concomitant subtalar arthrodesis and 69 underwent isolated TAA. Outcomes were Foot and Ankle Outcome Score (FAOS), visual analogue pain scale (VAS), 12-Item Short-Form Health Survey questionnaire physical component/mental component summary (SF-12 PCS/MCS), ankle ROM, radiological alignment (lateral distal tibial angle, α , tibiotalar surface (TTS), and Saltzman alignment), and complications. Analyses were performed using Welch's t-test or the Mann-Whitney U test for continuous variables and the chi-squared test or Fisher's exact test for categorical variables, together with multivariable regression, using a two-sided significance level of $\alpha = 0.05$ with Holm correction across angles. Post hoc minimum detectable effect (MDE) analysis at 80% power used observed SDs and group sizes.

RESULTS: Ankle osteoarthritis (OA) + STJ OA had worse baseline FAOS (24.8 (7.0) vs 42.6 (8.8); p [...])

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Patient-evaluated functional outcome is systematically different from physician-evaluated outcome after proximal humerus fracture fixation.

Sahu D, Chaubey A, Shah D

AIMS: The study aimed to investigate the agreement between Constant-Murley score (CMS) and the shoulder subjective value (SSV); and the association of poor self-evaluated outcome with shoulder pain, range of motion (ROM) limitation, and shoulder strength after fixation of proximal humerus fractures.

METHODS: The study included 74 shoulders in 72 patients (mean age 56 years (SD 14), 40 male (56%)) who underwent locking plate fixation of displaced proximal humerus fractures between January 2015 and January 2024. We evaluated CMS, SSV, pain scores (visual analogue scale), and ROM measurements at the most recent (minimum one-year) follow-up. Agreement statistics between CMS and SSV were used descriptively, rather than to test true substitutability, because CMS and SSV are nonequivalent constructs.

RESULTS: The Bland-Altman analysis showed differences between CMS and SSV, as the mean difference between CMS and SSV (-14.21 (95% CI -17.25 to -11.18)) did not include the zero-difference line. The Pitman's test suggested that the variances were significantly different ($p = 0.029$). Firth's penalized method of logistic regression revealed that only the presence of shoulder pain was associated with an increased likelihood of a poor functional outcome ($SSV \leq 69.4$) (coefficient 3.5 (95% CI 1.53 to 6.47); $p < 0.001$). The SSV threshold (69.4) was derived from the CMS ≤ 55 threshold on the sa [...]

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Assessment of the plane of maximum curvature in adolescent idiopathic scoliosis using 3D ultrasound : an exploratory study.

Yang ZQ, Wu C, Zhou Y, Schlösser TPC, Gardner A, Castelein RM, Chu WC, Cheng JC et al.

AIMS: The size of the curves in patients with adolescent idiopathic scoliosis (AIS) is traditionally assessed using 2D Cobb angles, which may underestimate the extent of this complex 3D deformity. The plane of maximum

curvature, measured using 3D reconstructions of the spine, including 3D ultrasound, offers more accurate 3D representation. However, the optimal 3D ultrasound-based method of measuring the plane of maximum curvature remains unknown. We evaluated the reliability and agreement of three methods of measuring the plane of maximum curvature (the maximum-curvature, endplate-apex-endplate, and maximum-axial-rotation methods) to provide radiation-free and severity-specific clinical recommendations for 3D ultrasound measurements of the plane of maximum curvature using the SRS-Lenke-Aubin 3D classification.

METHODS: A total of 88 patients with right major thoracic AIS, stratified by the severity of the curve (mild 29; moderate 34; severe 25), underwent 3D ultrasound. Conventional 2D parameters were quantified from the 3D ultrasound reconstructions. For each method of measurement of the plane of maximum curvature, we recorded the ultrasound curve angle (UCA) on the plane, its orientation, and the difference between it and the coronal UCA (Δ UCA). The reliability of all variables, the agreement between the three methods of measurement, comparisons between the severity groups, [...]

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Locking plate fixation compared with tension-band wiring for patellar fractures: a multicentre randomized controlled trial.

Larsen P, Thorninger R, Severinsen RP, Beuke JC, Jensen SS, Rasmussen MK, Petruskevicius J, Barckman J et al.

AIMS: Patellar fractures are associated with considerable postoperative morbidity, and tension-band wiring has high complication rates and suboptimal outcomes. Locking plate fixation is a recent surgical innovation which confers superior biomechanical stability. However, there is little information about the clinical outcomes of this method of treatment. The aim of this study was to compare locking plate fixation with tension-band wiring in patients requiring surgical treatment for a patellar fracture.

METHODS: In this patient- and assessor-blinded, multicentre randomized trial, 122 adults with a displaced patellar fracture were assigned 1:1 to locking plate fixation or tension-band wiring. The primary outcome was patient-reported knee function measured by the five sub-scales of the Knee Injury and Osteoarthritis Outcome Score (KOOS) at 12 months. Superiority was predefined as a statistically significant difference in at least three KOOS sub-scales exceeding the minimal clinically important difference (pain 10, symptoms 9, activities of daily living 6, sport and recreation 10, quality of life 10). Secondary outcomes included additional clinical and functional outcomes. Analyses were performed according to the intention-to-treat principle.

RESULTS: A total of 63 patients were assigned to locking plate fixation and 59 to tension-band wiring; 57 (90.5%) and 52 (88.1%), respectively [...]

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Hip arthroplasty following proximal femoral tumour resection in children : the impact of age and articulation on implant survival and the outcomes following acetabular revision.

Baird CAE, Green N, Evans S, Kurisunkal V, Parry MC, Stevenson J

AIMS: Hip arthroplasty is rarely required before skeletal maturity. However, it is appropriate after resection of a proximal femoral bone tumour. This retrospective single-centre cohort study was conducted to evaluate how age and choice of articulation affects modes of implant failure and revision-free survival, and to understand the longer-term outcomes following inevitable revision.

METHODS: Between January 1982 and April 2023, 60 patients aged between two and 16 years underwent excision of a malignant bone tumour and proximal femoral endoprosthetic reconstruction (PFEPR) combined with either a hemiarthroplasty (HA) or total hip arthroplasty (THA) articulation. The median follow-up was 12.6 years (IQR 4 to 21).

RESULTS: There were 41 HAs and 19 THAs. Revision-free survival was poor in pre-adolescent patients (age $\leq$ 12 years) regardless of articulation (54% at five years (95% CI 38 to 77) and 21% at ten years (95% CI 8 to 54)). Adolescent patients (age > 12 years) undergoing THA (n = 12) had better median, five-year, and ten-year revision-free implant survival (21.9 years (IQR 11.9 to 22.7), 89% (95% CI 71 to 100), and 78% (55 to 100), respectively) than similarly aged patients undergoing HA (n = 15); (7.1 years (IQR 4.9 to 13.5), 68% (95% CI 46 to 100), 39% (18 to 82); p = 0.048), respectively). The leading indication for revision across all ages was an acetabular complication [...]



Editorial: All Orthopaedic Subspecialties, General-interest Orthopaedic Topics, Education, Culture, and Related Research Spells "CORR".

Gebhardt MC, Manner PA, Porcher R, Rimnac CM, Wongworawat MD, Leopold SS

PubMed · DOI

Editor's Spotlight/Take 5: Most Contemporary TKA Implant Systems Meet International Benchmark Survivorship Standards: Evidence From a US Registry.

Leopold SS

PubMed · DOI

Most Contemporary TKA Implant Systems Meet International Benchmark Survivorship Standards: Evidence From a US Registry.

Son SJ, Sambare TD, Sun H, Fasig BH, Reddy NC, Kelly MP, Paxton EW, Prentice HA

BACKGROUND: Benchmarking seeks to evaluate implant survivorship at certain follow-up time points against predetermined standards. The International Society of Arthroplasty Registries (ISAR) International Prosthesis Benchmarking Working Group (IPBWG) developed a predefined set of standards in which receiving a benchmark indicates that an implant has acceptable revision-free survivorship, whereas not receiving a benchmark suggests that the revision rate exceeded these established standards. Although benchmarking of TKA implant systems has been performed in select international countries, applications of benchmarks to TKA systems used in a US-based cohort are lacking.

QUESTIONS/PURPOSES: We sought to apply the ISAR IPBWG benchmarking standards to our US-based healthcare system's total joint replacement registry and identify TKA implant systems meeting (1) early 2- and 5-year benchmarks, (2) midpoint 10-year benchmarks, and (3) late 15-year benchmarks. As a secondary aim, we sought to evaluate the consistency of these benchmarks when stratified by patient gender and age.

METHODS: Data from our US-based healthcare system's Total Joint Replacement Registry were used to conduct a retrospective cohort study. The registry is well suited for benchmarking analyses because it captures all primary and revision TKAs performed within a closed, longitudinal healthcare system, enabling near-c [...]

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CORR® Curriculum-Orthopaedic Education: Residency Procedure Volumes Should Mirror Actual Practice.

Kellam J, Dougherty PJ

PubMed · DOI

Residency Diary: Saying Goodbye.

Friedman LGM

PubMed · DOI

ArtiFacts: Malgaigne's Patellar Hooks.

Hawk AJ

PubMed · DOI

Your Best Life: Look for the Good and You Will Find It.

Kelly JD

[PubMed](#) · [DOI](#)

Art in Science: Why Two Leicas?

Green SA

[PubMed](#) · [DOI](#)

Editorial Comment: Diversity and Disparities in Orthopaedic Surgery: Update 2026.

Templeton KJ, Hogan MV

[PubMed](#) · [DOI](#)

Which Neighborhood-level Metric Is Most Appropriate for Pediatric Sports Medicine Disparities Research?

Maxwell BE, Raffman ES, Navarro MB, Rosenberg SI, Merritt EH, Patel NM

BACKGROUND: Neighborhood conditions are associated with access to care and outcomes. The Child Opportunity Index (COI) and Area Deprivation Index (ADI) are commonly used in pediatric research but without a clear rationale as to why one is chosen over the other. Despite an increasing volume of health equity research, there is little evidence that has directly compared the ADI and COI's associations with previously established pediatric orthopaedic disparities. Thus, the most appropriate neighborhood-level measure for pediatric orthopaedic research remains unclear.

QUESTIONS/PURPOSES: (1) Do COI and ADI correlate with each other? (2) How do COI and ADI compare in their associations with previously established pediatric orthopaedic disparities, including time to ACL reconstruction (ACLR) and the presence of concomitant meniscal and chondral pathology?

METHODS: This is a retrospective, comparative study of patients aged 18 years or younger who underwent primary ACLR between 2010 and 2023 at one tertiary center. We excluded patients who were missing COI or ADI data. Patients who underwent multiligament reconstruction, revision ACLR, intentionally staged or delayed procedures, or previous surgery on either knee were also excluded. We initially considered 806 patients, of whom 9% (72) were excluded for prespecified reasons. Consequently, 734 patients were included in the study (mean [...]

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CORR Insights®: Which Neighborhood-level Metric Is Most Appropriate for Pediatric Sports Medicine Disparities Research?

Kirchner GJ

[PubMed](#) · [DOI](#)

After ACL Injury in Children and Adolescents, Where in the Preoperative Timeline do Disparities in Timing and Clinical Course Originate?

Nutescu M, England P, Rosenberg SI, Merritt EH, Patel NM

BACKGROUND: Disparities in the timing of pediatric ACL reconstruction (ACLR) have been described along the lines of insurance and race. However, it is unclear where exactly in the preoperative timeline these inequities originate. Effective interventions to mitigate disparities cannot be designed without understanding this. This study aims to identify differences in the course and timing at each stage of the pre-ACLR process with respect to insurance and race or ethnicity. Such data can be used in the development of strategies to reduce delays for pediatric patients with ACL injuries.

QUESTIONS/PURPOSES: (1) When considering insurance and race or ethnicity, where in the preoperative timeline do disparate delays occur? (2) Do patients who are publicly insured or those in racially or ethnically minoritized groups experience differences in the type and number of clinicians seen before surgery?

METHODS: In this retrospective, comparative study, we identified 591 patients treated surgically for ACL injuries between 2011 and 2023. Of those, patients aged 18 years or younger who underwent primary ACLR were included. Fifty-seven were excluded due to intentionally delayed surgery, staged procedures, multiligamentous

reconstruction, or prior surgery in either knee, leaving 534 for analysis. The mean $\pm$ SD age was 16 ± 2 years, and 51% (271) of participants were boys. Demographic data were [...]

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CORR Insights®: After ACL Injury in Children and Adolescents, Where in the Preoperative Timeline do Disparities in Timing and Clinical Course Originate?

Polousky JD

[PubMed](#) · [DOI](#)

Higher Area Deprivation Index Is Associated With Greater Practice-initiated Perioperative Communication Workload in Patients With Primary Total Joint Arthroplasty.

Lam AD, Leipman JH, Meacock SS, Parikh N, Sherman MB, Fillingham YA, Krueger CA

BACKGROUND: Although deficits in social determinants of health (SDOH) have been previously associated with adverse clinical outcomes after primary THA and TKA, their role in the perioperative communication workload remains poorly characterized. Even though it remains essential to appropriately identify and address modifiable SDOH before a procedure, orthopaedic practices must also have the resources to handle the coordination of care effectively. Understanding how deficiencies in SDOH can impact communication workload would help support effective resource planning and equitable patient engagement strategies, particularly as more perioperative management takes place outside the hospital setting.

QUESTIONS/PURPOSES: (1) What are the differences in touchpoint utilization in patients who live in locations with varying Area Deprivation Index (ADI) scores, a surrogate measure for social deprivation? (2) Does social deprivation have an association with the length of stay (LOS) during primary total joint arthroplasty? (3) How are readmission rates and patient-reported outcome measures (PROMs) different in patients living in areas with varying degrees of social deprivation?

METHODS: In this retrospective, comparative study, there were 92,801 patients who underwent primary, elective THA (43% [39,963]) or TKA (57% [52,837]) for osteoarthritis at one high-volume, urban, academic institution [...]

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CORR Insights®: Higher Area Deprivation Index Is Associated With Greater Practice-initiated Perioperative Communication Workload in Patients With Primary Total Joint Arthroplasty.

Tyler WK

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Are Social Determinants of Health Disparities Associated With Worse Completion Rates for Patient-reported Outcome Measures After Total Joint Arthroplasty?

Meacock S, Hohmann A, Sellig MT, Sherman M, Fillingham YA

BACKGROUND: Patient-reported outcome measures (PROMs) are important markers to assess patient improvement after total joint arthroplasty (TJA). PROMs are increasingly relevant because of new PROM-reporting requirements for elective inpatient TJA from the Centers for Medicare & Medicaid Services. Social determinants of health (SDOH) disparities have been associated with various worse outcomes after TJA, but to our knowledge, it is not yet known how PROM completion may be affected by SDOH disparities.

QUESTIONS/PURPOSES: Among patients undergoing TJA, (1) are there SDOH disparities (such as insurance, transportation access, and living alone) that are associated with differences in PROM completion? (2) Are there neighborhood metrics, including Social Vulnerability Index (SVI) and the Area Deprivation Index (ADI), that are associated with differences in PROM completion?

METHODS: This study was a retrospective, comparative single-institution study of 12,842 patients who underwent primary, unilateral TJA for osteoarthritis between 2019 and 2022. Study participants had a mean $\pm$ SD age of 67 ± 10 years, 45% (5745) were men, and 86% (10,131 of 11,833) were White. Mean $\pm$ SD national ADI score was 34 ± 21 , and SVI score was 0.4 ± 0.3 . PROMs were collected within the first year preoperatively and at 6 months, 1

year, and 2 years postoperatively per institutional protocol. Patient demogra [...]

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CORR Insights®: Are Social Determinants of Health Disparities Associated With Worse Completion Rates for Patient-reported Outcome Measures After Total Joint Arthroplasty?

Balkissoon R

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Are LGBTQ+ Allyship Symbols Associated With Patient Trust in the Orthopaedic Surgeon? A Dual Cohort Survey.

Catley CD, Payne ER, Ballatori SE, Pangelinan J, Chen L, O'Keefe R, Cahill SV

BACKGROUND: Lesbian, gay, bisexual, transgender, and queer (LGBTQ+) patients often face barriers to trust in healthcare settings because of historical and ongoing discrimination. In orthopaedic surgery-a specialty sometimes perceived as less inclusive-visible symbols of allyship such as rainbow flags may influence patient perceptions, yet this remains unstudied.

QUESTIONS/PURPOSES: (1) Is the display of LGBTQ+ allyship symbols (rainbow flags) associated with patient trust in an orthopaedic surgeon? (2) Which demographic factors are associated with positive or negative responses to these symbols? (3) What opinions do patients have regarding LGBTQ+ surgeons or displaying these symbols in an orthopaedic setting?

METHODS: We conducted two randomized, blinded surveys; the first recruited from a tertiary urban and academic orthopaedic clinic (362 of 477 patients contacted) and the second cohort recruited nationally via the online Amazon Mechanical Turk (MTurk) platform (439 of 1258 patients contacted). Clinic participants had a mean $\pm$ SD age of 48 $\pm$ 17 years, with 62% (223) women, while online participants had a mean $\pm$ SD age of 45 $\pm$ 17 years, with 49% (215) women. Online participants were more frequently White, men, urban dwelling, and highly educated, with 82% (358 of 439) holding a bachelor's degree or higher. Prior reported experience with orthopaedic care was common in both gr [...]

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CORR Insights®: Are LGBTQ+ Allyship Symbols Associated With Patient Trust in the Orthopaedic Surgeon? A Dual Cohort Survey.

McAllister Nolan B

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Women Are Unequally Represented Among Clinical Trial Leadership by Orthopaedic Subspecialty.

Mao E, Glenn ER, Ryu D, Chang B, LaPorte DM, Aiyer A

BACKGROUND: Although men substantially outnumber women in orthopaedic surgery, prior studies have demonstrated that authorship disparities persist even after adjusting for this imbalance. If comparable patterns occur in clinical trial leadership, they may hinder women's academic advancement in our field. However, to our knowledge, no studies to date have examined proportional representation in this context.

QUESTIONS/PURPOSES: (1) Does the proportion of woman-led clinical trials differ among orthopaedic subspecialties? (2) Are women proportionally represented as PIs after adjusting for the percentage of women in each specialty? (3) Is the proportion of woman-led orthopaedic clinical trials associated with other characteristics, such as trial location or type of intervention?

METHODS: A retrospective analysis of orthopaedic surgery clinical trials registered on ClinicalTrials.gov from 2007 to 2025 was performed. Trials were manually reviewed for subspecialty relevance and PI identity. PI gender was determined via genderize.io, a validated online application that assigns a predicted gender to user-input names. Trials without orthopaedic surgeons as PIs and those with low-confidence gender predictions via genderize.io were excluded. Of the trials initially identified, 26% (1510 of 5842) met inclusion criteria, of which 55% (837 of 1510) were US-based and 42% (637 of 1510) were i [...]

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CORR Insights®: Women Are Unequally Represented Among Clinical Trial Leadership in Orthopaedic Subspecialty.

Mener A

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Program Size and City Characteristics May Be Associated With Gender Diversity in Orthopaedic Surgery Residency Programs.

Park YL, Grossman M, Collier C, Christiansen MJ, Musahl V, Mulcahey MK

BACKGROUND: Orthopaedic surgery remains one of the least gender-diverse specialties in medicine. Prior studies have identified factors that influence a woman applicant's rank list, such as surgical experience, women faculty, and resident satisfaction; however, little is known about how the geographic location of a program is associated with the gender composition of residency classes.

QUESTIONS/PURPOSES: (1) Is the geographic location of an orthopaedic surgery residency program associated with the proportion of women in its residency class? (2) After adjusting for potential confounders like program size and type, what program- and city-level factors are associated with a higher percentage of women in orthopaedic residency programs?

METHODS: Accreditation Council for Graduate Medical Education (ACGME)-accredited orthopaedic residency programs with updated resident lists for the 2025 to 2026 academic year were included; we included 204 such programs from a total of 212. Resident gender was determined using photographs, biographies, and preferred pronouns when available. The proportions of women residents were compared across US geographic divisions as defined by the Electronic Residency Application Service (ERAS®). Secondary analyses explored whether program characteristics (program type, women faculty proportion, gender of chair and program director) and city-level factors (po [...])

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CORR Insights®: Program Size and City Characteristics May Be Associated With Gender Diversity in Orthopaedic Surgery Residency Programs.

Lutnick E

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Minimum 10-year Results of Cementing a Polyethylene Liner Into an Acetabular Cup With a Deficient Locking Mechanism: Is It a Reliable Option?

Rhyu KH, Cho YJ, Chun YS, Lee MG

BACKGROUND: Cementing a new liner into a well-fixed acetabular cup with a deficient locking mechanism has been reported as a viable option, but concerns remain regarding complications such as dislocation and late loosening.

QUESTIONS/PURPOSES: What were the (1) durability of the cement-liner interface, (2) patient-reported outcome scores and complications, and (3) survivorship free from revision after liner cementation into a single acetabular cup design (Harris-Galante II, Zimmer) when the cup's locking mechanism was found to be deficient?

METHODS: Between May 2006 and January 2015, two surgeons performed 76 revision procedures with retention of the metallic shell by cementing a new polyethylene liner into the existing shell. During that time, this procedure was used when the acetabular shell was well fixed but the locking mechanism was found to be deficient, or when a compatible liner was unavailable. When the native locking mechanism appeared intact, liner exchange was performed using the original locking mechanism without cementation. All patients treated with this procedure were considered potentially eligible for inclusion in this retrospective study. Of those, 9% (7 of 76) were excluded because they had a shell other than the Harris-Galante II cup, which was the focus of this study. An additional 16% (12 of 76) were lost prior to the minimum study follow-up of 10 years [...]

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CORR Insights®: Minimum 10-year Results of Cementing a Polyethylene Liner Into an Acetabular Cup With a Deficient Locking Mechanism: Is It a Reliable Option?

Chen KL

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Is Preoperative Moderate to Severe Varus Alignment Associated With Survivorship and Outcome Scores at a Minimum of 5 Years After Mobile-bearing Total Ankle Arthroplasty With the Hinge Device?

Yoon YK, Park KH, Shim DW, Han SH, Lee JW

BACKGROUND: Analyses of long-term results after mobile-bearing total ankle arthroplasty (TAA) in ankles with preoperative moderate to severe varus deformity with adequate sample sizes remain limited. Because preoperative moderate to severe varus alignment may influence implant survival and functional outcomes, clarifying its long-term impact is important.

QUESTIONS/PURPOSES: (1) Do the clinical and radiologic outcomes of mobile-bearing TAA differ between ankles with preoperative neutral alignment and those with moderate to severe varus alignment? (2) What are the 5- and 10-year prosthesis survivorship, survivorship free from any revision including polyethylene exchange, and survivorship free from any unplanned reoperation in each group after mobile-bearing TAA? (3) Did the incidence of asymmetric polyethylene wear differ between the groups?

METHODS: Between September 2004 and May 2019, a single nondeveloper surgeon performed 417 TAAs. All procedures were performed using a single-design mobile-bearing device. The general indications for TAA were painful end-stage ankle arthritis. Of the 382 eligible ankles, 44% (168 ankles) were in patients with varus ankles (a preoperative coronal tibiotalar ankle $\geq 10^\circ$ of varus) and 56% (214 ankles) were in neutral ankles (defined as $< 10^\circ$ of varus or valgus). Sixty-one percent (103 of 168) of the varus group and 61% (131 of 214) of the neut [...]

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CORR Insights®: Is Preoperative Moderate to Severe Varus Alignment Associated With Survivorship and Outcome Scores at a Minimum of 5 Years After Mobile-bearing Total Ankle Arthroplasty With the Hinge Device?

SooHoo NF

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What Is the Interrater Reliability and Agreement Between the Decision Tree Patient-rated Wrist Evaluation and the Full-length Version?

Peters ST, Wouters RM, Selles RW, Slijper HP, and the Hand-Wrist Study Group

BACKGROUND: A frequently used strategy to improve patient-reported outcome measure (PROM) response rates and reduce patients' burden with PROM completion is item reduction. In this context, a shortened decision tree version of the Patient-Rated Wrist Evaluation (DT-PRWE) was developed, reducing the number of items from 15 to 5. The DT-PRWE demonstrated excellent psychometric properties in simulated data; however, its psychometric properties have not yet been evaluated in a real-world clinical setting.

QUESTIONS/PURPOSES: (1) What is the interrater reliability and agreement between the DT-PRWE and the Patient-Rated Wrist Evaluation (PRWE) in a clinical setting? (2) What is the difference in completion time between the DT-PRWE and the PRWE?

METHODS: We conducted a prospective study at Xpert Clinics in the Netherlands, a multicenter, referral-based outpatient practice, with both urban and regional locations, that specializes in hand and wrist surgery and hand therapy to assess the interrater reliability and agreement between the PRWE and the DT-PRWE. Between January and April 2025, a total of 427 adult patients were treated for wrist-related conditions and completed the PRWE at baseline as part of routine outcome measurement at our clinic. Subsequently, we asked patients to complete the DT-PRWE again 5 to 10 days after the initial assessment. Of those, we considered adult pa [...]

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CORR Insights®: What Is the Interverision Reliability and Agreement Between the Decision Tree Patient-rated Wrist Evaluation and the Full-length Version?

London DA

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Bilateral Open Adductor Longus Tenotomy Is Associated With Predictable Return to Play in Professional Soccer Players: A Case Series of 100 Patients.

Cugat R, Montaña F, Barastegui D, Barrueto D, Vázquez M, Ferré-Aniorte A, Láiz P, Seijas R

BACKGROUND: Dynamic pubic osteopathy is a clinical diagnosis characterized by chronic localized pain at the pubic insertion of the adductor longus that can limit high-level athletic performance in professional soccer players. Although open adductor longus tenotomy has been used for decades in selected athletes, little is known about return-to-play (RTP) timelines and outcomes in professional soccer players undergoing modern bilateral open release.

QUESTIONS/PURPOSES: We asked: (1) How long after bilateral open adductor longus tenotomy did professional soccer players return to unrestricted match participation? (2) What proportion returned to their preinjury competitive level? (3) Was player age associated with RTP duration? (4) What complications and reoperations occurred after surgery?

METHODS: We performed a retrospective case series of 100 professional men's soccer players (Tegner activity level 9 to 10) who underwent bilateral open adductor longus tenotomy between 2011 and 2024. All patients were diagnosed clinically with dynamic pubic osteopathy based on localized pubic insertion pain reproduced with resisted adduction. Radiographs were used to exclude fractures, stress fractures, tumors, or avulsion injuries. MRI was performed to evaluate associated findings such as bone edema or degenerative changes of the pubic symphysis; however, imaging findings were frequently withi [...]

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CORR Insights®: Bilateral Open Adductor Longus Tenotomy Is Associated With Predictable Return to Play in Professional Soccer Players: A Case Series of 100 Patients.

Owusu-Akyaw K

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Can Personalized 3D Kinematic Modeling Predict Loss of Pronation and Supination in Diaphyseal Forearm Malunions? A Clinical Validation Study.

van Loon DFR, van Es EM, Siemensma MF, Eygendaal D, Stockmans F, Veeger DHEJ, Colaris JW

BACKGROUND: Despite the clinical relevance of forearm fractures and malunions and the impact of a functional limitation, the link between forearm malalignment and limited pronation and supination remains poorly understood and still relies on anatomical alignment expressed as angulation. Using recently developed technologies, mechanisms that limit function can be automatically detected by modeling individual forearm kinematics using three-dimensional (3D) bone models of the radius and ulna.

QUESTIONS/PURPOSES: We evaluated the accuracy of a personalized 3D kinematic model to identify limitations in forearm rotation in pronation and supination and to answer the following questions: (1) How accurately does the model-predicted ROM agree with the corresponding clinical measurements? (2) How accurately does the model classify malunited forearms according to the presence of clinically relevant functional limitations, defined as a range of pronation or supination less than 50°? (3) What is the frequency at which the model detects bone impingement and central band block during pronation and supination?

METHODS: This retrospective study evaluated a diagnostic model using the preoperative CT scans of 45 patients with unilateral diaphyseal forearm malunions, all of whom underwent corrective osteotomy due to a clinically relevant limitation in pronation or supination function. In all, 53% [...]

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CORR Insights®: Can Personalized 3D Kinematic Modeling Predict Loss of Pronation and Supination in Diaphyseal Forearm Malunions? A Clinical Validation Study.

Beredjikian PK

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Letter to the Editor: Guest Editorial: Recalling a Recall.

Ekinici S

[PubMed](#) · [DOI](#)

Reply to the Letter to the Editor: Guest Editorial: Recalling a Recall.

Cornell CN

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Letter to the Editor: Compensatory Activation of Periscapular Muscles Aids Active Abduction in Patients With Massive Rotator Cuff Tears.

Qin K, Xu Y, Liang W

[PubMed](#) · [DOI](#)

Letter to the Editor: Can Nutritional and Inflammatory Markers Be Combined to Develop an Accurate Survival Model for Surgical Decision-making in Patients With Lung Cancer Bone Metastases?

Eckerle L

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Reply to the Letter to the Editor: Can Nutritional and Inflammatory Markers Be Combined to Develop an Accurate Survival Model for Surgical Decision-making in Patients With Lung Cancer Bone Metastases?

Yan Y, Zhang Y

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Letter to the Editor: Editorial: AAOS Orthobiologics Registry-Sometimes, More is Less.

Wang X, Shen W

[PubMed](#) · [DOI](#)

Letter to the Editor: Does First Metatarsal Head Lowering During Minimally Invasive Chevron Akin Osteotomy Yield No Significant Difference in Patient-reported Outcomes Compared With Second Distal Metatarsal Minimally Invasive Osteotomy for Treating Intractable Plantar Keratosis?

Zhao J, Gao J

[PubMed](#) · [DOI](#)

Reply to the Letter to the Editor: Does First Metatarsal Head Lowering During Minimally Invasive Chevron Akin Osteotomy Yield No Significant Difference in Patient-reported Outcomes Compared With Second Distal Metatarsal Minimally Invasive Osteotomy for Treating Intractable Plantar Keratosis?

Choi JY, Yi Y, Choi SM, Suh JS

[PubMed](#) · [DOI](#)

Reply to the Letter to the Editor: Editorial: AI-assisted Letters to the Editor-Scope of a Growing Ethical and Practical Concern, and CORR 's Approach to Managing It.

Daly T, Teixeira da Silva JA

[PubMed](#) · [DOI](#)



JBJS (American)

Volume 108, Issue 18 · September 2026 · 19 new articles

Toward More Reliable Survival Prediction in Osteosarcoma: The Case for Multi-Data-Set AI: Commentary on an article by Bishoy M. Galoaa, BS, et al.: "Enhancing Osteosarcoma Survival Predictions. A Comparative Study of a Multicomponent-Model Machine Learning Approach Integrating SEER and NCDB Data Sets Versus Conventional Single-Data-Set Modeling".

Ruggieri P

[PubMed](#) · [DOI](#)

Enhancing Osteosarcoma Survival Predictions: A Comparative Study of a Multicomponent-Model Machine Learning Approach Integrating SEER and NCDB Data Sets Versus Conventional Single-Data-Set Modeling.

Galoaa BM, Ubong SE, Girgis AG, Gonzalez MR, Lozano-Calderon SA

BACKGROUND: Osteosarcoma treatment decisions require accurate prognostic assessment, yet utilization of machine learning (ML) models is problematic because performance degrades consistently when models are applied across data sets. Current single-data-set models (single models) learn population-specific patterns rather than generalizable disease characteristics, limiting their clinical implementation. We developed a multicomponent-model (multi-model) ML framework using domain-adversarial training across 2 national registries, integrating structured clinical variables with text-based patient data to learn generalizable disease patterns and achieve reliable survival prediction across diverse populations.

METHODS: We conducted a retrospective study using data from 2 national cancer registries: SEER (Surveillance, Epidemiology, and End Results; n = 4,278 patients, 2004 to 2015) and NCDB (National Cancer Database; n = 4,049 patients, 2004 to 2018). We compared the cross-data-set performance of single models versus the multi-models. Primary outcomes were performance metrics for 2-year and 5-year overall survival predictions, measured by the area under the receiver operating characteristic curve (AUC), precision, recall, F1-score, and Brier score.

RESULTS: Single models achieved strong internal validation performance (AUC, 0.898 to 0.927) but performance declined substantially in cr [...]

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Erratum: One-Stage Versus Two-Stage Exchange Arthroplasty for Periprosthetic Joint Infection.

Fehring TK, Otero JE, Fehring KA, Curtin BM, Springer BD, Della Valle CJ, Parvizi J, Hietpas K et al.

[PubMed](#) · [DOI](#)

Importance of Medial Ligament Complex Tears Associated with Posterior Cruciate Ligament Avulsion Fractures: Commentary on an article by Binghao Wang, MD, et al.: " Medial Collateral Ligament Injury in Posterior Cruciate Ligament Tibial Avulsion Fractures. An Underrecognized Finding ".

Marshall SC

[PubMed](#) · [DOI](#)

Vertebral Body Tethering: We Have to Get It Just Right: Commentary on an article by Rachelle Imbeault et al.: " Outcomes and Complications of Vertebral Body Tethering in Skeletally Immature Patients with Idiopathic Scoliosis ".

Polly DW

[PubMed](#) · [DOI](#)

Nobody Said It Was Going to Be Easy: Commentary on an article by Alan D. Lam, BS, et al.: " Surgeons May Be 'Touched Out': The Ever-Increasing Workload of Telephone Calls and Electronic Messages in Total Joint Arthroplasty ".

Schenck RC

[PubMed](#) · [DOI](#)

Concentration and Frequency of Intraoperative Irrigation as a Strategy Against Implant-Associated Infection: Insights from a Preclinical Rat Model: Commentary on an article by Musashi Ima, MD, et al.: " Diluted Povidone-Iodine Irrigation for Prevention of Implant-Related Infection. A Comparative Analysis of Concentration and Frequency in a Rat Model ".

Kobryn A, Schwarz EM

[PubMed](#) · [DOI](#)

What's New in Hip Surgery.

Ekhtiari S, Ali MI, Hoveidaei AH, Luo TD, Köse Ö, Guerra-Farfán E, Citak M

[PubMed](#) · [DOI](#)

The Functional Vital Sign: Interpreting Physical Function to Guide Treatment.

Baumhauer JF, Cella D

[PubMed](#) · [DOI](#)

The Unexamined Practice: Five Philosophical Concepts in an Orthopaedic Encounter.

Parisien R

[PubMed](#) · [DOI](#)

Thoughts At Hand.

Ron Translateur T

[PubMed](#) · [DOI](#)

Orthopaedic Management During the First 24 Hours of the October 7 Mass Casualty Event in Israel: The Experience of a Frontline Trauma Center.

Rosinsky PJ, Artamonov A, Sasson T, Issa MA, Debbi R, Lubovsky O

➤ On October 7, 2023, a large-scale attack in southern Israel resulted in a major mass casualty event (MCE). One of the first hospitals to receive patients was Barzilai Medical Center (BMC) in Ashkelon, located approximately 10 km north of the Gaza border. ➤ During the first 24 hours, BMC managed a substantial extremity injury burden, placing orthopaedic surgeons in central operational roles. These extended beyond surgical care to include emergency department (ED) triage, operating room (OR) prioritization, and interhospital coordination. ➤ Israel's longstanding experience with MCE preparedness enabled rapid nationwide mobilization of medical personnel, and the country's compact geography and integrated trauma network facilitated early transfer of patients across hospitals. ➤ Patient arrival did not occur immediately and only peaked several hours after the onset of the attack. This delay allowed for staff mobilization, OR preparation, and establishment of coordinated patient transfer. ➤ The preservation of operative capacity required selective surgical management. Essential stabilization measures

performed in the ED, including hemorrhage control, tourniquet reassessment, vascular evaluation, wound debridement, and provisional immobilization, enabled safe stabilization and patient transfer, while reducing the risk of preventable ischemic injury. > These findings support the role [...]

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Nontraumatic Osteonecrosis of the Femoral Head: An Update.

Mont MA, Smitterberg CW, Jones LC, Goodman SB, Lieberman JR, Parvizi J, Cheng EY

> Nontraumatic osteonecrosis of the femoral head should be suspected in patients who are <50 years of age who have persistent hip pain despite normal radiographs. > Bilateral involvement is common; magnetic resonance imaging (MRI) of the contralateral hip is recommended. > MRI is the most sensitive modality for detecting early disease. > Femoral head sphericity is assessed on anteroposterior, lateral, and 30° view radiographs. > Small pre-collapse lesions have the best prognosis. Large lateral lesions and subchondral collapse predict rapid progression. > Systemic risk factors and multifocal disease should be actively evaluated. > Joint-preserving procedures are most effective before structural collapse. > Total hip arthroplasty remains the most reliable treatment after femoral head collapse.

[PubMed](#) · [DOI](#)

Compliance with the CONSORT AI Extension in Orthopaedic Randomized Controlled Trials.

Strasser T, Baker C, Cheminant J, Han B, Canumay S, Schlechter J

BACKGROUND: The utilization of artificial intelligence (AI), such as machine learning, in health care has risen exponentially, underscoring the necessity of transparency and traceability in the reporting of AI in randomized controlled trials (RCTs) that involve AI. In 2020, a CONSORT (Consolidated Standards of Reporting Trials) AI extension was published that included 14 additional items that should be reported when AI is used in RCTs. This study aimed to evaluate compliance with these additional guidelines.

METHODS: A systematic approach was used to identify all RCTs in orthopaedics that involved AI since the extension was published. Three databases were searched (PubMed, Embase, Cochrane) using the search terms ([orthopaedic] OR [orthopedic]) AND ([artificial intelligence] OR [AI] OR [machine learning] OR [deep learning]).

RESULTS: One hundred and seventy-four articles were identified, and 11 articles satisfied the inclusion and exclusion criteria and were assessed for compliance. The average number of CONSORT AI-extension items met per study was 11.4 ± 1.6 ($81.2\% \pm 12.1\%$). Eight items (1a [i], 1b [ii], 2a [i], 4a [i], 5 [ii], 5 [v], 5 [vi], 19) that were related to standard reporting of AI utilization were reported in all articles. The items with the lowest reporting rates were related to data transparency and included 4a (ii), 5 (iii), and 25, with only 9.1%, 36.4%, and 4 [...]

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Minimally Invasive Sacroiliac Joint Fusion Compared with Sham Operation (SIFSO Trial): A Concise Follow-up, at 2 Years, of a Previous Report.

Kibsgård TJ, Gerdhem P, Diarbakerli E, Randers EM

UNLABELLED: Minimally invasive fusion of the sacroiliac joint (SIJ) as a treatment for SIJ pain remains controversial. The SIFSO trial was a double-blinded, sham-controlled randomized trial with a primary end point of 6 months. Sixty-three patients were randomized to undergo either minimally invasive SIJ fusion or sham surgery. At the 6-month primary end point, a mean reduction of 2.6 points on the Numeric Rating Scale (NRS) was demonstrated in the surgical group, compared with a reduction of 1.7 points in the sham group (group difference: -1.0 point [95% confidence interval (CI), -2.2 to 0.3; $p = 0.13$]). Patients in the sham group were allowed to cross over to surgery after unblinding at 6 months. The current report describes the 2-year outcomes of all patients, including those who underwent SIJ fusion as part of the initial surgical group, those who crossed over to fusion after sham surgery, and those who remained in the sham group. After unblinding at 6 months, 26 of the 31 patients in the sham group crossed over to fusion. All 63 patients completed the 2-year follow-up. The mean age at baseline was 45 years (range, 26 to 63 years), and 59 of the 63 patients were female. Compared with baseline, the reduction in the NRS score at 2 years was 3.1 (95% CI, 2.0 to 4.1) in the initial fusion group ($n = 32$), 1.6 (95% CI, -1.8 to 5.0) in the sham group that did not cross over ($n = 5$ [...])

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Medial Collateral Ligament Injury in Posterior Cruciate Ligament Tibial Avulsion Fractures: An Underrecognized Finding.

Wang B, Ye T, Xie X, Zhang B, Wang Y, Sun L, Luo C, Zhu Y

BACKGROUND: Posterior cruciate ligament tibial avulsion fracture (PCLAF) is relatively rare. This fracture may be accompanied by soft tissue injuries, most commonly involving the medial collateral ligament (MCL). The present study aimed to determine the rate of MCL injury and its association with fracture characteristics in patients with PCLAF.

METHODS: Patients with PCLAF were identified with computed tomography, and associated ligamentous and meniscal injuries were evaluated with magnetic resonance imaging. Fracture morphology was assessed via heat maps and quantitative measurements. Receiver operating characteristic (ROC) curves and logistic regression analyses were utilized to identify predictors of MCL injury.

RESULTS: A total of 148 ethnic Chinese patients with PCLAF were included (mean age, 48.1 ± 12.9 years; 33.1% female), and 28.4% had concomitant MCL injuries. MCL injuries were significantly associated with posterior horn tears of the medial meniscus ($p < 0.001$). Patients with MCL injuries exhibited a larger fracture distribution area on heat maps. Consistent with that finding, quantitative analysis showed that these patients had a significantly smaller fracture medial border ($p < 0.001$) and a significantly larger fracture anteroposterior diameter percentage ($p < 0.001$). Multivariable analysis identified a fracture anteroposterior diameter percentage of $\geq 50.2\%$ as an [...]

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Outcomes and Complications of Vertebral Body Tethering in Skeletally Immature Patients with Idiopathic Scoliosis.

Imbeault R, Shen J, Alzakri A, Barchi S, Roy-Beaudry M, Turgeon I, Parent S

BACKGROUND: Vertebral body tethering (VBT) aims to gradually correct scoliosis using patients' growth while preserving spinal motion. We report 5 to 8-year outcomes and complications in skeletally immature patients.

METHODS: This prospective single-center cohort study included 74 patients who had idiopathic scoliosis and a ≥ 5 -year follow-up. Preoperative, first postoperative visit, 1-year, 2-year, and ≥ 5 -year radiographs were analyzed. A $\geq 5^\circ$ increase in the interscrew angle suggested tether breakage.

RESULTS: All 74 patients (5 male and 69 female) were skeletally immature at surgery. The mean age at surgery was 11.8 ± 1.3 years, and the mean follow-up time was 63.4 ± 8.4 months. Of the 74 patients, 68 patients were White, 4 were Black, and 2 were Middle Eastern or North African. VBT was performed on a mean of 7.4 vertebral levels. The maximum Cobb angle was $47.9^\circ \pm 9.4^\circ$ preoperatively, whereas the instrumented Cobb angle measured $17.2^\circ \pm 12.3^\circ$ at 2 years and $25.7^\circ \pm 14.0^\circ$ at ≥ 5 years postoperatively. An unplanned return to the operating room occurred in 16 patients (21.6%). Forty-nine patients (66%) had a suspected broken tether at the final follow-up. The mean time of the first tether breakage was 38.1 ± 15 months. Forty-nine patients (66%) also had a curve of $< 40^\circ$ without an unplanned return to the operating room at a minimum of 5 years postoperatively.

CONCLUSIONS: In our [...]

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Surgeons May Be "Touched Out": The Ever-Increasing Workload of Telephone Calls and Electronic Messages in Total Joint Arthroplasty.

Lam AD, Parikh N, Leipman JH, Abe EA, Hoffman E, Linton AA, Bido J, Purtill JJ et al.

BACKGROUND: With evolving patient expectations and practice models, perioperative communication through telephone and web encounters, termed "touchpoints," gains further importance. This study quantifies the workload created by telephone calls and electronic messages during patient episodes of care and analyzes their change over time.

METHODS: We retrospectively reviewed 277,729 telephone calls and 16,966 electronic messages within the 30-day preoperative periods and the 90-day postoperative periods of 45,216 patients who underwent primary total hip arthroplasty (THA) or total knee arthroplasty (TKA) from January 1, 2016, to December 31, 2022. Touchpoints were temporally trended by year to characterize utilization over time. Regression analysis was performed to determine factors predictive of increased touchpoint utilization.

RESULTS: Preoperatively, patients had a mean of 2.2 telephone calls (2.2 for THA and 2.1 for TKA) and 2.3 electronic messages (2.3 for both THA and TKA). Postoperatively, patients had a mean of 4.4 telephone calls (4.1 for THA and 4.6 for TKA) and 2.8 electronic messages (2.8 for both THA and TKA). From 2016 to 2022, the total perioperative touchpoints increased by 105% (8.7 to 17.8 touchpoints) for patients who underwent TKA and by 121% (7.3 to 16.1 touchpoints) for patients who underwent THA. Medicare insurance was independently associated with fewer t [...]

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Diluted Povidone-Iodine Irrigation for Prevention of Implant-Related Infection: A Comparative Analysis of Concentration and Frequency in a Rat Model.

Ima M, Kabata T, Inoue D, Yanagi Y, Iyobe T, Fujimaru N, Tokoro M, Nojima T et al.

BACKGROUND: Povidone-iodine (PVI) irrigation is widely used to reduce surgical site infection risk; however, the appropriate concentration and timing remain uncertain. We evaluated how the PVI concentration and irrigation interval influence the early bacterial burden and tissue response in a rat model of implant-related infection.

METHODS: Female rats received a stainless steel plate contaminated with methicillin-susceptible *Staphylococcus aureus*. The rats were randomized to receive 0.13% PVI, 0.35% PVI, or normal saline solution irrigation every 30 or 60 minutes during a 60-minute procedure. Irrigation consisted of a 3-minute exposure followed by a saline solution rinse. Outcomes included the bacterial count after sonication, soft-tissue infection score, peri-implant bone mineral density (BMD) on microcomputed tomography (μ CT), histological inflammation grading, and body weight trajectory.

RESULTS: Higher PVI concentrations and a shorter irrigation interval were associated with reduced recoverable bacterial burden. Among the 30-minute interval groups, no culturable bacteria were recovered in the 0.35% PVI group (i.e., the values were below the assay detection limit); in contrast, culturable bacteria were detectable in all of the 60-minute interval groups. PVI-treated rats demonstrated lower macroscopic infection scores and a trend toward more rapid body weight recovery comp [...]

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The Journal of Arthroplasty

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To the Editor: "Long-Term Outcomes After Conversion of High Tibial Osteotomy to Total Knee Arthroplasty in a United States Population".

Ma L, Sun J, Shi L

[PubMed](#) · [DOI](#)

In Reply: Outcomes After Conversion of High Tibial Osteotomy to Total Knee Arthroplasty: Nearly 14-Year Follow-Up in a United States Population.

Bax EA, Saris DBF

[PubMed](#) · [DOI](#)

Letter Regarding "Patients Who Have Metabolic Syndrome Have a Higher Chance of Postoperative Complications After Total Hip Arthroplasty".

Lu Z, Feng E, Lin Q

[PubMed](#) · [DOI](#)

Reply to the Letter Regarding: "Metabolic Syndrome Is Associated With Higher Odds of Postoperative Complications After Total Hip Arthroplasty".

Smitterberg CW, Katanbaf RM, Misch M, Nace J, Mont MA, Delanois RE

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The American Association of Hip and Knee Surgeons Position Statement on Outpatient Joint Replacement.

Ihekweazu UN, Walton S, Hannon CP

[PubMed](#) · [DOI](#)

The American Association of Hip and Knee Surgeons Position Statement on OrthoBiologics for Advanced Hip and Knee Osteoarthritis.

Carender CN, Grosso MJ, Browne JA, Nho SJ, Goodman SB, Della Valle CJ, Hannon CP

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Rethinking the Sequence of Arthroplasty-Focused Evaluation: Is It Time to Look at the Radiograph First?

Aflatooni JO, Smitterberg CW, Grynovicki EN, Scuderi GR, Delanois RE, Mont MA

Internal medicine training models intended for wide, symptom-driven differentials are the source of the conventional orthopaedic diagnostic sequence of history, physical examination, and subsequent imaging. However, this approach may cause inefficiencies in modern arthroplasty-focused clinics that are inconsistent with the structural basis of the majority of hip and knee pathologies. In high-volume clinics, patients are frequently roomed, sent for radiography, and then reroomed, which interferes with productivity and results in task switching and cognitive fragmentation. Early access to plain radiographs may improve patient safety and diagnostic efficiency given the likely pretest risk of structural joint disease in arthroplasty groups. This commentary suggests a triage-directed strategy wherein advanced care professionals, intake workers, or decision-support tools apply established intake criteria to gather relevant radiographs before the orthopaedic encounter. When utilized carefully, early imaging enables surgeons to place the physical examination and history in the context of known anatomy, promotes more targeted clinical inquiry, and may minimize needless or potentially uncomfortable examination procedures. Although there are legitimate worries about diagnostic anchoring, skilled orthopaedic doctors frequently modify diagnostic ideas when imaging and clinical results confl [...]

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Meeting Patient Expectations for Return to Recreational Activities.

Scuderi GR, Henry JP, Mont MA

[PubMed](#) · [DOI](#)

Using Early Gait Data From a Smart-Enabled Total Knee Arthroplasty to Identify Patient Function and Activity at 90 Days Postoperative.

Schwab JM, Raynor ME

BACKGROUND: A subset of total knee arthroplasty (TKA) patients experiences suboptimal recovery manifested by a decline in function, activity, or both. Smart implantable devices capable of continuous, adherence-independent kinematic monitoring offer an opportunity for early risk identification. This study aimed to determine whether early gait data from a smart implant could predict 90-day recovery outcomes reported as a three-class measure (green, yellow, and red).

METHODS: Kinematic data were obtained from 4,281 devices implanted in TKA patients. Data from 2,809 smart implantable devices formed the training dataset, and 1,472 formed the validation dataset. There were seven daily gait parameters from postoperative days eight to 90 that were reduced to two composite scores, function (tibial range of motion, knee range of motion, and stride length) and activity (step count, walk speed, distance, and cadence), based on principal components analyses. "Starting" (days, 8 to 21) and "outcome" (days, 77 to 90) ellipses were calculated for each patient, and a three-class outcome measure was assigned based on the proportion of the 95% confidence ellipse area within predefined function/activity quadrants. Multivariate logistic regression models using starting ellipse parameters were tested to predict the three-class outcomes at 90 days.

RESULTS: The distribution of outcomes was consiste [...]

Migration of Primary Total Knee Arthroplasty From Inpatient to Outpatient Facilities in the United States (2019 to 2022): Trends, Demographics, and Comorbidities.

Stern JM, Geller JS, Fernandez-Perez AM, Cruz-Ossa N, D'Apuzzo MR

BACKGROUND: Primary total knee arthroplasty (TKA) has shifted from inpatient to outpatient facilities in the United States, driven by surgical advances, the 2018 Center for Medicare & Medicaid Services inpatient-only list change, and COVID-19 pressures. Limited national data and unclear patient profiles in outpatient facilities versus hospitals prompt this study. Outpatient TKA patients are often younger and have fewer comorbidities, but safety debates persist. Using a national database, we examined (1) trends in knee arthroplasty settings, (2) demographic differences, and (3) comorbidity variations to inform patient selection, outcomes, and policy.

METHODS: We conducted a retrospective analysis using two national databases from 2019 to 2022. Adult patients undergoing elective, unilateral, primary TKA were identified using validated coding algorithms. We analyzed temporal trends in procedure location and compared demographic and comorbidity profiles between patients treated in hospitals versus outpatient facilities.

RESULTS: A total of 50% of the volume of primary TKA migrated to outpatient centers from 2019 to 2022. The proportion of hospital-based TKAs declined from 64 to 14%, whereas ambulatory surgical center (ASC)-based procedures rose from 36 to 86%. The ASC patients were younger (67.2 versus 67.6 years, $P < 0.01$), more likely to be White (78.6 versus 77.6%, $P < 0.01$), [...]

How Can Arthroplasty Surgeons Best Use Preoperative Patient-Reported Outcome Measures Across Multiple Domains to Predict Minimal Clinically Important Difference and Patient-Acceptable Symptom State Achievement in Medial Unicompartmental Knee Arthroplasty? Findings From a Prospectively Maintained Multi-Institutional Arthroplasty Registry.

Sauder N, Murrietta AJ, Booth M, Lim PL, Melnic CM, Bedair HS, Alpaugh K

BACKGROUND: Patient-reported outcome measures (PROMs) are increasingly utilized in medial unicompartmental knee arthroplasty (UKA). Total knee arthroplasty studies have shown preoperative PROMs across multiple domains (knee physical function, general physical health, mental health, etc.) influence achievement of minimal clinically important difference (MCID) or patient acceptable symptom state (PASS). However, the utility of preoperative PROMs in medial UKA is underexplored. We investigated the impact of preoperative PROMs across multiple domains on MCID and PASS achievement in medial UKA.

METHODS: We retrospectively sourced 911 medial UKAs from a prospectively maintained multi-institutional arthroplasty registry. The following preoperative PROMs were recorded: 1) Knee Osteoarthritis Outcome Score-Physical Function Short Form (KOOS-PS); 2) Patient-Reported Outcomes Measurement Information System (PROMIS) Physical Function-10a (PF-10a); 3) PROMIS Global Physical; and 4) PROMIS Global Mental. These PROMs measure the following domains, respectively: 1) knee-specific physical function; 2) general physical function; 3) general physical health; and 4) general mental health. Multivariable forward binary logistic regression models identified factors independently associated with the achievement of literature-derived, anchor-based KOOS-PS thresholds for MCID and PASS.

RESULTS: Achieve [...]

Opioid Consumption Trends in Resurfaced versus Unresurfaced Patellae in Total Knee Arthroplasty.

Leipman JH, Sutton R, Hohmann AL, Linton A, Bash HD, Lonner JH

BACKGROUND: Patellar resurfacing in total knee arthroplasty (TKA) is a highly debated topic, with equivocal results. While past literature has investigated pain, functional outcomes, and the cost of resurfaced versus unresurfaced patellae, examining opioid consumption is critical in the wake of the opioid epidemic. We sought to compare postoperative opioid consumption in patients who did and did not undergo patellar resurfacing in primary

TKA.

METHODS: This was a retrospective review of primary TKAs between 2018 and 2022 performed at a single high-volume institution with arthroplasty surgeons who practiced either selective or universal patellar resurfacing. Preoperative sunrise radiographs were reviewed to assess the severity of patellofemoral arthritis using the Iwano classification. Opioid prescription data were obtained from the Prescription Drug Monitoring Program and converted to morphine milligram equivalents (MMEs). Primary outcomes included total MME consumption and prescriptions filled. Secondary outcomes included length of stay, readmission, and patient-reported outcome measures. Of the 982 knees included, 447 were unresurfaced and 535 were resurfaced. There was a greater proportion of men in the unresurfaced versus the resurfaced group (52.8 versus 33.1%; $P < 0.001$); however, there was no difference in age (67.2 versus 67.7; $P = 0.431$).

RESULTS: For the first 30 da [...]

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Outcomes After Conversion of High Tibial Osteotomy to Total Knee Arthroplasty: Nearly 14-Year Follow-Up in a United States Population.

Bax EA, Ahmad RA, Kietselaer MA, Clark SC, Custers RJH, Taunton MJ, Sierra RJ, Hevesi M et al.

BACKGROUND: High tibial osteotomy (HTO) is a joint-preserving treatment for younger patients who have unicompartmental knee osteoarthritis. Although effective, concerns persist that prior HTO may compromise subsequent total knee arthroplasty (TKA). This study aimed to assess the revision-free survival of TKA following ipsilateral HTO and identify factors associated with failure of both procedures in a U.S.

METHODS: Patients who underwent TKA after prior ipsilateral HTO between 2000 and 2023 at a single academic institution were prospectively followed ($n = 134$), with a mean follow-up of 13.5 years (range, zero to 24.0) postoperative TKA and median ages at HTO and TKA of 52 and 63 years, respectively. Preoperative TKA radiographs were assessed using the Kellgren and Lawrence system. Kaplan-Meier analysis was performed to assess HTO and TKA survival. The mean HTO survival was 11.7 years (range, zero to 31.0), with a 10-year TKA conversion rate of 35.8%. Cox regressions identified factors associated with osteotomy survival.

RESULTS: Older age (hazard ratio [HR]: 1.06; $P < 0.001$), higher body mass index (HR: 1.03; $P = 0.033$), and lower Charlson Comorbidity Index (CCI) (HR: 0.79; $P < 0.001$) were significantly associated with earlier conversion. Following TKA, 6.0% underwent revision surgery. Revisions were more common in younger patients ($P = 0.01$) and those who had lower CCI ($P = \dots$)

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Survivorship and Functional Outcomes of Condylar-Stabilized Total Knee Arthroplasty Designs: An Analysis of the American Joint Replacement Registry.

Hegde V, Zaniletti I, Buddhiraju A, De A, Kagan R, Khanuja HS

BACKGROUND: The use of condylar-stabilized (CS) designs in total knee arthroplasty (TKA) is increasing. However, CS designs lack consistent classification and data examining survivorship. This study assessed revision risk and patient-reported outcome measures (PROMs) among CS designs and compared them to cruciate-retaining (CR) and posterior-stabilized (PS) designs.

METHODS: The American Joint Replacement Registry was queried for patients aged 65 years or older undergoing elective TKA with CR ($n = 111,622$), PS ($n = 282,487$), or CS ($n = 162,648$) designs between January 2012 and December 2021, with a minimum two-year follow-up. The CS designs were classified as anterior-stabilized ($n = 96,897$), ultracongruent ($n = 29,427$), medial-stabilized ($n = 21,555$), medial-pivot ($n = 11,171$), and lateral-stabilized ($n = 3,598$). Outcomes included all-cause revision, revision for infection, aseptic revision, and PROMs.

RESULTS: Compared to CR designs, medial-stabilized designs demonstrated lower all-cause (adjusted hazard ratio (aHR): 0.77, confidence interval (CI): 0.66 to 0.90, $P < 0.001$) and aseptic revision risk (aHR: 0.75, CI: 0.62 to 0.90, $P = 0.002$), lateral-stabilized designs had higher all-cause (aHR: 1.62, CI: 1.29 to 2.03, $P < 0.001$) and aseptic revision risk (aHR: 1.53, CI: 1.22 to 1.91, $P < 0.001$), and medial-pivot designs had higher all-cause revision risk (aHR: 1.23, CI: 1.02 [...])

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Body Mass Index of 45 Is a Safe Cut-Off for Cementless Total Knee Arthroplasty.

Madden-McKee CT, Bowsie KA, Cassidy RS, Napier RJ, Karayiannis PN, Beverland DE

BACKGROUND: It is widely perceived that morbid obesity is associated with unacceptable rates of complication after total knee arthroplasty (TKA), with many surgeons using a body mass index (BMI) of 40 as a cutoff. Also, cemented TKA is widely perceived as the gold standard. However, these two perceptions are not consistently supported by the literature. The aim of this study was to assess the outcomes of two patient groups undergoing cementless TKA in a high-volume unit: one with morbid obesity (BMI 40 to 44.9) and the other nonobese (BMI 20 to 29.9).

METHODS: This was a retrospective comparative single-center audit looking at 2,674 fully cementless TKAs, all performed with primary components and none having a patellar resurfacing. Of these, 568 were morbidly obese and 2,106 were nonobese. Median follow-up was 7.3 years (range, one to 13). Data were collected via electronic databases and radiology systems.

RESULTS: All-cause revision rates were 0.7% (four of 568) in the morbidly obese and 0.9% (19 of 2,106) in the nonobese ($P = 0.801$). There were no revisions for noninfective causes in the morbidly obese group. Other key outcomes, such as mortality, venous thromboembolism, and improvement in patient-reported outcomes scores, were also comparable. The reoperation rate for periprosthetic fracture was significantly lower in the morbidly obese (1.1%, [six of 568]) than the nonobe [...]

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Severe Coronal Plane Deformity Correction in Total Knee Arthroplasty Is Associated With Increased Surgeon Physiologic Demand.

Matheson DJ, Burnett RA, Kapron CR, Blackburn BE, Gililand JM, Anderson LA, Pelt CE, Peters CL et al.

BACKGROUND: Coronal plane deformity in total knee arthroplasty (TKA) can introduce major variability in difficulty between cases based on the type and degree of deformity. The purpose of this study was to compare the energy expenditure of TKA in varus and valgus knees.

METHODS: There were 85 patients undergoing primary, elective cemented TKA who were prospectively enrolled, and various demographic and surgical variables were collected. Surgeon physiologic parameters were collected via a smart garment that recorded heart rate variability (beats per minute), minute ventilation (mL/minute), and energy expenditure (kcal). Patients who had coronal plane deformities of various degrees were stratified into different groups based on severity: mild varus ($N = 31$), moderate/severe varus ($N = 28$), mild valgus ($N = 17$), and moderate/severe valgus ($N = \text{nine}$). A coronal plane deformity of 10° was the threshold for moderate/severe deformity. Analysis of variance models were used to compare demographics and surgeon physiologic stress between groups. There were differences in age and sex between cohorts ($P < 0.05$), whereas other demographics were comparable ($P > 0.05$).

RESULTS: Energy expenditure was significantly different across deformities, with valgus knees and increased deformity having the highest expenditure ($P < 0.001$). Heart rate variability and minute ventilation did not differ across [...]

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Preoperative Hip Flexion and Extension Range of Motion and Global Sagittal Alignment Affect Sagittal Spino-pelvic Alignment in Patients Undergoing Total Hip Arthroplasty.

Matsuki Y, Imagama T, Kawakami T, Numa M, Tanaka H, Sakai T

BACKGROUND: This study aimed to determine how preoperative hip flexion-extension range of motion (ROM) and sagittal vertical axis (SVA) affect postoperative ROM recovery, changes in sagittal spino-pelvic alignment, and patient-reported outcome measures (PROMs) following total hip arthroplasty (THA).

METHODS: We retrospectively reviewed 416 patients who underwent primary cementless THA. Spinopelvic parameters in standing and sitting positions and PROMs were evaluated preoperatively and at six months, one year, and two years postoperatively. Patients were divided into four groups based on preoperative hip flexion-extension ROM and SVA: the limited ROM (hip flexion $\leq 60^\circ$ and/or extension less than 0°) and high SVA (≥ 50 mm) group (LR-H group; $n = 60$), the limited ROM and normal SVA (less than 50 mm) group (LR-N group; $n = 77$), the preserved ROM (flexion greater than 60° and extension $\geq 0^\circ$) and high SVA group (PR-H group; $n = 56$),

and the preserved ROM and normal SVA group (PR-N group; n = 223).

RESULTS: All groups showed significant improvement in hip flexion-extension ROM postoperatively; however, the LR-H group maintained significantly lower values than the other groups at two years. This group showed the lowest sacral slope and lumbar lordosis in both standing and sitting positions, the highest SVA in both positions, and persistent highest spinopelvic mobility. Longitudinally [...]

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Metabolic Syndrome Is Associated With Higher Odds of Postoperative Complications After Total Hip Arthroplasty.

Smitterberg CW, Katanbaf RM, Misch M, Nace J, Mont MA, Delanois RE

BACKGROUND: Metabolic syndrome (MetS), a cluster of interrelated metabolic abnormalities typically including obesity, dyslipidemia, diabetes, and hypertension, has been linked to surgical complications, but its impact on total hip arthroplasty (THA) beyond 90 days is unclear. This study compared 90-days, 1-year, and 2-year THA outcomes in patients who had and did not have MetS, and identified which component of MetS (i.e., diabetes, hypertension, obesity, or hypertriglyceridemia) was most strongly associated with each complication. Outcomes included (1) infectious (surgical site infection [SSI; 90-days], periprosthetic joint infection [PJI]), (2) mechanical (mechanical loosening, periprosthetic fracture [PPFx]), (3) all-cause revision (ACR), and (4) deep vein thrombosis (DVT; 90-days).

METHODS: A nationwide database identified 706,602 THA patients (21.0% MetS) from 2010 to 2023. Outcomes were evaluated at 90 days, 1 year, and 2 years. Unadjusted comparisons used Pearson Chi-square and Student's t-tests; multivariable logistic regressions calculated odds ratios (ORs) with 95% confidence intervals.

RESULTS: Higher 90-day SSI (OR 2.41) and PJI at 90 days and at one and two years (OR 1.58, 1.44, and 1.31) were associated with MetS. Obesity was most strongly associated with infection (SSI OR 2.45; PJI OR 1.73 to 1.87), followed by diabetes (SSI OR 1.58; PJI OR 1.47 to 1.60) and hy [...]

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Reduced Lengths of Hospital Stay but No Difference in Survivorship Following Robotic Arm-Assisted Primary Total Hip Arthroplasty at 3.5 Years Follow-Up: A Propensity Score-Matched Prospective Cohort Study.

Poyser GT, Cheek TS, Toh Y, Vermeir JF, Donnelly WJ, Silva AM

BACKGROUND: Although robotic arm-assisted total hip arthroplasty (RA-THA) has been increasing in popularity, the outcomes following its use are still uncertain.

METHODS: We performed a propensity-matched prospective cohort study comparing all-cause revision, odds of instability/dislocation, odds of periprosthetic joint infection, and hospital length of stay (HLOS) between patients receiving an RA-THA versus a conventional THA (CO-THA). Consecutive patients undergoing a primary elective THA via a posterior approach at our institution between January 2019 and February 2024 were included. Those who received a bilateral simultaneous THA were excluded. We successfully matched 268 pairs of hips.

RESULTS: There was no difference in all-cause revision risk between the RA-THA and CO-THA groups (hazard ratio = 1.00; P = 1.000). Furthermore, there was also no significant difference in the odds of instability/dislocation (odds ratio = 1.00; P = 1.000) and the odds of periprosthetic joint infection (odds ratio = 0.75; P = 0.704). The HLOS was significantly shorter in the RA-THA cohort by 0.49 days (P = 0.044). The median follow-up duration was 3.46 years (interquartile range: 1.92 to 4.64).

CONCLUSIONS: Although there was no difference in short-term revision risk in patients receiving an RA-THA versus a CO-THA, likely, the benefits from improved implant positioning are yet to be realized [...]

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Ten-Year Functional Outcomes Following Robotic-Assisted Total Hip Arthroplasty: A Case Series.

Domb BG, Quesada-Jimenez R, Barda SR, Defort TM, Kuhns BD, Kahana-Rojkind AH

BACKGROUND: Robotic-assisted total hip arthroplasty (rTHA) has been shown to assist with precision in component placement. However, limited evidence exists evaluating its effects on minimum 10-year clinical outcomes. The purpose of this study was to report the minimum 10-year clinical outcomes in a case series of patients who underwent rTHA.

METHODS: Prospectively collected data from patients who underwent primary rTHA between June 2011 and April 2015 were analyzed. Patients who had a complete minimum 10-year follow-up, including radiographs and validated patient-reported outcomes, were included. A total of 215 hips met the inclusion criteria. The patient-reported outcomes included the Harris Hip Score, Hip Disability and Osteoarthritis Outcome Score-Joint Replacement, Forgotten Joint Score-12 (FJS-12), visual analog scale pain, and patient satisfaction. Radiographic assessment included acetabular cup inclination, version, leg-length discrepancy, and cup placement relative to the Lewinnek and Callanan safe zones.

RESULTS: At the final follow-up, patients reported excellent outcomes, with mean Harris Hip Score 93.0 ± 12.6 , Hip Disability and Osteoarthritis Outcome Score-Joint Replacement 89.1 ± 16.1 , FJS-12 85.2 ± 22.4 , visual analog scale pain 1.0 ± 1.9 , and satisfaction 9.1 ± 1.8 . The high FJS-12 score reflects that patients rarely or almost never thought about their hip in [...]

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Patellar Height and Functional Outcomes after Distal Femoral Replacement: Higher Patella Improves Flexion, While Patella Baja Predicts Worse Outcomes.

Aksoy T, Rumana R, Fitz SJT, Louka JG, Crawford BM, Temple HT

BACKGROUND: Distal femoral replacement (DFR) is a common limb salvage option for bone tumors. Postoperative outcomes partly depend on the integrity of the extensor mechanism, yet patellar height and joint line position have not been well studied in oncologic reconstructions.

METHODS: We retrospectively reviewed 86 patients who underwent modular DFR with rotating hinge prostheses (1992 to 2024). Demographics, surgical parameters, and functional outcomes were collected, including knee flexion, extensor lag, and Musculoskeletal Tumor Society (MSTS) scores. Postoperative patellar height was assessed with the Insall-Salvati ratio (ISR) and Blackburne-Peel index (BPI). Joint line position was measured relative to the fibular head. Statistical analyses included linear, multivariable, and segmented regression.

RESULTS: The mean ISR was 0.99 ± 0.22 , BPI 0.78 ± 0.20 , and joint line height was 17.7 ± 6.1 mm. Patellar height was classified as baja (14.0%), pseudobaja (18.6%), normal (59.3%), and alta (8.1%). Higher ISR correlated with greater knee flexion up to 120° ($P < 0.001$), after which the effect plateaued. BPI was associated with increased knee flexion ($P < 0.001$) and MSTS score ($P = 0.005$). A higher number of prior surgeries predicted lower BPI ($P = 0.009$). Patients who had patella baja demonstrated significantly reduced flexion, higher extensor lag, and inferior MSTS scores compa [...]

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External Validation of a New Classification for Bone Loss in Failed Stemmed Prostheses After Revision Total Knee Arthroplasty.

Herold JM, Shin J, Song YD, Nocon AA, Boettner F, Sculco PK

BACKGROUND: Evaluation and management of bone loss in revision total knee arthroplasty (rTKA) poses a demanding challenge. The rising number of knee revisions and rerevisions has led to increasing bone loss severity, and previous bone loss classification systems fail to provide adequate assessment of the diaphysis. The Scuderi classification was recently introduced to better assess diaphyseal bone loss and underwent internal validation. The purpose of this study was to perform an external validation using preoperative radiographs of failed stemmed rTKA and describe the interobserver and intraobserver reliability.

METHODS: From our institutional database, 128 preoperative radiographs of failed rTKAs with stemmed prostheses awaiting rerevision were identified. There were 120 stemmed femoral prostheses (60 cemented, 60 hybrid fixation) and 120 stemmed tibial prostheses (60 cemented, 60 hybrid fixation) that were subject to analysis. There were two readers who had similar experience levels who scored the cases according to the Scuderi classification, and each reader performed two independent reads at least two weeks apart from the first read. The levels of interobserver and intraobserver reliability were determined by the intraclass correlation coefficients.

RESULTS: Strong intraclass correlation coefficient for both femur (total: 0.96) and tibia (total: 0.89) has been demonstrated [...]

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Defining Preoperative Anemia Thresholds for Revision Total Knee Arthroplasty.

Sutton R, Leipman JH, Linton A, Sherman MB, Martinazzi BJ, Fellheimer HS, Fillingham YA

BACKGROUND: In an era of value-based medical management, anemia optimization before revision total knee arthroplasty (TKA) could improve outcomes. Using a simulation model, we evaluated whether outcomes would improve for revision TKA if patients were either approved/denied surgery based on incremental preoperative anemia thresholds.

METHODS: We retrospectively reviewed consecutive revision TKA patients from 2019 to 2024. Exclusion criteria were primary TKA, no preoperative hematocrit, and incomplete 1-year follow-up. Simulated preoperative hematocrit cutoff values of 39 to 33% were used in a sequential manner. Preoperative hematocrit values above each threshold "met the threshold" for surgery, while hematocrit values below each threshold "did not meet the threshold" for surgery. Intraoperative transfusion, postoperative transfusion, discharge disposition, 90-day readmission, and complications were evaluated. Area under the curve (AUC) determined anemia thresholds needed for an event not to occur.

RESULTS: If the strictest 39% hematocrit threshold was used, 669 of 1,322 (50.6%) patients would not meet the threshold for surgery, with these patients having higher readmissions (18.2 versus 8.4%), intraoperative transfusions (13 versus 0.5%), postoperative transfusions (26.3 versus 4.1%), nonhome discharge (53.8 versus 23.7%), and postoperative complications (26.8 versus 10.6%) compared [...]

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How Effective Are Constrained Liners at Preventing Dislocation After Revision Total Hip Arthroplasty?

Shah SN, Leal J, Stein MK, Deckey DG, Schwartz AM, Bolognesi MP, Ryan SP

BACKGROUND: Recurrent instability continues to challenge arthroplasty surgeons, and constrained liners may be utilized for cases of recurrent or multidirectional instability following revision total hip arthroplasty (rTHA). The purpose of this study was to evaluate dislocation rates and survivorship after rTHA using constrained liner (CL) prostheses.

METHODS: Patients who underwent rTHA using a CL prosthesis were retrospectively reviewed from January 1, 2013, to December 31, 2024. Patients who had antibiotic spacers or megaprotheses, or those who had less than one year of follow-up, were excluded. Survivorship analyses were performed to determine dislocation and reoperation-free survivorship through follow-up. A total of 98 rTHAs with CL were included, with a mean follow-up time of 3.9 years (range, one to 10.8).

RESULTS: The CL dislocation rate at final follow-up was 16.3% (16 of 98), and the 9-year dislocation-free survivorship was 71% (95% confidence interval [57 to 89]). There was neither statistically significant difference in 9-year dislocation-free survivorship among CL systems ($P = 0.73$), CLs inserted into cups that were retained versus revised ($P = 0.95$), nor CLs that were snapped in versus cemented ($P = 0.38$). The overall reoperation rate at final follow-up was 49.0% (48 of 98), and the 9-year all-cause reoperation-free survivorship was 33% (95% confidence interval [...])

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Diagnosing Seronegative Periprosthetic Joint Infection After Total Hip and Knee Arthroplasties.

Elmenawi KA, Poilvache H, Ducharme MT, Hannon CP, Abdel MP, Bedard NA

BACKGROUND: Serum erythrocyte sedimentation rate (ESR) and C-reactive protein (CRP) are used to screen for periprosthetic joint infection (PJI) of total hip arthroplasties (THAs) and total knee arthroplasties (TKAs). However, PJI may exist in the absence of elevated ESR and CRP. We aimed to report the incidence of seronegative PJIs in THAs and TKAs and the utility of arthrocentesis for PJI workup.

METHODS: We identified 925 total joint arthroplasties (312 THAs, 613 TKAs) that had surgical management of a chronic PJI from 2013 to 2022 at a single institution. All cases met the 2011 Musculoskeletal Infection Society (MSIS) criteria for PJI. Seronegative PJI was defined as meeting the MSIS criteria for PJI with a preoperative ESR of less than 30 mm/hour and CRP of less than 10 mg/L. Hips and knees were analyzed separately.

RESULTS: Out of 925 chronic PJIs, 10% of THAs ($n = 32$) and 4% of TKAs ($n = 24$) were seronegative PJIs. Among these, 94% of hips and 92% of knees met more than one major MSIS criterion, with 94 and 92% of seronegative hips and knees, respectively, having at least two positive intraoperative cultures with the same organism. A positive culture from preoperative aspiration was obtained from 84 and 81% of seronegative hips and knees, respectively. The most common infecting organism overall was *Staphylococcus epidermidis* (25% of hips and 38% of knees), followed by *C* [...]

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Withholding Prophylactic Antibiotics Improves Culture Yield in Revision Total Knee Arthroplasty for Periprosthetic Joint Infection With Negative Aspirations: A Multicenter Retrospective Study.

Paul BR, Verhey JT, Sullivan GR, Tarabichi S, Van Schuyver PR, Wyles CC, Ledford CK, Springer BD et al.

BACKGROUND: Preincision antibiotic prophylaxis reduces infection risk in total knee arthroplasty, but its role in revision procedures for periprosthetic joint infection (PJI) remains uncertain. Early antibiotic administration may suppress culture yield or alter concordance with preoperative aspiration, potentially leading to false-negative results that complicate postoperative antimicrobial selection.

METHODS: This retrospective multicenter study identified 576 chronic knee PJI patients who underwent two-stage exchange arthroplasty with a minimum of 1-year follow-up. A PJI was defined using the 2013 Musculoskeletal Infection Society criteria. Patients who did not have preoperative synovial aspirations or who were receiving outpatient antibiotic treatment within 2 weeks of aspiration or revision surgery were excluded ($n = 182$). Preoperative synovial aspirate cultures, intraoperative tissue cultures, and antimicrobial prophylaxis regimens were recorded. Antibiotic prophylaxis was administered before incision in 49% and withheld in 51%. Pearson Chi-square tests and one-way analyses of variance were used to determine whether antibiotic timing affected culture yield, proportion of positive culture results, and concordance with prior synovial aspirate results.

RESULTS: Intraoperative culture positivity was similar between early and delayed antibiotic groups (63 versus 63%, $P = 0.96$ [...])

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Preoperative Thrombocytopenia and Risk of Periprosthetic Joint Infection After Total Knee Arthroplasty: A Propensity-Matched Cohort Study.

Wiener JM, Hamad CD, Chun RF, Mehany JD, Yeaman MR, Sheppard WL, Bernthal NM

BACKGROUND: Periprosthetic joint infection (PJI) remains one of the most devastating complications following total joint arthroplasty. While platelets are recognized for their role in hemostasis, growing evidence suggests they also serve as immune effectors. Experimental data support a link between platelet depletion and infection risk, yet clinical evidence is limited.

METHODS: We performed a retrospective cohort study using a large national database to identify patients undergoing primary total knee arthroplasty between 2015 and 2023. Patients were stratified by preoperative platelet count: $\leq 50,000/\mu\text{L}$, 50,000 to 99,000/ μL , 100,000 to 149,000/ μL , and $\geq 150,000/\mu\text{L}$. The primary endpoint was PJI within one year, and the secondary endpoint was superficial surgical site infection (SSI). Multivariable logistic regressions and propensity score matching were used to adjust for demographics and comorbidities. Of 182,043 patients included in the study, 8,380 (4.6%) had platelet counts less than 150,000/ μL .

RESULTS: In adjusted models, risk of PJI increased stepwise with lower platelet counts: odds ratio (OR) 1.42 (95% confidence interval [CI]: 1.15 to 1.73) for 100,000 to 149,000/ μL , OR 3.67 (95% CI: 2.57 to 5.10) for 50,000 to 99,000/ μL , and OR 5.12 (95% CI: 1.73 to 12.19) for $\leq 50,000/\mu\text{L}$ compared with $\geq 150,000/\mu\text{L}$ (P -value trend < 0.001). Platelet count was not significantly associated [...]

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Repeat Two-Stage Revision Arthroplasty for Periprosthetic Joint Infection of the Hip: A Multicenter Study With a Mean 10-Year Follow-Up.

Kennedy IW, Howard LC, Somerville L, Masri BA, Garbuz DS, Vasarhelyi EM, Neufeld ME

BACKGROUND: Repeat two-stage revision remains a salvage option for managing recurrent periprosthetic joint infection (PJI) of the hip after a failed initial revision. However, the evidence base for this approach is limited. This study aimed to evaluate treatment outcomes and identify risk factors for failure following repeat two-stage revision arthroplasty for hip PJI.

METHODS: We conducted a retrospective review of prospectively maintained databases from two tertiary centers to identify patients who underwent repeat two-stage revision for chronic hip PJI between 1991 and 2022. Treatment success was defined using the 2019 Musculoskeletal Infection Society Tier 1 criteria. Kaplan-Meier survivorship analysis and Cox regressions were used to assess outcomes and potential predictors of reinfection. There were 52 patients who met the inclusion criteria. There were 16 patients who did not proceed to second-stage implantation, leaving 36 cases who completed repeat two-stage exchange. Mean follow-up of 10.7 years (range, 0.3 to 33.4).

RESULTS: Using Musculoskeletal Infection Society Tier-1 criteria, treatment success was achieved in 21 of 36 patients (58.3%). All-cause implant survival was 66.2% at five years, while survival free from PJI was 76.4%. At final follow-up, recurrent PJI occurred in 11 cases, with further surgery or long-term suppressive antibiotics required in 15 patient [...]

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Evaluation of Intraoperative and Early Postoperative Periprosthetic Fractures Following Total Knee Arthroplasty and Identification of Contributing Risk Factors.

Birinci M, Çağan MA, Hakyemez ÖS, Korkmaz O, Uzel K, Kara A, Azboy ■

BACKGROUND: Previous research on periprosthetic fractures (PPFs) following total knee arthroplasty (TKA) has primarily addressed late postoperative complications. Evidence concerning intraoperative and early (≤ 90 days) fractures remains limited, particularly within Eastern European and Middle Eastern populations. This study aimed to evaluate the incidence and identify risk factors for such fractures in a large national cohort.

METHODS: A retrospective analysis was conducted on 1,243 patients and 1,440 knees who underwent primary TKA between 2014 and 2023. Fractures developed in 35 knees (2.4%) intraoperatively or within 90 days after surgery and constituted the study group. Patients in the fracture group were older, with a mean age of 71 ± 8.8 years compared with 68 ± 7.6 years in the nonfracture group ($P = 0.014$). Demographic data, comorbidities, preoperative coronal alignment, and characteristics of the implants were compared between the two groups. Univariate and multivariate regression analyses were performed to determine risk factors based on the location of the fracture.

RESULTS: In multivariate regression analyses, independent risk factors for femoral PPF included the use of a constrained condylar insert without a femoral stem (odds ratio (OR): 54, 95% confidence interval (CI): 3.7 to 1,016; $P = 0.002$), a diagnosis of osteoporosis (OR: 20.5, 95% CI: 5.4 to 77.2; $P = 0$ [...])

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When Forced to Choose, Larger Femoral Head Size Confers Greater Protection Against Risk for Dislocation than a Lipped Acetabular Liner.

Friedman AJ, Haque H, Lama G, Magruder ML

BACKGROUND: Increasing femoral head size or using lipped polyethylene acetabular liners both reduce revision risk due to dislocation after total hip arthroplasty (THA). At some cup sizes, surgeons must choose between larger heads or lipped liners. This study compares 32-mm heads with 10° lipped liners against 36-mm heads with neutral liners for stability and revision risk.

METHODS: A national registry was queried for primary posterior-approach THAs. A total of 24,921 procedures were included: 17,386 with 32-mm femoral heads and 10° lipped cross-linked polyethylene liners, and 7,535 with 36-mm femoral heads and neutral cross-linked polyethylene liners. Kaplan-Meier analysis, multivariable logistic regressions, and t-tests evaluated 90-day, 2-year, and 8-year revision rates due to dislocation. A subanalysis was

performed for patients who had a body mass index ≥ 30 . The secondary outcomes included all-cause revision, infection, aseptic loosening, and periprosthetic fracture.

RESULTS: The 90-day, 2-year, and 8-year rates of revision for dislocation were higher in the 32-mm and 10° lipped liner cohort. The Kaplan-Meier curve analyses demonstrated the rate of revision for dislocation was significantly higher in the 32-mm and 10° lipped liner group during the first month following THA (hazard ratio, 5.58; $P = 0.019$), but was not significant at any later time points. Among obese pati [...]

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Early Greater Trochanteric Avulsion Fracture After Posterior Approach Primary Total Hip Arthroplasty: A Clinically Relevant and Potentially Avoidable Complication.

Flevas DA, Sokrab R, Anatone A, Selkridge I, El-Hassan M, Vigdorchik J, Sculco P

BACKGROUND: A greater trochanteric (GT) fracture after primary total hip arthroplasty (THA) is a clinically relevant complication that can lead to gait abnormalities and chronic lateral hip pain. Early postoperative GT avulsion fractures are rare, possibly resulting from poor bone quality or the use of two transosseous drill holes for posterior capsular repair. Computer- and robotic-assisted surgery may increase fracture risk due to cortical defects from temporary pins. This study aimed to assess the rate of early GT fractures and identify contributing factors and prevention strategies.

METHODS: This was a retrospective review of patients who underwent a posterior approach primary THA at our institution between January 1, 2016, and December 31, 2022, and subsequently presented with a GT avulsion fracture. Patient demographics, procedure details, implanted components, surgical technique, and postoperative fracture radiographs were reviewed, with descriptive statistics calculated for all variables.

RESULTS: There were 39 GT avulsion fractures identified among 18,873 posterior approach primary THAs, yielding an absolute risk of 0.2%. Of these, 97% occurred in women (mean age of 73 years, mean body mass index of 26). All avulsion fractures were atraumatic and occurred early, at an average of 27 days, during routine activities. The majority (59%) of the patients who had GT avulsio [...]

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Anatomical Relationship of the Popliteal Artery to Tibial Landmarks at Resection Levels in Total Knee Arthroplasty: A Cadaver Study.

Imai H, Kawai Y, Nakamura J, Suzuki T, Takahashi K, Ohtori S

BACKGROUND: Injury to the popliteal artery (PA) is a severe complication during total knee arthroplasty (TKA). Current information on the anatomical proximity of the PA to the posterior tibial cortex is mostly derived from imaging studies, which may not accurately represent surgical conditions. Additionally, anatomical differences between sexes remain unclear. This cadaver study aimed to determine the anatomical relationship between the PA and tibial landmarks at clinically relevant tibial resection levels during TKA, emphasizing potential sex-based differences.

METHODS: Overall, 102 knees from 51 formalin-embalmed cadavers (24 men and 27 women) were dissected. An Akagi line-based coordinate system defined the tibial landmarks, and three standard tibial resection levels employed in TKA (joint line, 10 mm distal, and 15 mm distal) were evaluated. The posterior, lateral, and shortest distances from the posterior tibial cortex to the PA were measured. Multivariate linear regression analyses that adjusted for lower leg length were used to assess sex-based anatomical differences.

RESULTS: The PA progressively approached the posterior tibial cortex distally; the median shortest distances significantly decreased from 10.0 mm (joint line) to 5.0 mm (15 mm distal). After adjusting for lower leg length, multivariate linear regression analysis revealed men as independently associated wi [...]

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Evaluating Measures of Clinical Meaningfulness for Patient-Reported Outcome Measures in Total Joint Arthroplasty: A Systematic Review.

Finley CL, Gray CF, Parvataneni HK, Prieto HA, Miley EN

BACKGROUND: Recently, the Centers for Medicare and Medicaid Services mandated the collection of patient-reported outcome measures (PROMs) for all primary total joint arthroplasties (TJAs), reinforcing their role in evaluating clinical outcomes. This systematic review investigated the calculation methods and anchor questions used to derive key PROM-based thresholds such as Minimal Clinically Important Difference (MCID), substantial clinical benefit (SCB), and the patient acceptable symptom state (PASS) for the Hip Disability and Osteoarthritis Outcome Score for Joint Replacement (HOOS JR) and the Knee Injury and Osteoarthritis Outcome Score for Joint Replacement (KOOS JR).

METHODS: Databases were queried for articles from 2000 to 2025 specific to the HOOS JR and KOOS JR. Articles were reviewed by two reviewers and included if they were in the English language and derived either the MCID, SCB, and/or PASS metrics specific to primary total joint arthroplasty procedures. Study characteristics, MCID, SCB, and/or PASS thresholds, and methods of derivation were extracted.

RESULTS: A total of 17 studies with 44,935 patients who met the final inclusion criteria were included. With respect to the HOOS JR, 10 studies derived a distribution-based MCID (range, 3.9 to 11.0 points), six studies derived an anchor-based MCID (range, 14.8 to 38.1 points), one study derived an SCB (22 points), [...]

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Cementless Unicompartmental Knee Arthroplasty: A Systematic Review of Survivorship and Revision Indications.

Basgul C, Kurtz MA, Rainey JP, Mont MA, Lombardi AV, Pelt CE, Gililand JM, Kurtz SM

BACKGROUND: Surgeons use cementless unicompartmental knee arthroplasty (UKA) as a joint-preserving alternative to total knee arthroplasty. However, cementless UKA survivorship and revision data remain fragmented. To address this gap, we systematically reviewed (1) survivorship of cementless UKAs reported at time points $\leq$ five years and $\geq$ 10 years and (2) the revision indications across implant types.

METHODS: The PubMed and EMBASE databases were searched to identify relevant studies. Inclusion criteria comprised cementless UKA devices with available survivorship and revision data. Study quality was assessed using the modified Coleman Methodology Score and the Journal of Bone and Joint Surgery Level of Evidence criteria.

RESULTS: We identified 27 studies on seven cementless device designs. Medial UKAs accounted for most implants (81.5%). Studies with 5-year time points or less reported a median survivorship of 96.1%. At time points $\geq$ 10 years, survivorship decreased to 92.0% ($P = 0.01$). The most frequent revision indications included instability (16%), progression of osteoarthritis (14%), and aseptic loosening (11%). In some cases, the indications for revision depended on the device: one device was associated with polyethylene-related complications (57%), while a blade-anchored device was frequently revised following loosening (63%).

CONCLUSIONS: Success was device specific, [...]

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Quadrangular Taper Cementless Designs Are Associated With Lower Risks of Postoperative Periprosthetic Fracture Compared With Cemented Stems and Other Cementless Designs Following Primary Total Hip Arthroplasty: A Systematic Review and Meta-Analysis.

Borazjani R, Ayatizadeh SH, Sattarian M, Moein SA, Mont MA, Kreuzer SW

BACKGROUND: Although cementless stems are increasingly favored in primary total hip arthroplasty (pTHA), the impact of stem designs on postoperative complications, such as periprosthetic femoral fracture (PFF), remains debated. This study compared PFF rates between cemented and cementless pTHA, focusing on cementless stem geometries.

METHODS: We systematically searched PubMed, Embase, Web of Science, Scopus, Google Scholar, and CENTRAL from inception through March 2025. Eligible studies compared cemented and cementless stems in adults undergoing elective pTHAs, reported PFF incidence, and specified cementless stem design. Using a random-effects model, we calculated pooled odds ratios (ORs) and 95% confidence intervals (95% CIs) for primary (PFF) and secondary outcomes (aseptic loosening, dislocation, periprosthetic joint infection, surgical site infection, and all-cause revision). There were 18 studies (55,942 pTHAs) included. Subgroup analyses were performed by

cementless stem designs and age. Metaregression was conducted to investigate heterogeneity across the study.

RESULTS: The pooled PFF rate was 1.3% (95% CI: 0.9 to 2.0) for cementless and 0.9% (95% CI: 0.6 to 1.5) for cemented pTHA (OR = 1.38; 95% CI: 0.95 to 2.02; P = 0.089; I² = 45.4%). Quadrangular stems (type B2) significantly reduced PFF risk (OR = 0.78; 95% CI: 0.64 to 0.96, P = 0.033; I² = 0.0%), whereas anatomy [...]

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Return to Sport and Exercise After Direct Anterior Approach Total Hip Arthroplasty: Minimum Five-Year Outcomes.

Mikula JD, Yanamala SR, Kelly JJ, Rohdenburg JE, Couch CG, Taunton MJ, Hevesi M

BACKGROUND: Younger, more active patients increasingly undergo total hip arthroplasty (THA) and commonly inquire about expectations and timelines regarding return to sport (RTS) and exercise. The direct anterior approach (DAA) may facilitate faster recovery and decreased instability risk, but long-term RTS data are limited.

METHODS: A retrospective registry review identified all primary DAA THAs from 2010 to 2020. Patients who had less than five-year follow-up were excluded. Eligible patients completed a survey on preoperative and postoperative sport participation, time to RTS, and potential factors limiting RTS. A total of 886 patients (466 women and 428 men) responded to the survey. Their mean age was 62 years (range, 28 to 93), and their mean body mass index was 29.0 (range, 17 to 49).

RESULTS: In the year prior to THA, 84% walked, 31% hiked, 27% cycled, and 11% ran. Postoperatively, 74% returned fully to prior activities, and 16% returned with modifications, with 90% of these patients achieving RTS within one year. High-impact sport participation decreased from 24% preoperatively to 10% postoperatively, and running decreased from 11 to 5% (P < 0.001). Those who discontinued running most commonly cited surgeon advice (34%), fear of damaging the joint (21%), and pain (20%) as contributing factors for running cessation. Overall rates of revision were 4.5% for the running cohort [...]

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Kinematic and Mechanical Alignment Yield Similar Outcomes in Total Knee Arthroplasty: A Systematic Review and a Meta-Analysis of Randomized Controlled Trials.

Blackman BW, Burow CA, Macciachera MP, Hayes EW, Garceau SP

BACKGROUND: Although mechanical alignment (MA) is the current gold standard for total knee arthroplasty (TKA), suboptimal patient satisfaction rates have prompted the exploration of alternative alignment strategies. This review examined whether kinematic alignment (KA) improves outcomes following TKA compared to MA.

METHODS: There were four databases searched through September 23, 2024, to identify randomized controlled trials (RCTs) investigating TKA using KA versus MA. Patient demographics, survivorship, and patient-reported outcome measures (PROMs) were abstracted. Meta-analyses were performed to compare survivorship and PROMs. Risk of bias was assessed using the RoB 2 tool. There were 11 RCTs and 972 patients included (KA: 484, MA: 488). The mean follow-up was 3.9 years (range, one to 13).

RESULTS: There was no statistically significant difference in all-cause reoperation rate between groups (relative risk (RR): 1.34, 95% confidence interval (CI): 0.71 to 2.52, I² = 0%, P = 0.37). A meta-analysis of two studies with greater than 10-year follow-up found no statistically significant difference in all-cause reoperations (RR: 1.21, 95% CI: 0.6 to 2.47, I² = 0%, P = 0.59) and component revisions (RR: 1.26, 95% CI: 0.38 to 4.14, I² = 0%, P = 0.71) between groups. There was no statistically significant difference in PROMs. In two studies including patients who underwent bilaterally [...]

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Risk of Revision for Periprosthetic Fracture Following Cementless Total Hip Arthroplasty Using a Specific Type 1 Blade Design Stem: A Registry-Based Cohort Study.

Sheth DS, Chang RN, Prentice HA, Fasig BH, Mohan V, Paxton EW

BACKGROUND: Implant survival following total hip arthroplasty (THA) may differ by femoral stem design. In a series of patients, a consistent pattern of low-energy periprosthetic fracture (PPF) with a specific blade design stem was observed. Therefore, we conducted a cohort study comparing revision risk with a specific stem to other stems.

METHODS: Adult patients who underwent primary cementless THA for osteoarthritis with a highly crosslinked polyethylene liner and large femoral head (≥ 28 mm) were identified using data from a United States-based health care system's registry (2010 to 2022). There were 9,425 THAs using the specific blade stem compared to 67,484 THAs using other stems. Multivariable Cox regressions were used to evaluate the 90-day revision due to PPF and aseptic revision.

RESULTS: The crude PPF revision incidence was 0.8 and 0.4% for the specific blade stem and other stems, respectively. In adjusted analyses, a higher risk of PPF revision (hazard ratio [HR] = 2.19, 95% confidence interval [CI] = 1.60 to 3.01) and aseptic revision (HR = 1.84, 95% CI = 1.40 to 2.42) with the specific blade stem was observed. This higher risk was observed when comparing the specific blade stem to other type 1 stems (PPF: HR = 2.19, 95% CI = 1.54 to 3.11; aseptic: HR = 1.56, 95% CI = 1.14 to 2.12) as well as to type 2 stems (PPF: HR = 2.12, 95% CI = 1.48 to 3.05; aseptic: HR = 1.9 [...])

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Is the Posterior Cruciate Ligament Necessary in Cruciate-Substituting Total Knee Arthroplasty? A Cadaveric Comparison of Knee Kinematics With or Without the Posterior Cruciate Ligament Versus the Native Knee.

Shigekiyo S, Hamada D, Wada K, Tamaki Y, Sairyo K

BACKGROUND: This study sought to determine knee kinematics after cruciate-substituting total knee arthroplasty (TKA) using an insert with or without the posterior cruciate ligament (PCL) in the same-specimen fresh-frozen cadavers using a within-knee repeated-measures design.

METHODS: There were eight specimens that underwent TKA with a medial-dished (MD) type insert, one of the cruciate-substituting designs, using an image-free robotic-assisted surgical system. Knee kinematics were evaluated using an image-free knee navigation system. We recorded the kinematics of the native knee, TKA with MD and PCL preservation (MD PCLplus), and TKA with MD and PCL dissection (MD PCLminus) from full extension to maximum flexion using the navigation system. Differences in the rotational and antero-posterior (A-P) positions between the native knee and TKA in each MD group were examined for statistical significance.

RESULTS: At full extension, knees in both MD groups were initially in a more internally rotated position compared with the native knee. In the initial-flexion phase, the MD PCLplus group exhibited rotational position changes similar to those of the native knee, whereas the MD PCLminus group did not. There was no significant difference in rotational position beyond midflexion between knees in either of the MD groups and the native knee. The A-P position through to the midflexion pos [...]

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Intraoperative Vancomycin Does Not Change Periprosthetic Joint Infection Risk in Patients Undergoing Total Joint Arthroplasty.

Jiang WM, Sanchez JG, Halperin SJ, Molloy IB, Grauer JN, Rubin LE

BACKGROUND: Cephalosporins are the primary prophylactic antibiotic in primary total hip and knee arthroplasty (THA and TKA). There is interest in augmenting this effect by adding intraoperative vancomycin via intrawound, intraosseous, or intravenous routes. However, low periprosthetic joint infection (PJI) rates have hindered statistical power and consensus within prior research. This study aimed to compare 90-day infectious and medical complications and 5-year revision-free survival among patients who did and did not have intraoperative vancomycin within a large, nationally representative population.

METHODS: Within a national database, adult patients undergoing primary THA or TKA for osteoarthritis from January 2010 to April 2023 who received same-day cefazolin prophylaxis were identified. Exclusion criteria included methicillin-resistant *Staphylococcus aureus* or PJI history, prior infection within 90 days, and any antibiotic prescription within 90 days before and three days following surgery. Patients who did and did not have the addition of intraoperative vancomycin were matched (1:1) on age, sex, Elixhauser Comorbidity Index, body mass index greater than 40, type II diabetes, insulin use, smoking status, and geographic region. Among 78,778 THA patients, 20,841 (26.5%) received intraoperative vancomycin, yielding 17,051 matched patients per group. Among 171,274 TKA patient [...]

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Comparing Outcomes in Sexual Function After Total Hip and Knee Arthroplasty: A Systematic Review.

Pendyala SY, Akram N, Asopa V, Sochart DH

BACKGROUND: Sexual function is an important contributor to quality of life for people undergoing lower limb arthroplasty, yet it is rarely discussed in clinics and inconsistently measured in research. Evidence suggests improvement after surgery, but findings are scattered across hips and knees, instruments vary, and a direct joint-wise comparison is lacking. This review compares sexual outcomes after total hip arthroplasty (THA) and total knee arthroplasty (TKA) across three prespecified domains: return to sexual activity (RTSA), frequency of sexual activity (FSA), and sexual satisfaction (SS) or fulfillment of preoperative expectations.

METHODS: A systematic search of major biomedical databases and trial registries was conducted per the Preferred Reporting Items for Systematic Reviews and Meta-Analyses guidelines. Eligible studies reported on sexual function after primary THA or TKA, specifically RTSA, FSA, or SS. There were two independent reviewers who screened, extracted, and assessed the risk of bias. There were 27 studies, including 9,267 patients (5,350 THA and 3,917 TKA), that were eligible.

RESULTS: The RTSA tended to be earlier following TKA compared with THA; however, through exploratory pooling, a greater proportion of THA patients reported higher SS (75.4%, 703 of 932) than TKA patients (69.3%, 843 of 1,216; $P = 0.0013$) and demonstrated greater increases in FSA ([...]

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Attainment of Substantial Clinical Benefit Following Primary Total Knee Arthroplasty Is Impacted by Preoperative Patient-Reported Outcome Measures.

Call CM, Walsh ZA, Mackenzie JA, Babikian GM, McGrory BJ, Rana AJ

BACKGROUND: The Centers for Medicare & Medicaid Services (CMS) has made the collection of patient-reported outcome measures (PROMs) mandatory for inpatient total knee arthroplasty (TKA). The reporting of the proportion of patients who reach the substantial clinical benefit (SCB) threshold between preoperative and postoperative PROMs based on Knee Dysfunction and Osteoarthritis Outcome Score for Joint Replacement (KOOS JR) scores is also necessary for reimbursement. Our study evaluated characteristics among patients who did and did not meet SCB to understand trends that may help surgeons to comply with CMS policies and maximize patient outcomes following TKA.

METHODS: A retrospective review was performed of patients undergoing primary TKA between January 2021 and August 2024 at a single large academic center. Demographic, operative, and outcome variables were compared between patients who met SCB and those who did not. Multivariable analysis was performed to identify risk factors for failure to achieve SCB. A total of 807 of 3,538 patients (23%) were eligible for inclusion based on completion of both preoperative and postoperative KOOS JR surveys; 62% of whom reached SCB.

RESULTS: There were few differences in demographics and no significant differences in complications between subgroups. Patients had strikingly different PROM profiles; the group failing to meet SCB reported I [...]

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Long-Term Dislocation Risk Following Primary Total Hip Arthroplasty Due to Osteoarthritis: A Population-Based Cohort Study From the Danish Hip Arthroplasty Register.

Chabaita M, Iljazi A, Sørensen MS, Overgaard S, Petersen MM

BACKGROUND: Population-based data on long-term dislocation risks are currently lacking in literature. Thus, the purpose of this study was to investigate the long-term cumulative incidence of dislocation after total hip arthroplasty (THA) stratified by femoral head size. Furthermore, we investigated the difference in cumulative incidence of dislocation over different time periods.

METHODS: We conducted a population-based cohort study using data from the Danish Hip Arthroplasty Register and the Danish National Patient Register. The cohort comprised 56,515 THAs. The study included patients who had a primary THA due to primary osteoarthritis, performed in Denmark from 2000 to 2016. Follow-up was extended until death, implant removal, or December 31, 2021, to ensure a potential minimum 5-year follow-up period. We used a recently validated algorithm to include dislocations that were treated with closed reduction

without revision. Cumulative incidence of dislocation was estimated using the Aalen-Johansen estimator with death and implant removal as competing risks. Cumulative incidence of dislocation was stratified by femoral head size and the three time periods: 2000 to 2004, 2005 to 2009, and 2010 to 2016.

RESULTS: The 10-year cumulative incidence of dislocation was 8.8% (95% confidence interval [CI], 8.4 to 9.3) for 28-mm femoral heads, 7.4% (95% CI, 7.0 to 7.9) for 32-mm, and 5.7 [...]

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Rising Body Mass Index Increased Early Complications, But Not Early Reoperations Following Aseptic Revision Total Knee Arthroplasty.

Shannon MF, Wong VR, Frear AJ, Lin RT, Plakseychuk EN, Santana A, Urish KL

BACKGROUND: Elevated body mass index (BMI) contributes to osteoarthritis and demand for total knee arthroplasty (TKA). Increased BMI is an established risk factor for complications following primary TKA. However, literature on BMI and outcomes of revision TKA (rTKA) is less conclusive. This study evaluated the association between BMI and outcomes in the early postoperative period following aseptic rTKA. We hypothesized that increasing BMI would be associated with a higher risk of short-term complications.

METHODS: A retrospective review of 470 patients who underwent aseptic rTKA within one regional health system from 2017 to 2022 was performed. Revisions were due to infection (39.8%), instability (23.7%), arthrofibrosis (11.8%), and aseptic loosening (11.8%). Patient BMI was obtained from electronic medical records at the time of rTKA. The primary outcome was early reoperation, assessed at 30 and 90 days postoperatively. The secondary outcomes included readmission, major complications, and minor complications at 30 and 90 days postoperatively. Multivariate regression models considering BMI as a continuous variable were used to determine the impact of BMI on outcomes.

RESULTS: Elevated BMI was associated with slightly increased risk of overall reoperation (odds ratio: 1.03, $P = 0.036$) and 30-day readmission (odds ratio: 1.05, $P = 0.029$). Rising BMI was also associated with gre [...]

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Timing of Periprosthetic Joint Infections Following Primary and Revision Arthroplasty in Ontario: A Population-Based Retrospective Cohort Study Using Administrative Databases From 2003 to 2017.

Kandel CE, Daneman N, Widdifield J, Jenkinson R, Hansen BE, McGeer AJ

BACKGROUND: Periprosthetic joint infections (PJIs) of the hip and knee are a devastating outcome of arthroplasty, resulting in profound morbidity. As more arthroplasties are performed, there is a need to monitor the long-term risk and trends of infection.

METHODS: A population-based retrospective cohort of all primary hip and knee arthroplasties performed in Ontario from April 1, 2003, until March 31, 2017, was created using linked administrative databases with patients followed postoperatively until March 31, 2018. There were 504,332 primary hip and knee arthroplasties performed, with the number of annual procedures increasing an average of 5.8% annually over the study. The primary outcome was hospital readmission for a PJI. Poisson models were used to determine trends over time.

RESULTS: Overall, 7,331 PJIs were identified. The incidence of a PJI following primary arthroplasty was 0.59 out of 100 (95% confidence interval (CI): 0.57 to 0.61) at 90 days and 1.29 of 100 (95% CI: 1.26 to 1.33) at five years, whereas following revision arthroplasty, it was 1.93 of 100 (95% CI: 1.70 to 2.16) at 90 days and 4.89 of 100 (95% CI: 4.52 to 5.26) at five years. The 1-year incidence of PJI following primary arthroplasty increased from 0.70 of 100 in 2003 to 0.97 of 100 in 2016 ($P < 0.001$).

CONCLUSION: The PJIs are increasing, both because of an increase in the numbers of primary arthro [...]

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The Association Between Social Determinants of Health and Total Hip Arthroplasty Outcomes: A Study of Neighborhood Disadvantage Indices.

Elnaggar A, Mehaidli A, Hodson N, Driessche A, Harati M, Silverton C

BACKGROUND: Total hip arthroplasty (THA) is a highly effective procedure, but outcomes may be influenced by social determinants of health. We investigated the association between neighborhood disadvantage indices, the Social Vulnerability Index (SVI) and Area Deprivation Index (ADI), and postoperative outcomes following THA in a large, multicenter cohort.

METHODS: A retrospective review of 9,519 primary THAs performed between 2014 and 2022 was conducted. Patient addresses were geocoded to US Census tracts to assign SVI and ADI scores. We analyzed associations between disadvantage indices and postoperative complications, health care utilization, and patient-reported outcomes using multivariable regression models.

RESULTS: Our analysis found that neighborhood disadvantage was associated with specific postoperative outcomes. Patients who were from higher socioeconomic disadvantage quartiles had an increased risk of periprosthetic fracture (odds ratio = 2.07; 95% confidence interval: 1.21 to 3.54). Conversely, a higher disadvantage in the housing type and transportation domain was associated with a decreased risk of 90-day readmission (odds ratio = 0.72; 95% confidence interval: 0.56 to 0.91). Higher minority status disadvantage was associated with a significant decrease in lengths of hospital stay. There were no significant associations observed for overall SVI or ADI with most [...]

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Assessing Medicaid Accessibility to Total Knee Arthroplasty: Comparison of Ambulatory Surgery Centers and Hospitals.

Singh R, Fuller Z, Dawar A, Lingam S, Patel J, Kim H, Abdo ZE

BACKGROUND: Ambulatory surgery centers (ASCs) are increasingly becoming the preferred setting for elective orthopaedic procedures, including total knee arthroplasty (TKA). The ASC's favorable cost containment, reimbursements, and patient outcomes have driven their growth, but access to the Medicaid population remains an issue. This study aimed to evaluate these barriers for patients seeking TKA at ASCs compared to public hospitals using a cross-sectional survey design.

METHODS: All current ASCs registered within the United States were queried, and ASCs offering orthopaedic surgery services were isolated. Trained researchers contacted ASCs and public hospitals using a standardized inquiry script. Key data points included Medicaid acceptance, appointment availability, and out-of-pocket costs. Data were analyzed utilizing chi-square and Student t-tests. A total of 98 ASCs and 98 matched public hospitals were contacted for the study (Vermont and South Dakota had one ASC meeting initial criteria), with 77 ASCs and 94 public hospitals having complete data for analyses.

RESULTS: Public hospitals demonstrated greater Medicaid acceptance compared to ASCs (95 versus 78%, $P = 0.002$). Both ASCs and public hospitals offered similar appointment availability (14.4 versus 19.1 d, $P = 0.258$). There were no differences between ASC and hospital out-of-pocket consultation (\$470 versus \$200, $P = [...]$

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Teenage Total Hip Arthroplasty Yields High Satisfaction and Excellent Survival up to 20-Year Follow-Up.

Hopman AGM, Te Velde JP, Sierevelt IN, Buijs GS, Schaafroth MU, Blankevoort L, Kievit AJ

BACKGROUND: Total hip arthroplasty (THA) is increasingly performed in patients younger than 20 years who have destructive hip pathology. The unique anatomical and developmental characteristics of this population, along with the high likelihood of future revision surgery, have led to cautious clinical decision-making. Data on safety, effectiveness, and long-term (20 years) outcomes in teenagers remain limited. This study aimed to evaluate implant survival, patient satisfaction, functional outcomes, and complications following THA in teenagers.

METHODS: This retrospective cohort study with cross-sectional evaluation was conducted at a tertiary academic center. All patients aged 10 to 19 years at the time of surgery with $\geq$ one year follow-up were included. The study population comprised 68 teenagers (75 THAs), who had a median age at surgery of 16 years (range, 11 to 19). The median follow-up was 5.4 years (range, 1.0 to 20.4). The most common indications were developmental dysplasia of the hip (28%), slipped capital femoral epiphysis (20%), and osteonecrosis (17.3%). Of the 75 hips, 37 (49%) had undergone prior hip-preserving procedures. Implant survival was assessed using Kaplan-Meier analysis, with revision as the primary endpoint. In addition, Hip disability and Osteoarthritis Outcome Score for

Physical Function (0 to 100) and patient satisfaction (Likert scale; 1 to 10) were [...]

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Outcomes of Curved Intertrochanteric Varus Osteotomy for Osteonecrosis of the Femoral Head With a Beak-Shaped Healthy Area.

Osawa Y, Funahashi H, Ido H, Asamoto T, Takegami Y, Imagama S

BACKGROUND: Indications for curved intertrochanteric varus osteotomy (CVO) for osteonecrosis of the femoral head (ONFH) with a beak-shaped healthy area (BHA) remain unclear. This study aimed to evaluate the clinical outcomes and radiographic findings of CVO for ONFH with BHA.

METHODS: This comparative study included 77 patients who were Japanese Investigation Committee type B and C1 ONFH. From January 2005 to December 2022, 21 patients (22 hips) who had a BHA underwent CVO (BHA group), whereas 56 patients (62 hips) who did not have a BHA underwent CVO (control group). The average follow-up period was 98.4 and 97.2 months in the BHA and control groups, respectively. Clinical and radiographic evaluations included the Harris Hip Score, complication rates, radiographic parameters, and survival rates. Endpoints were defined as conversion to total hip arthroplasty or radiographic failure.

RESULTS: The preoperative and postoperative Harris Hip Score values were similar between the BHA and control groups. The complication rates were comparable between the groups. The postoperative intact ratio and varus angle showed no significant differences. In the BHA group, three cases of beak fractures occurred within the first year postoperatively. The 8-year survival rate based on conversion to total hip arthroplasty and radiographic failure did not differ significantly between the groups.

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Contemporary Outcomes of Highly Porous Metaphyseal Cones in Revision Total Knee Arthroplasty: A Systematic Review and Meta-Analysis.

Goh GS, Lee S, Singh K, Travers HI, Kuznetsov M, Thomas TL, Smith EL

BACKGROUND: Metaphyseal bone loss represents a major challenge in revision total knee arthroplasty (RTKA). Highly porous metaphyseal cones have potential for osseointegration and durable fixation. While early evidence was limited to tantalum cones, newer designs, such as three-dimensional-printed titanium cones, have been introduced by multiple manufacturers, necessitating an updated synthesis of the literature.

METHODS: PubMed, Scopus, and Cochrane databases were searched from inception through October 1, 2024. Eligible studies included randomized controlled trials and observational cohorts reporting clinical or radiological outcomes of metaphyseal cones in RTKA with a minimum 1-year follow-up. Demographics, implant characteristics, functional scores, survivorship, cone removal rates, and radiographic outcomes were recorded. Pooled analyses were performed using random-effects models, with subgroup analyses by cone material and fixation type. There were 63 studies comprising 4,433 patients and 5,435 cones that were included, with 79.9% of femora and 75.4% of tibiae involving Anderson Orthopedic Research Institute type 2B or 3 defects.

RESULTS: Significant improvements in function were observed, with pooled Knee Society Score Knee and Function scores of 77.4 (95% confidence interval [CI], 70.6 to 84.3) and 67.5 (95% CI, 58.4 to 76.6), respectively. The pooled all-cause re-rev [...]

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Is Septic Revision Knee Arthroplasty Associated With Higher Thromboembolic Events? A Matched Cohort Study.

Hoveidaei AH, Pirahesh K, Over D, Reich J, Suresh S, Adolf J, Conway JD

BACKGROUND: There is a paucity of literature assessing the incidence and characteristics of acute and chronic thromboembolic events following septic versus aseptic revision knee arthroplasty. The current study aimed to evaluate these complications between these populations.

METHODS: This retrospective cohort study used a national database (2010 to 2022) to examine postoperative outcomes in patients who underwent aseptic or septic revision total knee arthroplasty. Outcomes of interest

included acute thromboembolic events such as pulmonary embolism (PE), deep vein thrombosis, myocardial infarction, cerebrovascular disease, precerebral occlusion, and chronic thromboembolic events such as chronic thromboembolic pulmonary hypertension, postthrombotic syndrome, limb ischemia, lower-limb amputation, chronic PE, and secondary pulmonary hypertension. A 1:1 matching was used to control for baseline differences, and multivariate logistic regressions were conducted to compare complication rates between the matched groups.

RESULTS: Septic revision arthroplasty cohorts demonstrated increased rates of PE (odds ratio [OR] = 2.65, 2.45; $P < 0.001$), deep vein thrombosis (OR = 3.83, 3.35; $P < 0.001$), myocardial infarction (OR = 3.49, 3.33; $P < 0.001$), cerebrovascular disease (OR = 3.39, 2.62; $P < 0.001$), and precerebral occlusion (OR = 3.21, 2.22; $P < 0.001$) at 30- and 90-day time points, respectively [...]

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Aspirin May Be a Suitable Prophylaxis for Cancer Patients Undergoing Primary Total Knee Arthroplasty.

Tarabichi S, Verhey JT, Braithwaite C, Hall RP, Lebens R, Christopher ZK, Spangehl MJ, Bingham JS

BACKGROUND: Aspirin is the prophylactic agent of choice for venous thromboembolism prevention in low-risk patients undergoing primary total knee arthroplasty (TKA). However, there remains a paucity of data in the literature on the efficacy of aspirin as a mode of chemoprophylaxis in patients who have a diagnosis of cancer. The purpose of this study was to determine the efficacy of aspirin versus other anticoagulant medications in primary TKA patients who had a diagnosis of active visceral organ malignancy within one year of their arthroplasty procedure.

METHODS: This study utilized a national database to identify patients who underwent primary TKA between 2012 and 2022. Only patients who had a diagnosis code for active visceral organ malignancy within 1 year before their primary TKA were included. Using demographic data, medical comorbidities, and preoperative anticoagulation use, primary TKA patients who have cancer and were prescribed aspirin for thromboprophylaxis ($n = 2,833$) were propensity score matched on a 1:1 basis to those who were on non-aspirin medications ($n = 2,833$). The odds ratios (ORs) for developing 90-day medical and surgical complications were determined for patients prescribed aspirin only, when compared to those prescribed other types of chemoprophylaxis.

RESULTS: Patients in the aspirin group had significantly lower odds of developing deep vein thrombosis [...]

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Fractures of the coronoid process: state of the art.

Marinelli A, Riva M, Coliva F, Minerba M, Carbone G, Guerra E

Coronoid fractures are rarely isolated and are much more frequently associated with other osseous or ligamentous structures injuries. On the basis of the coronoid fracture patterns, described by the O'Driscoll classification, it is possible to recognize three main patterns of injury that differ on traumatic mechanism and on associated lesions: posterolateral rotatory instability, posteromedial rotatory instability, and axial load injuries. The management of coronoid fractures is challenging and varies according to characteristics of the fracture, associated lesions, and amount of elbow instability. In general, operative treatment is indicated in every case the fracture is at least 50% of the whole coronoid, whether the sublime tubercle is involved, and whether the ulno-humeral joint is not perfectly reduced. In conclusion, the correct management of the coronoid, especially in the setting of complex elbow instability, represents a predictive factor for patient outcomes and functional results. The stability of the elbow, rather than the size of the coronoid fragment, is the main parameter for surgical indication, aimed to fix the coronoid and/or repair the associated lesions.

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Total hip arthroplasty in Italy: an observational, population-based study on surgical volume growth from 2001 to 2023 and forecasts until 2050 with six different statistical models.

Ciminello E, Cuccu A, Romanini E, Venosa M, Cazzato G, Tucci G, Boniforti F, Carpanese L et al.

BACKGROUND: The number of total hip arthroplasty (THA) procedures has been steadily increasing worldwide, driven by aging population, improvements in surgical techniques and implant design. This study aimed to analyze the temporal trends of elective THA in Italy since 2001-2023 and forecast THA volumes up to 2050 to provide insights for healthcare planning.

MATERIALS AND METHODS: International Classification of Diseases, 9th Revision, Clinical Modification (ICD9-CM) coding system was used to extract records of interest (elective THA) from the Italian National Hospital Discharge Record database. Six statistical models were applied to forecast future THA volumes: logistic regression; Poisson regression; logarithmic regression; inverse/power regression; Poisson log-normal regression; and hierarchical Poisson regression with temporal effects (HPTE). Model performances were assessed by using error metrics and internal validation on the basis of a rolling-origin approach. An out-of-sample validation was conducted to ensure a robust assessment of forecasting reliability. THA volume forecasts were provided with 95% prediction intervals.

RESULTS: A total of 1,318,400 records for primary elective THAs performed in Italy since 2001-2023 were analyzed. The number of THAs increased by approximately 80%, rising from 68.270 in 2001 to 122.777 in 2023. Among the tested models, HPTE generally [...]

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Clinical relevance of patient-reported outcome measures in the surgical management of focal chondral defects of the knee: a systematic review.

Migliorini F, Maffulli N, Memminger MK, Hofmann UK

INTRODUCTION: To evaluate clinical outcome following surgical management of focal chondral defects in the knee, patient-reported outcome measures (PROMs) are used. To give these measures meaning, parameters such as the minimal clinically important difference (MCID), patient-acceptable symptom state (PASS), minimally detectable change (MDC), clinically important difference (CID) and substantial clinical benefit (SCB) have been introduced. This systematic review investigated the MCID, SCB, CID, PASS and MDC of the most commonly used PROMs for assessing patients following surgical repair of focal chondral defects of the knee.

METHODS: This systematic review was conducted in accordance with the 2020 Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) guidelines and the recommendations of the Cochrane Handbook for Systematic Reviews of Interventions. All clinical studies investigating tools to assess the clinical relevance of PROMs in the surgical repair of focal chondral defects of the knee were reviewed. In April 2025, the following databases were accessed: PubMed, Web of Science and Embase. The PROMs of interest included: the International Knee Documentation Committee (IKDC) questionnaire, the Knee Injury and Osteoarthritis Outcome Score (KOOS) and its related subscales activities of daily living (ADL), pain, quality of life (QoL), sports and recreational [...]

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Simultaneous versus staged bilateral total hip arthroplasty: a meta-analysis.

Soler F, Hernández J, Lamo-Espinosa JM, Benlloch M, Mariscal G

BACKGROUND: There is debate regarding the optimal timing of bilateral total hip arthroplasty (THA), with simultaneous or staged approaches considered. Cost-effectiveness is an important factor that influences resource allocation. The main aim of this study was to assess the cost implications of bilateral simultaneous (sim-THA) versus staged (st-THA) total hip arthroplasty. The secondary objective was to evaluate the efficacy and safety of sim-THA compared those with of st-THA.

METHODS: A systematic review and meta-analysis were conducted according to the Preferred Reporting Items for Systematic reviews and Meta-Analyses (PRISMA) guidelines. Studies comparing simultaneous and staged bilateral THA in terms of cost, complications, length of hospital stay, and patient outcomes were eligible. Odds ratios (ORs), mean differences (MD), and standard mean differences (SMD) with 95% confidence intervals (CIs) were calculated. The risk of bias was assessed using the Methodological Index for Non-randomised Studies (MINORS). Meta-analyses were performed using Review Manager version 5.4 (Cochrane, Oxford, United Kingdom).

RESULTS: A total of 20 observational studies including 13,984 patients were included. Sim-THA was associated with significantly lower total costs (SMD -0.54, 95% CI -0.92 to -0.16; $p = 0.005$) and shorter hospital stay (MD -2.90, 95% CI -4.38 to -1.42; $p = 0.0001$) than tho [...]

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Long-term outcomes of the modified Dunn procedure in moderate and severe slipped capital femoral epiphysis: a prospective case series with 7-year follow-up.

Fahmy M, Abdelazeem AH, Shawky MA

INTRODUCTION: Slipped capital femoral epiphysis (SCFE) is the most common hip disorder in adolescents and may lead to femoroacetabular impingement, early osteoarthritis, and long-term functional disability if inadequately treated. While in situ pinning remains the standard treatment for mild slips, it fails to correct the deformity in moderate and severe cases, potentially predisposing to degenerative changes. The modified Dunn procedure (MDP) was developed to restore proximal femoral anatomy through surgical hip dislocation while preserving vascular supply. The aim of the study is to evaluate the long-term radiological and functional outcomes of the MDP in patients with moderate (14 cases) and severe (10 cases) SCFE, and to assess the incidence of avascular necrosis (AVN), osteoarthritis, and other complications.

METHODS: A prospective case series was conducted between August 2015 and January 2019 at a single tertiary institution. A total of 24 hips with moderate-to-severe SCFE and open physis were treated using the MDP via surgical hip dislocation. Mild and acute-only slips were excluded. MDP was used as a primary procedure, performed early in severe slips and in selected moderate slips after clinical assessment. Patients were followed clinically and radiologically for a mean duration of 84 ± 2.6 months (range 80-88 months). Functional outcomes were assessed using the Harris [...]

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The presence of a positive blood culture should be regarded a risk factor for DAIR failure in cases of haematogenous total knee arthroplasty infection.

Pedemonte-Parramón G, Reynaga E, Molinos S, de Los Ríos JD, Vivero A, García-Oltra E, López-Pérez V, Paredes R et al.

PURPOSE: Late acute haematogenous total knee arthroplasty (LAH TKA) infections represent approximately 20% of all prosthetic infections. Debridement, antibiotics, and implant retention (DAIR) is a widely accepted treatment; however, it carries a significant risk of failure. The study aims to identify risk factors for DAIR failure in LAH TKA infections, focusing on blood cultures (BC) as a potential contributor.

METHODS: A retrospective cohort study was conducted on 37 cases of LAH TKA infections from 2015 to 2023. Patients were divided into two groups: 20 in the success group (SG) and 17 in the failure group (FG). The study analyzes various factors, including demographics, comorbidities, serum and joint fluid biomarkers, blood and intraoperative cultures, prior infection or antibiotic use, and surgical and post-surgical variables.

RESULTS: Positive BC were more frequent in the FG compared with the SG ($P = 0.03$), and were also more common in patients with a history of prior infection ($P = 0.03$). Logistic regression identified positive BC as the only significant predictor of DAIR failure (odds ratio (OR) 12, 95% confidence interval (CI) 1.1-18, $P = 0.04$), even after adjusting for other variables. Positive intraoperative cultures were more frequent in the FG, particularly in those with a prior infection history ($P = 0.08$), even if they were receiving antibiotics ($P = 0.05$).

CON [...]

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Iatrogenic femoral neck fractures or separation of the proximal femoral epiphysis during closed reduction of irreducible femoral head fracture-dislocations in children: a review of 12 cases.

Lu Y, Xue C, Canavese F, De Salvo S, Yao H, Huang D, Lin R, Zheng J et al.

OBJECTIVES: This study aimed to analyze the radiologic characteristics of irreducible pediatric femoral head fracture-dislocations to prevent potential iatrogenic femoral neck fractures (FNF) or separation of the proximal femoral epiphysis (SPFE).

METHODS: This is a retrospective review of patients who were skeletally immature and diagnosed with traumatic hip dislocations combined with femoral head fractures. The collected data included patient demographics, fracture classification, fragment ratio, combined injuries, urgent reductions, treatment strategies, complications, and final outcomes.

RESULTS: We treated 12 patients with femoral head fractures and dislocations; 11 out of 12 patients underwent urgent closed reduction (91.7%). Six of the patients failed closed reduction and experienced FNF (n = 2, 33.3%) or SPFE (n = 4, 66.7%). Five patients presented with avascular necrosis of the femoral head after open reduction with internal fixation via a surgical hip dislocation approach, and two patients required further surgical treatment. Analysis of radiographs and computed tomography (CT) scans of irreducible femoral head fracture-dislocations revealed that the fractured femoral head was perched on the sharp angle of the posterior wall of the acetabulum, with a fragment ratio of 18-27%. After recognizing the irreducibility, one case with a fragment ratio of 26% underwent immed [...]

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Antibiofilm-coated intramedullary nailing for tibial and femoral diaphyseal fractures at high-risk infection: outcomes of 103 consecutive patients.

Vicenti G, Migliorini F, Loiodice M, Scarpino A, D'attis P, Schäfer L, Colasuonno G, Solarino G

BACKGROUND: Fracture-related infection (FRI) is a devastating complication of intramedullary nailing of tibial and femoral diaphyseal fractures. The present study evaluates the clinical efficacy of an antibacterially coated intramedullary nail in patients with high-risk tibial and femoral diaphyseal fractures. Open fractures, fractures requiring fasciotomy or closed fractures with Tscherne grade II or III soft-tissue damage were considered fractures at high risk of infection.

METHODS: A single-centre observational study was conducted between January 2022 and December 2024, in accordance with the Strengthening the Reporting of Observational Studies in Epidemiology (STROBE) guidelines. Patients who underwent definitive intramedullary nailing with Bactiguard (Bactiguard AB, Tullinge, Sweden)-coated Natural Nail (Zimmer Biomet, Warsaw, IN, USA) for tibial or femoral diaphyseal fractures at high risk of infection were included. The primary outcome was the incidence of FRI, defined according to the internationally validated consensus criteria. Secondary outcomes included time to union, rates of nonunion, malunion, unplanned secondary procedures and functional outcomes at a minimum follow-up of 12 months.

RESULTS: A total of 103 patients were enrolled: 55 with tibial fractures and 48 with femoral fractures. The combined cohort comprised 90 males and 13 females, with a median age of [...]

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Orthopaedic publications receive fewer citations than general medicine articles: a scientometric analysis of temporal trends.

Migliorini F, Vaishya R, Maffulli N, Eschweiler J, Kobbe P, Betsch M, Schäfer L, Oliva F et al.

BACKGROUND: Orthopaedics, as a surgical speciality with a structurally narrower citing audience and lower citation density than general medicine, is systematically disadvantaged in any cross-disciplinary comparison relying on absolute citation counts. Whether this disadvantage is stable or has changed over time has not been previously quantified.

METHODS: The 100 most-cited articles indexed in the Web of Science Core Collection were identified for medicine and orthopaedics in 2014 and 2024. Citation data were extracted from a single snapshot taken on 19 January 2026. Total citations and citation velocity, defined as citations per year since publication, were compared between fields and across time points using Mann-Whitney U tests. Ratio-based analyses and distributional characteristics were also assessed.

RESULTS: Median citation counts were substantially higher in medicine than in orthopaedics in both years, reaching 5055 versus 288 in 2014 and 720.5 versus 34.0 in 2024, corresponding to 17.6- and 21.2-fold differences, respectively. Citation velocity followed the same pattern, with medicine maintaining an approximately 17- to 21-fold advantage at both time points. Within-field temporal analysis showed that citation velocity remained broadly stable in medicine between cohorts, whilst orthopaedics demonstrated reduced early citation accrual in the more recent cohort. Citatio [...]

Sleep quality and nocturnal pain in patients with elbow disorders: a prospective multicenter study.

Schneider MM, Nietschke R, Migliorini F, Schoch C, Burkhart K, Hollinger B, Kimmeyer M, Lehmann L et al.

BACKGROUND: Mental health disorders have become a growing global concern. Sleep quality is known to have a crucial impact on mental well-being. Poor sleep contributes to increasing economic costs owing to reduced productivity as well as the development of mental disorders. Conversely, anxiety and depression often lead to sleep disturbances. Pain is a key factor affecting sleep quality. Studies have shown that musculoskeletal disorders, especially in the shoulder, may cause sleep disturbances. Research on the relationship between elbow pain and sleep quality remains limited. This study aims to evaluate the impact of common elbow pathologies on sleep quality.

METHODS: This prospective multicenter cohort study used self-reported outcome measures in patients with elbow disorders, including the Pittsburgh Sleep Quality Index (PSQI), to assess sleep quality. The PSQI was compared with other outcome measures.

RESULTS: A total of 664 patients (284 female, 380 male) were included, of whom 81% reported nocturnal pain. The mean age was 47.9 ± 14.2 years, body mass index (BMI) 26.9 ± 6.7 kg/m² and symptom duration 15.8 ± 7.3 months. The most common diagnoses were lateral epicondylar pain (66%), medial epicondylar pain, arthritis, ulnar nerve entrapment and biceps/triceps tendinopathies. The overall PSQI was 7.3 ± 3.8 , disabilities of arm, shoulder and hand questionnaire (DASH) 35.0 ± 19 . [...]

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A novel figure-of-eight polymeric cerclage wiring technique for greater trochanter fixation in revision arthroplasty and conversion of failed proximal femur fracture fixations to total hip arthroplasty.

Leggieri F, Martín Cocilova FN, Secci G, Stimolo D, Zanna L, Civinini R, Innocenti M

INTRODUCTION: Periprosthetic femoral fractures occur in 0.8-10% of primary total hip arthroplasties (THA) and up to 17.5% for revision THA (revTHA), with isolated greater trochanter (GT) fractures being the most predominant pattern. This study aimed to evaluate a novel figure-of-eight cerclage wiring technique for greater trochanter fixation in hip replacement cases.

METHODS: These retrospectively collected data included 36 hips (36 patients) undergoing revision THA or conversion of failed intertrochanteric fracture to THA with figure-of-eight cerclage wiring between April 2021 and June 2023. Patients under 18 years, with incomplete records, or lost to follow-up before 24 months were excluded. Primary outcomes included Trendelenburg sign, periprosthetic infections, and hip dislocation rates. Secondary outcomes evaluated THA stem subsidence and trochanteric migration using calibrated digital imaging. Statistical analysis included descriptive statistics, independent t-tests, linear regression, and correlation analyses with significance set at $p < 0.05$.

RESULTS: Mean follow-up was 404.76 ± 22.51 days. Patient demographics showed mean age 76.5 ± 12.3 years, body mass index (BMI) 25.8 ± 4.5 kg/m², with 73.8% female patients. At final follow-up, one patient (2.8%) experienced post-surgical infection, one patient (2.8%) had dislocation, and three patients (8.3%) exhibited positive T [...]

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Minimal clinically important difference (MCID), patient-acceptable state (PASS), minimally detectable change (MDC), and substantial clinical benefit (SCB) in patients who have undergone arthroscopic surgery for femoroacetabular impingement: a systematic review.

Migliorini F, Maffulli N, Schäfer L, Jeyaraman M, Betsch M, Mahmoud M

INTRODUCTION: Hip arthroscopy has been established as a successful management for femoroacetabular impingement (FAI) syndrome. Patient-reported outcome measures (PROMs) are frequently used to assess treatment results in patients. However, clinically important outcome values have recently been calculated to better understand and interpret PROMs and define successful treatment. This systematic review aimed to investigate the

minimal clinically important difference (MCID), patient-acceptable state (PASS), substantial clinical benefit (SCB), and minimally detectable change (MIC) of the most commonly used PROMs for assessing patients who have undergone arthroscopic surgery for FAI.

MATERIAL AND METHODS: This study was conducted according to the Preferred Reporting Items for Systematic Reviews and Meta-Analyses: the 2020 PRISMA statement. In October 2025, the following databases were accessed: PubMed, Web of Science, and Embase. All clinical studies investigating tools to assess the clinical relevance of PROMs used to evaluate patients who have undergone arthroscopic surgery for FAI were accessed. Only studies that assessed the minimal MCID, PASS, SCB, and/or MIC were considered.

RESULTS: The methodological quality of the 21 included studies was assessed, and the overall risk of bias was found to be low-to-moderate. The main characteristics of the included studies were analyzed and [...]

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Fibular allograft augmentation shows potential benefits in treating medial column comminuted proximal humeral fractures with osteoporosis: a secondary analysis of a randomized controlled trial.

Chai Y, Shi T, Jiang H, Chen Y, Xu Z, Zhu Y, Ruan Z, Chen Y et al.

BACKGROUND: Fibular allograft (FA) augmentation is proposed to address the lack of medial column support in plate-fixed proximal humeral fractures; however, the role of FA augmentation in these fractures remains inadequately elucidated. Our previous clinical trial did not show additional benefit in inserting an FA when treating adults with medial column comminuted proximal humeral fractures. Nonetheless, it is possible that some subgroups may still derive benefits from FA. This study aimed to investigate whether there were any subgroups benefit from FA.

MATERIALS AND METHODS: Our randomized controlled trial (RCT) recruited adults with a medial column comminuted proximal humeral fracture and randomly allocated them to receive a surgery fixed with a locking plate (LP group) or with an LP augmented with an FA (FA group). This study was a subsequent subgroup analysis of the previous randomized controlled trial, in which we included 72 participants who completed follow-up at the time of 12 months for the current subgroup analysis. The primary outcome was the Disabilities of the Arm, Shoulder and Hand (DASH) score at 12 months postoperatively. The secondary outcomes included the DASH score at other time points, changes in neck-shaft angle and humeral head height. We used generalized linear models for subgroup analysis to evaluate the effect of intervention (FA or LP) on 12-month DAS [...]

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Thicker polyethylene inserts for varus deformity versus thinner inserts for flexion contracture in total knee arthroplasty: a retrospective comparative study.

Lee SH, Kwon HM, Lee WS, Park KK

BACKGROUND: The success of total knee arthroplasty (TKA) depends on achieving precise mechanical alignment and stable soft tissue balance, with the thickness of the final polyethylene (PE) insert serving as a critical component. However, the impact of preoperative factors on PE thickness remains unclear. In this study, we investigated the effects of preoperative knee alignment and clinical deformities on PE thickness.

MATERIALS AND METHODS: The medical records of patients who underwent primary TKA between August 2024 and July 2025 were retrospectively evaluated. Preoperative radiographic parameters, including the hip-knee-ankle (HKA) angle, medial proximal tibial angle, lateral distal femoral angle (LDFA), joint-line convergence angle, weight-bearing line ratio (WBLR), and posterior tibial slope, were measured, along with the flexion contracture (FC) as a clinical factor. Intra-observer reliability was assessed using intraclass correlation coefficients (ICC). Linear regression analyses were performed to evaluate the effects of these preoperative factors on PE thickness.

RESULTS: This study included 233 knees (158 patients). The intra-observer reliability of the radiographic parameters was excellent (ICC > 0.9). Multivariable linear regression revealed that preoperative HKA angle ($\beta = 0.033$, $p < 0.001$), WBLR ($\beta = -0.008$, $p < 0.001$), and LDFA ($\beta = 0.065$, $p = 0.003$) were signifi [...]

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PediLoc® blade plate yields faster consolidation rates than LCP pediatric condylar plate in distal femoral osteotomies: a retrospective comparison and description of surgical technique.

Tschudi S, Delay C, Krautwurst B, Grisch D, Dreher T

BACKGROUND: The PediLoc® Locking Cannulated Blade Plate System was originally designed for proximal femoral osteotomies but has also been used for multidimensional correction of distal femoral deformities. The present study delineates the surgical technique and undertakes a retrospective analysis of the consolidation rates of distal femoral osteotomies performed using the aforementioned system, comparing them with those performed using the DePuy Synthes® 90° LCP Pediatric Condylar Plate. We hypothesized that the semi-rigid design of the blade plate results in faster osteotomy healing rates compared with other locking screw plate systems.

MATERIALS AND METHODS: This retrospective single-centre study included patients who underwent distal femoral osteotomy with the PediLoc® Blade Plate (Blade Plate) and the DePuy Synthes® 90° LCP Pediatric Condylar Plate (90° LCP-PCP) at the University Children's Hospital Zurich (2020-2024). Consolidation rates were assessed using "Regenerating Bone Defects Using New Biomedical Engineering" (REBORNE), "Radiographic Union Score for Tibial Fractures" (RUST) and modified RUST (mRUST) scores at 6 weeks and 3 months postoperatively and compared using Wilcoxon signed-rank test. For the comparison between both intervention groups, the unpaired t-test was used for normally distributed data and the Mann-Whitney U test for non-normally distributed data [...]

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Thin cortical translucent area of the iliac wing: morphometric analysis and implications for surgical safety.

Yüceda■ Gündo■du H, Pasl■ B, Erçakmak Güne■ B

BACKGROUND: The iliac wing is a frequently used fixation site in the surgical management of pelvic and acetabular fractures. Achieving safe implant placement in this region necessitates a comprehensive understanding of local morphology. However, the morphometric characteristics and surgical safety margins of the thin cortical translucent area (PLA) located in the central iliac wing have not been sufficiently defined. This may pose a significant challenge in evaluating the risk of cortical perforation, particularly during implant placement. This study aimed to evaluate the morphometric characteristics of the PLA and its relationship to fixation corridors.

METHODS: Fifty-one unpaired dry hip bones were examined. The thin cortical translucent region was identified using photoluminescence. Photograph-based measurements were conducted utilising ImageJ software on standardised photographs. Dimensions, area, distances to reference landmarks and relationships with the iliac pillar and supraacetabular corridors were assessed. The safe angular deviation range for anterior inferior iliac spine (AIIIS)-origin screw fixation was calculated. Cortical thickness was measured using a digital thickness gauge.

RESULTS: The vertical and horizontal diameters and area of the PLA were 56.24 ± 16.56 mm, 59.92 ± 24.83 mm and 28.10 ± 16.47 cm², respectively. Mean distances to the anterior superior ilia [...]

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Efficacy and safety of MIPO versus ORIF in distal tibial fractures: a systematic review and meta-analysis.

Hassan AM, Hassan AA, Abdelkhalik WS, Saad AM, Hosny AS, Elfeky BAEH, Mandour I, Ragheb MG et al.

BACKGROUND: Distal tibial fractures are challenging to manage owing to limited soft-tissue coverage and compromised blood supply. Minimally invasive plate osteosynthesis (MIPO) and open reduction and internal fixation (ORIF) are among the widely used surgical options, but the optimal approach remains unclear. MIPO preserves periosteal blood flow with smaller incisions, while ORIF offers direct visualisation but increases soft-tissue damage. With growing comparative evidence, an updated evaluation of these techniques is necessary.

AIM: To compare the efficacy and safety of MIPO versus ORIF in adult patients with distal tibial fractures.

METHODS: Following PRISMA guidelines and the Cochrane Handbook, we searched in PubMed, Embase, Cochrane, Scopus, and Web of Science till November 2025. Randomised trials and cohort studies comparing MIPO and ORIF were included. Data extraction was performed independently by two reviewers. Risk of bias was

assessed using RoB 2 for RCTs and the Newcastle-Ottawa Scale for observational studies. Random-effects models were used to estimate pooled mean differences and odds ratios, and heterogeneity was assessed using I². Sensitivity and subgroup analyses were conducted by study design.

RESULTS: Nine studies involving 530 patients met our inclusion criteria. No significant differences were found between MIPO and ORIF in AOFAS scores, union time, oper [...]

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Comparative efficacy and safety of aspirin, low-molecular-weight heparin, and rivaroxaban for deep venous thrombosis prophylaxis after hip arthroplasty for femoral neck fracture: a single-center randomized controlled trial.

Miao S, Yue Q, Zhang X, Lu J, Zhang Y, Lu N, Wei L

OBJECTIVE: To evaluate the efficacy and safety of aspirin, low-molecular-weight heparin (LMWH), and rivaroxaban for deep vein thrombosis (DVT) prophylaxis after total hip arthroplasty (THA) or bipolar hemiarthroplasty (BHA) for femoral neck fracture, with subgroup analysis stratified by surgical type.

METHODS: This single-center randomized controlled trial (RCT) was conducted between June 2024 and June 2025. A total of 217 consecutive patients with femoral neck fracture undergoing hip arthroplasty were initially screened, and 161 eligible patients were finally enrolled and randomly assigned to the aspirin group, LMWH group, or rivaroxaban group at a 1:1:1 ratio. All patients received standardized intermittent pneumatic compression (IPC) and graduated compression stockings for mechanical DVT prophylaxis, as well as a unified postoperative rehabilitation protocol. The primary outcome was the incidence of lower extremity DVT within 12 weeks postoperatively, confirmed by color Doppler ultrasound. The secondary outcome was the incidence of bleeding-related complications graded by the International Society on Thrombosis and Hemostasis (ISTH) criteria. All patients were followed up at 6, 8, and 12 weeks postoperatively, with routine bilateral lower extremity ultrasound performed at 12 weeks regardless of symptoms. Subgroup analysis was performed on the basis of surgical type (THA ver [...])

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Does the surgical sequence of lumbar spine fusion and total hip arthroplasty affect the dislocation and revision risk? A meta-analysis and systematic review.

Liu F, Jia C, Wu X, Zhou Y

BACKGROUND: Patients who underwent primary total hip arthroplasty (THA) after lumbar spine fusion (LSF) had a higher incidence of complications compared with those who had not previously undergone LSF. This study aimed to determine whether the timing of THA before or after LSF impacts the incidence of THA complications.

METHODS: Following the Preferred Reporting Items for Systematic reviews and Meta-Analyses (PRISMA) guidelines, we searched PubMed, Web of science, Embase, and Cochrane Library from their inception until 1 October 2025. Data from all eligible published studies meeting the inclusion criteria were extracted and subsequently analyzed using a random-effects model to calculate the rates of all-cause revision, dislocation, periprosthetic joint infections, and aseptic loosening.

RESULTS: Our analysis included 10 studies involving 27,605 patients who underwent THA followed by LSF and 32,002 patients who received LSF prior to THA. There are no significant differences in the rates of dislocation (odds ratio [OR] 1.19, 95% confidence interval (CI) 0.73-1.86, $p = 0.45$), periprosthetic joint infections (OR 0.91, 95% CI 0.52-1.62, $p = 0.76$), and aseptic loosening (OR 0.89, 95% CI 0.74-1.08, $p = 0.23$) between the two surgical sequences, while for all-cause revision, the OR was (OR 1.01, 95% CI 0.70-1.47, $p = 0.94$), which reached statistical significance.

CONCLUSIONS: Compare [...]

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Effects of anterior cruciate ligament resection and total knee arthroplasty on lower limb alignment and tibiofemoral rotation.

Fan Y, Chen S, Jiang L, Chen Z, Wu M, Wang C

BACKGROUND: The anterior cruciate ligament (ACL) is a key structure that restricts excessive anterior translation of the tibia and maintains rotational stability. This study aims to dynamically and quantitatively assess the direct effects of ACL resection and total knee arthroplasty (TKA) on lower limb alignment and tibiofemoral joint rotation under passive, unloaded conditions and in the pathological state of osteoarthritis.

METHODS: This retrospective study included 110 patients with varus knee osteoarthritis who underwent Mako for robotic-assisted TKA. Dynamic intraoperative measurements of the lower limb mechanical axis (flexion angle at maximum extension; varus angle) and tibiofemoral rotation at multiple flexion angles (min to max flexion) were recorded at three intervals: pre-ACL resection, post-ACL resection and post-TKA. The variation in rotation amplitude was calculated for successive flexion intervals.

RESULTS: Post-ACL resection, a significant decrease in the flexion angle at the maximum extension and varus angle (both $p < 0.001$) was observed, which was accompanied by increased tibial internal rotation at flexion angles of 60° and above ($p < 0.05$). Varus alignment was successfully corrected following TKA ($p < 0.001$). However, tibiofemoral rotational kinematics were significantly modified: the amplitude of internal rotation decreased in the initial flexion arc (min [...])

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Comparison of Bankart repair with remplissage and Latarjet procedure for anterior shoulder instability: a systematic review and meta-analysis.

Nong K, Liu G, Jiang S, Li Q, Kwabena BR, Yuan D, Xiao W, Zhang K et al.

BACKGROUND: Anterior shoulder instability is the most common type of shoulder instability. For cases with subcritical glenoid bone loss, both Bankart repair with remplissage (BR) and the Latarjet procedure are viable surgical options. The aim of this study was to compare the postoperative outcomes in patients undergoing BR and Latarjet procedure.

METHODS: This review was conducted in accordance with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) guideline. The methodological index for nonrandomized studies (MINORS) was used to characterize the methodological quality. A literature search was conducted using PubMed, the Cochrane Library, Embase, Web of Science, and Scopus. The analyzed outcomes included postoperative Rowe score, visual analog scale (VAS) score, the Single Assessment Numeric Evaluation (SANE) score, range of motion (ROM), return-to-sport (RTS) rate, RTS time, recurrence rate, revision rate, and complications. Subgroup analysis was performed on the basis of follow-up duration.

RESULTS: Overall, 12 studies involving 1186 patients were included. The meta-analysis showed that BR was associated with a significantly lower complication rate than the Latarjet procedure. No significant differences were observed in functional outcomes, ROM, recurrence, revision, RTS, or RTS time. Subgroup analysis showed a significant interaction between f [...]

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Risk of periprosthetic joint infection within 1 year following robotic-assisted versus conventional primary total knee arthroplasty: a propensity-score-matched cohort study.

Tian S, Li Y, Mu W, Ji B, Zhang X, Cao L, Zhao J

BACKGROUND: Robotic-assisted total knee arthroplasty (RA-TKA) is increasingly being adopted for its ability to enhance bone-resection accuracy and component alignment. However, whether these technical gains influence the risk of periprosthetic joint infection (PJI) remains uncertain, especially in the context of prolonged operative duration. This study aimed to compare the 1-year rate of PJI following conventional total knee arthroplasty (cTKA) and RA-TKA in a propensity-score-matched cohort.

METHODS: We retrospectively reviewed 1284 consecutive patients who underwent primary TKAs at a single centre between 2021 and 2023. The patients were stratified according to surgical technique (cTKA versus RA-TKA) and subsequently matched 1:1 using propensity score analysis (age, sex, body mass index [BMI], American Society of Anesthesiologists [ASA] score, Charlson Comorbidity Index [CCI] score, CCI components and smoking), resulting in 522 pairs (1044 patients) for the final comparative analysis. Operative time and 1-year PJI were assessed using multivariable logistic regression. Infections were stratified according to timing: ≤ 90 days and from 90 days to 1 year after surgery.

RESULTS: The 1-year rate of PJI was 0.77% (4/522) after RA-TKA and 0.96% (5/522) after cTKA (P = 1.000). All PJIs in patients who underwent RA-TKA occurred within 90 days, whereas PJIs in patients who underwent [...]

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Measured resection, gap balancing, and force-sensor-guided total knee arthroplasty result in different femoral and tibial rotational alignment but similar combined knee rotation and clinical outcomes: a randomized controlled trial.

Hernández-Hermoso JA, Nescolarde L, Yañez-Siller F, Calle-García J, Pérez-Andrés R, García-Perdomo D

BACKGROUND: Different alignment philosophies and balancing methods may alter femoral and tibial component rotation in distinct ways within the same knee. This study aimed to (1) determine which of three surgical techniques-measured resection (MR), gap balancing with computer-assisted surgery (GB-CAS), or a force-sensor soft-tissue balancing device (FS-STB)-most closely reproduces native femoral, tibial, and combined rotational alignment in mechanically aligned total knee arthroplasty (TKA), and (2) assess whether differences in rotational alignment affect outcomes at 5 years postoperatively.

MATERIAL AND METHODS: A total of 60 patients undergoing primary mechanical alignment TKA were randomly assigned to one of three surgical approaches (n = 20 per group): MR, GB-CAS, or FS-STB. Blinded observers assessed the Knee Society Score (KSS), Western Ontario MacMaster Universities Osteoarthritis Index (WOMAC) score, and hip-knee-ankle angle preoperatively and at the 5-year follow-up visit. Pre- and postoperative two-dimensional (2D)-computed tomography scans were used to measure femoral rotation (BFA), tibial rotation, and combined femur-tibia rotation (TE_PTCA and BC_PTCA). Statistical analyses included paired t-tests, one-way analysis of variance, and effect size calculations.

RESULTS: Femoral rotation remained unchanged in the MR and GB-CAS groups, but decreased slightly (1° exter [...])

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Risser growth plate injury in unstable paediatric pelvic fractures: a multicentre retrospective study.

Oransky M, Aulisa AG, Roncoroni A, Zoppi AR, Mata M, Rohayem MA, Falciglia F, Toniolo RM

INTRODUCTION: Unstable pelvic fractures in children are rare but severe injuries and are associated with high-energy trauma. Unlike adults, children have open growth plates, particularly the Risser growth plate (RGP), which is vulnerable to injury. The presence of growth plates, such as the triradiate cartilage and Risser's plate, represents point of vulnerability that determines distinctive fracture patterns, similar to those seen with Salter-Harris growth plate injuries. Lateral compression fracture is the most common mechanism and results in an irreducible crushing of the sacral alae, causing a rotational deformity. Both vertical and lateral compression displaced pelvic fractures require careful evaluation of the posterior iliac apophysis-associated injury. Failure to treat Risser growth plate injury (RGPI) can lead to severe long-term problems such as limb length discrepancy and scoliosis. This study aimed to determine how commonly RGPI occurs in unstable paediatric pelvic fractures, link it to specific fracture patterns and propose a subgroup type to improve treatment.

MATERIALS AND METHODS: This multicentre retrospective study included 40 children aged up to 12 years with unstable pelvic fractures (AO/OTA 61B and 61C). The patients were treated between 1987 and 2022 at trauma centres in Italy, Argentina and Venezuela. We reviewed patient demographics, injury mechanisms a [...]

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Total femur replacement: function, quality of life, and gait: a multicentric EMSOS study.

Valentini M, Svehlik M, Leithner A, Bergovec M, EMSOS TFR Study Group, Smolle MA

BACKGROUND: Total femur replacements (TFRs) involve the replacement of the whole femoral bone, reconstruction of the hip and knee joints, and soft tissue reconstruction for function and joint stability. The aim of this multicentric European Musculo-Skeletal Oncology Society (EMSOS) study is to report on the functional outcome, quality of life, as well as gait analysis in patients with TFR.

MATERIALS AND METHODS: Retrospective data from 12 international participating centers were collected. In total, 65 patients [median (IQR) age at surgery 40 (18-56) years, 60% male] who received a TFR owing to

oncologic or nononcologic indications between 1 January 1990 and 31 March 2024 were included. Patient-related outcome measures (PROMs), range of motion (ROM) of the hip and knee, and gait videos were collected.

RESULTS: At a median (IQR) follow-up of 28 (14-126) months, the median (IQR) percentage Musculoskeletal Tumor Society (MSTS) score was 63% (43-73%), 12-Item Short-Form Health Survey (SF-12) was 70% (55-79%), Harris Hip Score (HHS) was 63% (52-77%), and Oxford Knee Score (OKS) was 63% (52-79%); the median (IQR) extension-to-flexion ROM were 80° (70-97.5°, hip) and 70° (40-90°, knee). Gait videos were available for 24/65 patients (37%). The median (IQR) total Edinburgh Visual Gait Score of the operated side was 8.5 (6-13), with deviations from norm in all patients (24/24). Compens [...]

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Vascularized versus nonvascularized free fibular grafts in reconstruction of post-traumatic critical long bone defects, a comparative study.

Fouaad AA, Massoud AH, Shaban M, Abdel-Naby M, Abulsoud MI

BACKGROUND: Reconstruction of critical-sized long bone defects is a complex orthopedic challenge. Free fibular grafts, vascularized (FVFG) and nonvascularized (NVFG), are established reconstructive options, but comparative clinical outcomes remain uncertain.

PURPOSE: To compare clinical and radiological outcomes of FVFG and NVFG in patients with post-traumatic critical bone defects more than 10 cm.

METHODS: A randomized controlled trial was conducted with 50 patients assigned equally to FVFG or NVFG groups. The primary outcome was time to union, while secondary outcomes included graft hypertrophy, functional scores (DASH and LEFS), complication rates, and donor site morbidity.

RESULTS: The mean time to union was 5.78 months in the FVFG group and 6.17 months in the NVFG group, showing no statistically significant difference ($p = 0.447$). Rates of graft hypertrophy, functional recovery, and complications were comparable between the groups.

CONCLUSIONS: Both FVFG and NVFG provide effective reconstruction for critical bone defects, with nearly similar healing times and functional outcomes. NVFG may represent a less complex alternative in selected cases.

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Five-year outcomes of medial open-wedge high tibial osteotomy with lateral supplemental lag screw in patients with obesity.

Tian YH, Lee KH, Huang CW, Lin SY, Yang JC

BACKGROUND: Obesity contributes to the accelerated progression of knee osteoarthritis. Medial open-wedge high tibial osteotomy (MOWHTO) is a joint-preserving surgical intervention for unicompartmental knee osteoarthritis; however, its efficacy in patients with obesity remains controversial. This study aimed to evaluate the 5-year clinical and radiographic outcomes of MOWHTO with lateral supplemental lag screw fixation in patients with obesity.

MATERIALS AND METHODS: This retrospective cohort study included patients with obesity who underwent MOWHTO between 2017 and 2020, with a minimum follow-up of 5 years. All procedures were performed by a single surgeon using three-dimensional printed, patient-specific instrumentation, locking plates, and a lateral supplemental lag screw. Clinical outcomes were assessed using the visual analog scale (VAS) and Western Ontario and McMaster Universities Osteoarthritis Index (WOMAC). Radiographic parameters, including the weight-bearing line percentage (WBL%) and medial proximal tibial angle (MPTA), were evaluated.

RESULTS: Significant improvements in VAS and WOMAC scores were observed at all postoperative time points ($p < 0.001$) and were accompanied by improved radiographic alignment, with WBL% shifting toward the Fujisawa point and increased MPTA values. At 5 years, mild regression in alignment was noted; however, the overall correction was [...]

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Outcomes of surgical fixation of osteochondral fracture: a systematic review.

Pang AS, Oviya R, Tan JJ, Lim KSA, Hui JHP, Tan SHS

BACKGROUND: Dealing with knee osteochondral defects presents a substantial clinical challenge due to the variable nature of repair outcomes and intricate biomechanical complexities inherent to the knee joint. Numerous surgical alternatives exist for addressing knee osteochondral lesions, yet there is limited evidence available for comparing their respective outcomes.

METHODS: According to the Preferred Reporting Items for Systematic Review and Meta-analyses (PRISMA) guidelines, a systematic literature search was conducted to identify publications from inception to September 2023 on PubMed, Cochrane, and Medline academic search engines with the MeSH terms "osteochondral" AND ("patella" OR "patellar" OR "patellofemoral" OR "femoral condyle lesions" OR "trochlear lesions"). Inclusion criteria were clinical human studies, English language, subjects who underwent surgical management (fixation, reconstruction, or debridement) for knee osteochondral lesions with reported treatment outcomes, and a minimum sample size of ten patients. The methodological index for non-randomized studies (MINORS) was used to evaluate the non-randomized studies' methodological quality. Main outcome measurements Pediatric International Knee Documentation Committee (Pedi IKDC) scale, Tegner-Lysholm Scoring Scale, Tegner Activity Score, Visual Analog Scale, Knee Society Score (KSS) Score, Kujala Score, and F [...]

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Do modular tapered long stems differ in stability and function? A prospective comparison of two stems in complex femoral revision THA.

Fahmy M, Shawky MA

BACKGROUND: Revision total hip arthroplasty (rTHA) in the setting of substantial proximal femoral bone loss is technically challenging. Modular tapered fluted stems provide predictable diaphyseal fixation while allowing independent adjustment of version, offset, and limb length. Among these, two commonly used systems-modular tapered fluted stem Type A (Revitan™) and Type B (LIMA Modulus™)-have limited direct comparative evidence. This study aimed to prospectively compare radiographic stem subsidence (primary outcome), as well as functional outcomes, complications, survivorship, and secondary outcomes, between Type A and Type B modular long stems in femoral rTHA.

METHODS: In this single-center randomized prospective study, 110 patients undergoing femoral revision rTHA were randomly assigned to receive either Type A (n = 55) or Type B (n = 55) stems. All procedures were performed by experienced revision surgeons under standardized perioperative and rehabilitation protocols. Radiographs were analyzed for stem subsidence, osseointegration, and limb-length restoration. Functional outcomes were assessed using the Harris Hip Score (HHS), Oxford Hip Score (OHS), and European Quality of Life Visual Analogue Scale (EQ-VAS) at baseline and final follow-up (mean 61.4 months). Complications and stem survivorship were recorded prospectively. Statistical analysis included paired and unpaired [...]

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Clinical comparison of single posterolateral plate with medial-cannulated-screw fixation and double-plate fixation for extra-articular distal humerus fractures.

Jung HS, Chu MS, Lee JS

BACKGROUND: Double-plate fixation is the gold standard for extra-articular distal humerus fractures, but it carries a substantial risk of postoperative ulnar neuropathy. Fixation using a single posterolateral plate with a medial cannulated screw may reduce ulnar neuropathy while maintaining fracture stability. This study aimed to compare the clinical outcomes of single-plate-with-medial-screw fixation versus double-plate fixation for extra-articular distal humerus fractures.

MATERIALS AND METHODS: Fifty-six patients who underwent surgery for extra-articular distal humerus fractures (Arbeitsgemeinschaft für Osteosynthesefragen/Orthopaedic Trauma Association [AO/OTA] classification A2 or A3) between January 2018 and August 2024 were divided into a double-plate group and a single-plate-with-medial-screw group. We conducted a retrospective, nonrandomized comparative study. The double-plate fixation was used in 30 patients from January 2018 to October 2021, while the single-plate fixation with a medial screw was used in 26 patients from November 2021 to August 2024. All surgeries were performed using a posterior paratricipital approach. Bony union, radiographic healing, and loss of reduction were evaluated. Postoperative pain scores (visual analog scale at 2 days after the operation), operative time (minutes), elbow range of motion, elbow function (Mayo Elbow Performance Score [MEP [...]

A study on the association between tibial plateau fractures and intra-articular soft-tissue injuries under valgus injury mechanisms.

Duan S, Wang S, Zhu T, Li Q, Zhang M

BACKGROUND: While recent investigations have focused on injury mechanism classifications of tibial plateau fractures (TPFs), the association between valgus TPFs and concomitant soft-tissue damage involving menisci and ligaments remains insufficiently elucidated. This study aimed to characterize intra-articular soft-tissue injuries associated with various valgus TPFs and assess the predictive value of lateral plateau depression (LPD) and widening (LPW). Additionally, the analysis extended to other injury mechanisms.

MATERIALS AND METHODS: This study included adult patients with acute tibial plateau fractures who had complete imaging data, excluding patients with open fractures and multiple fractures throughout the body. Imaging examinations were used to assess the fracture mechanism and intra-articular soft-tissue damage.

RESULTS: The study retrospectively analyzed the clinical data of 138 patients with valgus injury TPFs in our hospital. The study compared the incidence of TPFs with intra-articular soft-tissue damage under different injury mechanisms. The result demonstrated that the incidence of valgus hyperextension TPFs combined with medial collateral ligament injuries was relatively high (61.5%) compared with TPFs with other injury mechanism. Multivariable logistic regression and smooth curve fitting revealed significant dose-response relationships of LPD (odds ratio [OR] [...])

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The coronoid as the key fragment of trans-ulnar fracture-dislocations of the elbow: Insights from a retrospective cohort comparison using the coronoid-centric Mayo classification system.

Henssler L, Kerschbaum M, Zanklmaier M, Wieczorek LA, Alt V, Klute L

BACKGROUND: Trans-ulnar fracture-dislocations of the elbow are rare injuries with complex fracture patterns and variable outcomes. Traditional classification systems offer limited prognostic value. A recently introduced coronoid-centric Mayo classification distinguishes injury subtypes based on coronoid attachment and identifies trans-ulnar basal coronoid (TUBC) fractures as a particularly challenging entity. This study evaluated outcomes across Mayo fracture types and explored factors associated with inferior results in TUBC injuries.

MATERIALS AND METHODS: In this retrospective cohort study, surgically treated trans-ulnar elbow fracture-dislocations managed at a level I trauma center between 2010 and 2022 were identified and classified according to the Mayo system. Demographic data, injury characteristics, surgical management, radiographic outcomes, and complications were recorded. Functional outcomes were assessed after a minimum follow-up of 12 months using the Mayo Elbow Performance Score (MEPS); Oxford Elbow Score (OES); Quick Disabilities of Arm, Shoulder and Hand Questionnaire (QuickDASH); European Quality of Life Five-Dimension, Five-Level Version (EQ-5D-5L); and range-of-motion measurements. Radiographs were analyzed for union, instability, heterotopic ossification, and post-traumatic osteoarthritis (OA).

RESULTS: A total of 52 patients were included (14 trans-olecr [...])

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Feasibility study of a robot-assisted endoscopic technique for plate osteosyntheses of the acetabulum and anterior pelvic ring.

Hinz N, Althoff G, Thomaschewski M, Bonke LA, Eberenz A, Münch M, Behrends L, Männel G et al.

BACKGROUND: Minimally invasive stabilization of acetabular and pelvic ring fractures using endoscopic techniques has become increasingly important. The logical advancement of conventional endoscopic techniques is a robot-assisted approach, which benefits from the advantages of robot-assistance systems (e.g., more degrees of freedom for instruments and improved visualization). The aim was therefore to investigate the feasibility of a robot-assisted endoscopic technique for plate osteosyntheses of the acetabulum and anterior pelvic ring.

MATERIALS AND METHODS: In this two-part feasibility study, four different plate osteosyntheses were performed endoscopically using the Hugo™ robot-assisted surgery system, first on ten synthetic pelvic models and then on

ten human cadavers. During robot-assisted dissection for the preperitoneal approach, identification of ten relevant anatomical landmarks was also assessed. In both parts, the success, number of drilling errors, and time for each plate were described as learning curves and analyzed using linear regression.

RESULTS: The infrapectineal plate could be successfully performed in 100% of synthetic models, the posterior column plate in 100%, the suprapectineal plate in 90%, and the superior pubic ramus plate in 80%. Learning curves could be observed for the number of drilling errors per plate (e.g., 0.67 to 0, $p = 0.009$) and the time r [...]

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Favorable subjective clinical outcomes after revision hip arthroscopy for femoroacetabular impingement syndrome despite a high rate of capsular defects and increased capsular thickness.

Cao Z, Gao G, Lin W, Zhu Y, Zhou X, Wang J, Xu Y

BACKGROUND: The capsular healing status and capsular thickness changes in patients with femoroacetabular impingement syndrome (FAIS) following revision hip arthroscopy are poorly documented, and their relationship with subjective clinical outcomes remains unclear. The purpose of this work is to evaluate the incidence of capsular defects and changes in capsular thickness using magnetic resonance imaging (MRI) following hip arthroscopy in patients with FAIS, and to compare the subjective clinical outcomes between patients with and without capsular healing after revision hip arthroscopy.

PATIENTS AND METHODS: Consecutive patients with FAIS who underwent revision hip arthroscopy between 2013 and 2023 were included. Patients were categorized into two groups on the basis of capsular healing status after revision hip arthroscopy. Patient-reported outcomes (PROs) were collected preoperatively and at minimum 2-year follow-up, including the modified Harris Hip Score (mHHS), International Hip Outcome Tool-12 (iHOT-12), Hip Outcome Score-Activity of Daily Living Scale (HOS-ADL), Hip Outcome Score-Sport Specific Subscale (HOS-SSS), and visual analog scale (VAS). Achievements in minimal clinically important difference (MCID) and patient acceptable symptom state (PASS), patient satisfaction, and re-revision rates were evaluated and compared between groups. Capsular thickness was assessed by [...]

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From bench to bedside: a novel suture-augmented prosthesis for greater trochanteric fixation in osteoporotic hip arthroplasty.

Yang M, Yang J, Wang X, Wu Z, Bian S, Sheng R, Liao F, Qin K et al.

BACKGROUND: Stable fixation of the greater trochanter during hip arthroplasty for unstable osteoporotic intertrochanteric fractures remains a significant challenge. Conventional techniques depend on securing the bone-implant interface, which is particularly compromised in osteoporotic bone. Here, we propose a novel conceptual approach that establishes a direct mechanical bridge from the abductor tendon to the prosthesis, thereby reducing reliance on the fragile bone-implant interface.

METHODS: In this two-stage translational study, we progressed from biomechanical validation to clinical application. First, using a decalcified caprine femoral model simulating osteoporosis, three fixation constructs were compared: locking plate (LP), suture-augmented locking plate (LPSA), and Kirschner-wire tension band (KWTB). Ultimate load and construct stability were evaluated. Biomechanical testing confirmed the principle of suture-mediated load sharing and highlighted the intrinsic weakness of screw fixation in osteoporotic bone. Guided by these results, we designed a novel femoral prosthesis that eliminates screw fixation, employs sutures as the primary load-bearing element, and incorporates integrated suture anchor tunnels. This prosthesis was then assessed in a retrospective series of 15 consecutive elderly patients with osteoporotic intertrochanteric fractures. Clinical outcomes were ev [...]

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Updating radiographic parameters for the healthy growing hip: are current parameters still valid?

Hütter K, Smolle MA, Butter S, Schroedter R, Tschauer S, Kraus T

BACKGROUND: This study aimed to evaluate whether the acetabular angle (AA) by Tönnis and lateral center-edge angle (LCEA) by Wiberg-key radiographic parameters in pediatric hip assessment-have changed in a contemporary pediatric population compared with historical reference values, considering trends in earlier skeletal maturation and body composition.

MATERIALS AND METHODS: This retrospective diagnostic accuracy study assessed changes in AA and center-edge angle (CE) angles in a contemporary cohort. A total of 1774 anteroposterior pelvic radiographs (3548 hips) from patients aged 0-18 years without hip pathology were analyzed. Radiographs were obtained between 2006 and 2018 and measured using the Supervisely digital platform. Standardized anatomical landmarks were applied. Data were stratified by age and sex and compared with historical values using two-sample t-tests ($p < 0.05$). To minimize measurement bias, two independent observers conducted all assessments using standardized digital protocols.

RESULTS: A total of 1774 patients aged 0-18 years (mean age, 8.30 ± 5.20 years; 666 females, 1108 males) were evaluated. AA values did not show statistically significant differences in 11 of 16 age groups compared with historical data ($p > 0.05$). In contrast, LCEAs were significantly higher in the contemporary cohort, especially in the 5-8, 9-12, and 12-16 age groups (all $p < 0.01$) [...]

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In-hospital deep vein thrombosis after tibial plateau fractures: incidence, laterality/anatomy, and risk factors in a multicenter retrospective cohort of 3366 patients.

Yang S, Li G, Li Y, Long Y, Zhang J, Liu L, Zhao Z, Sun C et al.

BACKGROUND: We aimed to determine the cumulative in-hospital incidence, anatomic distribution, and predictors of duplex ultrasonography-detected lower-extremity deep vein thrombosis (DVT) in patients hospitalized with tibial plateau fractures under routine thromboprophylaxis and protocolized surveillance.

METHODS: We retrospectively analyzed consecutive adults (≥ 18 years) admitted to two level-I trauma centers (November 2014-January 2024). Bilateral duplex ultrasonography was performed on admission/preoperatively (as soon as feasible after arrival and prior to surgery) and repeated serially during hospitalization (approximately every 5-7 days and when clinically suspected). The primary outcome was cumulative in-hospital DVT from admission to discharge; isolated intermuscular calf-muscle vein thrombosis (soleal/gastrocnemius) was recorded descriptively but excluded from the primary endpoint. Multiple injuries (polytrauma) were defined as ≥ 1 additional acute traumatic lesion beyond the index tibial plateau fracture documented on admission. Pulmonary embolism (PE) was assessed only when clinically suspected and confirmed by computed tomography pulmonary angiography (CTPA). Multivariable logistic regression identified independent predictors; receiver operating characteristic (ROC) analysis evaluated discrimination for key continuous predictors.

RESULTS: Among 3366 patients, 675 [...]

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Retained articulating spacers (1.5 stage) in chronic knee periprosthetic joint infections: an analysis of associate factors and outcomes in a single center cohort of patients.

Balato G, Ascione T, Festa E, De Mauro D, Di Gennaro D, Rosa D, Mariconda M

BACKGROUND: Two-stage revision remains the gold standard for chronic periprosthetic joint infection (PJI), but it can significantly impact patients' quality of life and functional recovery. The 1.5-stage revision, involving indefinite spacer retention, has emerged as a potential alternative in selected high-risk patients. Our questions were: (1) What are the early functional outcomes, including their infection-free survival? (2) Which pre- and perioperative factors are associated with spacer retention? and (3) What is the mechanical survivorship of retained spacers?

MATERIALS AND METHODS: This was a retrospective observational study including 108 consecutive patients with chronic PJI between 2016 and 2020 who were followed prospectively for clinical outcomes. Functional status and quality of life were assessed using the EuroQol-5-Dimension-5-Level (EQ-5D-5L), Western Ontario and McMaster Universities Osteoarthritis Index (WOMAC), and Knee Society Score (KSS) before spacer placement and prior to scheduled reimplantation. Infection-free status was defined as the absence of clinical, radiographic, and laboratory signs of infection recurrence over a 24-month follow-up. Univariate and multivariate logistic regression were used to identify factors associated with spacer retention.

RESULTS: All patients initially underwent articulating spacer implantation as part of a planned two-st [...]

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Moderate leg length discrepancy: a long-term risk factor for hip osteoarthritis?

Aprato A, Donis A, Via RG, Scandurra A, Tuè F, Audisio A, Fusini F, Massè A

BACKGROUND: Leg length discrepancy (LLD) has been implicated as a biomechanical factor contributing to hip osteoarthritis (OA), yet the extent of its influence remains unclear. This study examines the correlation between moderate LLD (≥ 10 mm) and hip OA progression, focusing on asymmetrical OA distribution in patients over 65 years old.

MATERIALS AND METHODS: A retrospective analysis was conducted from a database of 1672 full-length standing X-rays. Patients under 65 years, with deformities, unilateral limb issues, or prosthetics, were excluded; therefore, the study group was composed of 220 patients. Tibial and femoral lengths were measured bilaterally, and hip OA severity was assessed using the Tönnis classification. Statistical analyses included Pearson's Chi-squared test and linear regression to explore correlations between LLD and OA distribution.

RESULTS: Among the sample, 18% showed an LLD ≥ 10 mm. A significant correlation was found between LLD and the asymmetrical distribution of hip OA ($p = 0.002$), with higher OA severity observed in the hypometric limb. Linear regression analysis suggested that each millimeter of LLD corresponded to a 0.74-unit change in OA severity difference between hips.

CONCLUSIONS: This study highlights a significant association between moderate LLD and contralateral hip OA in the elderly, emphasizing the biomechanical impact of asymmetrical [...]

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Authorship, titles and open access as drivers of citation performance in orthopaedics: a scientometric analysis.

Migliorini F, Vaishya R, Rivera F, Eschweiler J, Kobbe P, Betsch M, Oliva F, Maffulli N

BACKGROUND: Bibliometric analyses are increasingly used to explore how scientific knowledge is created, disseminated, and perceived. In orthopaedics, research output has expanded rapidly over the past decade, yet the factors determining whether an article achieves wide visibility and scholarly impact remain poorly understood. Beyond the inherent quality of a study, elements such as authorship patterns, title construction, and open access (OA) availability may play an essential role in shaping citation performance. However, evidence in this field is still limited and sometimes contradictory, highlighting the need for large-scale, field-specific analyses.

METHODS: Orthopaedic publications from 2010 to 2020 were identified in Scopus using the keyword 'orthopaedic'. After duplicate removal, 97,806 unique articles were included with complete data on authorship, titles, citation counts, study design, and OA status. Citation rates were normalised per year since publication. Associations between bibliographic features and citation performance were assessed using multiple linear regression, while differences across title styles and study designs were evaluated with comparative statistical testing. Exploratory modelling was performed to identify combinations of authorship and title characteristics linked to the highest predicted citation rates.

RESULTS: Larger author teams were associa [...]

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Interpreting nonsignificant findings: Reassessing the safety of self-locking standalone cages in octogenarian patients undergoing ACDF.

Abudayeh A, Fishchenko I

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A novel, simple, and affordable technique for arthroscopic soft-tissue tenodesis of the long head of the biceps tendon.

Fossati C, Ioppoli A, Ravaglia R, Vaja F, Menon A, Randelli PS

BACKGROUND: The purpose of this study was to describe a novel, simple, and implant-free arthroscopic soft-tissue tenodesis technique of the long head of the biceps tendon (LHBT) to the rotator cuff using an absorbable monofilament suture and to evaluate its clinical and functional outcomes at a minimum 1-year follow-up.

METHODS: A retrospective case series of 23 patients (mean age 58.0 ± 7.8 years) who underwent arthroscopic rotator cuff repair with concomitant LHBT soft-tissue tenodesis between June 2021 and June 2023 was analyzed. Functional outcomes were assessed using Constant-Murley Score (CMS), American Shoulder and Elbow Surgeons (ASES) score, Single Assessment Numeric Evaluation (SANE), visual analog scale (VAS) for pain, and the Long Head of Biceps (LHB) score. Supination strength was measured with a handheld dynamometer and compared with the contralateral side. Incidence of Popeye deformity and tenderness over the bicipital groove were recorded.

RESULTS: Objective Popeye deformity was observed in 13% of patients, with subjective concern reported by only 4.3%. Supination strength and LHB scores were similar to the contralateral side (means of 104.65 versus 104.64 N; LHB 93.7 versus 94.6 points). The mean CMS and ASES scores were 90.3 ± 12.4 and 89.6 ± 15.9 points, respectively. The SANE score averaged 87.4 ± 20.9 , and the VAS for pain was low (1.65 ± 2.59 cm).

CONCL [...]

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ORIF of three- and four-part proximal humeral fractures: a retrospective study of long-term clinical and complication outcomes of two-plate fixation methods.

Conteduca J, Gasbarra E, Rausa I, Casto A, Valentino L, Solarino G, Meccariello L, Rollo G

BACKGROUND: The aim of this study was to compare clinical outcomes and complication rates during long-term follow-up (≥ 6 years) after treatment for three- and four-part proximal humeral fractures using two different types of locking plates.

MATERIALS AND METHODS: A total of 113 patients with three- and four-part proximal humeral fractures who underwent surgery between September 2012 and January 2019 were enrolled retrospectively. Data for 49 patients [9 males, 40 females; mean age [standard deviation] (SD) 68.9 (5.8) years] treated with a PGR (intrauma) plate (group A) and 51 patients [10 males, 41 females; mean age (SD): 66.0 (7.5) years] treated with a PHILOS humeral plate (PHP, group B) were available at the last follow-up. The mean follow-up periods in groups A and B were 10.8 and 10.2 years, respectively, with a minimum follow-up of 6 years. At the final follow-up evaluation, functional outcomes were assessed using the Oxford Shoulder Scale (OSS), Simple Shoulder Test (SST), and Disabilities of the Arm, Shoulder and Hand (DASH) questionnaires. X-ray evaluation was also performed, and complications were recorded.

RESULTS: The mean OSS score in the PGR and PHP groups at follow-up was 42.9 and 39.1, respectively ($p > 0.05$). The SST score (PGR: 7.5 ± 2.0 ; PHP: 7.3 ± 4.0) and DASH score (PGR: 22.1 ± 5.5 ; PHP: 20.5 ± 4.0) were similar in both groups ($p > 0.05$). In our serie [...]

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Incidence rates and treatment of the transcervical fracture of the neck of femur in Italy: is total hip arthroplasty an increasingly preferred approach? A population study on trends between 2001 and 2023 based on 1,120,770 hospital discharge records.

Ciminello E, Romanini E, Venosa M, Cazzato G, Tucci G, Boniforti F, Carpanese L, Cuccu A et al.

INTRODUCTION: Transcervical femoral neck fractures (TFNFs) are among the most devastating fragility fractures in the elderly. TFNF are associated with excess 1-year mortality rates ranging from 15% to 30%. Treatments include conservative methods, internal fixation, and arthroplasty (partial or total hip arthroplasty). This study aims to analyze the changes in incidences of TFNF in the Italian population between 2001 and 2023 and the evolution of the choices of treatment.

MATERIALS AND METHODS: Using hospital discharge record (HDR) data from 2001 to 2023, records with ICD9-CM codes for femoral neck fractures (820.0 and 820.1) among diagnoses were selected and categorized into four treatment groups: total arthroplasty, partial arthroplasty, fixation, and conservative. Time series were analyzed with stratification by sex and age.

RESULTS: The extracted data included 1,120,724 records of TFNFs, with 871,161 cases treated surgically (total or partial arthroplasty or internal fixation) and 249,563 treated conservatively; the average patient age was 79.1 years, with a higher proportion of women (72.8%). Partial hip arthroplasty was the preferred treatment overall. For younger patients, in the age classes < 45 and 45-54 years, fixation was the most chosen treatment. Over time, the use of the conservative treatment decreased from 27.5% in 2001 to 14.6% of cases in 2023. The use of par [...]

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Does latissimus dorsi tendon transfer provide a normal scapular rhythm and external rotation strength in posterosuperior massive irreparable rotator cuff tears? A kinematic analysis in a retrospective cohort.

Calvo C, Fiumana G, Brigo A, Ruggiero ED, Bonfatti R, Donà A, Micheloni GM, Giorgini A et al.

BACKGROUND: Posterosuperior massive irreparable rotator cuff tears (PMIRT) are rare and disabling conditions. When conservative treatment fails, latissimus dorsi tendon transfer (LDTT) is a viable surgical option for symptom relief in carefully selected patients. However, its effectiveness in restoring glenohumeral function and its influence on scapulothoracic rhythm remain subjects of ongoing debate.

PURPOSE: The purpose of this study was to evaluate the clinical outcomes of LDTT and assess its impact on scapulothoracic rhythm using kinematic and electromyographic analysis.

MATERIAL AND METHODS: A total of 18 patients with PMIRT underwent LDTT. Functional scores consisted of Constant-Murley score (CMS), America Shoulder and Elbow Surgeons (ASES) score, and Quick Disabilities of the Arm, Shoulder, and Hand (QuickDASH) score. Electromyography (EMG) activity of the latissimus dorsi muscle was evaluated in conjunction with three-dimensional (3D) kinematic tracking system. Differences in external rotation strength were compared with and without active adduction.

RESULTS: The mean age was 55.9 years (range 40-69 years), with a mean follow-up of 20.4 months (range 6-39 months). At final follow-up, the mean CMS was 68.2 (95% CI 64.9-71.5), ASES was 76.9 (95% CI 66.1-87.6), and QuickDASH was 17.6 (95% CI 9.5-25.6). A significant difference in external rotation strength was observed [...]

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Is there a need for exploration in pulseless supracondylar fractures of the humerus? A systematic review and individual patient data meta-analysis.

Haoyang C, Chen JG, Lai H, Lim AKS, Tan SHS, Hui JHP

BACKGROUND: Different approaches have been proposed to treat patients with pulseless supracondylar humeral fractures (SHF). We aim to analyze the current outcomes of patients who have undergone different treatment options for pulseless SHF, namely watchful waiting versus urgent surgical exploration.

METHODS: Electronic databases including PubMed, Embase, and Cochrane Library were explored. Our study included patients from all age groups but only included English-language articles. Single case reports and case studies with adequate description of patient population, injury, and outcomes were included. An individual patient data meta-analysis was done to evaluate key outcomes of surgical exploration versus no surgical exploration in pulseless patients with SHF.

RESULTS: Overall, the data for 1070 individual patients from a total of 48 studies were included. Patients with pulseless SHF with open fractures have a higher probability of requiring vascular intervention ($p < 0.001$) as they are more likely to have disrupted arteries ($p = 0.050$) and more likely to require vascular repairs ($p < 0.001$) than patients with closed fractures. Similarly, patients with pulseless SHF with pucker sign ($p = 0.003$) and ecchymosis ($p = 0.002$) were more likely to undergo surgical exploration. However, the neurological status had no relation to them undergoing surgical exploration ($p = 0.382$), and al [...]

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Increasing complexity and plateauing outcomes in AO type C distal humerus fractures: a 50-year institutional case series.

Hönck K, Wenzelburger P, Eibinger N, Sadoghi P, Puchwein P

BACKGROUND: Distal humerus fractures in adults are rare but complex injuries, particularly AO type C3 fractures, often involving comminution, instability, and osteoporotic bone. Demographic trends indicate rising incidence, especially among elderly patients, with increasing functional demands challenging modern osteosynthesis. Open reduction and internal fixation (ORIF) with bicolumnar plating remains the preferred reconstructive approach, although complications remain substantial. This study aimed to evaluate trends in fracture complexity, patient demographics, and functional outcomes over five decades, focusing on the most recent cohort (2009-2018, series E) and comparing with historical cohorts (series A-D, 1969-2008).

METHODS: This retrospective analysis included five consecutive 10-year institutional cohorts (series A: n = 43; B: n = 29; C: n = 47; D: n = 58) of 233 patients with 235 supradiacondylar distal humerus fractures (AO type C1-3) treated with ORIF. Functional outcome parameters (Jupiter and Cassebaum Scores) and demographic data were available for all cohorts; additional parameters (range of motion (ROM), QuickDASH Score) were available for series D and E, Mayo Elbow Performance Score (MEPS), Short Form (SF)-36, and strength were assessed specifically in series E (n = 56; follow-up n = 21). Cross-cohort comparisons of categorical variables used chi-square tests; [...]

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Magnetic resonance imaging-based fat infiltration grading improves reparability prediction in massive rotator cuff tears.

Hu Q, Zhang G, Wang D, Tung TH, Ding J, Zhou X

PURPOSE: This study aims to investigate the extent of fat infiltration (FI) in the rotator cuff (RC) muscles in more medial slices from the Y-shaped view (Y-view) on shoulder magnetic resonance imaging (MRI) and assess whether these slices provide a more accurate prediction of the reparability of massive RC tears (massive RCTs).

METHODS: The retrospective study included 57 patients with massive RCTs who successfully underwent arthroscopic surgery between 1 January 2023 and 31 December 2023. Patients were categorized into two groups: the irreparable group and the repairable group. All patients underwent shoulder MRI covering the region from the acromion to the medial border of the scapula in oblique coronal and oblique sagittal orientations. The FI stage of the RC was assessed across three different views to determine which view most effectively predicts the reparability of patients with massive RCTs.

RESULTS: The FI stage of the infraspinatus (IS) muscle across three distinct views demonstrated a significant correlation with the reparability of massive RCTs. The FI stage was more severe in the irreparable group. Receiver operating characteristic (ROC) analysis revealed that the area under the curve for the 1/2 scapula view (0.737) and the most severe view (0.745) exceeded that of the traditional Y-view (0.644). Paired-sample ROC curve analysis revealed a significant difference [...]

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Mid-term clinical and functional outcomes after reverse shoulder arthroplasty with latissimus dorsi transfer.

Colombini AG, Rab P, Macken AA, Soares MN, Kimmeyer M, Shirinskiy IJ, Popescu IA, Lafosse L et al.

BACKGROUND: Although reverse total shoulder arthroplasty (rTSA) with concomitant latissimus dorsi transfer (LDT) has been shown to effectively treat external rotation (ER) deficits, there are limited data regarding its outcomes with modern implants and its impact on activities of daily living (ADLs) requiring ER. The purpose of this study was to assess the mid-term clinical and radiographic outcomes of rTSA with concomitant isolated LDT in patients with an ER lag sign and posterior rotator cuff deficiency.

METHODS: This retrospective cohort study with prospective follow-up included consecutive patients who underwent rTSA with concomitant isolated LDT between 2010 and 2022 with a minimum follow-up of 2 years. Primary outcomes included the resolution of ER lag sign, active ER, and the Activities of Daily Living and External Rotation (ADLER) score. Secondary outcomes included the Auto-Constant Score (CS), Subjective Shoulder Value (SSV), activities of daily living requiring internal rotation (ADLIR) score, visual analog scale (VAS) for pain, and radiographic analysis of standardized radiographs.

RESULTS: In total, 32 procedures in 32 patients were identified. Of these, 22 procedures in 22 patients (68% female, 72.9 ± 8.4 years at surgery) were available for follow-up at 4.8 ± 2.2 years postoperatively (response rate

73%). The ER lag sign resolved in 95.5% of patients, the active [...]

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Partial versus full revision in cementless single-stage exchange for hip periprosthetic joint infection: a multicenter comparative study with 10-year survivorship analysis.

Mu W, Jarusriwanna A, Xu B, Brown SA, Zhang X, Dawes A, Parvizi J, Cao L

BACKGROUND: Periprosthetic joint infection (PJI) is one of the most challenging complications following total hip arthroplasty (THA). Traditional management typically involves full revision to ensure comprehensive infection eradication. However, for patients with well-fixed implants, partial revision in a single-stage cementless approach may provide a viable alternative, potentially preserving bone stock and reducing operative time. The relative efficacy of this approach compared with full revision remains to be fully explored. This multicenter study aims to determine whether partial revision in cementless single-stage exchange surgery offers comparable infection control outcomes to full revision for select patients with well-fixed implants.

METHODS: We conducted a retrospective, multicenter cohort study involving 226 patients who underwent cementless single-stage exchange hip arthroplasty for PJI between 1 October 2013 and 31 July 2022. Patients were divided into partial revision (n = 61) and full revision (n = 165) groups. The primary outcome was treatment success, defined as the absence of clinical symptoms and signs of infection at a minimum follow-up of 2 years.

RESULTS: The success rates were 77.0% for the partial revision group and 80.0% for the full revision group, with no significant difference (p = 0.629). Both groups showed comparable 10-year survival rates for ove [...]

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Methodological Concern Regarding the Multivariable Analysis of Rare Implant Breakage Events.

Yang Y, Liu J

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Author's Response Re: Methodological Concern Regarding the Multivariable Analysis of Rare Implant Breakage Events.

Van Rysselberghe NL, Gardner MJ

[PubMed](#) · [DOI](#)

Treatment Effects of Bisphosphonates Compared With Oxandrolone on Burn-Induced Bone Loss and Recovery in Mice.

Tseng MC, Wise PM, Osipov B, Song H, Lin YQ, Yeh J, Gallardo O, Wang A et al.

OBJECTIVE: Severe burn injuries can lead to systemic bone loss, increasing the risk of secondary fragility fractures. Evaluating treatments aimed at mitigating post-traumatic systemic bone loss is critically important for preventing secondary fractures in patients with burn. This study evaluated the efficacy of bisphosphonate (alendronate), oxandrolone, and vitamin D/calcium supplementation in mitigating post-traumatic bone loss and promoting recovery of bone in a murine model of burn-induced systemic bone loss, in which a full-thickness thermal skin burn involving approximately 15% total body surface area was used to model severe burn injury .

DESIGN: Male C57BL/6J mice were divided into 5 groups: control, burn with no treatment, burn + alendronate, burn + vitamin D/calcium, and burn + oxandrolone. Full thickness burn injuries were induced on the back of mice by submerging in 65°C water for 20 seconds; debridement and matrix application were performed 48 hours after injury, and treatments were administered starting on the day of injury.

OUTCOME MEASURES: Whole-body and site-specific bone mineral density (BMD) and bone mineral content (BMC) were assessed weekly using dual-energy X-ray absorptiometry. Micro-computed tomography and 3-point bending were performed at weeks 2 and 6 postinjury to assess bone microstructure and mechanical properties.

RESULTS: Untreated burned mice [...]

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Preoperative Proton Pump Inhibitor Use Was Associated With Increased Risk of Revision Surgery and Impaired Bone Healing After Pelvic Fracture Fixation: A Retrospective Cohort Study.

Wang J, McGowan E, Horani K, Hagedorn JC

OBJECTIVES: The aim of this study was to evaluate the association between preoperative proton pump inhibitor (PPI) use and postoperative outcomes after pelvic fracture fixation.

DESIGN: Retrospective cohort study.

SETTING: TriNetX Research Network, a federated database of electronic medical records from approximately 103 health care organizations.

PATIENT SELECTION CRITERIA: Adult patients of 18 years of age and older who underwent surgical fixation of pelvic fractures between 2000 and 2023 were identified. All operatively managed pelvic fracture types were included, encompassing OTA/AO classifications 61 A-C; however, specific fracture classification was not available within the database. Patients were stratified into cohorts based on preoperative PPI use.

OUTCOME MEASURES AND COMPARISONS: Postoperative outcomes, including sepsis, revision surgery, hardware removal, nonunion, and malunion, were compared between patients with and without preoperative PPI use at 90 days and 2 years after surgery.

RESULTS: Before matching, 2648 patients were included in the preoperative PPI cohort and 7655 patients in the control cohort. After propensity score matching, 2403 patients remained in each cohort. The mean age was 44.0 ± 17.3 years in the PPI cohort and 43.7 ± 17.0 years in the control cohort, with 37.3% and 38.6% female patients, respectively. At 90 days postoperatively, preopera [...]

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Periprosthetic Femur Fractures: The Screw-In-Washer Technique for Bicortical Small-Fragment Screw Fixation Through Large-Fragment Plates. A Technical Trick and Case Series.

Mahmoud AN, Horwitz DS

When treating periprosthetic femur fractures, bicortical screw fixation around a medullary-filling femoral stem may present a technical challenge, potentially leading surgeons to opt for unicortical locking screws or cerclage wires. These solutions may not achieve desirable stability, particularly in osteoporotic bone. In this technical note, we describe a simple approach using bicortical small-fragment screws-in-washers through large-fragment plates to facilitate secure bicortical screw passage around the stem. We also present preliminary outcomes from a case series in which this technique was used.

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Factors Affecting Outcomes of Hindfoot Fusion Nails for Acute Injury: A Multicenter Study: Erratum.

Kim E, Tornetta P, Carlson JB, Schultz A, Wireman G, Ollivere B, Zheng A, Spitler C et al.

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Modified CT Sagittal Angle Measure: Accuracy in Determining Hip Stability for Posterior Wall Acetabulum Fractures and Observer Reliability and Agreement Among Inexperienced Observers.

Jaebon T, Harrington C, Khan U, Clark K, Kaplan S, McGlone P, Panarello N, Talwar S et al.

OBJECTIVES: The aim of this study was to describe modifications to the original CT sagittal angle measure (SAM) for predicting hip stability after posterior wall acetabular fractures (PWF), and to assess modified SAM (mSAM) accuracy and resident agreement and accuracy with the mSAM relative to examination under anesthesia (EUA).

DESIGN: Retrospective review of prospectively collected data.

SETTING: Academic Level 2 Trauma Center.

INTERVENTION: Results of a modification of the sagittal CT based PWF measurement (mSAM) and gold standard EUA were collected and evaluated.

PATIENT SELECTION CRITERIA: Patients with PWFs (AO/OTA 62A.1) requiring an EUA from June 2017 through July 2025 were included.

OUTCOME MEASURES AND COMPARISONS: The accuracy of mSAMs by the lead author and residents in predicting hip stability was quantified by sensitivity, specificity, positive predictive value, and negative predictive value using EUA, performed by fellowship trained orthopaedic surgeons, as the reference standard. A receiver operating characteristic curve analysis was used to determine whether mSAM reproduced the published SAM threshold angles most closely associated with hip stability, using EUA as the reference standard. Modified SAM reliability and agreement was assessed with intraclass correlation coefficients (ICC) after measurements in 2 rounds, separated by 1 month. Resident agreement [...]

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A Dual-Implant Approach to Vancouver C Femur Fractures: Improved Outcomes With Retrograde Nail and Lateral Plate Fixation Compared With Lateral Locked Plating.

Page BE, Shaath MK, Rechter GR, Nasir BA, Baker TJ, Yawman JP, Jacobs E, Langford JR et al.

OBJECTIVES: To compare lateral locked plating (LLP) with a combined treatment approach using a retrograde intramedullary nail alongside a minimally invasive lateral locked periprosthetic plating system (Smith & Nephew; Memphis, TN) in the management of Vancouver C periprosthetic femoral fractures.

DESIGN: Retrospective chart review.

SETTING: Single, academic, Level-1 Trauma center.

PATIENT SELECTION CRITERIA: All adult patients who underwent fixation of a Vancouver C periprosthetic femur fracture (AO/OTA Type 32C [IVC/D]) with a retrograde intramedullary nail and periprosthetic plating system (NP Group) or LLP (LLP group) between 2019 and 2025 with follow-up to union, reoperation, or a minimum of 1 year were included. Patients in the NP group were recommended to fully bear weight immediately while patients in the LLP group were recommended to remain non-weight-bearing for 8 weeks.

OUTCOME MEASURES AND COMPARISONS: The primary outcome was fracture union. Secondary outcomes included implant failure, infection, and alignment immediately postoperatively and at the final follow-up. Primary and secondary outcome measures were compared between the NP and LLP groups.

RESULTS: A total of 52 patients were included. In total, 31 in the NP group (68% female, mean age 79 years (range, 67-99)) and 21 in the LLP group (67% female, 69 years, range: 53-85, $P = 0.54$ for sex, $P = 0.26$ for age [...])

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Evaluation of an AI-Driven Risk Stratification System for Clinical Deterioration in Patients With Geriatric Hip Fracture.

Lin CQ, Ruiz M, McLellan M, Gardner MJ

OBJECTIVES: To determine whether an artificial intelligence-driven Clinical Deterioration Index (CDI) could identify geriatric patients with hip fracture at risk of early postoperative complications and to establish an orthopaedic-specific cutoff that identified patients at risk of deterioration.

DESIGN: Retrospective cohort study.

SETTING: Single Level I trauma center.

PATIENT SELECTION CRITERIA: Patients younger than 65 years of age who underwent fixation of OTA/AO 31-A/B/C hip fractures between June 2022 and December 2023 were included using ICD-10 codes S72.0-, S72.1-,

and S72.2-. Exclusion criteria were pathologic fracture, revision surgery, or absence of 30-day follow-up. The CDI uses 31 clinical measures to generate a score from 0 to 100 reflecting risk of rapid deterioration. Patients were separated into 2 groups based on peak CDI in the immediate 48-hour postoperative period: CDI ≥ 65 and CDI < 65 . This cutoff of 65 was established by previous institutional validation.

OUTCOME MEASURES AND COMPARISONS: Primary outcomes were in-hospital complications (cognitive changes, cardiac complications, DVT/PE, etc). Secondary outcomes included length of stay, ambulation distance, and 1-year mortality. Outcomes were compared between CDI groups.

RESULTS: Of 197 patients (age range 65-98, 66% female), 15 (7.6%) exceeded the baseline CDI threshold of 65. This group had higher comp [...]

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Blood Culture Testing in Fracture-related Infections: Low Yield and Lack of Concordance With Deep Tissue Pathogens.

Merrell LA, Solasz SJ, Ganta A, Konda SR, Egol KA

OBJECTIVES: To assess the concordance between blood culture isolates and intraoperative deep tissue cultures in patients with confirmed fracture-related infection (FRI).

DESIGN: Retrospective Cohort Study.

SETTING: Academic Medical Center.

PATIENT SELECTION CRITERIA: This Institutional Review Board-approved study included patients 18 years and older diagnosed with a confirmed FRI according to the FRI Consensus Group criteria who, at the time of irrigation and debridement (I&D), underwent deep tissue culture (TC) as well as concurrent blood culture (BC) testing (in the Emergency Department or inpatient setting). The decision to perform BC testing was left to the discretion of the initial treating providers at the time of this presentation.

OUTCOME MEASURES AND COMPARISONS: Microbiological data were reviewed from the electronic medical record. Infections were classified as monomicrobial (either gram-positive or gram-negative), polymicrobial, or culture negative. Pathogen concordance between blood and intraoperative tissue cultures was analyzed.

RESULTS: Eighty four patients were included with both intraoperative deep TC and concurrent BC. This cohort had a mean age of 56.2 ± 20.3 years and consisted of 33 women (39.3%). BC were never ordered by the orthopaedic surgeon. Microbial analysis of deep tissue specimens identified 29 gram-positive infections, 18 gram-negative infect [...]

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NonTobacco Nicotine Dependence Is Associated With Increased Fracture Risk in Adolescents and Young Adults.

Zhang J, Meydan Y, Chen B, Barsi J

OBJECTIVES: The aim of this study was to evaluate whether nontobacco nicotine dependence (NTND) in adolescents and young adults is associated with an increased risk of new-onset fractures over a medium-term and long-term follow-up.

DESIGN: retrospective cohort query of TriNetX network.

SETTING: Multicenter study conducted using deidentified electronic health records from the TriNetX network of participating health care organizations.

PATIENT SELECTION CRITERIA: Adolescents and young adults (≤ 21 years) were included if they had a diagnosis of NTND between June 1, 2005, and June 1, 2020. Exclusions: any previous tobacco use, environmental tobacco exposure, tobacco use disorder, preexisting bone/joint pathology (eg, osteoporosis, osteoarthritis, osteogenesis imperfecta, and malignant neoplasms of bone/cartilage), or previous fracture. Patients with NTND were propensity score matched 1:1 with controls without any documented nicotine or tobacco use.

OUTCOME MEASURES AND COMPARISONS: The primary outcome was incident fracture after the most recent eligible clinical encounter documented between June 1, 2005, and June 1, 2020. Fracture risk was assessed over a fixed 5-year period after the index date and across the longest available follow-up in TriNetX. Risk ratios (RR)

and 95% confidence intervals (CI) were calculated. Kaplan-Meier survival analysis was used to estimate hazard rat [...]

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Outcomes of Primary Repair of Heel Pad Degloving Injuries.

Roddy E, Dalstrom D, Sangeorzan B, Benirschke S, Vu S, Keys K, Firoozabadi R

OBJECTIVES: The aim of this study was to report the outcomes of patients with heel pad degloving injuries, and to identify patient, injury, or treatment factors associated with successful repair.

DESIGN: Retrospective cohort study.

SETTING: Single level 1 academic trauma center.

PATIENT SELECTION CRITERIA: All patients admitted with an acute heel pad degloving injury between 2005 and 2024 were included.

OUTCOME MEASURES AND COMPARISONS: The primary outcome was successful repair of a heel pad degloving injury. Univariate analysis was performed to examine risk factors for primary repair failure.

RESULTS: A total of 58 patients with acute heel pad degloving injuries were included, of whom 50 were male with average age of 35 years (range 6-69, SD 17). Forty patients were treated with an attempt at primary repair, 3 were treated primarily with a flap, and 15 were treated with primary amputation. Primary repair was successful in 15 patients (38%). Repair method (suture alone, augmentation with k-wires, and incisional wound vac) was not associated with repair success ($P > 0.05$ for all). Of the patients with failed primary repair, 6 underwent secondary amputation, whereas 19 underwent attempted salvage with a flap (13), skin graft (5), or dermal substitute (1). Of the 19 with attempted salvage, 10 (53%) had complications including persistent ulceration, infection, and/or wound br [...]

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What is the Rate of Occult Vessel Abnormalities in High-Energy Tibial Plateau Fractures (OTA/AO 41)?

Zhang D, Eurich J, Pitz-Goncalves L, Schimizzi G, Erdman MK, Christiano AV

OBJECTIVES: The aim of this study was to determine the rate of occult vascular abnormalities in high-energy tibial plateau fractures using computed tomography angiography (CTA).

DESIGN: Prospective Cohort Study.

SETTING: Single Academic Level I Trauma Center.

PATIENT SELECTION CRITERIA: Adult patients with a tibial plateau (OTA/AO 41) fracture, without clinical indication for CTA, that underwent knee-spanning external fixation were prospectively enrolled.

OUTCOME MEASURES AND COMPARISONS: After external fixation, patients underwent CTA to evaluate fracture morphology and the vascular tree. Fisher exact test was used to determine correlations between arterial abnormalities and wound complications in patients with ≥ 90 days of follow-up.

RESULTS: A total of 26 patients with 27 high-energy tibial plateau fractures were enrolled. Patients were predominantly male ($n = 16$, 61.5%) with a mean age of 45.1 ± 13.3 years. All patients had a blunt injury mechanism resulting in mostly closed ($n = 25$, 92.6%), bicondylar ($n = 22$, 81.5%) tibial plateau fractures. CTA demonstrated nonatherosclerosis vascular abnormalities in 3 patients (11.5%). Two had disrupted flow in the popliteal artery, and 1 had disrupted flow in the anterior tibial artery. No patient required vascular intervention. Atherosclerosis was appreciated in 6 (26.9%) patients. One of 3 patients (33%) with nonatherosclerosis [...]

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BMP-2 Enhanced Bone Regeneration Using Clinically Available Grafting Strategies in a Chronic Caprine Tibial Defect Model.

Piuzzi NS, Luangphakdy V, Ramos MS, Zhang C, Pluhar GE, Carlson CS, Bechtold JE, Muschler GF

OBJECTIVES: The aim of this study was to assess bone grafting strategies in the chronic caprine tibial defect (CCTD) model.

METHODS: Design: Preclinical investigation. Setting: Chronic caprine tibial defect model. Patient Selection Criteria: Female Spanish-Boer goats, 4 to 6 years old. Outcome Measures and Comparisons: Primary outcome was total bone volume measured by MicroCT at 12 weeks. Secondary outcomes were radial percent bone volume and moment angle of inertia on MicroCT, and cortical bone bridging on radiograph.

RESULTS: Animals with MCA + BMP-2 (4542.1 mm³, P = 0.026) and MCA + BMP-2+BMA (3633.1 mm³, P = 0.006) grafts showed higher median tBV within the central 2.5 cm of the defect compared with animals with MCA + BMA grafts (12.3 mm³). Median tBV within the central 2.5 cm did not differ significantly between animals grafted with ACB (486.8 mm³) and those with MCA + BMP-2 (4542.1 mm³) or MCA + BMA + BMP-2 (3633.1 mm³) grafts. Adding unprocessed BMA to either MCA (MCA: 2.7 mm³ vs. MCA + BMA: 12.3 mm³) or MCA + BMP-2 (MCA + BMP-2: 4542.1 mm³ vs. MCA + BMA + BMP-2: 3633.1 mm³) did not significantly increase median tBV within the central 2.5 cm.

CONCLUSIONS: In the CCTD model, BMP-2 had a positive effect on tBV, comparable with and trending greater than ACB. This study supports using a composite of MCA + BMP-2 for large segmental bone defects, whereas the [...]

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A Tibial Osteotomy of the Medial Collateral Ligament Insertion to Improve Visualization of the Medial Tibial Plateau: A Cadaveric Study.

Barth KA, Tijerina RR, Amin A, Novak AA, Mayer EN, Ismaily SK, Munz JW, Choo AM et al.

OBJECTIVES: To compare visualization of the medial tibial plateau articular surface using 3 surgical approaches: medial collateral ligament (MCL) split, femoral epicondyle osteotomy of the MCL origin, and tibial osteotomy of the MCL insertion.

METHODS: Seven matched pairs of fresh-frozen adult cadaveric lower extremity specimens (14 knee specimens total) with no evidence of previous surgery were used. Through a medial approach to the proximal tibia, the MCL split technique was performed followed by a submeniscal arthrotomy. Standardized distraction of the joint was applied using a universal distractor. A three-dimensional digitizer probe was used to map the visible articular surface. These steps were repeated for the femoral osteotomy on the same extremity as the MCL split and for the tibial osteotomy on the contralateral matched pair extremity. The tibial osteotomy preserved the meniscomfemoral portion of the deep MCL, posterior oblique ligament, and proximal meniscocapsular attachments. Visible articular surface area (mm²) for each of the 3 techniques was calculated from the mapped data points and compared among techniques.

RESULTS: The mean age of the specimens was 77.1 ± 7.2 years. MCL split technique visualized 87.2 ± 28.4 mm² of articular surface. The medial femoral epicondyle osteotomy provided 614.7 ± 217.6 mm² of visualization (677% more than MCL split, P < 0.001) [...]

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Biomechanical comparison of innovative suture techniques for repair of Achilles tendon rupture in a porcine flexor tendon model.*Weigand L, Liebrand B, Scheible S, Meffert R, Hoelscher-Doht S*

INTRODUCTION: In recent years, evidence-based progressive rehabilitation protocols following the surgical treatment of Achilles tendon rupture have been shown to be safe and appear superior to more restrictive treatment regimens. These protocols mainly rely on early mobilization and weight bearing. To enable this during postoperative rehabilitation, a biomechanically stable surgical treatment must be ensured. Different solutions for suturing ruptured Achilles tendons are currently favored by orthopedic surgeons and have been investigated in the past, including promising further developments of traditional approaches such as the combination of pretension and the Bunnell technique, as well as minimally invasive or percutaneous techniques using systems like the Arthrex PARS. However, the best solution in this regard remains unclear.

METHODS: We analyzed the biomechanical stability of four different suture techniques using porcine deep flexor tendons as a biomechanical model for human Achilles tendons. The tendons were harvested, prepared and sutured according to their respective group (A: Bunnell, B: Bunnell + pretension, C: PARS-to-PARS, D: PARS + anchor). The four groups (n = 12) underwent progressive, cyclic, uniaxial tensile loading in the biomechanical testing machine (ZwickRoell Z020).

RESULTS: Group D (PARS + anchor) showed the highest maximum force at ultimate failure, r [...]

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Comparative outcomes of delta-on-delta and delta-on-highly cross-linked polyethylene bearings in total hip arthroplasty: a nationwide cohort study.*Lee YK, Kang D, Kim H, Kang E, Cho J, Park JW*

INTRODUCTION: The purpose of this study was (1) to evaluate the national trends of Delta ceramic and highly crosslinked polyethylene (HXLPE) liner in total hip arthroplasty (THA), and (2) to compare survivorship of Delta ceramic versus HXLPE liner when used with a Delta ceramic head.

MATERIALS AND METHODS: Using the K-NHIS database, we identified all patients who underwent primary THA using Delta ceramic head in South Korea between 2003 and 2013 (n = 16,250). Cumulative incidences of dislocation, periprosthetic joint infection (PJI), and periprosthetic fracture were evaluated according to bearing type: Delta-on-Delta ceramic (DoD) and Delta-on-HXLPE (DoHXLPE). Survivorship for revisions was calculated to determine differences. Multivariable Cox proportional hazard regression models were used for comparisons, and propensity score matching was additionally performed as a sensitivity analysis.

RESULTS: A total of 15,299 patients underwent THA with Delta ceramic head between 2003 and 2013. Delta liner was used in 14,255 patients (93.2%) and HXLPE in 1,044 patients (6.8%). Utilization of Delta liner has noticeably increased since 2007. The DoHXLPE group had a higher risk of PJI (aHR = 2.10, 95% CI = 1.29-3.41) compared to the DoD group within 1 year. There were no significant differences in risk of dislocation (aHR = 1.41, 95% CI = 0.97-2.05) and periprosthetic fracture (aHR = 0.9 [...])

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A comparative study on the clinical efficacy and complications of proximal femoral nail antirotation and femoral neck system in the treatment of basicervical femoral neck fracture (AO/OTA 31-B3).*Jiang X, Lin D, Huang Y, Liu J, Lin F*

OBJECTIVES: Basicervical femoral neck fractures (FNFs) are prone to internal fixation failure owing to their inherent biomechanical instability. This study compares the efficacy of femoral neck system (FNS) and proximal femoral nail anti-rotation (PFNA) in treating basicervical FNF.

MATERIALS AND METHODS: A retrospective analysis was conducted on 99 patients with basicervical FNF admitted to fuzhou second general hospital from January 2020 to June 2024. Patients were divided into two groups based on the internal fixation method used: the FNS group (53 cases, average age 63.83 ± 18.80 years) and the PFNA group (46 cases, average age 69.78 ± 16.65 years). Main observation indicators included reduction quality and internal fixation-related complications, while secondary indicators included tip-apex distance, surgical time, occult blood loss, wound length, postoperative hospital stay, mortality, and Harris hip score.

RESULTS: The average follow-up time was 24.03 ± 9.4 months. Preoperative data were comparable between groups ($P > 0.05$). The PFNA group had a significantly lower overall incidence of implant-related complications than the FNS group (2.17% vs. 16.98%, $P = 0.035$), and the distribution of implant-related complication subtypes also differed significantly between groups ($P = 0.042$). No significant differences were observed in fracture reduction quality, tip-apex distance, [...]

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Management and outcome of displaced acetabular fractures in the oldest-old: a retrospective analysis of orif and acute total hip arthroplasty.

Mamoutakis I, Horst K, Alabdulrahman H, Groven RVM, Bläsius FM, Hildebrand F, Berk T, Bolierakis E

PURPOSE: The optimal surgical management of geriatric acetabular fractures remains controversial. This study compared short-term outcomes following open reduction and internal fixation (ORIF) or acute total hip arthroplasty (THA) with or without additional osteosynthesis (THA $\pm$ ORIF) and evaluated the impact of a new treatment algorithm (standard operative procedure, SOP) that favors the use of THA in elderly and medically comorbid patients.

METHODS: In this retrospective cohort study, patients aged ≥ 70 years with isolated trauma-associated unilateral displaced acetabular fractures who underwent surgical treatment at a single Level I trauma center between January 2016-March 2019 (old SOP) and January 2022-December 2025 (new SOP) were included. Patients underwent either ORIF or THA $\pm$ ORIF. The transitional implementation phase of the new SOP was excluded. The primary outcome was postoperative time to gait mobilization. Secondary outcomes included pain at discharge, functional mobility, complications, mortality, quality of fracture reduction, and conversion to THA following initial ORIF. Because treatment allocation followed different institutional algorithms across the study periods, the study primarily evaluates an evolving treatment strategy rather than directly comparing ORIF and THA.

RESULTS: Sixty patients (median age 82 years, IQR 78-87; median Charlson Comorbidity Inde [...])

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Race, ethnicity, and language are underreported in hip fracture CPGs.

Mulakaluri A, Snyder EM, Young B, Kamal RN, Shapiro LM

INTRODUCTION: Clinical Practice Guidelines (CPGs) are systematically developed recommendations based on peer-reviewed literature that guide patient care. Considering the impact of sociodemographic factors on patient outcomes, this study aims (1) to review the racial, ethnic, and linguistic representation of patients in literature utilized to inform the CPGs for hip fractures in older adults and (2) to assess the use of linguistically- and culturally-adapted Patient Reported Outcome Measures (PROM) in these studies.

MATERIALS AND METHODS: Publications listed under the American Academy of Orthopaedic Surgeons (AAOS) CPGs for the Management of Hip Fractures in Older Adults were identified and extracted. Variables extracted included: title, author, publication year, inclusion/exclusion criteria, country/ies of study, language of study, patient language, race and ethnicity, patient language proficiency, outcome measures utilized, translation or cultural-adaptation of outcome measures, and clinical trial registration page. The extracted variables were reported descriptively.

RESULTS: Of the 196 unique articles included, 12/196 (6.12%) reported race/ethnicity data and 9/196 (4.59%) reported patient language, of which 6/9 (66.7%) reported patient language proficiency. Of the studies that reported patient language or language proficiency, 8/9 (88.9%) specified this data in their inclu [...]

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Level-dependent changes in cervical multifidus composition following anterior cervical discectomy and fusion: bridging quantitative MRI and clinical perspectives.

Folkerts T, Tripathi R, Caffard T, Verna B, Zhu J, Sama AA, Cammisa FP, Hughes AP

INTRODUCTION: Fatty infiltration (FI) of the cervical paraspinal muscles is associated with greater disease severity and less favorable outcomes in degenerative cervical spine disease. However, longitudinal data on whether muscle composition changes following anterior cervical discectomy and fusion (ACDF) are scarce. This study aimed to evaluate changes in FI and functional cross-sectional area (fCSA) of the cervical multifidus and rotatores after ACDF and to assess whether these changes differ across surgical levels.

MATERIALS AND METHODS: This single-center retrospective study analyzed patients who underwent ACDF for degenerative cervical stenosis between 2015 and 2018. Patients were included if both preoperative and postoperative cervical spine MRI (obtained 1-5 years after surgery) were available. The multifidus and rotatores colli were segmented bilaterally on T2-weighted axial images. Measurements were categorized by position within the fusion construct as upper (most cranial fused vertebra), middle (intervening vertebrae), and lower (most caudal fused vertebra) levels. FI and fCSA were quantified using Otsu thresholding. P-values were adjusted for multiple testing using the Benjamini-Hochberg procedure.

RESULTS: Thirty-five patients met inclusion criteria (median age 58 years, 40% female, 1-4 fused levels). Bilateral FI decreased significantly at the lower level (52.1 [...])

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Superior reduction quality without improved outcomes: open versus closed reduction for displaced femoral neck fractures in adults under 60 years.

Chi JE, Liu CH, Lo YH, Lai CY, Chen IJ, Yu YH

INTRODUCTION: The optimal reduction strategy for displaced femoral neck fractures (DFNFs) in non-elderly adults remains controversial. Although open reduction may improve radiographic alignment, its impact on outcomes remains unclear.

MATERIALS AND METHODS: Adults aged < 60 years with DFNFs who were treated with internal fixation between 2017 and 2023 were retrospectively reviewed. Patients were categorized into closed and open reduction groups. Reduction quality was assessed using the Garden alignment index and the Haidukewych reduction criteria. Multivariate analyses were performed for the overall cohort and patients with Pauwels type III fractures. Outcomes included non-union, osteonecrosis, femoral neck shortening, and major reoperations.

RESULTS: In total, 98 patients were included, among whom 83 and 15 underwent closed and open reduction, respectively. Although open reduction achieved a higher rate of excellent reduction (53.3% vs. 16.9%, $p = 0.012$), it was not associated with lower rates of non-union or osteonecrosis. In the overall cohort, age was associated with non-union (odds ratio [OR], 1.09; $p = 0.024$), whereas poorer reduction quality was associated with major reoperation (fair: OR 7.03, $p = 0.012$; poor: OR 47.6, $p = 0.007$). In patients with Pauwels type III fractures, poorer reduction quality predicted conversion to arthroplasty (fair: OR 9.84, $p = 0.014$), and [...]

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Longer hospital stay but similar one-year mortality and readmission after combined proximal humerus and proximal femur fragility fractures: a Propensity-Score-Matched Cohort Study.

Murphy S, Kellett C, Roberts D, McGurk C, Ahern E, O'Loughlin P

BACKGROUND: Fragility Proximal Humerus (PH) fractures are common and often associated with significant morbidity and mortality. Unlike Proximal Femur (PF) fractures, PH fractures are often omitted from established orthogeriatric multidisciplinary team (MDT) pathways. Combined Proximal Humerus and Proximal Femur fractures (PH&PF) are rare. Combined injuries would be expected to result in worse outcomes compared to isolated PH fractures due to greater injury burden. This study assessed outcomes between isolated PH and combined PH&PF fractures, exploring whether differences in care pathways may contribute to outcome.

METHODS: A retrospective propensity score-matched cohort study was performed over a 2-year period identifying patients sustaining isolated fragility PH fractures and over a 5-year period identifying patients sustaining combined

PH&PF fractures secondary to low-energy trauma. Propensity score matching was performed using age, sex, comorbidity burden (as defined by Charlson Comorbidity Index (CCI)), mobility status, cognitive impairment and fracture pattern. Primary outcome was 1-year mortality. Secondary outcomes included 1-year readmission and length of stay (LoS). Mortality was analysed using Firth logistic regression, readmission using clustered logistic regression, and LoS using Gamma regression with cluster-robust standard errors.

RESULTS: Sixty-five patients we [...]

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Are kneeling stress radiography and the posterior sag sign associated with subjective flexion instability after posterior-stabilized total knee arthroplasty?

Akın M, Çalar Ö, Tokgözoğlu AM, Atilla B

INTRODUCTION: Flexion instability is a common but underdiagnosed cause of patient dissatisfaction after posterior-stabilised total knee arthroplasty (TKA). Standard clinical tests have limited reliability for this condition. We investigated whether kneeling stress radiography and selected physical examination findings can help identify it.

MATERIALS AND METHODS: We retrospectively reviewed 145 posterior-stabilised TKA knees in 119 patients, divided into two groups by the presence of subjective knee instability. The posterior sag sign, 90° anterior drawer test, and kneeling stress radiographic parameters-particularly the change in tibiofemoral translation under load (Δ -TPFA)-were compared between groups. Post-hoc analyses included ROC curve analysis, diagnostic accuracy with Wilson 95% confidence intervals, and multivariable logistic regression adjusted for age, sex, BMI, follow-up duration, implant type, and lumbar pathology.

RESULTS: Thirty knees (20.7%) reported subjective instability. Mean Δ -TPFA was 1.40 ± 3.75 mm in the unstable group versus -1.17 ± 3.28 mm in the stable group ($p = 0.001$; AUC 0.68, 95% CI 0.56-0.80) and remained an independent predictor on multivariable analysis (OR 1.30 per mm, 95% CI 1.11-1.52, $p < 0.001$). The posterior sag sign had a specificity of 82.6% (95% CI 74.6-88.4%) and was independently associated with instability (OR 3.26, 95% CI 1.54-6.91, [...])

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Pelvic injuries in elderly trauma patients: epidemiology, injury characteristics, in-hospital care and factors influencing mortality.

Enocson A, Lundin A, Granqvist M

BACKGROUND: Pelvic and acetabular fractures are increasing in incidence in elderly patients. Whether the fracture contributes independently to mortality, or merely accompanies the factors that drive it, remains contested, and large national data on elderly trauma patients are limited. The primary aim of this study was to determine factors influencing 30-day mortality in elderly trauma patients, with particular focus on pelvic injuries. Secondary aims included to analyse the influence of these injuries on in-hospital care and discharge destination.

METHODS: All patients aged ≥ 60 years admitted as trauma alerts in Sweden between 1 January 2022 and 31 December 2024 were identified through the Swedish Trauma Registry (SweTrau). The primary outcome was 30-day mortality. The association between pelvic injury (pelvic or acetabular fracture) and 30-day mortality was assessed using multivariable logistic regression, adjusting for gender, ASA class, NISS, and head injury.

RESULTS: Of 11,624 patients (median age 76 years, 39.7% males), 776 (6.7%) sustained a pelvic injury. Patients with pelvic injury more frequently sustained high-energy trauma, had a higher median NISS (17 vs. 11), longer hospital length of stay (6 vs. 3 days), higher ICU admission rate (30.9% vs. 20.0%), and was less frequent discharged direct home (34.0% vs. 55.8%; all $p < 0.001$). Crude 30-day mortality was 16.1% vs [...]

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Pain, quality of life, and functional outcomes of total hip arthroplasty in female patients with crowe type III-IV developmental dysplasia of hip: retrospective cohort study.

Güzel N, Genç AS, Gümü NE, Takın O, Güzel A

INTRODUCTION: The aim of this study was to investigate physical and psychosocial recovery including pain and quality of life measures, as well as lower limb functional outcomes in patients with Crowe type III-IV developmental dysplasia of the hip (DDH) who underwent total hip arthroplasty (THA) with subtrochanteric shortening osteotomy.

MATERIALS AND METHODS: This study was conducted with a total of 45 female patients aged 32-65 with Crowe type III-IV DDH. Pre-op 30-second chair-stand Test (30-s CST), 6-minute walk test (6MWT) and Visual Analogue Scale (VAS) were assessed clinically, whereas Harris Hip Score (HHS), EuroQol 5-Dimension 5-Level (EQ-5D-5 L) scale, Forgotten Joint Score-12 (FJS-12), Hip disability and Osteoarthritis Outcome Score (HOOS), and SF-36 Quality of Life Scale (SF-36) scores were measured and recorded based on patient self reports. At least two years after the surgery, the same measurements were repeated in the patients.

RESULTS: At the mid-term follow-up, a significant increase was observed in the 30-s CST and 6MWT distance compared with the pre-op period ($p < 0.01$). Furthermore, statistically significant differences were observed in the VAS, HHS, EQ-5D-5 L, FJS-12, HOOS and SF-36 scores ($p < 0.01$).

CONCLUSIONS: An evaluation after THA with subtrochanteric shortening osteotomy in female patients revealed a significant improvement in functional performance [...]

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Bioabsorbable vs. titanium screws in first metatarsophalangeal joint arthrodesis: a pilot randomized controlled trial with 2-year follow-up.

Partio N, Mattila V, Waris V, Mäenpää H

INTRODUCTION: To assess the feasibility and preliminary clinical outcomes of bioabsorbable screw fixation in first metatarsophalangeal joint arthrodesis compared to conventional titanium screw fixation.

METHODS: In this prospective pilot randomized controlled trial, 30 patients were allocated 1:1 to bioabsorbable or titanium cannulated lag screw fixation. Arthrodesis was performed using a cup-and-cone technique at two Finnish hospitals. Full weight-bearing in an orthopaedic shoe was allowed after six weeks. Outcomes included AOFAS and VA-FAS FA-VAS scores, union rates, and complications assessed at 3, 6, 12, and 24 months.

RESULTS: Both groups showed significant functional improvement with no between-group differences. At 24 months, mean AOFAS scores were 89.1 (SD 2.7) in the titanium group and 90.0 (SD 0.0) in the bioabsorbable group ($p = 0.23$). Mean FA-VAS total scores (sum of pain, function, and complaints subscales) were 1948 (SE 59.4) in the titanium group and 1914 (SE 61.4) in the bioabsorbable group ($p = 0.655$). Union rate was 90% overall; nonunion occurred in one titanium and two bioabsorbable patients, all requiring revision. No implant-related complications or hardware removal surgeries were recorded. Degradation of bioabsorbable screws caused no adverse clinical or radiographic reactions.

CONCLUSIONS: Bioabsorbable screws appear safe and feasible in first metatars [...]

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Intraoperative patellofemoral load varies between total knee replacement and patellofemoral joint resurfacing, and across TKR implant systems.

Phillipson L, Naybour J, Watson A, London N, Barrett D

INTRODUCTION: Patellofemoral joint loading is influenced by implant design and surgical execution during knee replacement. Altered patellofemoral mechanics have been implicated in postoperative symptoms, yet the magnitude and variability of intraoperative load changes across different replacement procedures and implant systems remain incompletely characterised. The aim of this registry study was to describe and quantify intraoperative patellofemoral joint load changes following total knee replacement (TKR) and patellofemoral joint resurfacing (PFJR), focusing on cumulative and compartment-specific load redistribution.

MATERIALS AND METHODS: This observational registry study analysed prospectively collected intraoperative patellofemoral joint load measurements from 291 knee replacement procedures. Load was recorded in medial, lateral, superior, and inferior compartments before and after trial component implantation. Absolute and percentage changes in compartmental and cumulative load were calculated. Descriptive analyses examined the distribution and variability of load change across procedure type and implant system, with implant comparisons restricted to TKR procedures. No formal hypothesis testing was pre-specified, and no adjustment for potential confounding variables was undertaken.

RESULTS: Cumulative patellofemoral load change demonstrated substantial inter-individual v [...]

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Cortical hypertrophies in cementless short stem total hip arthroplasty in young patients under 60 years and elderly patients over 75 years-analysis of outcome and risk factors for cortical hypertrophies.

Schwarz PM, Stützle S, Stadler C, Holzbauer M, Pisecky L, Gotterbarm T, Luger M

BACKGROUND: Cortical hypertrophy (CH) is a common radiological finding associated with short-stem total hip arthroplasty (THA), but its clinical significance remains unclear. This study evaluates differences in outcomes between young patients under 60 years of age and elderly patients over 75 years of age who underwent cementless short stem THA. The aim was to evaluate if there are differences in these patients depending on the presence of CH.

METHODS: A retrospective analysis of 208 short-stem THAs performed between 2014 and 2017 was conducted. Patients were divided into younger (< 60 years, n = 119) and older (> 75 years, n = 89) groups. Clinical outcomes, including Harris Hip Scores (HHS) and Oxford Hip Scores (OHS), were compared between patients with and without CH. Multivariate regression analysis was performed to identify risk factors for CH, focusing on delta hip offset, canal fill index, and stem alignment.

RESULTS: Younger patients with CH demonstrated significantly better HHS (96.1 ± 7 vs. 90.8 ± 14.3 , $p = 0.010$) and OHS (45.8 ± 5.2 vs. 42.9 ± 8.8 , $p = 0.028$) compared to those without CH. In older patients, the presence of CH did not significantly impact outcomes. Regression analysis identified younger age ($p = 0.012$), higher Canal Fill Index III ($p = 0.047$), and stem alignment ($p = 0.049$) as significant predictors of CH.

CONCLUSION: Cortical hypertrophy is associ [...]

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Comparative effectiveness of single-stage versus two-stage revision for hip prosthetic joint infection: emulation of the INFORM target trial using electronic health record data.

Zhang C, Li Y, Su C, Shen Y, Lu S

OBJECTIVE: To estimate the comparative effectiveness of single-stage versus two-stage revision for hip prosthetic joint infection (PJI) in Chinese tertiary hospitals by emulating the INFORM randomized trial using the target trial emulation framework.

DESIGN: Retrospective cohort study emulating a target trial, reported per the TARGET guideline. The protocol and analysis plan were written before analysis but were not prospectively registered.

SETTING: Three tertiary hospitals in China. Index revision surgeries were performed between 1 January 2018 and 30 June 2022, with follow-up data locked on 31 December 2023, ensuring a minimum potential follow-up of 18 months for every patient.

PARTICIPANTS: 754 adults with confirmed hip PJI undergoing revision surgery (280 single-stage; 474 two-stage), identified from electronic health records and confirmed by manual chart review. Treatment groups reflected the intended surgical strategy documented before or at the index operation.

MAIN OUTCOME MEASURES: Primary outcome: Harris Hip Score (HHS) at 18 months.

SECONDARY OUTCOMES: surgically treated reinfection, reoperation, mortality, and complications. The primary analysis used inverse probability of treatment weighting (IPTW) on the full cohort with multiple imputation of missing covariate and outcome data; 1:1 propensity score (PS) matching (280 pairs) was confirmatory. Patients who di [...]

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Cemented dual mobility component in newly implanted porous tantalum shells for severe acetabular defects: medium-term outcomes from a high-volume single-surgeon series.

Loppini M, Di Matteo V, Bulgarelli A, La Camera F, Chiappetta K, Morengi E, Grappiolo G

BACKGROUND: The management of severe acetabular bone defects remains challenging due to extensive bone loss and poor soft tissue quality during revision total hip arthroplasty (rTHA). The aim of this study was to assess clinical and radiological outcomes of the use of dual mobility components (DMC) cemented into a newly implanted porous tantalum (PT) acetabular shell during acetabular rTHA.

METHODS: Patients who underwent cementation of a DMC into a newly implanted PT cup between 2014 and 2020 were included. Bone defects were classified according to the Paprosky classification. The primary outcome was to determine the reintervention-free survival rate. The secondary outcome was clinical and radiographic assessment at last follow-up.

RESULTS: Thirty-six patients (36 hips), with an average age of 67.9 years (range, 46-90) were included. Paprosky type III defects (61.1%) were more common than type II defects (38.9%), with type IIIa (36.1%) and IIIb (25.0%) predominating. Median follow-up was 67.3 months (range; 17-109). The dislocation rate was 8.3% (n = 3). Four (11.1%) of 36 hips underwent acetabular component revision: 2 (5.5%) for infections, 1 (2.7%) for aseptic loosening, and 1 (2.7%) for dislocation. Cup survival rate was 89.08% (CI 69.54% to 96.38%) at 60 months. The Harris hip score (HHS) improved from 31.8 ± 12.0 to 87.0 ± 14.2 points ($p < 0.001$). The vertical and horiz [...]

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From load-bearing to load-sharing: contact-driven transitions in locked plate constructs.

Hollensteiner M, Schindler M, Greinwald M, Mühling M, Baumeister D, Augat P

Locked plate osteosynthesis functions as an internal fixator, yet load-induced plate-bone and fragment-fragment contacts - key transitions from load-bearing to load-sharing - remain poorly characterized. This study quantified their effects on construct stiffness, plate strain, and interfragmentary motion under axial loading. Twenty-seven configurations were tested using epoxy-glass surrogate bone fixed with titanium LCP plates, varying fracture gap (3-9 mm), working length (49-121 mm), and plate-bone distance (0-3 mm). Digital image correlation measured strain and motion. Contact events were identified manually from characteristic transitions in the force-displacement response, supported by corresponding changes in plate-surface strain progression. Geometric parameters dominated initial stiffness (125-442 N/mm), decreasing with plate elevation and working length in elevated setups. Within the investigated load range up to 600 N, 16/27 configurations showed non-linear transitions associated with contact events: fragment-fragment contact caused pronounced stiffness increases and strain stagnation at the fracture gap, whereas plate-bone contact was associated with local plate-surface strain stagnation at the location overlying the contact while strain at the fracture gap continued to increase. Interfragmentary motion stagnated after contact, confirming contact-dependent shifts in [...]

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Factors associated with periprosthetic joint infection following total knee arthroplasty: an updated systematic review and meta-analysis.

Gao H, Deng Y, Ma Y, Li X, Zhang F, Mei Y

BACKGROUND: This study aimed to systematically evaluate factors associated with periprosthetic joint infection (PJI) following total knee arthroplasty (TKA), and thereby to provide evidence-based references for clinical prevention and perioperative risk stratification.

METHODS: Computerized searches were conducted in the following databases from their inception until May 26, 2025: PubMed, Web of Science, Embase, the Cochrane Library, CINAHL, China National Knowledge Infrastructure, Wanfang Database, Chinese Scientific Journal Database, and Chinese Biomedical Literature Database. Two researchers independently screened the literature, extracted data, and assessed study quality. The methodological quality was assessed using the Newcastle-Ottawa Scale. Quantitative synthesis was performed when at least two studies reported comparable exposure definitions and sufficient comparator information; otherwise, narrative synthesis was used. Review Manager 5.4 software was used for the primary analysis. This study is registered on PROSPERO (CRD420251079339).

RESULTS: A total of 25 observational studies were included in the qualitative synthesis, of which 24 contributed to the primary quantitative synthesis. Quantitatively pooled factors associated with PJI included male sex (OR = 1.39, 95% CI 1.27-1.51), BMI ≥ 30 kg/m² (OR = 5.72, 95% CI 2.65-12.36), prolonged operative time, type 2 diabetes [...]

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Early surgery of trochanteric femoral fractures in patients on direct oral anticoagulants (DOACs) reduces the length of stay (LOS) and the 1-year mortality rates without increasing the risk for revision surgery: a matched-pair analysis.

Weihs V, Laggner R, Humenberger M, Duma A, Frossard M, Hajdu S

BACKGROUND: It remains unclear whether early surgical fixation of trochanteric femoral fractures in patients with direct oral anticoagulation (DOAC) therapy has an impact on the long-term outcome.

METHODS: A retrospective cohort study on patients with trochanteric femoral fractures and DOAC therapy between 2016 and 2024 was conducted. Patients were divided into two groups according to their time to surgery and matched with age and ASA score as covariates. The primary outcome of interest was the impact of early surgery on the long-term outcome.

RESULTS: Of 233 included patients, 102 patients were assigned in each group after propensity score matching. Patients with early surgery had lower in-hospital complication rates (16.0% vs. 26.3%; $p = 0.077$) and a significantly shorter median length of stay (LOS) (11.5 days (IQR 6) vs. 15 days (IQR 6); $p < 0.001$). No difference in the rates of revision surgery (9.8% vs. 5.9%; $p = 0.298$) was observed. Kaplan-Meier curves revealed a significant difference in the mortality rates during the follow-up period ($p = 0.0057$) with lower in-hospital (2.0% vs. 6.9%; $p = 0.088$), 30-day (3.9% vs. 9.8%; $p = 0.097$) and 1-year mortality rates (20.6% vs. 31.4%; $p = 0.079$).

CONCLUSION: Early surgical fixation of trochanteric femoral fractures in patients on DOAC therapy leads to a significant reduction in the LOS without increasing the risk for peri- and [...]

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Platform-specific learning curve and early tibial radiographic agreement of ROSA®-assisted medial unicompartmental knee arthroplasty in an experienced high-volume surgeon: a consecutive case series.

Petrillo S, Ardiri D, Marullo M, Romagnoli S

PURPOSE: To evaluate the platform-specific learning curve and early tibial radiographic agreement of imageless ROSA®-assisted medial unicompartmental knee arthroplasty (UKA), with short-term clinical findings assessed exploratorily.

METHODS: This retrospective consecutive case series included 30 patients who underwent ROSA®-assisted medial UKA with the Persona® Partial Knee implant between January and April 2026 at a high-volume arthroplasty referral centre. A continuous learning-curve cumulative summation (LC-CUSUM) analysis of total surgical duration was performed against an independently defined surgeon-specific historical benchmark derived from institutional operative-register skin-to-skin times for manual UKA performed by the same surgeon (20 ± 4 min). Adequate performance was defined as a mean duration of 25 min, inadequate performance as 30 min, and the reference standard deviation as 4 min. Type I and type II error targets were 0.05 and 0.10, respectively; the decision boundary was calibrated by Monte Carlo simulation over a 30-case monitoring horizon. Robotic time and a supportive comparison of cases 1-5 versus 6-30 were secondary efficiency analyses. Postoperative radiographic measurements were performed by an observer blinded to the planned and robot-validated ROSA® values. Agreement was quantified using Bland-Altman bias and 95% limits of agreement.

RESULTS: Mean [...]

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Cartilage disruption patterns associated with percutaneous intramedullary screw fixation for metatarsal fractures: a cadaveric study.

Keith B, Burke J, Hynes K, Baker J, Huh J

PURPOSE: Intramedullary screw fixation is an emerging technique for operative metatarsal fractures, yet the extent and location of articular cartilage disruption from retrograde screw insertion remain undefined. This study quantifies cartilage surface disruption and maps injury locations in the metatarsal head following screw placement.

METHODS: Fourteen fresh-frozen cadaveric feet (56 lesser metatarsals) underwent standardized retrograde intramedullary screw fixation using 3.6 mm screws. Following dissection, the total articular surface area and cartilage defect were measured to calculate the percentage of disruption. The cartilage surface was divided into

quadrants (dorsal-to-plantar) and thirds (medial-to-lateral) to determine screw trajectory.

RESULTS: Mean cartilage disruption increased progressively from the 2nd to 5th metatarsals: 10.17%, 11.2%, 14.93%, and 17.43%, respectively. All screws were located within the middle third in the medial-to-lateral plane. In the dorsal-to-plantar plane, the majority of screws were positioned in the 2nd and 3rd quadrants. No screws were placed in the medial or lateral thirds.

CONCLUSION: Retrograde intramedullary screw fixation results in measurable cartilage disruption, with a tendency for screw placement in the middle third in the medial-to-lateral plane and 2nd and 3rd dorsal-to-plantar quadrants. While the relative cartilage surf [...]

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Locking plate fixation for proximal humerus fractures: are four proximal screws adequate? A clinical and radiological analysis of 105 consecutive cases.

Michelitsch C, Stillhard PF, Zindel C, Sommer C, Acklin YP

INTRODUCTION: Proximal humerus fractures are common injuries, and achieving stable fixation is essential to prevent complications such as varus collapse and screw perforation. While optimal screw positioning has been emphasized biomechanically, clinical evidence regarding the required number of screws in the humeral head, particularly when minimally invasive aiming devices limit placement to four screws, remains limited. This study evaluated whether fixation with four cranial screws provides adequate clinical and radiological stability.

MATERIALS AND METHODS: A retrospective cohort study was conducted including 105 patients with displaced proximal humerus fractures treated with plate osteosynthesis. Patients were grouped according to the number of proximal screws used: four screws ($n = 58$) or more than four screws ($n = 47$). The primary outcome was the Constant-Murley Score. Secondary outcomes included secondary screw perforation, head-shaft angle maintenance, restoration of medial cortical support, and avascular necrosis. Mean follow-up was 18 ± 6 months.

RESULTS: Functional outcomes were comparable between groups, with Constant-Murley Scores of 78 ± 7 in the four-screw group and 73 ± 13 in the multi-screw group ($p = 0.23$). Rates of screw perforation (8.6% vs. 10.6%; $p = 0.72$) and postoperative head-shaft angles were likewise similar, and no significant alignment change occur [...]

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Nighttime intramedullary nailing of femoral fractures is not associated with worse outcomes: a 13-year Level I trauma center cohort.

Abialevich A, Benkovich V, Osinsky V, Adiniaev Y, Tzaytlin I, Heinemann Y, Acker A

BACKGROUND: Whether nighttime orthopedic trauma surgery adversely affects outcomes remains controversial. Evidence specifically addressing intramedullary fixation across femoral fracture locations remains limited. This study evaluated whether nighttime surgery was associated with a higher rate of recorded postoperative complications than daytime surgery.

METHODS: This retrospective observational cohort included adult patients with nonperiprosthetic femoral fractures treated with intramedullary fixation between January 2010 and January 2023 at a single Level I trauma center. Of 311 screened records, 23 were excluded because the patient was younger than 18 years ($n = 17$) or the fracture was periprosthetic ($n = 6$), leaving 288 procedures (242 daytime and 46 nighttime). The primary outcome was any recorded postoperative complication during available follow-up. Secondary outcomes included radiographic abnormality, delayed union (> 24 weeks), nonunion, revision surgery, and mortality. Surgeon experience was categorized as senior surgeon or resident. Exploratory fracture-location subgroup analyses and multivariable logistic regression were performed.

RESULTS: Nighttime patients were younger (35.1 ± 19.1 vs. 46.6 ± 24.6 years, $p < 0.001$), more frequently male (82.6% vs. 64.0%, $p = 0.014$), more often injured by a high-energy mechanism (80.4% vs. 56.6%, $p = 0.002$), and underwent surgery [...]

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Patient-reported outcomes after lateral unicompartmental and total knee arthroplasty: a propensity score-matched cohort study.

INTRODUCTION: Comparative evidence on patient-reported outcomes (PROs) after lateral unicompartmental knee arthroplasty (UKA) and total knee arthroplasty (TKA) remains limited. We compared PROs between cohorts matched for demographic characteristics and baseline knee function.

MATERIALS AND METHODS: Prospectively collected single-center data were retrospectively analyzed. Lateral UKA was performed for isolated lateral-compartment osteoarthritis and TKA predominantly for multicompartmental disease. One-to-one propensity score matching incorporated age and body mass index, exact matching on sex, and restrictions on age and preoperative Oxford Knee Score (OKS), yielding 31 pairs. Postoperative OKS was the primary outcome. Secondary outcomes included OKS improvement, Forgotten Joint Score (FJS), University of California, Los Angeles (UCLA) activity score, OKS minimal clinically important difference (MCID) achievement, and FJS patient acceptable symptom state (PASS) attainment.

RESULTS: All matching covariates achieved acceptable balance (absolute standardized mean differences < 0.20). Postoperative OKS did not differ significantly (UKA, 38.9 ± 7.0 ; TKA, 39.8 ± 7.5 ; mean paired difference [TKA minus UKA], 0.94; 95% confidence interval [CI], - 2.57 to 4.44; $p = 0.590$). No significant differences were detected for OKS improvement (difference, 0.77; 95% CI, - 2.62 to 4.17; $p = 0.644$) [...]

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Pre- and postoperative 4D CT analysis of thumb CMC joint kinematics after first metacarpal extension osteotomy.

Guebeli A, Honigmann P, Dobbe JGG, Keller M

INTRODUCTION: First metacarpal extension (Wilson) osteotomy combined with modified Brunelli ligamentoplasty is performed for early thumb carpometacarpal (CMC1) osteoarthritis to improve pain and function. The purpose of this study was to quantify in vivo changes in CMC1 joint contact distribution and kinematics after this combined procedure and to determine whether postoperative patterns resemble those of healthy volunteers. We hypothesized that the procedure would reduce cumulative palmar contact during opposition-retroposition.

MATERIALS AND METHODS: In a prospective single-center observational cohort study, six adults with early CMC1 osteoarthritis (Eaton-Littler I-II) underwent standardized preoperative and 6-month postoperative four-dimensional computed tomography (4D CT) of the CMC1 joint during active opposition-retroposition. Five healthy volunteers served as reference with a single 4D CT scan. First metacarpal-trapezium motion was registered across frames. Joint contact was estimated using a 2.0 mm proximity surrogate on the trapezial surface. The primary outcome was change in cumulative palmar contact area. Secondary outcomes included change in dorsoradial contact, temporal variability of contact curves, and contact-centroid path length and mean position. Within-subject pre- versus postoperative comparisons were performed; healthy scans provided reference patterns. [...]

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Minimal kyphotic progression following pediatric vertebral fractures: implications for reduced radiographic surveillance.

Wegmann S, Lenz M, Babasiz T, Eysel P, Mueller LP, Weber M

INTRODUCTION: Pediatric spinal fractures are rare injuries (14.24 per 100,000 per year), representing only 0.6-3% of all spinal injuries. While most stable compression fractures are managed conservatively, concerns exist regarding progressive vertebral collapse and kyphotic deformity. Unlike extensive adult literature, pediatric-specific data on radiographic progression following conservative management remain limited, creating uncertainty about the clinical utility of routine follow-up imaging in this radiosensitive population.

MATERIALS AND METHODS: Retrospective single-centre cohort study of 54 pediatric patients (mean age 12.3 years) with traumatic vertebral body fractures diagnosed between 2012 and 2024. Kyphosis angles were measured using Cobb methodology at baseline and follow-up intervals (1, 3, and 6 weeks). The primary outcome was the change in kyphotic angle; secondary outcomes included the identification of predictors of progression. Paired t-tests and multivariable linear regression analysis were performed.

RESULTS: Kyphosis angles remained essentially unchanged across all follow-up intervals ($\Delta\text{FU1} + 0.19^\circ$, $\Delta\text{FU2} + 1.27^\circ$, $\Delta\text{FU3} - 0.14^\circ$), with no statistically significant differences detected ($p > 0.05$ at all timepoints). No clinical or

radiographic factors predicted progression of kyphosis. Only three patients (5.6%) required treatment escalation. Subgroup analyses [...]

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Correction: Different sagittal cutting plane orientations do not affect posterior tibial slope in open wedge high tibial osteotomy: a Sawbone model study.

Gurbanov E, Cochard B, Bauer DE, Miozzari HH, Tscholl PM

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Resident autonomy in orthopedic trauma: independent femoral IMN by senior residents yields comparable outcomes to fellowship-trained surgeons.

Balziano S, Nesher G, Ben Nun N, Segev T, Freidlander A, Prat D

INTRODUCTION: Intramedullary nailing (IMN) of femoral shaft fractures is a fundamental cornerstone procedure in orthopedic trauma and an important benchmark of resident surgical competency. While independent resident operating is essential for skill development, its safety in complex trauma cases remains debated. This study evaluated whether senior residents can safely perform femoral IMN without direct supervision compared with fellowship-trained trauma surgeons.

MATERIALS AND METHODS: A retrospective cohort analysis was performed of adult patients undergoing IMN for isolated femoral shaft fractures between 2015 and 2023 at a level I trauma center. Cases were grouped by primary surgeon: senior residents (postgraduate year ≥ 4) versus attending trauma surgeons. Demographics, fracture characteristics, operative parameters, complications, and revision rates were compared. To minimize selection bias, 1:1 propensity score matching (PSM) was performed based on preoperative covariates including age, comorbidity, fracture type, injury mechanism, and severity.

RESULTS: A total of 207 patients met inclusion criteria; 150 (75 per group) were matched. Operative duration was longer in resident-led cases (median 120 vs. 91 min, $p = 0.003$). Postoperative hospital stay (5.0 vs. 6.0 days, $p = 0.674$) and weight-bearing protocols ($p = 0.218$) were similar. Overall complication rates did not dif [...]

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Racial and ethnic disparities in perceived health and barriers to care among adults with hip osteoarthritis: a national analysis of 16,000 + patients from the all of us program.

Khan ST, Devadiga SA, Emara AK, Brahmbhatt S, Elmenawi KA, Delaney PG, Piuze NS

OBJECTIVE: To explore racial and ethnic disparities in perceived health, healthcare access, and perceived barriers among adults with hip osteoarthritis (OA) in a large, nationally representative U.S.

METHODS: Cross-sectional analysis of 16,743 adults with hip OA from the All of Us Research Program. Participants self-identified as Black, Hispanic, or White. Outcomes included self-reported general, mental, physical, and social health, provider communication, and structural barriers to care. Between-group comparisons used Chi-square tests.

RESULTS: Black and Hispanic participants were younger, had lower income and education, and relied more on Medicaid than White participants (all $p < 0.001$). They reported higher rates of unaffordable deductibles, difficulty accessing follow-up care, insurance coverage issues, transportation challenges, and medication cost-related nonadherence (all $p < 0.001$) Table 3. Minority groups also more often experienced poor provider respect, trust, and communication. Across all health domains, Black and Hispanic participants reported poorer outcomes and greater emotional distress (all $p < 0.001$).

CONCLUSION: Significant racial and ethnic disparities exist in structural and perceptual barriers to care among adults with hip OA. These findings identify potentially modifiable barriers that warrant further study and may inform future efforts to improve equi [...]

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Postoperative periprosthetic fractures associated with compressive osseointegration: a systematic review of the Zimmer Biomet Compress® device.

The Zimmer Biomet Compress® Compliant Pre-Stress (CPS) device uses a compressive osseointegration technique for endoprosthetic fixation and provides an alternative to stemmed implants. Despite having benefits, such as bone stock retention, periprosthetic fractures continue to remain a concern. Therefore, this systematic review presents the prevalence, risk factors, and outcomes of the CPS device and periprosthetic fractures after treatment. Medical databases (PubMed, Embase, and Google Scholar) were searched to identify studies reporting complications related to the CPS device. Peer-reviewed studies with at least 2 years of follow-up for fracture rates, revision results, and implant survivorship were included. The primary data collected were fracture incidence and timing of complications, and the CPS device was compared with the traditional cemented stem. This review highlights important studies, including a cohort of 221 patients and pediatric data from 36 patients. Adult populations showed a periprosthetic fracture rate of 2.7-3.9%. A distinct temporal pattern emerged: the frequency of fractures was most pronounced within the first 2 years after surgery. In the pediatric context, spindle survivorship was 86.3% at 5-10 years, despite higher rates of mechanical complications. Notably, for most fracture cases, the bone-implant interface was radiographically stable, providing ease [...]

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Hip resurfacing arthroplasty at a minimum of 14 years: revision is concentrated among female patients with dysplasia and small components.

Lin YC, Chang CH, Hsieh PH, Huang YY, Chen DW, Lau NC

INTRODUCTION: Hip resurfacing arthroplasty (HRA) is a bone-preserving option for young, active patients. The influence of the underlying diagnosis on long-term outcome remains unclear, particularly the distinction between primary osteoarthritis (OA), developmental dysplasia of the hip (DDH) and non-OA conditions.

MATERIALS AND METHODS: We retrospectively reviewed 104 consecutive metal-on-metal HRAs in 101 patients (three bilateral) performed by one surgeon between 2006 and 2011, stratified into Primary OA (n = 50), DDH (n = 29) and Non-OA (n = 25: avascular necrosis [AVN] n = 15, inflammatory arthritis n = 5, post-traumatic arthritis n = 3, Legg-Calvé-Perthes disease n = 2). AVN was analysed additionally as an independent group, and exploratory analyses restricted to female patients and to cups ≤ 50 mm addressed the collinearity of dysplasia, sex and size. Survivorship used Kaplan-Meier estimation with revision as the endpoint.

RESULTS: Implant status was known for all 104 hips; the 91 unrevised assessed hips had a mean follow-up of 16.2 ± 1.5 years. Overall 15-year survivorship was 95.2%. Primary OA, Non-OA and AVN each showed 100%; DDH 82.8% (versus Primary OA p = 0.002, versus Non-OA p = 0.031). All five revisions occurred in female DDH patients with a cup ≤ 50 mm; DDH survivorship remained lower within female patients (p = 0.025) and within small cups (p = 0.006), with no [...]

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Elective arthroplasty on "high-risk" calendar dates: no evidence for worse outcomes in 43,000 procedures.

Walter N, Lowenberg DW, Heiss C, Lau EC, Rupp M

BACKGROUND: Concerns regarding the timing of elective surgery remain common among patients and may influence surgical scheduling. Superstition-associated calendar dates such as Friday the 13th are frequently perceived as "high-risk" time points, yet evidence regarding their impact on arthroplasty outcomes is lacking. This study investigated whether primary total hip arthroplasty (THA) and total knee arthroplasty (TKA) performed on these dates are associated with differences in revision risk, mortality, or procedure frequency.

METHODS: A nationwide retrospective cohort study was conducted using Medicare physician service records (2009-2019). Primary THA and TKA procedures were identified in a 5% sample of Medicare beneficiaries aged ≥ 65 years. Outcomes included aseptic revision, septic revision, and mortality. Competing-risk survival analyses (Fine-Gray) and multivariable Cox regression models adjusted for demographic, clinical, and socioeconomic covariates were applied. Procedure volumes were compared between superstition-associated and control dates.

RESULTS: A total of 945 arthroplasties were performed on Fridays falling on the 13th. No significant differences were observed in aseptic revision, septic revision, or mortality compared with control dates. For Friday the 13th, hazard ratios were 0.80 (95% CI 0.44-1.47) for aseptic revision, 0.83 (95% CI 0.31-2.25) for septic r [...]

Evaluation of a novel strap fixation method for biomechanical testing of sheep Achilles tendons.

Basci O, Aydemir S, Husemoglu RB, Çeltik M

BACKGROUND: Reliable fixation of tendon specimens is a major challenge in biomechanical testing, as slippage and tissue damage can reduce accuracy and reproducibility. Although various clamp- and suture-based fixation methods have been reported, there is still no widely accepted, practical, low-cost technique that preserves tendon integrity during testing. This study aimed to compare commonly used fixation techniques and to evaluate whether a novel strap-based fixation method provides a more stable and tissue-preserving alternative for biomechanical testing of sheep Achilles tendons.

METHODS: Forty fresh-frozen cadaveric sheep Achilles tendons obtained from a local butcher were used, and the specimens were stored at - 20 °C until testing. After controlled thawing at room temperature, all tendons were repaired using the Bunnell suture technique with No. 0 Vicryl. Four different proximal fixation methods were evaluated: (1) sutured fixation with strap, (2) direct clamp fixation, (3) Kocher clamp fixation, and (4) Krackow suture fixation. All specimens were mounted on a universal testing machine and subjected to a load-to-failure protocol at a constant loading rate of 10 mm/s. Maximum force (N), displacement at failure (mm), and mechanical failure mechanisms were recorded for each sample. Data were analyzed using the Kruskal-Wallis test with Bonferroni-corrected Mann-Whitney U test [...]

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Implementation and early results of same-day hip and knee replacement in a public hospital without prior same-day surgical experience, a feasibility study.

Jørgensen CC, Sveen TM, Johansen B, Sander M, Steen-Hansen C, Sundstrup MS, Rothe C, Lange KH

BACKGROUND: Hip and knee replacements are increasingly performed as same-day surgery (SDS). However, the feasibility of SDS pathways in departments without prior experience is unknown. We report on the implementation process, feasibility and early results of implementing a SDS protocol in a department without prior experience with same-day hip and knee replacement.

METHODS: The study design was based on Bowens feasibility study framework on implementation processes. Data were collected prospectively through patient reported questionnaires and medical records during, and from the local Department of Digitalisation and Analytics after implementation. Outcome data included discharge on day of surgery, 30-days primary-care contacts, 90-days readmissions and patient satisfaction.

RESULTS: Implementation was initiated as a dedicated fellowship from September 2023 to October 2024. Patients were admitted and discharged through a dedicated day-surgical department with transfer to the regular ward in case of admission. The first SDS patient underwent surgery the 4th of September 2023. By October 2024, 150 of 713 operated patients (21.0% CI:18.2-24.2) had SDS. Of these, same-day discharge was achieved in 125 (83.3% CI:76.6-88.5) patients of which 6 (4.0% CI:0.9-7.1) were readmitted within 90-days. From October 2024 to October 2025, 178 of 800 patients were planned for SDS (22.3% CI:19.5 [...])

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Relationship of foot anthropometry with Modified Pirani score during Ponseti correction of idiopathic congenital talipes equinovarus: a prospective observational study.

Suresh M, Pinto DJV, Rajasekharan P

BACKGROUND: The Ponseti method is the gold standard treatment for idiopathic congenital talipes equinovarus (CTEV). Clinical monitoring during correction primarily relies on the Modified Pirani score, which evaluates deformity severity based on clinical findings. However, objective quantitative assessment of morphological foot changes during treatment remains limited. This study aimed to evaluate the relationship between serial foot anthropometric measurements and Modified Pirani scores during Ponseti correction of idiopathic clubfoot.

METHODS: A prospective observational cohort study was conducted in 30 children with idiopathic unilateral CTEV undergoing Ponseti correction at a tertiary care center between June 2024 and February 2026. Five anthropometric parameters were measured weekly for eight weeks: great toe to midheel distance (GT-MH), little

toe to midheel distance (LT-MH), second toe to midheel distance (ST-MH), medial malleolus to midheel distance (MM-MH), and lateral malleolus to midheel distance (LM-MH). Modified Pirani scores were assessed simultaneously by two independent observers. Pearson correlation analysis and inter-observer reliability testing were performed.

RESULTS: Significant improvement was observed in all anthropometric parameters and Pirani scores over the study period ($p < 0.001$). Mean GT-MH increased from 80.73 ± 3.36 mm to 87.56 ± 3.41 mm, LT-MH f [...]

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Union outcomes and associated factors in adolescent scaphoid fractures.

Torun Ö, Girgin AB, Kaşıkçı M, Acar A, Çevik HB

PURPOSE: Scaphoid fractures (SF) in adolescents differ from adult fractures in fracture pattern, healing capacity, and skeletal maturity. Limited data exist on factors affecting union and healing time in this population. This study evaluated clinical, radiological, and treatment-related factors associated with union and healing time in adolescent SF.

METHODS: This study included 77 adolescents (aged 10-18 years) treated for SF. Chronological age, bone age, fracture characteristics, treatment modality, union status, and healing time were recorded. Fractures were classified by anatomical location, displacement, and time to diagnosis (< 6 weeks or ≥ 6 weeks). CT-based morphometric measurements, including scaphoid height, lateral intrascaphoid angle, and dorsal cortical angle, were obtained from initial computed tomography images. Functional outcomes were assessed using the QuickDASH and patient-rated wrist evaluation (PRWE) scores.

RESULTS: Union was achieved in 63 patients (81.8%), with a median union time of 8 weeks (range, 6-10 weeks). Union rates were highest in distal pole fractures (100%) and lowest in proximal pole fractures (33.3%). Non-displaced fractures had higher union rates than displaced fractures (92.5% vs. 56.5%), and fractures diagnosed within 6 weeks demonstrated better union outcomes. Surgical treatment, performed in 16.9% of patients, was associated with lower [...]

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The effect of surgeon volume on outcomes after total hip arthroplasty: a systematic review and meta-analysis.

Meissner N, Ortwig K, Ramos-Pascual S, Agu C, Stoeve J, Schrednitzki D, Rolvien T, Halder AM

PURPOSE: To synthesize, critically appraise and systematically review studies comparing outcomes after primary total hip arthroplasty (THA) performed by low- vs. high-volume surgeons, and to propose evidence-based threshold definitions for these categories.

METHODS: This review followed the PRISMA guidelines and was registered in PROSPERO. Medline and Embase were searched on March 24, 2025. Prospective and retrospective clinical studies were included if they reported THA thresholds for low- and high-volume surgeons. Articles not written in English or German were excluded. Outcomes were pooled and presented in forest plots. Methodological quality was assessed.

RESULTS: Of 872 identified articles, 18 were included, reporting on 796,061 patients undergoing THA. Methodological quality was high in all studies (>6 of 7 points). Thresholds used to define high- vs. low-volume were determined using arbitrary values ($n = 11$), tertiles ($n = 1$), quartiles ($n = 3$), percentiles ($n = 2$), or receiver operating characteristic curves ($n = 1$). High-volume surgeons had significantly lower 1-month readmission rates [odds ratio (OR) = 1.6; $p = 0.033$] and 3-month mortality rates (OR = 1.8; $p = 0.008$). In contrast, there were no significant differences between high- and low-volume surgeons for 3-month revision rates (OR = 2.0; $p = 0.228$), 12-month revision rates (OR = 1.8; $p = 0.228$), and 3-month re [...]

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Hexapod-assisted limb salvage after talectomy: outcomes of tibiocalcaneal arthrodesis using the TL-hex system.

Guerrero Serrano JA, Sánchez Morata E, García López JM, Mencía González S, Vilá Y Rico J

BACKGROUND: Severe talar destruction due to infection, trauma, avascular necrosis, or neuroarthropathy represents a major reconstructive challenge. Talcotomy followed by tibiocalcaneal arthrodesis (TCA) is a limb-salvage option in selected high-risk patients, although optimal fixation remains controversial. Modern hexapod external fixators offer computer-assisted multiplanar control that may improve alignment and fusion after talcotomy. The aim of this study was to evaluate the clinical and radiographic outcomes of talcotomy followed by TCA using the TL-HEX system in a series of high-risk patients.

METHODS: A retrospective observational study was conducted between January 2020 and January 2024 including all patients who underwent total talcotomy followed by TCA using a TL-HEX circular external fixator. Indications included talar destruction secondary to infection, avascular necrosis, post-traumatic collapse, Charcot neuroarthropathy, and aseptic loosening of total ankle arthroplasty. Primary outcomes were radiographic fusion and hindfoot alignment. Secondary outcomes included time to union, functional scores (AOFAS, EFAS), pain (VAS), complications, and patient satisfaction. Preoperative and final follow-up outcomes were compared using paired statistical tests, with significance set at $p < 0.05$.

RESULTS: Thirteen patients (mean age 57 years) were included with a mean follow-up [...]

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Radiographic changes in midfoot alignment after hallux valgus correction with and without concomitant second tarsometatarsal arthrodesis: a comparative study.

Kim J, Kyung MG, An Y, Kim NY, Lee KM, Lee DY

INTRODUCTION: Second tarsometatarsal (TMT) joint osteoarthritis occurs in a subset of patients with hallux valgus (HV). While second TMT arthrodesis is performed to address symptomatic arthritis, its association with midfoot alignment changes during HV correction remains unclear. This study aimed to compare changes in radiographic arch parameters between patients undergoing HV correction with and without concomitant second TMT arthrodesis.

MATERIALS AND METHODS: This retrospective comparative study included 73 feet with HV and medial column malalignment (lateral Meary's angle $> 10^\circ$). The TMT group ($n = 37$) underwent HV correction combined with second TMT arthrodesis for symptomatic osteoarthritis, while the control group ($n = 36$) underwent isolated HV correction. These patients were matched to achieve comparable baseline characteristics. Radiographic parameters of the transverse arch (M2-M5 angle, C1-M2 distance) and medial column alignment (lateral Meary's angle) were evaluated preoperatively and at a minimum 1-year follow-up. Functional outcomes were assessed as a secondary measure.

RESULTS: The TMT group demonstrated greater changes in transverse arch parameters compared with the control group. The M2-M5 angle decreased by 2.1° (95% CI, 1.4° to 2.7°) in the TMT group, whereas no significant change was observed in controls (-0.2° ; 95% CI, -0.7° to 0.4°). The C1-M2 distance [...]

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Coronal shear fractures of the distal humerus in children and adolescents: proposal for a modified classification.

Lieber J, Esser M, Fernandez FF, Schmittenbecher PP, Schneidmueller D, Strohm P, Kaiser MM, Kraus R

PURPOSE: Coronal shear fractures (CSF) of the capitulum and trochlea humeri are rare injuries during growth age. No systematic, prospective analysis of the classification, procedure, and outcome has yet been found in the literature.

MATERIALS AND METHODS: Based on the results of a multicentre study of CSF in growing patients, we propose a redesign of the AO Paediatric Comprehensive Classification of Long-Bone Fractures (PCCF) for children and adolescents. We also assessed inter-rater agreement consistency with the modified classification and calculated the intra-class correlation coefficient (ICC) and its 95% confident interval (CI).

RESULTS: A detailed classification has been proposed that refers to the location of the fragment, the extent to which the trochlea is involved, and the distinction between single and multiple fracture injuries. This modified AO classification distinguishes between five types of injury (13-E/3.1I - 13-E/3.1IV and 13-E/3.2), replacing the current code 13r-E/8.1. Inter-rater reliability using this modified classification was moderate (ICC 0.452 (95% CI 0.284; 0.661). This is more accurate than all previous classifications tested for CSF.

CONCLUSION: A revised classification of coronal shear fractures of the distal humerus in children and adolescents is required. Our proposed classification considers the various types of this injury and their implic [...]

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Correction: Prevalence and risk factors for stem subsidence following reverse shoulder arthroplasty using on-lay press-fit short stems.

Nishiura R, Nakazawa K, Manaka T, Hirakawa Y, Ito Y, Terai H

[PubMed](#) · [DOI](#)

Reverse shoulder arthroplasty vs. hemiarthroplasty for proximal humerus fracture: in-hospital outcomes in a national trauma registry.

Raizah A

BACKGROUND: Reverse shoulder arthroplasty (RSA) has rapidly supplanted hemiarthroplasty for acute proximal humerus fracture, driven by superior functional outcomes and lower revision rates. Whether this shift has altered perioperative morbidity at the population level remains poorly characterized. This study compared in-hospital outcomes between RSA and hemiarthroplasty in a national trauma cohort.

METHODS: This retrospective cohort study used the National Trauma Data Bank (2017-2022). Adults (≥ 18 years) admitted with proximal humerus fracture who underwent RSA ($n = 2657$) or hemiarthroplasty ($n = 428$) during the index hospitalization were included. The primary outcome was non-home discharge; secondary outcomes were any in-hospital complication, extended length of stay exceeding 7 days, and intensive care unit (ICU) admission. Multivariable logistic regression and inverse probability of treatment weighting (IPTW) were used to address confounding by indication.

RESULTS: Among 3085 patients (median age, 73 years; 2304 [74.7%] female), RSA patients were older (median, 74 vs. 64 years; $P < .001$) and more frequently female (77.3% vs. 58.6%; $P < .001$). After multivariable adjustment, RSA was not significantly associated with non-home discharge (adjusted odds ratio [aOR], 1.20; 95% confidence interval [CI], 0.93-1.55), any complication (aOR, 1.01; 95% CI 0.58-1.77), extended length o [...]

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Impact of cognitive impairment on postoperative delirium and discharge disposition following knee arthroplasty: a Japanese nationwide propensity score-matched study.

Mori Y, Tarasawa K, Tanaka H, Kanabuchi R, Mori N, Fushimi K, Aizawa T, Fujimori K

PURPOSE: As populations age, the number of older patients undergoing knee arthroplasty continues to increase. Cognitive impairment is prevalent among older adults and has been associated with adverse perioperative outcomes; however, its impact on postoperative delirium and discharge disposition following knee arthroplasty remains insufficiently defined, particularly in large Asian populations.

METHODS: We conducted a nationwide retrospective cohort study using Japan's Diagnosis Procedure Combination database from April 2016 to March 2023. Older patients who underwent primary total knee arthroplasty (TKA) or unicompartmental knee arthroplasty (UKA) were identified. Cognitive impairment was defined using ICD-10 diagnostic codes at admission. The primary outcome was postoperative delirium, and secondary outcomes included discharge to home, length of hospital stay, and perioperative blood transfusion. Propensity score matching (1:1) was performed to adjust for demographic factors, comorbidities, and surgical characteristics. Multivariable logistic regression and sensitivity analyses were conducted.

RESULTS: Among 259,319 eligible patients, 3,934 matched pairs were identified after propensity score matching. Patients with cognitive impairment had a significantly higher risk of postoperative delirium compared with those without cognitive impairment (absolute risk difference, 3.2%; [...])

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Association of primary tumor surgery with survival outcomes in patients with bone metastases across multiple cancer types.

Xu PH, Shi B, Li T, Tian M, Fang M, Cheng M, Huang W, Ren F et al.

BACKGROUND: Synchronous bone metastases (BM) have a notable impact on the survival rates of patients across various cancer types. While primary tumor surgery (PTS) has shown potential benefits, its impact across different cancers and patient subgroups needs further clarification. This study aimed to investigate the survival benefits of PTS across various cancer types.

METHODS: This retrospective cohort study analyzed data from the Surveillance, Epidemiology, and End Results (SEER) database, covering 53,015 patients diagnosed with BM from 2010 to 2019. Propensity score matching (PSM) was performed to reduce baseline imbalance. Overall survival (OS) and cancer-specific survival (CSS) were evaluated using Kaplan-Meier analysis and Cox proportional hazards regression.

RESULTS: PTS substantially improved overall and cancer-specific survival in patients with lung, kidney, breast and bladder cancers. All surgical modalities benefited breast and renal cancer survival. In prostate cancer, PTS produced no overall survival gain; survival varied by surgery type, being better after prostatectomy and worse with local resection. For liver cancer, PTS improved cancer-specific but not overall survival, and the small matched cohort necessitates circumspect interpretation.

CONCLUSION: The findings indicate that PTS can significantly enhance survival in selected patients with BM, particularly i [...]

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Liquid nitrogen-treated autograft combined with megaprosthesis versus total femoral replacement for large-volume malignant femoral bone tumors: a comparative study.

Sang NTQ, Trung ND, Huy LD, Phuc LH, Thang VD, Anh LN, Hung LT, Hien TTT et al.

BACKGROUND: Limb-salvage surgery for extensive femoral osteosarcoma presents major reconstructive challenges. Conventional options such as total femoral replacement (TFR) and long-segment biological reconstructions are associated with significant complications. We evaluated the clinical outcomes of a novel hybrid approach combining a liquid nitrogen-treated autograft with a megaprosthesis (LN-MP) and compared them with those of TFR.

MATERIALS AND METHODS: This retrospective study included 28 patients (range 8-24 years old) with malignant femoral bone tumors treated between 2020 and 2024. Patients underwent either LN-MP (n = 10) or TFR (n = 18). Perioperative outcomes, functional results (Musculoskeletal Tumor Society score-MSTS), bone union, oncological control, and survival were analyzed.

RESULTS: LN-MP demonstrated better functional outcomes (MSTS 27.0 ± 2.1 vs. 24.4 ± 2.2 , $p = 0.014$), shorter operative time, and lower blood loss compared with TFR. Length of stay and follow-up duration were comparable. Continuous disease-free survival was higher in the LN-MP group (90% vs. 44%, $p = 0.058$), with lower recurrence (10% vs. 28%). Bone union was achieved in 70% of LN-MP cases at a mean of 9 months, with longer graft length associated with delayed union.

CONCLUSIONS: LN-MP reconstruction may serve as a joint-preserving alternative to total femoral replacement for patients with e [...]

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Tibial bone transport using external fixation in adults: a systematic review.

Giuli C, Di Pietro G, Farine F, Bocchi MB, Vitiello R, Palmacci O

INTRODUCTION: Tibial bone transport using external fixation is widely employed for the reconstruction of large and complex tibial bone defects; however, indications and surgical strategies remain highly heterogeneous. Tibial bone transport using external fixation is a demanding but reliable reconstructive technique, characterized by low failure rates and generally favorable outcomes despite frequent complications. Adequate soft-tissue management, selective docking-site grafting, and early functional rehabilitation were commonly reported. Further prospective studies are needed to standardize treatment strategies and improve clinical decision-making. **MATERIALS AND METHODS:** A systematic review was conducted in accordance with PRISMA guidelines in April 2025, searching PubMed/MEDLINE and the Cochrane Library. Extracted data included patient demographics, comorbidities, indications, defect length, surgical technique, frame type, external fixation time, healing index, complications, failures, and ASAMI bone and functional outcomes. Methodological quality was assessed using the MINORS score and results were synthesized descriptively.

RESULTS: A total of 3543 patients from 89 studies were included. Post-traumatic infection or osteomyelitis accounted for 58.5% of indications. The mean defect length was 7.04 cm. Bifocal bone transport was performed in 82.4% of cases, and circular exte[r] [...]

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Accuracy and risk factors for poor cup orientation in total hip arthroplasty: a comparison between supine and lateral registration using a portable navigation system.

Matsumoto S, Tetsunaga T, Yamada K, Okuda R, Masada Y, Yamamoto T, Okazaki Y, Ozaki T

INTRODUCTION: Accurate acetabular cup orientation is essential in total hip arthroplasty (THA). Portable inertial navigation may improve reproducibility, but accuracy in lateral decubitus THA may depend on the pelvic registration workflow. We compared cup placement accuracy between supine and lateral registration workflows using NAVBIT (Smith & Nephew PLC) and explored factors associated with large navigation errors.

MATERIALS AND METHODS: We retrospectively reviewed 110 consecutive primary cementless THAs performed in the lateral decubitus position using NAVBIT. The supine-registration workflow included 52 hips (supine pelvic registration followed by repositioning), and the lateral-registration workflow included 58 hips (registration and cup implantation in the lateral decubitus position). NAVBIT recorded radiographic inclination (RI) and radiographic anteversion (RA) intraoperatively; postoperative RI and RA were measured via computed tomography. Accuracy was defined as the absolute difference between navigation records and postoperative measurements. We compared navigation accuracy outcomes between propensity score-matched groups. Outliers were defined as an absolute error > 5° in RI or RA; factors associated with outliers were evaluated in the overall cohort.

RESULTS: After propensity score matching, 46 matched pairs (n = 92) were analyzed. Postoperative cup orientation w[as] [...]

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Differences in the coronal plane alignment of the knee (CPAK) measurements using long-leg radiographs and computed tomography.

Kawaguchi K, Tay ML, Young S

INTRODUCTION: Coronal plane alignment of the knee (CPAK) parameters are increasingly used to characterize native coronal alignment and guide alignment strategies in total knee arthroplasty (TKA) based on long-leg radiographs (LLR). Computed tomography (CT) based robotic TKA software enables calculation of CPAK parameters based on anatomical reference points. However, potential differences in CPAK measurements between LLR and CT remain incompletely defined. This study compared CPAK coronal alignment parameters and phenotype distributions between LLR and CT, and evaluated factors associated with measurement discrepancies.

METHODS: This was a radiographic subanalysis of a prospective randomised controlled trial including 241 patients undergoing robotic-assisted TKA. Medial proximal tibial angle (MPTA) and lateral distal femoral angle (LDFA) were measured and arithmetic hip-knee-ankle-angle (aHKA), joint line obliquity (JLO) and CPAK phenotype distribution were calculated and classified from preoperative LLR and CT. Discrepancy groups were defined as LLR = CT ($\leq 2^\circ$), LLR < CT ($> 2^\circ$) and LLR > CT ($> 2^\circ$). Associations between discrepancy groups and preoperative deformity and flexion contracture were assessed.

RESULTS: Compared with LLR, CT demonstrated a more varus MPTA (mean 86.7° vs. 87.1°, $p = 0.04$), a more valgus LDFA (87.1° vs. 87.8°, $p < 0.01$) and a lower JLO (173.9° vs. 174. [...])

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Conservative and surgical treatment of wrist osteoarthritis: a systematic review.

Boever J, Unglaub F, Wulf J, Spies CK, Langer MF, Kußmaul AC

INTRODUCTION: Wrist osteoarthritis leads to pain and functional impairment. Despite conservative treatment options, surgery remains a central therapeutic component. In Germany, the number of patients rose from over 472,000 cases in 2018 to more than 580,000 in 2024. This review examines the most common surgical treatment approaches, focusing on Proximal Row Carpectomy (PRC) and Four-Corner Fusion (4CF), discussing indications, contraindications and potential complications.

METHODS: A systematic and narrative literature review was conducted on treatment options for wrist osteoarthritis. The systematic review focusing on PRC and 4CF followed PRISMA guidelines and included searches in PubMed, Cochrane and Embase. Included were RCTs, systematic reviews and meta-analyses from January 2010 to May 2025 that compared both procedures in terms of pain, range of motion, grip strength, function, complication rates and reoperation rates.

RESULTS: Eight studies were included. Both procedures significantly reduced pain and improved function. PRC showed advantages in range of motion and lower complication rates, while 4CF was associated with higher grip strength. Partial arthrodesis (e.g., scaphotrapeziotrapezoid (STT) arthrodesis or radioscapulunate (RSL)) arthrodesis may be selectively used but are linked to higher nonunion rates. Total wrist arthrodesis or wrist arthroplasty may be consi [...]

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Factors associated with selection of total hip arthroplasty versus hemiarthroplasty for femoral neck fracture in older adults: a nationwide analysis of 99,084 cases.

Maman D, Steinfield Y, Berkovich Y

BACKGROUND: Surgical management of displaced femoral neck fractures in older adults typically involves hemiarthroplasty or total hip arthroplasty (THA). Although clinical guidelines suggest that THA may be considered in selected healthier and cognitively intact patients, real-world procedure selection varies widely. This study evaluated patient factors associated with selection of THA versus hemiarthroplasty in a contemporary U.S.

METHODS: A retrospective cohort study was conducted using the 2022 Nationwide Readmissions Database (NRD). Patients ≥ 65 years hospitalized with femoral neck fracture were identified using ICD-10-CM codes. Those treated with internal fixation or non-arthroplasty procedures were excluded. Weighted analyses characterized demographics, comorbidities, and hospital utilization between THA and hemiarthroplasty groups. A multivariable logistic regression model identified factors independently associated with receiving THA. All analyses accounted for NRD survey design.

RESULTS: Among 142,013 operative cases, 99,084 met inclusion criteria (81.8% hemiarthroplasty; 18.2% THA). Patients receiving THA were younger (76.6 vs. 81.9 years), had shorter length of stay (6.06 vs. 7.24 days), and were more frequently discharged home (17.5% vs. 6.2%). Metabolic conditions were associated with increased odds of THA, including obesity (OR 1.17) and sleep apnea (OR 1.12). F [...]

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Effect of distal screw number on axial deformation and fragment-specific stability in AO/OTA 2R3 C2.1 distal radius fractures.

Dietz L, Spiegel C, Kohler FC, Biedermann U, Zderic I, Feist A, Gueorguiev-Rüegg B, Lenz M et al.

PURPOSE: To investigate the biomechanical influence of distal screw number in palmar plate fixation of unstable intra-articular distal radius fractures with radiodorsal metaphyseal comminution.

METHODS: Nine matched pairs of cryopreserved human radii were prepared using a standardized AO/OTA 2R3 C2.1 fracture model with a defined radiodorsal metaphyseal defect zone. For each pair, specimens were randomized to palmar fixation using a 2.4-mm variable-angle locking plate with either four or six distal screws. After embedding, constructs were subjected to cyclic axial loading (150 N, 5,000 cycles) simulating early postoperative rehabilitation. Global axial deformation and fragment-specific three-dimensional interfragmentary motion were recorded using an optical motion-tracking system. Statistical analysis was performed using non-parametric tests ($p < 0.05$).

RESULTS: Two specimens from one donor pair were excluded, leaving 16 radii (8 pairs) for analysis. Bone mineral density and initial construct stiffness did not differ between groups. The six-screw configuration showed significantly lower axial deformation after cyclic loading ($p = 0.047$) and lower radial fragment-shaft translation at both time points ($p = 0.028$; $p = 0.045$). Radial fragment-shaft rotation was lower in the six-screw group at baseline ($p = 0.018$), but no rotational difference remained after cyclic loading. In the [...]

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Crowe severity and femoral stem geometry are associated with lower-limb alignment after total hip arthroplasty for unilateral developmental dysplasia of the hip-related secondary osteoarthritis: valgus alignment trajectories.

Afacan MY, Gurcinar MG, Davulcu CD, Unlu MC, Kaynak G

BACKGROUND: Total hip arthroplasty (THA) for secondary osteoarthritis related to developmental dysplasia of the hip (DDH) restores hip mechanics. We aimed to quantify pre-to-postoperative changes in ipsilateral knee and global lower-limb alignment after unilateral THA to determine whether alignment changes differ by Crowe classification and femoral stem geometry.

METHODS: This retrospective cohort study included 74 patients who underwent unilateral primary THA for DDH-related secondary osteoarthritis, with a mean follow-up of 45.4 months. The predefined primary radiographic outcome set comprised preoperative-to-postoperative changes and final follow-up values in coronal lower-limb alignment parameters, including mechanical and anatomic lateral distal femoral angles (mLDFA and aLDFA), mechanical medial proximal tibial angle (mMPTA), joint line convergence angle (JLCA), global offset (GO), hip-knee-ankle angle (HKA), knee mechanical axis deviation ratio (KMADR), and femoral mechanical-anatomic axis angle (FMMA). The contralateral reference limb was used for side-to-side radiographic comparison. Patients were stratified by Crowe classification (I-II vs. III-IV), and femoral stem geometry analyses were performed according to conical, wedge, and rectangular stem designs. Clinical outcomes included hip/knee visual analog scale, Harris Hip Score, Knee Society Score, Oxford Knee Score [...]

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Can ChatGPT pass the polish national medical specialization examination in orthopedics and traumatology?

Maciąg B, Bujak K, Jegierski D, Stolarczyk A

INTRODUCTION: Artificial intelligence (AI) has evolved rapidly in recent years and is becoming increasingly integrated into many areas of medicine. In the medical field, these systems have attracted considerable attention because of their potential applications in clinical decision support, medical education, scientific communication, and postgraduate training. The aim of the present study was to evaluate whether ChatGPT could achieve a passing score on the Polish National Medical Specialization Examination (Panstwowy Egzamin Specjalizacyjny, PES) in orthopedics and traumatology and to determine how different prompting strategies influenced its performance.

MATERIALS AND METHODS: Authors systematically assessed the performance of ChatGPT-4 across five consecutive official PES exams (from Autumn 2023 to Spring 2025) in orthopedics and traumatology. Each exam was administered using three distinct prompting strategies: Professor prompt, Specialist prompt, Resident prompt. For each exam session (e.g., Spring 2024, Autumn 2023), the prompts were submitted to ChatGPT sequentially. The model's responses were evaluated against the official answer key published by the Polish Center of Medical Exams (Centrum Egzaminów Medycznych, CEM).

RESULTS: The results were averaged across all prompts. The overall accuracy was 81% (range: 68.33%-85.83%). The highest score (85.83%) was recorded mult [...]

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Two-year outcomes following concomitant ACL and extra-articular ligament reconstruction: a nationwide analysis of over 5,000 patients.

Abid R, J Burkhart R, J Moyal A, M Adelstein J, Kodszy M, G Calcei J, E Voos J, M Apostolakos J

PURPOSE: The anterior cruciate ligament (ACL) is the most commonly injured and reconstruction ligament in the United States. Associated injuries are common, oftentimes including components of the extra-articular knee. However, complications following concomitant ACL and extra-articular ligaments (EAL) has not been defined in a large-scale, population-based sample. We aimed to analyze outcomes following concomitant ACL and EAL reconstruction using the TriNetX nationwide database. **METHODS:** Deidentified data from the TriNetX Collaborative Network was used to identify all patients who received ACL and EAL reconstruction on the same day. Demographic information including age, sex, race, and geographic data were collected. Rates of medical and orthopaedic complications were analyzed at 30, 90, and 180 days and at 2 years. A propensity matched

sub-analysis comparing outcomes patients with concomitant and staged repair was performed. Patients were identified using Current Procedural Terminology (CPT) coding.

RESULTS: We identified 5944 patients who received concomitant ACL and EAL reconstruction with postoperative outcomes available at 30, 90, and 180 days. 5040 patients had data available at 2 years. Serious medical complications like surgical site infection, deep vein thrombosis, acute kidney failure, and sepsis were rare at each point of follow-up. Rates of revision surgery were [...]

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Recovery of gait after ankle fracture surgery following early mobilization and weight-bearing.

Bauer L, Schlegel AS, Scheuer A, Böcker W, Polzer H, Baumbach SF

BACKGROUND: Ankle fractures are among the most common fractures in adults. Despite significant advancements in surgical techniques, the postoperative treatment regimens remain conservative. This study aims to objectively evaluate gait recovery following early mobilization and weight-bearing after surgical ankle fracture treatment using instrumented gait analysis.

METHODS: This prospective, single-armed, longitudinal study enrolled adult patients following surgical treatment for isolated ankle fractures with early pain dependent weightbearing. Gait analysis, utilizing a treadmill integrated with a pressure plate and IMU-based motion capture systems, was conducted at 6 weeks, 3-, 6-, and 12-months post-surgery, assessing spatio-temporal and kinematic parameters. Results were compared longitudinally and against a healthy control group.

RESULTS: Longitudinal data were available for 44 patients. Significant ($p < .001$) improvements of self-paced walking velocity (SPV) were observed from 6 weeks (2.2 ± 1.1 km/h), to 3 (2.9 ± 0.9 km/h) and 6 months (3.4 ± 0.9 km/h). The spatio-temporal parameters showed significant changes between 6 weeks and all further time points only at SPV. Kinematic parameters showed significant longitudinal improvements at SPV for hip, knee, and ankle motion (including all three ankle planes). Patient gait parameters at 6 and 12 months were comparable to healthy [...]

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A comparative analysis of three knee arthrodesis techniques to manage failed total knee arthroplasty due to infection: intramedullary nailing, LRS external fixator, and compression plating.

Rajasekaran RB, Ponniah HS, Palanisami DR, Sundaram VP, Palanivelayutham SS, Jayaramaraju D, Dhanasekaran S, Rajasekaran S

INTRODUCTION: This study compared external fixation (EF), intramedullary nailing (IMN), and compression plating (CP) for knee arthrodesis (KA) following failed total knee arthroplasty (TKA) due to periprosthetic joint infection, focusing on complications, radiological, and functional outcomes.

METHODS: Patients aged ≥ 18 who underwent knee arthrodesis following failed TKA due to infection from 2008 to 2023 were included. Four surgeons used three techniques: EF, IMN, and CP. Retrospective data on demographics, surgical techniques, complications, and outcomes were collected. Postoperative patient-reported outcome measures (PROMs), pain, and functional status were assessed. Radiographic data was reviewed for limb shortening and union status.

RESULTS: 46 patients (mean age 61.9 years, BMI 28.2), predominantly male (84.8%) were involved in this study. Twenty patients underwent EF, 16 underwent CP, and 10 underwent IMN, all for infected primary TKA. *Pseudomonas* was the most common infecting organism (15.2%). Average follow-up was 29.1 months. Bony fusion was achieved in 41 knees (89.1%), with five requiring additional surgery; four eventually required amputation due to non-fusion. EF group had the shortest time to union and no major complications or amputations. Time to union was significantly shorter in the CP group compared to the IMN group ($p < 0.001$), and EF group also had a si [...]

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Accuracy of iliac crest reference pin placement in total hip arthroplasty: a CT-based analysis of risk factors for cortical perforation.

Masada Y, Tetsunaga T, Yamada K, Inoue T, Okuda R, Tetsunaga T, Okazaki Y, Ozaki T

INTRODUCTION: Accurate iliac crest reference pin placement is essential for safe navigation-assisted total hip arthroplasty (THA). Although complications are uncommon, inaccurate placement may cause neurovascular injury. This study aimed to evaluate pin insertion accuracy using postoperative computed tomography (CT), identify risk factors for cortical perforation, and determine whether CT-detected cortical perforation was associated with postoperative acetabular cup positioning accuracy.

MATERIALS AND METHODS: We retrospectively analyzed 200 primary THAs performed using portable or CT-based navigation between December 2022 and July 2025. Pin position was assessed on CT 1 week postoperatively and classified as complete intraosseous insertion or cortical perforation. Risk factors were examined using univariable and multivariable logistic regression analyses. Perforation direction was evaluated using Firth's penalized logistic regression. Postoperative cup positioning accuracy was assessed using CT-based radiographic inclination (RI), radiographic anteversion (RA), and absolute errors from target angles of 40° for RI and 15° for RA.

RESULTS: Cortical perforation occurred in 70 of 200 hips (35.0%): 32 medial and 38 lateral. On a pin-by-pin basis, 113 of 400 pins (28.3%) demonstrated perforation. In multivariable analysis, smaller ASIS width, lateral decubitus positioning, and 4-m [...]

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Long-term outcomes of arthroscopic single-anchor fixation versus microfracture for osteochondral fractures of the knee weight-bearing area in adolescents and young adults.

Yang W, Pan Q, Wang H, Peng Y, Meng C, Huang W

PURPOSE: Osteochondral fractures involving the weight-bearing area of the knee in young patients require stable restoration of the native osteochondral fragment whenever possible. This study aimed to evaluate the long-term clinical and radiological outcomes of arthroscopic single-anchor fixation and to compare them with those of microfracture.

METHODS: This retrospective comparative study included 24 adolescents and young adults with osteochondral fractures of the femoral condyle or trochlear region treated between 2016 and 2017. Twelve patients underwent arthroscopic single-anchor in situ fixation, and 12 underwent microfracture. The fixation technique used one double-loaded suture anchor to obtain multi-point compression of the osteochondral fragment while preserving the subchondral plate. Clinical outcomes were assessed using the Lysholm, International Knee Documentation Committee, and Tegner scores. Magnetic resonance imaging was evaluated using the MOCART 2.0 score. The mean follow-up was 93.7 months.

RESULTS: The single-anchor group showed a significantly higher intraoperative contact area ratio than the microfracture group ($96.5\% \pm 3.2\%$ vs. $85.8\% \pm 5.6\%$, $P < 0.001$). At final follow-up, the single-anchor group had superior Lysholm scores (91 ± 5 vs. 83 ± 8 , $P = 0.009$), IKDC scores (86 ± 6 vs. 78 ± 7 , $P = 0.010$), and Tegner activity levels (5.0 ± 1.1 vs. 3.9 ± 0.9 , $P = 0$ [...])

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Isometric points in cerclage for patellar tendon repair: a 3-dimensional, weightbearing computed tomography kinematic simulation.

Zindel C, Hodel S, Luo Y, Meisterhans M, Fucentese SF, Vlachopoulos L

PURPOSE: The gold standard for the management of ruptured patellar tendon is the surgical repair with nonabsorbable sutures and additional reinforcement e.g. cerclage wires as described by McLaughlin in 1947. Despite the early description, the exact placement of such cerclage wires remains unclear. The aim of the study was (1) to analyze how the cerclage elongates during knee flexion and (2) to define the insertion points with the most isometric behavior at the patella and tibial tuberosity (TT).

METHODS: 3D simulations of 10 healthy knees in the range from 0° to 120° of flexion using weightbearing computed tomography scans were generated. 12 insertion points were defined at the patella in equal distances

along the lateral and medial border from the proximal to the distal pole. 42 insertion points were defined at the tibial tuberosity (TT) in a 3.0 × 3.0 cm large grid around its center. An automatic string simulation algorithm was used to calculate the length for all possible combinations of the cerclage from 0° to 120° flexion. The most isometric combination of insertion points was found using the isometric score.

RESULTS: The most isometric insertion at the TT is distally ($p < 0.01$) and towards the center of the TT ($p < 0.01$). The most isometric points at the TT is located 61 mm distal to the joint line and 5 mm from the anterior border of the TT in a true lateral view fluo [...]

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Patient mortality exceeds implant revision risk with equivalent survival following unicompartmental versus total knee arthroplasty in octogenarians.

Innocenti M, Leggieri F, Massenzi M, Stimolo D, Akkaya M, Matassi F, Civinini R

BACKGROUND: While traditional surgical decision-making emphasizes long-term implant survivorship for octogenarians knee reconstruction, this paradigm may be inappropriate when patient mortality risk substantially exceeds revision risk. This study compared mid-term mortality between unicompartmental knee arthroplasty (UKA) and total knee arthroplasty (TKA) in octogenarians, hypothesizing that implant type would not significantly influence patient survival.

METHODS: A retrospective cohort study was conducted on 238 consecutive octogenarian patients (age ≥ 80 years) who underwent primary knee arthroplasty at a single tertiary orthopaedic center between January 2018 and December 2020. Patients with revision arthroplasty, fracture, tumor, inflammatory arthropathy, or incomplete data were excluded. The primary outcome was all-cause mortality at minimum 5-year follow-up. Secondary outcomes included age-stratified mortality (80-84, 85-89, ≥ 90 years) and identification of independent mortality predictors. Statistical analyses included independent t-tests, chi-square/Fisher's exact tests, standardized mean differences, and multivariable logistic regression adjusting for age, follow-up duration, and registry year.

RESULTS: Of 238 patients, 185 (77.7%) underwent TKA and 53 (22.3%) underwent UKA. Overall mortality was 35.3% (84/238): 33.5% in TKA versus 41.5% in UKA (OR = 1.41, 95% CI: 0 [...]

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Short- to mid-term clinical outcomes and return to sport and work after arthroscopic debridement of the extensor carpi radialis brevis in patients with chronic lateral epicondylopathy.

Siebenlist S, Doucas A, Hatt FL, Lappen S, Vieider RP, Lacheta L, Kadantsev P

BACKGROUND: This study aimed to evaluate the short- to mid-term clinical outcomes following arthroscopic extensor carpi radialis brevis (ECRB) debridement in patients with chronic epicondylopathy humeri radialis (cEHR). We hypothesized that arthroscopic ECRB debridement would result in favorable short- to mid-term patient-reported outcomes, improving pain levels, strength, range of motion (ROM) and patient satisfaction.

METHODS: A retrospective case series was conducted on patients who underwent arthroscopic ECRB debridement for cEHR with symptoms > 6 months. Patients were assessed at a minimum of 12 months postoperatively using the Mayo Elbow Performance Score (MEPS), Patient-Rated Tennis Elbow Evaluation (PRTEE), Disabilities of the Arm, Shoulder, and Hand (DASH) score, and Visual Analog Scale (VAS) for pain. Pre- and postoperative scores were compared using DASH and VAS. Maximum isometric wrist extension strength and postoperative ROM were measured bilaterally. Return to sport and work, patient satisfaction, and complications were documented.

RESULTS: Of the 35 eligible patients, 30 completed follow-up (86% follow-up rate; mean age 48.4 ± 6.6 years; range: 36-65 years; 47% male). The mean duration of symptoms before surgery was 12 months (IQR 8-20, range 6-62). The mean follow-up duration was 28.13 ± 10.9 months (range 12-45). At final follow-up, median outcome scores were [...]

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Complications and outcomes after operative treatment of low-energy pelvic ring and acetabular fractures.

Salonpää KB, Nurkkala J, Lantto I, Kakko S, Kaakinen T, Aitavaara-Anttila M, Liisanantti J

BACKGROUND: Fragility fracture of the pelvis (FFP) with a low-energy mechanism of injury is relatively common among the growing geriatric population. The incidence of postoperative complications after surgically treated FFP is not well documented. The aim of this study was to report complications after operatively treated FFP and to determine whether these complications would have a detrimental effect on outcomes.

METHODS: Retrospective study of 200 consecutive patients who underwent operative treatment at Oulu University Hospital, Oulu, Finland, between 1.1.2010 and 15.4.2021 for FFP with a low-energy injury mechanism. Postoperative complications were recorded, and associations with outcomes were depicted.

RESULTS: Postoperative complications were recorded in 31 (15.5%) patients. Patients with complications had higher intraoperative bleeding (OR 2.8 (1.0-7.9), $p = 0.048$) and higher ASA classification 3.4 (1.3-8.4), $P = 0.009$). Patients with complications had longer hospital stays, but no clear association between complications and mortality or discharge location was observed.

CONCLUSIONS: The incidence of postoperative complications was comparable with that reported in previous literature. Patients who experienced complications had longer hospital stays; however, no definitive association between complications and outcomes was identified.

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Minced cartilage implantation for cartilage regeneration: a survey of current clinical practices.

Yilmaz T, Leutheuser S, Niemeyer P, Behrendt P, Roessler P, Gille J, Mumme M, Vogt S

INTRODUCTION: The objective of this study is to evaluate how minced cartilage implantation (MCI) is currently applied in clinical settings, focusing on the selection criteria, procedural execution, handling of concomitant joint pathologies, and postoperative management.

MATERIAL AND METHOD: An online survey was conducted among all 4,915 members of the German Society for Arthroscopy and Joint Surgery (AGA). Invitations with a survey link were emailed, and the questionnaire was accessible anonymously from January 10 to February 20, 2024. A total of 927 responses were received and analyzed, resulting in a 19% response rate. Demographic data were described descriptively; survey answers were reported as percentages. The 25-item questionnaire, developed by AGA and the Quality Circle for Cartilage Repair & Joint Preservation (QKG e.V.), included single- and multiple-choice questions on experience with cartilage therapy, minced cartilage procedure application and postoperative care, and the overall assessment of this technique within cartilage repair.

RESULTS: MCI has emerged as one of the three most commonly used cartilage repair techniques in the knee joint. Defect sizes up to 4 cm² represent the largest treated group. The application of the procedure is quite heterogeneous, with some practitioners using a shaver and others manually mincing with a scalpel. Fixation methods also var [...]

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Variability in United States online rehabilitation protocols after open and endoscopic hip abductor repair.

Anwunah C, Rosenberg S, Hartwell M

PURPOSE: The purpose of this study was to evaluate readily accessible U.S. based online physical therapy protocols for open and endoscopic hip abductor repairs and to assess the variability in post-operative rehabilitation protocols.

METHODS: Online physical therapy protocols from U.S. based orthopaedic institutions for gluteus medius/minimus repair were collected using a web-based query utilizing search terms "hip abductor repair rehabilitation protocol" and "gluteus repair rehabilitation protocol." Published protocols from both Electronic Residency Application Service (ERAS) academic orthopaedic surgery programs and private groups were collected. All publicly available protocols were included. Protocols were analyzed using a standardized spreadsheet to assess rehabilitation components and timelines. Descriptive statistics (median values, percentages, ranges) were calculated.

RESULTS: 54 rehabilitation protocols were assessed, including 22 (41%) from ERAS-affiliated academic programs or associated physicians. Postoperative weight-bearing was addressed in 53/54 (98%) protocols, with 41 (77%) recommending toe-touch/touchdown/flat-foot/20-lb weightbearing. Median duration of restricted weight-bearing was 6 weeks (range, 2-8 weeks). Regarding range of motion restrictions, 37 (69%) protocols limited hip flexion to 90°, 42 (78%) advised against passive adduction past neutral, and [...]

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The incidence and epidemiology of patellar fractures following medial patellofemoral ligament reconstruction: a systematic review and meta-analysis.

Abelleyra Lastoria DA, Hunt A, Ross CK, Yemane M, Keynes S, Ghosh A, Hing CB

INTRODUCTION: Patellar instability can arise from a traumatic event with anatomical predisposing factors increasing the risk of dislocation. Medial patellofemoral ligament reconstruction (MPFLR) is commonly performed to treat patellar instability. The aim of this systematic review was to determine the incidence of and risk factors for patellar fractures after MPFLR.

MATERIALS AND METHODS: The protocol for this review was registered on PROSPERO with registration number CRD420251066107. The PRISMA 2020 checklist was followed. Electronic databases and conference proceedings were searched to the 15th of May 2025. Studies were eligible if they reported incidence of patellar fractures after MPFLR. A random-effects meta-analysis was performed. Included studies were appraised using tools respective of study design.

RESULTS: A total of 4,972 records were screened in the initial search. A total of 110 studies satisfied the inclusion criteria, evaluating 115,496 patients. The evidence was moderate to low in quality. The incidence of patellar fracture ranged from 0 to 9.50%, with a pooled incidence of 1.17% (95% confidence interval [CI] 0.76- 1.68%). There was no difference in incidence of patellar fractures between patients who did and did not undergo bone tunnel fixation (0.94%, 95% CI 0.69-1.24 vs. 1.00%, 95% CI 0.15-2.44%, respectively). There was no difference in fracture incidence [...]

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Predictive accuracy of a general-purpose artificial intelligence model for cut-out following proximal femoral nailing.

Ben Arie G, Warschawski Y, Amzallag N, Gan-Or H, Ben-Tov T, Khoury A, Horev I, Graif N et al.

INTRODUCTION: Cut-out is the most consequential mechanical complication after proximal femoral nailing and requires prompt recognition. General-purpose artificial intelligence (AI) models with image-interpretation capability are increasingly accessible, yet their diagnostic performance for this task is unknown. We evaluated ChatGPT's accuracy for predicting cut-out following proximal femoral nailing and compared TFNA and Gamma nail subgroups.

MATERIALS AND METHODS: In this retrospective predictive-accuracy study, 989 patients (683 TFNA, 306 Gamma nail; mean age 83.8 years; cut-out prevalence 2.5%) were analysed. For each case, three baseline radiographs - the injury anteroposterior and lateral views and the immediate postoperative control radiograph, all obtained before any complication could be radiographically present - together with the full clinical data set (excluding complication-related information) were submitted to ChatGPT, which returned a binary prediction (yes/no) and probability estimate (0-100%). The follow-up radiographs on which cut-out becomes apparent were not provided to the model. Ground truth was established on subsequent follow-up radiographs by a departmental review board requiring agreement of two senior surgeons. Accuracy metrics were calculated with Wilson 95% confidence intervals (CI); exact McNemar's and Mann-Whitney U tests assessed directional bias [...]

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Staged management of a compound combined pelvi-acetabular injury with perineal degloving in a young female: a case report and literature review.

Dhankhar K, Kekatpure AL, Kekatpure AL

We present the case of a 30-year-old woman, the patient who sustained a compound combined pelvi-acetabular fracture with extensive perineal internal degloving after high-energy trauma. Computed tomography revealed an unstable pelvic ring injury (Tile type C1; anteroposterior compression type III) with right transverse acetabular

fracture. The occurrence of extensive internal degloving was confirmed intraoperatively. The injury was treated using a graduated, principle-based approach incorporating early soft-tissue debridement, dead-space management and temporary pelvic stabilization, infection control, and delayed definitive internal fixation. An intercurrent pin-site infection was treated successfully without progression to deep infection. The patient demonstrated progressive radiological union at 4 months with a Harris Hip Score (HHS) of 80 and a Majeed pelvic score of 55. At one-year follow-up, complete union occurred and functional recovery was achieved, characterized by HHS of 90 and a Majeed score of 70. In this case, infection-guided staging as well as extremely aggressive soft tissue treatment may allow for safe delayed fixation even where pelvi-acetabular trauma has been compromised.

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Concomitant upper extremity fractures do not worsen outcomes in conservatively managed pelvic fractures: a cohort study of 1840 geriatric patients.

Graif N, Elbaz E, Kazum E, Rachevsky G, Gurel R, Factor S

INTRODUCTION: Pelvic fractures in elderly patients are associated with significant pain, functional decline, and prolonged rehabilitation, with the majority managed non-operatively. In the conservative management paradigm, intact upper extremity function is critical for mobilization with assistive devices; concurrent upper extremity (UE) fractures may therefore complicate recovery. The aim of this study was to evaluate the clinical outcomes associated with this injury combination in patients with conservatively managed pelvic fractures.

METHODS: A retrospective cohort study was conducted at a Level 1 trauma center, including all patients aged ≥ 65 years admitted with pelvic ring or acetabular fractures managed non-operatively between January 2010 and January 2024. Patients were stratified by the presence of a concomitant UE fracture sustained during the same traumatic event. Primary outcomes were hospital length of stay (LOS) and all-cause mortality at multiple time points. Secondary outcomes included readmission rates and systemic infection rates.

RESULTS: Of 1840 patients, 1693 (92.0%) sustained isolated pelvic fractures and 147 (8.0%) had concomitant UE fractures. Groups were comparable at baseline across all measured covariates. LOS did not differ significantly between groups (11.0 ± 14.2 vs. 11.8 ± 16.9 days; $p = 0.796$). Mortality was similar at all evaluated time points [...]

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Single double-loaded anchor repair of isolated Lafosse III subscapularis tears in patients not involved in physically demanding activities provides satisfactory functional and radiological outcomes at minimum 5-year follow-up.

Aral F, Oklaz EB, Ahmadov A, Asiret E, Delice C, Guler Oklaz EB, Akdulum I, Kanatli U

PURPOSE: To evaluate the outcomes of single-anchor repair for isolated Lafosse III subscapularis tears in patients not engaged in physically demanding activities.

METHODS: Patients with isolated subscapularis tears who underwent arthroscopic surgery between 2014 and 2020 were retrospectively evaluated. The inclusion criteria were patients who underwent single double-loaded anchor repair for Lafosse III tears and were not involved in physically demanding activities. Assessments included the American Shoulder and Elbow Surgeons (ASES) score, Constant-Murley Score (CMS), Visual Analog Scale (VAS), range of motion, specific clinical tests, and magnetic resonance imaging encompassing tendon integrity, cross-sectional area (CSA), vertical diameter, and transverse diameters.

RESULTS: 27 patients (mean age 52.4 ± 10.3 years; mean follow-up 80.8 ± 18.6 months) were included in the study. The final follow-up demonstrated significant improvements in the ASES (44.0 ± 12.4 to 82.4 ± 14.1), CMS (49.1 ± 15.3 to 85.3 ± 11.1), VAS (5.7 ± 2.1 to 1.3 ± 2.2), and internal rotation (L4 to T8) ($p < 0.001$ for all). The positivity of clinical tests decreased substantially: Bear-Hug, from 93% to 18%; Belly-Press, from 89% to 15%; and Lift-Off, from 89% to 18% ($p < 0.001$ for all). Radiological analyses indicated maintained tendon integrity in 93% of patients and increases in CSA (1.292 ± 453 to 1.544 [...])

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Shoulder injuries in rugby union: a systematic review and meta-analysis.

Doyle J, Bellamy M, Fowler K, Keiller H, Gunasekera R, Funk L

Shoulder injuries are common in Rugby Union, resulting in prolonged absence and considerable burden. Reported incidence, severity, and mechanisms vary widely, reflecting the multiple high-impact actions in the sport. Various management approaches exist to treat a wide range of pathologies. This systematic review and meta-analysis aimed to synthesise evidence on the epidemiology, mechanisms, and treatment of shoulder injuries in rugby. PUBMED, SCOPUS, and WEB OF SCIENCE were searched from database inception until 1st June 2025. From 1,099 abstracts screened, 37 studies were included. The pooled match incidence was 11.02 injuries/1,000 player-hours (95% CI 7.23-16.82), though heterogeneity was very high ($I^2 = 97\%$) and the 95% prediction interval was wide (2.11-57.61 injuries/1,000 player-hours), indicating the pooled rate should be read as an average across diverse populations rather than a single representative value. Higher rates were seen in male elite (13.74/1,000 h) and high school/university players (12.28/1,000 h). Training incidence was considerably lower (0.20/1,000 h). Substantial between-study heterogeneity was observed across all incidence analyses, reflecting genuine variation across playing levels, sex, and surveillance methods. Tackling was the predominant injury mechanism, most often affecting the tackler. Bankart, Latarjet, and Bristow procedures all produced fav [...]

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The pre-operative physical component subscale of the 12-item Short Form Health Survey was the only factor associated with 2-year clinical outcome and satisfaction after hip abductor tendon repair.

Ebert J, Edwards P, Klinken S, Janes G

INTRODUCTION: Surgical repair in the context of symptomatic hip abductor tears unresponsive to non-operative management has demonstrated encouraging outcomes. This study sought to investigate demographic, injury/management and post-operative factors associated with clinical outcome after hip abductor tendon repair.

METHODS: A total of 104 patients undergoing repair were included. Patients were assessed pre-surgery and 2-years post-operatively with the modified Harris Hip Score (mHHS), Oxford Hip Score (OHS), physical (PCS) and mental (MCS) component subscales of the 12-item Short Form Health Survey (SF-12), and satisfaction. Regression analysis assessed the contribution of pre-operative demographic (age, sex, body mass index, PCS, MCS), injury/treatment (symptom duration, prior corticosteroid injections, gluteus minimus/medius tear grade and fatty infiltration) and post-operative (hip abduction strength symmetry) variables, to the 2-year mHHS and being 'very satisfied'.

RESULTS: Mean improvement in the mHHS and OHS from baseline to 2 years was 36.0 points (95% CI, 31.9 to 40.2; $p < 0.001$) and 17.5 points (95% CI, 15.8 to 19.3; $p < 0.001$), respectively. Seven (6.7%) patients presented with symptomatic re-tearing. Of the 97 patients with 2-year follow-up, 78 (80.4%) were 'very satisfied'. The pre-operative SF-12 PCS was the only factor associated with the 2-year mHHS ($B = 0.46$; [...])

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Temporising external fixation reduces loss of reduction compared with plaster splinting in ankle fracture-dislocations: a systematic review and meta-analysis of cohort studies.

Suravaram V, Solomon L, Bhat S, Wittauer M

PURPOSE: The optimal temporising strategy for ankle fracture-dislocations is a highly debated topic. These unstable injuries are commonly managed with staged fixation to allow for soft tissue recovery, using either plaster splinting or ankle-spanning external fixation. This study aimed to compare the efficacy and safety of these two approaches.

METHODS: A systematic search of MEDLINE, Embase, Scopus, Cochrane CENTRAL, and Google Scholar was conducted. Comparative studies evaluating temporary external fixation versus plaster splinting in skeletally mature patients with ankle fracture-dislocations were included. Meta-analyses were performed to assess differences in loss of reduction prior to definitive fixation, soft tissue complications, and time to definitive fixation. Risk of bias was assessed using ROBINS-I tool, and certainty of evidence was appraised using GRADE.

RESULTS: Nine retrospective cohort studies comprising 1638 patients were included (589 external fixation, 1049 splinting). Loss of reduction occurred in 3% of injuries in the external fixation cohort compared with 14.6% in those managed with plaster splinting (RR 0.22; 95% CI 0.09-0.52). Rates of overall soft-tissue complications were similar between cohorts. The data regarding time to definitive fixation were highly heterogeneous across studies. Overall certainty of evidence was moderate for LOR and very low for [...]

Comprehensive ultrasound assessment of thumb metacarpophalangeal collateral ligament injuries reveals frequent concomitant abnormalities.

Bain O, Farkash U, Atzmon R, Shemesh S, Ohana N

INTRODUCTION: Ulnar collateral ligament (UCL) tears of the thumb are well-recognized injuries, while radial collateral ligament (RCL) and dorsal extensor-related abnormalities are less frequently studied. The pattern of concomitant ultrasound abnormalities in patients after thumb trauma is not well documented. The aim of this study was to characterize concomitant UCL, RCL, and dorsal extensor-related abnormalities within a selected cohort referred to thumb ultrasound after trauma, and to explore associations with injury mechanism.

MATERIALS AND METHODS: A retrospective review was performed of 42 consecutive adult patients who underwent high-resolution ultrasound of the thumb metacarpophalangeal joint between April 2014 and February 2025 for suspected ligamentous injury after acute trauma. Patients were included if ultrasound demonstrated pathologic findings in at least one stabilizing structure, UCL or RCL, and the extensor mechanism was systematically assessed. Collateral ligament injuries were graded using standard sonographic criteria, while dorsal extensor-related findings were recorded descriptively. Associations between injury severity and mechanism of injury were tested using chi-square or Fisher's exact analysis, as appropriate.

RESULTS: UCL pathology was present in 35/42 (83.3%) patients, RCL pathology in 23/42 (54.8%), and dorsal extensor-related abnormalities in 17 [...]

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Holding the line: predictors of coronal and sagittal loss of reduction after tibial plateau fracture fixation.

Hesmerg MK, Benner JL, Joosse P, Keijser LCM

INTRODUCTION: Tibial plateau fractures (TPFs) are challenging fractures that often suffer from secondary Loss of Reduction (LoR) after fracture fixation. In this study, we aimed to investigate the factors and characteristics associated with LoR after surgical fixation, using measurements of both coronal and sagittal displacement.

METHODS: A retrospective analysis of 213 TPF patients (79.3% female, mean age 57.5 ± 14.4 years) surgically treated between 2017 and 2024 in a level 1 trauma center was performed. Patients were included if they had a minimum of 100-days radiological follow-up. Loss of reduction was defined as late postoperative depression ≥ 3 mm, condylar widening ≥ 5 mm, varus/valgus or tibial slope collapse > 5 degrees, breakage, loosening, or conversion to total knee arthroplasty (TKA). Radiographic images were assessed for preoperative, direct postoperative, and late postoperative (up to 1 year) displacement. Univariable and multivariable logistic regression analyses were employed to study factors associated with LoR..

RESULTS: At a mean follow-up of 325 days (SD ± 161) 77 patients (36.2%) suffered from LoR. The adjusted multivariable logistic regression model revealed that patient age (OR 1.03, $p = 0.028$), involvement of the posterior column (OR 2.87, $p = 0.014$), preoperative step-off (OR 1.08, $p < 0.001$), postoperative step-off (OR 1.61, $p = 0.007$), and postope [...]

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Using an App based algorithm to determine axial rotation and pelvic tilt at the time of an AP radiograph: a sawbone study.

Boettner F, Premkumar A, Sterneder M, Aboaf C, Haralambiev L

INTRODUCTION: Axial rotation of the pelvis impacts inclination and anteversion measurements of the acetabular component on plain radiographs. Currently the axial rotation is not taken into account when providing acetabular component measurements on plain radiographs. The current study evaluates a novel App based algorithm to determine pelvic axial rotation at the time of an AP pelvis radiograph.

MATERIALS AND METHODS: AP and lateral radiographs were taken of the sawbone pelvis to calibrate the App. The sawbone was then rotated 11 times to the left (average 5.3° , range 0.2° - 10.5°) and 10 times to the right (average 5.2° , range 1.2° - 11.3°). The sawbone was then tilted forward 14 times (average 5.6° range 0.1° - 11.6°) and 13 times backward (average 5.9° , range 0.1° - 11.2°). The amount of axial rotation and tilt of the sawbone pelvis

was determined using a digital caliper. AP pelvis radiographs were taken for each position. Using a novel App based calculation algorithm the amount of axial rotation and pelvic tilt was determined from the AP Pelvis radiograph.

RESULTS: The algorithm determined the axial rotation of the pelvis at the time of the AP pelvis radiograph with an average difference of 0.6° (range 0°-1.8°) compared to the digital caliper measurement. For measuring pelvic tilt, the difference was 2.3° (range 0.4°-4.5°) compared to the digital caliper measurement.

CONCLUSIONS [...]

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Risk factors for implant-associated infection after bone allograft reconstruction: a 20-year retrospective cohort study.

Scheurer F, Smolle M, Hauer G, Ruckenstein P, Valentin T, Leithner A

AIMS: Reconstruction of bone defects is a key challenge in orthopaedic surgery. While autologous bone grafts are considered the gold standard, osseous allografts are increasingly used as an alternative as they allow for treatment of large bone defects. However, they may be associated with an increased risk of postoperative infections. This study aims to analyze the frequency of and risk factors for implant-associated infections after bone allograft transplantation.

METHODS: All patients who underwent reconstruction with bone allografts at a single orthopaedic and trauma unit between 2003 and 2023 were retrospectively included. Data on demographics, disease-associated parameters, risk factors at index surgery, and development of implant-associated infections during follow-up were documented. Potential risk factors for development of implant-associated infections during follow-up were investigated with logistic regression models.

RESULTS: A total of 220 patients were included (46.8% males; median age at surgery 46.5 years [IQR: 22-67]; n = 23 with implant-associated infections during follow-up). Factors independently associated with subsequent implant-associated infections were smoking (p = 0.001), BMI (p = 0.015) and radiotherapy (p = 0.013). Gender, age, preoperative chemotherapy, allograft material, preoperative C-reactive protein, hemoglobin and reason for surgery (tumor, n [...])

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Why it's better to believe in a larger definition of the diaphyseal junction zone in pediatric distal radius fractures.

von Schrottenberg C, Beck SM, Schwert P, Fitze G, Schultz J

INTRODUCTION: Diaphyseal radius fractures (DMRF) in children are characterized by special biomechanical behavior. They present with a stereotypical loss of reduction (radial translation and ulnar tilt) of the distal fragment when stabilized with elastic stable intramedullary nailing (ESIN), the standard osteosynthesis for pediatric diaphyseal forearm fractures. No consensus has been found on the definition of the diaphyseal junction zone (DMJZ). Some authors claim that diaphyseal fractures immediately proximal to the metaphysis, as defined by the AO Pediatric Comprehensive Classification of Long Bone Fractures (AO-PCCF), can safely be stabilized with ESIN. They confine the DMJZ to a small part within the metaphysis. Other definitions of the DMJZ extend slightly more proximally, including a small part of the distal diaphysis. The forearm fracture index (FFI) calculates the ratio of the fracture's distance to the radius' growth plate over its width and defines DMRF to have an FFI between 1 and 2. The aim of this case series was to demonstrate that these particular fractures, which can be considered diaphyseal according to the AO-PCCF, still exhibit biomechanical characteristics typical of DMRF.

MATERIALS AND METHODS: Radiologic and clinical outcomes of 11 DMRF, that would be considered diaphyseal by the AO-PCCF classification but fall into the DMJZ when using the FFI, are [...]

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Optimizing initial biomechanical strength ("Time Zero") of posterior repair in total hip arthroplasty: a biomechanical comparison of suture materials and knot configurations.

Chen M, Liu X, Chen D, Li X, Du H, Sun Y, Wang H

INTRODUCTION: Posterior capsule repair (PCR) is commonly performed during total hip arthroplasty (THA) via the posterolateral approach to improve postoperative stability. Because biological healing of the repaired posterior structures requires time, the repair construct must provide sufficient immediate mechanical support after surgery. This study aimed to identify the suture material and knot configuration with favorable initial mechanical performance in a standardized in vitro model.

MATERIALS AND METHODS: This in vitro biomechanical study was conducted in two sequential phases. Phase 1 evaluated the baseline tensile properties of three commonly used suture materials: #2 Ethibond, #0 Vicryl, and 3 - 0 PGLA. Phase 2 used a standardized porcine dermal surrogate model to compare eight suture configurations, including single- and double-strand techniques. The primary biomechanical outcomes included maximum failure load, load at 2 mm gap formation, and construct stiffness during uniaxial quasi-static load-to-failure testing.

RESULTS: In Phase 1, non-absorbable #2 Ethibond showed significantly greater ultimate tensile strength (92.44 ± 8.72 N) and higher stiffness than the absorbable alternatives ($P < 0.001$). In Phase 2, double-strand configurations outperformed all single-strand techniques. Among them, the double-strand Nice knot achieved the highest maximum failure load (104.04 [...])

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Intravenous versus oral tranexamic acid in shoulder arthroplasty.

Parmar T, Adio A, Daher M, Khan A, Horneff J, Abboud J

PURPOSE: Tranexamic acid (TXA) is used in shoulder arthroplasty to reduce blood loss. While intravenous (IV) TXA is standard, oral TXA offers advantages in cost and administration. This study compares 90-day complications and postoperative laboratory values between IV and oral TXA in total shoulder arthroplasty (TSA).

METHODS: A retrospective cohort study using TriNetX identified adults undergoing primary TSA (2005-2025) receiving TXA on the day of surgery, stratified by route (IV vs. oral). One-to-one propensity score matching was performed. Ninety-day outcomes included transfusion, thromboembolic events, cardiac and renal complications, neurologic events, infection, wound dehiscence, readmissions, emergency department visits, opioid utilization, and mortality. Postoperative laboratory outcomes included hemoglobin, erythrocyte count, hematocrit, and platelet count. Outcomes were compared using odds ratios (ORs) with 95% confidence intervals (CIs) and t tests where appropriate.

RESULTS: After matching, 5722 patients were included in each cohort. No significant differences were observed between IV and oral TXA in 90-day transfusion rates ($P = 0.873$) or thromboembolic events, including deep vein thrombosis and pulmonary embolism. Rates of myocardial infarction, stroke, cardiac ischemia, acute renal failure, infection, readmission, emergency department visits, opioid use, and mo [...]

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Robotic-arm assistance in revision total hip arthroplasty improves the accuracy of acetabular component positioning: a cadaver-based study.

Fitzgerald K, Borukhov I, Chandra V, Smitterberg C, Mont MA, LiArno S

INTRODUCTION: Acetabular component positioning is a key factor for stability, impingement, wear, and biomechanics in revision total hip arthroplasty (THA). However, anatomic distortion and previous implants may complicate positioning. Although evidence on computed-tomography (CT)-based robotic assistance in revision THA is limited, its planning features and precision may improve surgical plan execution. This study aimed to measure the precision and accuracy of acetabular component positioning of revision robotic-arm assisted total hip arthroplasty (RA-THA) compared with a published manual primary total hip arthroplasty (M-THA) benchmark for: (a) acetabular inclination; (b) acetabular version; and (c) center of rotation (COR), with comparison by surgeon case volume.

MATERIALS AND METHODS: There were seven cadavers (nine hips) that previously underwent primary THA revised with RA-THA by independent surgeons with varying primary RA-THA case volumes, surgical approaches, and acetabular component types. Due to surgical protocol deviations, two hips were excluded leaving seven hips from six cadavers. Pre- and postoperative CT scans were compared to calculate absolute error for inclination, version, and COR; normality was established with the Anderson-Darling test and two-variance tests assessed differences in standard deviations ($\alpha = 0.05$). Mean differences between cadaver-based rev [...]

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Correction: Modified rotational wedge distal metatarsal osteotomy versus chevron osteotomy for hallux valgus: long-term radiographic and clinical outcomes.

Aybar A, Gokkus K, Özden E, Çetin MH, Çetin MÜ

[PubMed](#) · [DOI](#)

Isolated compartment arthrosis of the knee: contemporary concepts, persistent controversies, and future directions in unicompartmental knee arthroplasty.

Akkaya M, Innocenti M

[PubMed](#) · [DOI](#)

Preoperative lower radiographic osteoarthritis severity in primary total knee arthroplasty increases revision risk by tenfold compared to severe osteoarthritis.

Stroobant L, Vermue H, Droogsma T, Van Onsem S, Arnout N, van Hellemond G, Victor J, Heesterbeek PJC

BACKGROUND: Dissatisfaction following primary total knee arthroplasty (pTKA) is higher in patients with preoperative low-grade osteoarthritis (OA), but its impact on revision risk remains unclear.

QUESTIONS/PURPOSES: This study aimed to identify predictors for revision surgery within 10 years following pTKA, focusing on radiographic severity of OA using the Kellgren-Lawrence (KL) grading system, along with other demographic and radiographic factors.

PATIENTS AND METHODS: This retrospective case-control study was conducted at two European tertiary referral centers. The case group included 142 patients who underwent aseptic revision total knee arthroplasty (rTKA) between 2007 and 2023, within 10 years following pTKA. A 2:1 control group of 284 patients who had pTKA between 2011 and 2014, with no revision surgery within 10 years, was selected. Collected data included age, sex, body mass index (BMI), side of surgery, and American Society of Anesthesiologists (ASA) classification. Radiographic data were collected, requiring a preoperative radiograph to assess the KL grade. Univariate analyses identified potential predictors for rTKA, and multivariable logistic regression assessed their relationship with revision risk.

RESULTS: Patients requiring rTKA were significantly younger at the time of pTKA. For each additional year of age, the revision risk decreased by 6% (OR 0.94, 95% CI [...])

[PubMed](#) · [DOI](#)

Correction: Non-tobacco nicotine dependence and postoperative complications following operative fixation of tibial shaft fractures: a retrospective cohort analysis.

Berk A, Bogart M, Eghtedari C, Good L, Florentino S, Moyal A, Burkhart R, Ochenjele G et al.

[PubMed](#) · [DOI](#)

Comparison of pediatric proximal femoral locking plates and dynamic hip screws in the fixation of pediatric intertrochanteric fractures, a biomechanical study.

Wang JH, Shih CA, Wang CH, Lee CC, Wu KW, Wang TM, Kuo KN, Hong CK

Treatment options for pediatric intertrochanteric fractures range from conservative casting in young children to surgical fixation in older patients, most commonly using dynamic hip screws (DHS) or pediatric proximal femoral locking plates (PF-LCP). As there is limited biomechanical evidence favoring one method over another, this study aimed to compare the biomechanical stability of DHS and PF-LCP for the treatment of pediatric intertrochanteric fractures using a synthetic bone model. Sixteen synthetic composite femurs were allocated into two groups stabilized with either a DHS or a PF-LCP. To simulate Delbet type IV intertrochanteric fractures, a vertical osteotomy was created in each specimen. Biomechanical evaluation was carried out using a universal testing machine, assessing axial stiffness, vertical, horizontal, and total displacement, as well as ultimate load to failure under standardized loading conditions. Comparative statistical analyses were then performed to determine differences in biomechanical performance between the two fixation methods. Group P demonstrated significantly

smaller vertical and total displacements (0.15 ± 0.15 mm and 0.41 ± 0.31 mm) compared with the DHS group (0.59 ± 0.42 mm and 0.75 ± 0.33 mm) ($p = 0.018$ and $p = 0.045$), whereas horizontal displacement showed no difference ($p = 0.142$). Axial stiffness and ultimate failure load were comparable bet [...]

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Impact of severe varus deformity on the outcome of mobile-bearing unicompartmental knee arthroplasty: a comparison of patients with varus deformity of less than and more than 15 degrees using matched-pair analysis.

Mullaji AB, Gupta AK, George GK, Kazi R

INTRODUCTION: The outcome of patients undergoing UKA with larger preoperative deformity ($> 15^\circ$ varus) and lesser deformity is sparsely compared in the literature. This matched-pair retrospective study attempts to fill this research gap by comparing the effect of preoperative and postoperative residual varus on outcome.

METHODS: Patients undergoing UKA with preoperative varus deformity $> 15^\circ$ (HKA $> 15^\circ$ group) were selected and matched for age and gender with those with $< 15^\circ$ of preoperative varus deformity (HKA $< 15^\circ$ group). There were 72 age and gender-matched pairs knees. HKA was measured and Forgotten Joint Score, OKS, VAS and WOMAC scores were obtained at a minimum follow up of 5 years. Postoperative residual varus was classified into 3 groups based on postoperative HKA: varus $< 3^\circ$, varus 3 to 7° and varus $> 7^\circ$.

RESULTS: Mean correction of deformity was significantly more ($p < 0.001$) in the HKA $> 15^\circ$ group (10.5°) when compared to HKA $< 15^\circ$ group (4.5°). There was no significant difference in outcome measures OKS ($p = 0.21$), FJS ($p = 0.35$) and VAS ($p = 0.6$) between patients in the two groups. There was no significant correlation between the preoperative and postoperative HKA of the patient with postoperative OKS, FJS and VAS in the two groups. There was no statistically significant difference between FJS ($p = 0.73$) of the three postoperative varus groups. Excellent to good [...]

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A multimodal imaging approach to predict carpal tunnel syndrome after distal radius fractures.

Önüt M, Alkol N, Önalolu Y, Özer M, Apaydın E, Talmaç MA

OBJECTIVE: Distal radius fractures are commonly associated with the development of carpal tunnel syndrome (CTS), yet current evidence primarily focuses on radiographic alignment parameters. The role of carpal tunnel morphology, particularly total carpal tunnel cross-sectional area (CSA) measured on computed tomography (CT), remains insufficiently investigated. This study aimed to evaluate the relationship between CSA, volar tilt, and the development of CTS in conservatively managed distal radius fractures, and to assess the predictive performance of these parameters when used in combination.

METHODS: This retrospective case-control study included 150 patients, comprising 50 patients who developed CTS and 100 controls without CTS. CSA was measured on CT scans obtained at initial presentation, while volar tilt was assessed using standard radiographs. Additional variables, including the presence of ulna fracture and fracture type, were analyzed. Group comparisons were performed using nonparametric tests. Multivariable logistic regression analysis was conducted to identify independent predictors of CTS. Diagnostic performance was evaluated using receiver operating characteristic (ROC) curve analysis.

RESULTS: Patients who developed CTS had significantly lower CSA and higher volar tilt values (both $p < 0.001$). The presence of an ulna fracture was inversely associated with CTS ($p = [...]$)

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Unicompartmental knee arthroplasty and total knee arthroplasty in patients with 15 or more degrees of varus deformity: age- and gender-matched comparative study of radiographic and clinical outcomes.

Mullaji A, Chopperla S, George GK, Kazi R

INTRODUCTION: To date, few studies have directly compared matched cohorts of patients with severe varus deformity undergoing UKA versus TKA. The current study seeks to address this gap. The hypothesis was that, in

patients with deformity of $> 15^\circ$, postoperative PROMs following UKA would be inferior to those achieved with TKA.

METHODS: Records of patients operated for UKA and TKA from January 2016 to December 2021 were evaluated, and the data for patients with deformity of $\geq 15^\circ$ were selected. The UKA patients were then matched for age and gender with TKA patients. 52 matched pairs of patients were obtained. The HKA, mLDFA, MPTA were measured. OKS, SF12, and FJS were collected. The magnitude of change in the mLDFA and MPTA was assessed using the Estimated Marginal Means.

RESULTS: The mean follow-up times for UKA and TKA were 3.7 years and 5.5 years, respectively. The mean deformity correction (Δ HKA) was significantly greater in TKA ($16.3^\circ \pm 4.5^\circ$) than UKA ($10.6^\circ \pm 3.1^\circ$) ($p < 0.001$), with significant changes in mLDFA and MPTA ($p < 0.001$). In UKAs, 68% of patients demonstrated a postoperative HKA between 171.2° and 175.7° , whereas in TKAs, values clustered between 174.5° and 179.8° . Mann-Whitney U test demonstrated no statistically significant differences between the UKA and TKA cohorts across all assessed PROMs, including OKS, SF-12, and FJS ($p > 0.05$ in all). 98% of the UKA an [...]

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Acetabular superior wall fractures: a distinct acetabular fracture entity.

Gänsslen A, Krappinger D, Liodakis E, Clausen JD, Omar-Pacha T, Lindtner RA, Sehmisch S, Osche DB

INTRODUCTION: A transitional "grey-zone" exists between typical locations of classical trapezoidal anterior wall and large-fragment posterior wall acetabular fractures. Part of this grey zone is the superior acetabular dome, which is commonly involved in acetabular fracture patterns, especially in geriatric patients. The aim of this study was to characterize isolated acetabular superior wall fractures and to assess their relevance as a distinct fracture pattern.

METHODS: A retrospective analysis of pelvic fracture databases from three tertiary trauma centers was performed. Acetabular fractures with superior dome involvement were screened, and cases with isolated superior articular involvement were included. Overall, seven patients were identified. Clinical and radiological data were analyzed regarding fracture morphology, associated fracture characteristics (including dislocation, comminution, marginal impaction, intra-articular fragments, and femoral head injury), concomitant pelvic ring injuries, treatment, and clinical outcomes.

RESULTS: Isolated superior wall acetabular fractures occurred following both high-energy and low-energy mechanisms, including simple falls. The mean age was 48 years and four of seven patients were male. Hip dislocation or subluxation was observed in four cases (anterior-superior or purely superior). Surgical treatment was performed in six cases, p [...]

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A novel biomechanical model for reproducing and analysing the causal mechanisms of PFNA "cut-in" phenomenon.

Rasappan K, Joshua K, Chou SM, Shaw LKRM, Huang D, Yew A, Kwek EBK

INTRODUCTION: "Cut-in" is a rare but serious complication associated with cephalomedullary nail fixation for intertrochanteric hip fractures, particularly with Proximal Femoral Nail Antirotation (PFNA). This phenomenon involves paradoxical superomedial migration of the helical blade through the femoral head. Although increasingly recognised, the underlying mechanisms remain incompletely defined. This study aims to describe a new superolateral tensile loading set up in a bidirectional loading model, with the aim of incorporating possible mechanical deviations introduced by abductor weakness.

MATERIALS AND METHODS: This study was conducted in two stages. An initial stage with synthetic femurs (SYNBONE® 2420) was used to calibrate the set up. Within this stage, intramedullary canal diameter was compared (12 mm vs. 18 mm). In the final stage, 5 osteoporotic synthetic femurs (Sawbones® 3503) were used. In both stages, standardised AO/OTA 31-A1.1 intertrochanteric fracture were created in the synthetic femurs and fixed with PFNA-II implants. All femurs were subjected to cyclical bidirectional loading with a universal testing machine. Each cycle consisted of axial compression (720 N) and superolateral tensile loading (120 N) at 2 Hz. This configuration was designed to simulate altered hip biomechanics postulated with the trendelenburg gait. Testing was continued until mechanical fail [...]

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Routine intraoperative infection testing in presumed aseptic hip and knee revision: a matched cohort retrospective study.

Loppini M, Rocchi C, Di Maio M, Chiappetta K, Bulgarelli A, Grappiolo G

INTRODUCTION: Periprosthetic joint infections (PJIs) complicate 1-2% of primary and up to 4% of revision arthroplasties, and their diagnosis remains challenging due to the lack of a diagnostic gold standard. Differentiating PJI from aseptic failure is particularly difficult in low-grade or biofilm-related infections. Routine intraoperative microbiological screening is not recommended in presumed aseptic revisions. This study aimed to evaluate the association between routine intraoperative microbiological screening and implant survival in presumed aseptic revision arthroplasty. A secondary objective was to identify preoperative predictors of unexpected PJIs.

METHODS: Retrospectively collected data of patients undergoing presumed aseptic THA and TKA revisions between 2013 and 2019 were included. Two matched cohorts were compared: a screened group (2016-2019) undergoing routine intraoperative microbiological sampling, and an unscreened one (2013-2015) without systematic screening. Kaplan-Meier survival analysis with log-rank testing assessed overall and infection-free survival. Univariate conditional logistic regression and receiver operating characteristic (ROC) analyses evaluated potential associations between preoperative serum markers and unexpected PJI.

RESULTS: A total of 559 patients were included, of whom 295 underwent screening and 264 did not. Unexpected infections occurred [...]

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Artificial intelligence advancements for orthopaedic clinical reasoning: longitudinal assessment of newer models (ChatGPT-5, Grok-3, Gemini 2.5 Flash) compared to clinicians.

Agharia S, Soroush S, Ameen D, Zhou Y

INTRODUCTION: This descriptive study aimed to longitudinally evaluate the performance of contemporary large language models - ChatGPT-5, Gemini 2.5 Flash, and Grok-3 - on orthopaedic clinical multiple-choice tasks, benchmarked against pooled clinician consensus. A secondary aim was to assess whether recent advances in generative AI translated into improved alignment with clinician consensus compared with previous AI models.

MATERIALS AND METHODS: A total of 97 multiple-choice clinical cases spanning eight orthopaedic subspecialties were sourced from OrthoBullets and previously benchmarked against aggregated responses from thousands of practising clinicians. Using identical methodology to our 2023 study of ChatGPT-3.5, ChatGPT-4, and Bard, each model was prompted with standardised case stems and response options. The primary outcome was the proportion of AI responses matching the most popular clinician response; secondary analyses assessed agreement within 10% and 20% of clinician consensus, performance on 'controversial' (< 25% margin) questions, and inter-model concordance using Cohen's kappa coefficients.

RESULTS: Gemini 2.5 Flash achieved the highest alignment with clinician consensus (69.1%), followed by Grok-3 (66.0%) and ChatGPT-5 (58.8%). None of the LLMs refused to respond to any prompts, representing a reduction from 7.2% from our 2023 study. Subspecialty analysis de [...]

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Trajectory of urinary incontinence symptoms following total hip arthroplasty for hip osteoarthritis.

Kadowaki M, Fukutani S, Ushio K, Uchio Y

INTRODUCTION: Urinary incontinence (UI), a prevalent quality-of-life-limiting condition, frequently coexists with hip osteoarthritis, potentially through pelvic floor dysfunction related to obturator internus muscle involvement. Several reports suggest improvement after total hip arthroplasty (THA), although postoperative time-course changes remain unclear. This study aimed to clarify the temporal trends in UI after THA and examine association of postoperative UI trajectories with functional recovery.

MATERIALS AND METHODS: This retrospective study included 118 female patients who underwent THA for hip osteoarthritis between October 2021 and March 2024. UI, assessed using the International Consultation on Incontinence Questionnaire-Short Form (ICIQ-SF), was defined as ICIQ-SF ≥ 1 preoperatively and at 3, 6, 9, and 12 months postoperatively, and classified as stress (SUI), urge (UUI), or mixed (MUI). Hip-related patient-reported

outcomes were evaluated using the Japanese Orthopaedic Association Hip Disease Evaluation Questionnaire (JHEQ). Associations with BMI, age, surgical approach (direct lateral/Bauer vs. modified Watson-Jones/OCM), and UI type were tested using Mann-Whitney U, chi-squared, and repeated-measures ANOVA tests.

RESULTS: Of 118 patients (mean age at surgery, 67 years; mean BMI, 24.3 kg/m²) preoperatively, 73 (61.9%) reported UI (mean ICIQ-SF 7.88), including S [...]

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The grand piano ratio: an adjunctive intraoperative screening tool for detecting excessive femoral component external rotation in total knee arthroplasty.

Avci Ö, Öztürk A, Güler BO, Bayrak HÇ, Kaya AÖ, Doğan Y

BACKGROUND: Accurate femoral component rotation is essential for optimal outcomes in total knee arthroplasty (TKA), yet reliable intraoperative assessment remains challenging. The "grand piano sign" has been described as a qualitative visual cue, but its quantitative clinical value has not been clearly established. This study aimed to evaluate the diagnostic performance of the grand piano ratio as an intraoperative tool for detecting excessive femoral component external rotation using advanced postoperative imaging as a reference standard.

METHODS: A retrospective analysis was conducted on 170 knees undergoing primary TKA. The intraoperative grand piano ratio was measured after completion of all femoral resections and before trial component implantation using the anterior femoral resection surface. Postoperative femoral component rotation was assessed using MAVRIC-sequence magnetic resonance imaging referenced to the surgical transepicondylar axis (sTEA). Excessive femoral component external rotation was defined as postoperative external rotation greater than 3° relative to the sTEA reference. Receiver operating characteristic (ROC) analysis was performed to evaluate diagnostic performance of the grand piano ratio and determine the optimal cutoff value, while multivariable logistic regression analysis was used to identify independent predictors of excessive external rotation. [...]

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The shoulder stiffness scale: a validated tool for monitoring stiffness in arthroscopic rotator cuff reconstructions.

Bouaicha S, Biner M, Selman F, Wieser K, Müller AM, Audigé L

BACKGROUND: Postoperative shoulder stiffness after arthroscopic rotator cuff repair is common and clinically heterogeneous. Established shoulder scores assess pain and function, but do not specifically isolate stiffness during follow-up. The aim of this study was to validate the newly developed Shoulder Stiffness Scale (SSS) as a pragmatic monitoring tool for postoperative shoulder stiffness after arthroscopic rotator cuff repair (ARCR).

METHODS: From a cohort of 973 primary arthroscopic rotator cuff reconstructions, patients without concomitant joint pathology of the same or contralateral arm were included. The new SSS was based on three items related to shoulder pain, subjective and objective ROM ranging from 0 to 10 points. It was collected before surgery, as well as 6 and 12 months postoperatively, along with other pain level parameters, Constant Score (CS), Subjective Shoulder Value (SSV), Oxford Shoulder Score (OSS). Changes of the SSS and its subscales over time, Cronbach's alpha for internal consistency, bottom and ceiling effects were reported. Validity was investigated through SSS associations with other parameters of similar dimension, and functional scores.

RESULTS: The selected cohort with 740 patients showed postoperative shoulder stiffness in 12.7 %. The SSS averaged 5.1 points (SD 2.5) preoperatively, and decreased significantly to mean 2.9 (SD 2.4) and 1.6 (S [...])

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Isolated screw fixation of posterior wall fractures.

Clausen JD, Özbek H, Omar-Pacha T, Sehmisch S, Gänsslen A

INTRODUCTION: Posterior wall (PW) fractures are common acetabular fractures and are most often treated by open reduction and internal fixation. The gold-standard of surgical stabilization is screw fixation of large wall fragments with additional buttress plating. Some reports focussed on screw fixation alone presenting adequate results.

MATERIAL AND METHODS: From a total of 208 PW-fracture treated between 1972 and 2008, 57 patients were identified with open reduction and internal fixation (ORIF) using isolated screw fixation. These patients were analyzed regarding demographical data (patient age, sex), type of accident, injury mechanism, Injury Severity Score (ISS), concomitant pelvic ring injuries, associated injuries, perioperative parameters and long-term results.

RESULTS: 44 patients were male and 13 were female with a mean age of 36.8 years. More than 90% sustained high-energy trauma with 34 sustaining multiple injuries or polytrauma. The mean Injury Severity Score was 11.5 points. All patients had fracture dislocations, which were reduced within 24 h after admission except one patient with secondary transfer. 11 patients presented with an initial sciatic nerve deficit. 29 patients had a single PW-fragment, 16 presented with two PW-fragments and in 12 patients marginal impactions were present. Surgery was performed in average after 7 days using the Kocher-Langenbeck appr [...]

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Research progress on debridement, antibiotics, and implant retention (DAIR) for the treatment of periprosthetic joint infection after artificial joint replacement.

Feng W, Zhang Z, Pu R, Wang H, Liu R, Sun Y, Zhang G

Periprosthetic joint infection (PJI) is one of the most serious complications following artificial joint replacement, posing significant treatment challenges and a heavy clinical burden. Debridement, antibiotics, and implant retention (DAIR) has become the preferred treatment strategy for acute PJI due to its advantages of minimal invasiveness, functional preservation, and cost-effectiveness. This article reviews the latest advances in optimizing the indications, timing of intervention, key technical details, and multimodal anti-infection strategies for DAIR. It focuses on discussing patient selection models (e.g., KLIC score), the impact of modular component exchange on biofilm eradication, the synergistic role of chemical debridement, and the central position of biofilm-active therapy in penetrating biofilms. Furthermore, this article proposes that future efforts should integrate molecular diagnostics, intelligent antimicrobial materials, and artificial intelligence prediction models to achieve individualized precision therapy for PJI, providing a theoretical basis for clinical decision-making.

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Tranexamic acid in high-risk shoulder arthroplasty patients: safety across thromboembolic, cardiac, renal, and neurologic risk profiles.

Parmar T, Adio A, Handy A, Daher M, Kellish A, Khan A, Abboud J

INTRODUCTION: Tranexamic acid (TXA) effectively reduces blood loss and transfusion requirements in shoulder arthroplasty. However, concerns regarding thromboembolic, neurologic, cardiac, and renal complications have limited its use in medically high-risk patients. This study evaluated temporal trends and the safety of TXA in high-risk patients undergoing total shoulder arthroplasty (TSA).

METHODS: A retrospective cohort study was conducted using TriNetX, identifying patients who underwent shoulder arthroplasty (2012-2025). Patients were stratified by preexisting high-risk conditions, including prior thromboembolism, renal failure, atrial fibrillation, seizure disorders, and visual disturbances. Within each subgroup, patients receiving TXA were propensity score-matched to those who did not. Perioperative TXA utilization trends were assessed. Ninety-day postoperative outcomes were compared, including transfusion requirements, thromboembolic events, cardiac complications, renal failure, neurologic events, infections, readmissions, emergency department visits, and mortality. Outcomes were reported as odds ratios (ORs) with 95% confidence intervals (CIs).

RESULTS: Reported perioperative TXA use began in 2012 and increased through 2025, with over 70% of patients in both standard- and high-risk cohorts receiving TXA. TXA use was associated with significantly lower transfusion rates [...]

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Osteoporosis treatment gap prior to femoral fracture and prevalence of pharmacological risk factors: a prospective observational study.

Böck L, Kerschhan-Schindl K, Frossard M, Hajdu S, Crevenna R, Anditsch M, Stemer G, Antoni A

INTRODUCTION: Hip fractures are a major complication of osteoporosis and are associated with high morbidity, mortality, and costs. Despite clear guideline recommendations, pharmacological osteoporosis treatment remains underutilized. In addition, medications that increase fall and fracture risk further contribute to fracture occurrence. The aim of this study was to assess guideline concordance of osteoporosis therapy and to quantify the prevalence of medications associated with increased fall and fracture risk prior to the fracture in patients admitted with femoral fractures.

MATERIALS AND METHODS: In this prospective observational study, 145 patients aged ≥ 55 years admitted with femoral neck, per- and subtrochanteric, femoral shaft and distal femur fractures to a tertiary academic trauma center between April and June 2025 were included. Pre-admission medication was assessed using structured medication reconciliation. Guideline concordance of pre-fracture osteoporosis therapy was evaluated according to the current Austrian osteoporosis guideline (Dimai et al., 2024). Fall-risk-increasing drugs (FRIDs) were identified using the STOPPFall criteria, and fracture-associated medications were recorded based on Austrian osteoporosis guideline-defined drug classes.

RESULTS: Among 145 included patients (mean age 80.4 years; 71% female), 117 (81%) met criteria for pharmacological oste [...]

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Gait speed recovery after iliofemoral ligament-preserving hip arthroscopy for femoroacetabular impingement: associations with early postoperative gait kinematics.

Machida S, Tsutsumi M, Utsunomiya H, Kudo S

INTRODUCTION: Whether iliofemoral ligament-preserving hip arthroscopy facilitates recovery of gait speed-an indicator of functional recovery following hip arthroscopy in patients with femoroacetabular impingement-remains unclear. We investigated whether gait speed improved after iliofemoral ligament-preserving hip arthroscopy and examined whether pelvic jerk, defined as the time derivative of acceleration and assessed using inertial sensors at 1 month postoperatively (1M), was associated with gait speed at 6 months (6M).

MATERIALS AND METHODS: Eighteen patients who underwent primary hip arthroscopy for femoroacetabular impingement were included. Gait speed and pelvic jerk were assessed during a 10-m walk at a self-selected speed, using an inertial sensor positioned at the third lumbar level. Peak pelvic jerks during the 1st and 2nd halves of the stance phase (1st - and 2nd -peak pelvic jerks) were calculated. Measurements were obtained preoperatively and at 1M and 6M. Gait parameters were compared across time points using a linear mixed model. Bayesian regression was used to compare the observed change in gait speed from preoperatively to 6M with previously reported values. Associations between gait variables at 1M and gait speed at 6M were also examined.

RESULTS: Gait speed was significantly higher at 6M than that pre surgery (preoperative, 1.15 [95% confidence interval, 1.0 [...]

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Computed tomography findings in 11,504 adult patients with traumatic brain injury: a large real-world cohort study with a S100B subgroup analysis.

Clar C, Puchwein P, Moshhammer M, Sadoghi P, Kramer D, Leithner A, Reinbacher P

OBJECTIVE: The aim of this study was to characterize acute cranial CT findings in a large cohort of adult patients with traumatic brain injury and to explore the diagnostic performance of S100B in a selected subgroup. The hypothesis was that most cranial CT scans in adult patients with traumatic brain injury would not demonstrate acute traumatic abnormalities, and that, within a selected subgroup, negative S100B levels would be associated with the absence of such findings on CT.

METHODS: This retrospective cohort study included 11,504 adult patients presenting with traumatic brain injury to a level I trauma center between 04/2016 and 05/2024. All patients underwent cranial CT, while serum S100B measurements were available in a selected subset. CT findings were classified as normal or showing acute traumatic abnormalities. In the S100B subgroup, biomarker levels were analyzed in relation to CT findings to assess their diagnostic performance for excluding acute pathology.

RESULTS: A total of 11,504 adult patients with traumatic brain injury were included; 27.4% were inpatients and 72.6% outpatients. Overall, 81.9% of CT scans showed no acute pathology, while 10.7% demonstrated

abnormalities. 7.4% of all cases were excluded. Acute TBI-related interventions were rare (3.7% of inpatients vs. 0.3% of outpatients). In patients with dementia and those receiving anticoagulation, CT sc [...]

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Early post-operative CRP is a better predictor of DAIR failure than pre-operative CRP in total knee PJI.

Beadel H, Chaffey R, Kim K, Zhu M, Coleman B

INTRODUCTION: Debridement, antibiotics and implant retention (DAIR) surgery is often performed as a first-line treatment for periprosthetic joint infection (PJI), however, success rates are variable. This study aimed to determine whether post-operative C-reactive protein (CRP) levels predict DAIR failure.

MATERIALS AND METHODS: A multi-centre retrospective cohort study was performed over a 15-year period. All total knee arthroplasty (TKA) patients undergoing DAIR for first episode PJI were included. CRP was measured at admission and for 6 weeks post-operatively. Treatment success was defined as implant retention without the need for revision surgery or long-term suppressive antibiotics. Trends in CRP were compared between two groups: failed and successful DAIR. Receiver operating characteristic (ROC) curves were analysed.

RESULTS: 189 DAIR procedures were included, of which 49% (92) failed and 51% (97) were successful. Mean follow-up was 7.5 years. Overall, CRP trended down following surgery. Mean CRP was significantly higher in the failed DAIR group at all time points. The greatest difference was at week one post-operatively (mean 76 vs. 101, $P < 0.01$). This remained significant when patients experiencing DAIR failure within 90 days were excluded (mean 76 vs. 107, $P < 0.01$). CRP was most accurate as a predictor of failure at week one post-operatively, with an area under the [...]

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Tuberosity refixation in reverse proximal humerus fracture arthroplasty: a prospective cohort study with two-year follow-up.

Oberrauter CH, Hess F, Welter J, Pape HC, Mayer S, Dullenkopf A, Mazzucchelli R, Marintschev I

INTRODUCTION: To evaluate the outcomes of a simplified tuberosity refixation technique using a two-suture construct in patients with complex proximal humerus fractures treated with reverse shoulder arthroplasty (RSA). The technique reduces the number of fixation sutures rather than the traditional four to five, aiming to decrease operative complexity while promoting greater tuberosity (GT) healing and improving the healing rate. A secondary exploratory analysis compared clinical and radiological outcomes between cemented and uncemented humeral stem fixation.

MATERIALS AND METHODS: This prospective study included 73 patients who underwent RSA (Global Unite Fracture Reverse System, DePuy Synthes, Warsaw, Indiana, USA). Clinical and radiological outcomes were assessed at two-year follow-up. Bone quality was evaluated using the Deltoid Tuberosity Index (DTI). Functional outcomes and GT healing were analyzed, and stem fixation type was assessed in a secondary subgroup analysis.

RESULTS: The mean patient age was 79 ± 8 years (range 61-95), and 65 (89%) were female. Osteoporotic bone quality ($DTI < 1.4$) was observed in 51 (70%) of cases. At two years, the median outcome scores were: Subjective Shoulder Value, 90 (IQR 80-95); Constant-Murley score, 76 (IQR 73-81); active forward flexion, 140° (IQR 120-160); abduction, 140° (IQR 120-160); external rotation, 8 (IQR 8-8); and internal r [...]

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Is it time for a systematic and objective assessment of knee laxity? The impact of objective knee laxity after TKA on patient-reported outcomes: a comprehensive review.

Vermue H, Klincke V, Stroobant L, Tampere T, Victor J, Arnout N

PURPOSE: Over the past decade, robot assistance has emerged as an innovative and rapidly advancing tool in total knee arthroplasty (TKA). Despite technological advancements, the evaluation of the soft-tissue envelope during TKA often relies on subjective and rudimentary assessments by the surgeon. This literature review explores the relationship between objective intraoperative and postoperative laxity measurements in the coronal and sagittal planes and patient-reported outcome measures (PROMs) following TKA.

METHODS: A literature review was conducted using PubMed/MEDLINE and Embase to identify studies on intra- or postoperative laxity measurements in TKA and their correlation with PROMs. Inclusion criteria required studies on TKA patients with quantifiable coronal and/or sagittal laxity measurements and postoperative PROMs linked to these measurements.

RESULTS: Thirty-four studies met inclusion criteria: ten on sagittal laxity, twenty-seven on coronal laxity, and three on both. Follow-up ranged from 1 month to over 8 years. Sagittal laxity thresholds of 5-10 mm under applied loads of 89-133 N were associated with better functional outcomes, whereas excessive (> 10 mm) or overly tight (< 5 mm) laxity correlated with reduced satisfaction. Medial laxity in the coronal plane was linked to lower PROMs, while the impact of lateral laxity on PROMs was less conclusive.

CONCLUSIONS: [...]

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Silver-impregnated occlusive dressing is associated with a lower rate of acute surgical site infection after direct anterior total hip arthroplasty.

Tsai YY, Shen PH, Wu CC, Pan RY, Lin LC, Wang SH

INTRODUCTION: The direct anterior approach (DAA) for total hip arthroplasty (THA) has gained increasing adoption in recent years; however, it has been associated with a higher risk of wound-related complications compared with other surgical approaches. Silver-impregnated occlusive dressings have demonstrated antimicrobial potential, though their role in DAA THA remains underexplored.

MATERIALS AND METHODS: We conducted a retrospective cohort study with a historical control group, reviewing 321 consecutive primary DAA THAs performed by a single surgeon (September 2020-December 2025). Standard sterile gauze was used before August 2021 ($n = 76$) and a silver-impregnated occlusive dressing (AQUACEL® Ag SURGICAL; ConvaTec, Deeside, UK) thereafter ($n = 245$). Despite the chronological difference between cohorts, baseline demographic and clinical characteristics did not differ significantly between groups (Table 1). The primary endpoint was acute surgical site infection (SSI) within 30 days (CDC criteria); all patients had ≥ 3 months follow-up to capture early wound-related reoperations as secondary outcomes.

RESULTS: Four acute SSI events occurred (1.25%). SSI incidence was 3.94% (3/76) with gauze versus 0.41% (1/245) with the silver-impregnated dressing (Fisher's exact $p = 0.043$). This corresponds to an absolute risk reduction (ARR) of 3.54% (95% CI, 0.34-10.57) and a number needed to treat [...]

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Minimally invasive stabilisation of anterior pelvic ring injuries through anterior infix versus percutaneous anterior retrograde pubic screw: a prospective comparative cohort study.

Goda El-Hamrawy A, Abdelmoneim M, Al-Sayed M, Youness M, Sadek F, Kassem E

BACKGROUND: Management of unstable anterior pelvic ring fractures has shifted toward minimally invasive fixation techniques. This prospective randomized cohort study compares anterior subcutaneous internal fixator with percutaneous retrograde pubic ramus screw fixation, evaluating radiological outcomes, functional scores, complications, and peri-operative parameters.

METHODS: Fifty adult patients with unstable anterior pelvic ring injuries (Tile B & C; Young-Burgess lateral compression and vertical shear mechanisms) were enrolled between 2023 and 2025 and randomized into two equal groups: group 1 (internal fixator) and group 2 (retrograde pubic screw). Both groups underwent individualized posterior ring fixation as required. Outcomes assessed included operative time, blood loss, fluoroscopy time, union rate, Matta radiological score at 6 months, Majeed and Pelvic Outcome Scores at 12 months, and post-operative complications.

RESULTS: Management of anterior pelvic injury showed that retrograde screw fixation demonstrated significantly shorter operative time (32.0 ± 9.2 vs. 50.3 ± 8.4 min, $p < 0.001$) and less intraoperative blood loss (68.3 ± 16.2 vs. 122.3 ± 31.8 ml, $p < 0.001$). Radiological outcomes were comparable: excellent Matta score in 68% vs. 64% ($p = 0.834$). Union time showed no significant difference (14.1 ± 2.4 vs. 15.1 ± 1.5 weeks, $p = 0.076$). Functional outcomes at [...]

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Prioritisation of assessments, diagnostic classifications, and outcome measures in Perthes disease: a Delphi survey of international health professionals.

Ball S, Davies LM, Gray K, Pacey V

INTRODUCTION: The aim of this study is to reach consensus and prioritise the most clinically important assessments, diagnostic classifications, and outcome measures by an international panel of experts for Legg-Calve-Perthes disease.

MATERIALS AND METHODS: A three-round modified e-Delphi survey was conducted between April and May 2025. Participants rated the clinical importance of 14 clinical assessments, 26 radiological assessments, eight diagnostic classifications, and 26 outcome measures identified by a scoping review. Consensus was defined as $\geq 75\%$ agreement. In the final round, items in each category were prioritised for overall condition and by specific age groups (< 6 years, 6-8 years, > 8 years).

RESULTS: Thirty-eight health professionals, from nine countries participated in round 1. Thirty-four completed all three rounds, a retention rate of 89.5%. Consensus and prioritisation for clinical use were achieved for 10 clinical assessments, three radiological assessments, three diagnostic classifications, and three outcome measures.

CONCLUSION: Paediatric specialists have reached consensus and prioritised methods to evaluate and diagnose Legg-Calve-Perthes disease in clinical practice. These results represent an important step towards improving continuity of care and the foundation for future development of clinical practice guidelines.

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Compensatory activation of the deltoid and remaining rotator cuff muscles in patients with rotator cuff tears using real-time tissue elastography.

Hoshikawa K, Oishi R, Yuri T, Uno T, Nagai J, Giambini H, Mura N

INTRODUCTION: Rotator cuff (RC) tears are a prevalent condition leading to dysfunction and shoulder pain. Spatial changes in the behavior of the deltoid and remaining RC muscles during arm elevation in patients with RC tears have not been investigated. The purpose of this study was twofold: (1) to investigate factors related to the changes in the deltoid muscle behavior during scapular plane abduction (scaption) in patients with RC tears, and (2) to examine the functional relationship between the deltoid and remaining RC muscles, particularly remaining infraspinatus (ISP) and teres minor (TMin) muscles, in these patients using ultrasound elastography.

METHODS: Twenty-four patients (mean age, 66 ± 6 years) with RC tears (mean antero-posterior diameter, 17.4 ± 8.3 mm) were evaluated. Antero-posterior and medio-lateral diameters of the posterosuperior RC and Subscapularis (SSC) tears were measured intra-operatively. Muscle elasticity was evaluated at passive and active states, as a proxy for muscle activation, at 30° , 60° , and 90° during scaption for the deltoid, ISP, and TMin muscles using real-time tissue elastography. Based on the deltoid muscle activity ratio at 30° and 90° scaption, patients were divided into normal and abnormal activation groups.

RESULTS: Activity of the deltoid muscle in the abnormal activation group exhibited consistently high activity across various ele [...]

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Pin orthosis extension block pinning versus conservative treatment for doyle type 4B mallet fractures.

Col KA, Demirsu O, Aydin M, Surucu S, Yilmaz M, Atlihan D

INTRODUCTION: Although numerous treatment options have been reported for mallet fractures, a universally accepted gold-standard approach has not yet been established. The purpose of this study was to compare the clinical and radiographic outcomes of the pin-orthosis extension-block pinning technique (PO-EBPT) with those of conventional conservative treatment in patients with Doyle type 4B mallet fractures.

MATERIALS AND METHODS: This study included 62 patients with Doyle type 4B mallet fractures involving 20-50% of the distal interphalangeal (DIP) joint articular surface, treated between March 2022 and April 2024. Patients were randomized into two groups: Group 1 (n = 33) underwent PO-EBPT, whereas Group 2 (n = 29) received conservative treatment with splint immobilization. Outcome measures included DIP joint extension lag, range of motion, fracture union, complication rates and functional outcomes according to the Crawford criteria.

Follow-up evaluations were performed at 2, 4 and 6 weeks and at 3, 6 and 12 months.

RESULTS: A total of 62 patients were analyzed (33 PO-EBPT; 29 conservative). No statistically significant differences were observed between the groups with respect to sex, affected side, injured finger, or complication rates ($p = 0.461$, $p = 0.658$, $p = 0.763$ and $p = 0.165$, respectively). However, the PO-EBPT group demonstrated significantly improved DIP joint exten [...]

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Impact of cannabis use and tobacco smoking on outcomes of open carpal tunnel release surgery: a nationwide study in the United States.

Hoveidaei AH, Esmaeili S, Gilmor R, Chen Z, Conway JD, Ingari JV

PURPOSE: To evaluate the impact of cannabis use, tobacco smoking, and their concurrent use on postoperative complications following open carpal tunnel release (CTR) using a nationwide database.

METHODS: A retrospective cohort study was conducted using the PearlDiver database (2010-2022) to identify adult patients who underwent open CTR. Patients were categorized into four groups: cannabis-only users ($n = 800$), tobacco-only users ($n = 167,803$), concurrent users ($n = 3529$), and non-users ($n = 167,803$ matched controls). Postoperative complications, surgical site infection, wound disruption, joint stiffness, and complex regional pain syndrome, were assessed at 6 weeks and 3 months postoperatively.

RESULTS: At 6 weeks and 3 months, surgical site infection was more common among concurrent cannabis and tobacco users, cannabis-only users, and tobacco-only users. Wound disruption occurred more frequently in concurrent users and tobacco-only users, but not in cannabis-only users. Complex regional pain syndrome was associated only with tobacco use. No association was found between any substance use and joint stiffness.

CONCLUSION: Cannabis use, tobacco smoking, and especially concurrent use increase the risk of surgical site infection and wound disruption following open CTR. Tobacco use is independently associated with complex regional pain syndrome, while cannabis may have a protectiv [...]

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Timing and microbiological profile influence long-term outcomes after debridement, antibiotics, and implant retention (DAIR) in acute hip periprosthetic joint infection.

Muñoz-Mahamud E, Perdomo-Lizarraga JC, Combalia A, Alías A, Serra A, Fernández-Valencia JÁ, Verdejo MÁ, Soriano Á

INTRODUCTION: The strategy of debridement, antibiotics, and implant retention (DAIR) represents one of the main therapeutic modalities for acute periprosthetic joint infection (PJI). However, reported outcomes remain highly variable, with success rates ranging from 16 to 92%. This study aimed to evaluate long-term treatment failure-free survival and identify risk factors for treatment failure in patients with acute hip PJI treated with DAIR using time-to-event analytical methods.

MATERIAL AND METHODS: This retrospective study evaluated the treatment failure rate in 115 patients treated with the DAIR strategy for acute PJI following primary or aseptic revision hip arthroplasty between 1999 and 2018. Potential predictors of treatment failure-free survival, including patient characteristics, infection microbiology, and surgical timing, were analyzed using univariate tests, Cox proportional hazards regression (hazard ratio [HR]), and Kaplan-Meier survival estimates.

RESULTS: Among the 115 patients included, 56 were women and 59 were men; 48 were aged 75 years or younger and 67 were older than 75 years. In 88 cases, the infection occurred after primary arthroplasty, whereas 27 followed aseptic revision surgery. The mean follow-up was 7.1 years (SD 4.4). At five years, treatment failure occurred in 30.4% of cases. Procedures performed within seven days of diagnosis showed lower fai [...]

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Surgical treatment of varus unicompartmental knee osteoarthritis: indications, trends, and outcomes-a narrative review.

Burgio C, Bosco F, Cobisi C, Zepeda K, Giustra F, Capella M, Mayman D, Debbi E et al.

INTRODUCTION: Varus unicompartmental knee osteoarthritis (UC-KOA) is a frequent and heterogeneous clinical condition characterized by compartment-specific cartilage degeneration and frequently associated with lower-limb malalignment. The progressive understanding of knee biomechanics and alignment has led to the development of surgical strategies specifically tailored to address isolated compartment disease. The purpose of this narrative review is to evaluate indications, current trends, and clinical outcomes of the main surgical options for varus UC-KOA, with particular focus on realignment osteotomies and unicompartmental knee arthroplasty (UKA).

MATERIALS AND METHODS: A narrative review of the literature was conducted using major biomedical databases. Evidence related to biomechanics, alignment correction, osteotomy techniques, UKA indications, implant evolution, complication profiles, and comparative outcomes was analyzed and synthesized thematically.

RESULTS: Biomechanical and clinical evidence consistently identifies malalignment as a key driver of isolated compartment degeneration. Corrective osteotomies effectively restore physiological load distribution and provide durable outcomes in selected patients with extra-articular deformity. Advances in implant design and surgical techniques have substantially improved UKA survivorship, allowing broader patient selection with [...]

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Preoperative NarxCare overdose risk scores greater than 100 are associated with worse PROMs improvements and dissatisfaction after primary THA.

Khan ST, Emara AK, Patel SV, Elmenawi KA, Ibaseta A, Pasqualini I, Zhang C, Cleveland Clinic Adult Reconstruction Research Group et al.

INTRODUCTION: The NarxCare Overdose Risk Score (ORS) is a weighted measure of patient-specific prescription drug use. In patients undergoing primary total hip arthroplasty (THA), this study aimed to evaluate (1) the trend of the ORS from preoperative to 1-year postoperatively, and (2) the association of preoperative ORS with achievement of clinically meaningful improvements in patient reported outcome measures (PROMs), and satisfaction at 1-year.

METHODS: All patients who underwent primary unilateral elective THA at a USA tertiary healthcare system from January 2016-December 2022 were eligible. Patients with incomplete PROMs or missing baseline NarxCare ORS were excluded, leading to the inclusion of 5,424 patients. Multivariable regression models were used to assess the association between ORS and 1-year PROMs, including achievement of the minimum clinically important difference (MCID) and Patient Acceptable Symptom State (PASS) for the Hip Disability and Osteoarthritis Outcome Score (HOOS) subdomain scores, as well as satisfaction.

RESULTS: Preoperatively, 46.1% (n = 2501) had a NarxCare ORS of 0, 24.6% (n = 1336) had scores between 100 and 199, and 16.2% (n = 876) in the 200-299 range. After adjusting for confounding variables, a preoperative ORS of 100-199 was associated with failure to achieve MCID in HOOS Pain (OR 1.91;CI 1.32-2.77;p = 0.001), and HOOS JR (OR 1.54;1.10-2 [...])

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Only a partial shield: local vancomycin postpones but does not prevent hip and knee prosthetic infections.

Darup A, Ettinger M, Savov P, Brand S, Seeber GH, Stauss R

INTRODUCTION: Periprosthetic joint infection (PJI) represents a severe complication following total hip arthroplasty (THA) and total knee arthroplasty (TKA). The intraarticular application of vancomycin powder is increasingly integrated into perioperative standards as an innovative strategy for PJI prevention. Yet the efficacy of this method remains a topic of debate. The aim of this retrospective cohort study was to evaluate whether topical vancomycin application reduces the incidence of PJI after primary total joint arthroplasty (TJA) and whether it is associated with an increased risk of wound complications.

MATERIALS AND METHODS: In this retrospective monocentric cohort study, all primary THA and TKA procedures performed between January 1, 2022, and December 31, 2023, were analysed. From January 1, 2023, intraarticular vancomycin powder was implemented as part of the standard perioperative protocol for PJI prevention. The PJI rates, postoperative surgery-related complications, and time to infection were compared between the vancomycin and control groups.

RESULTS: A total of 1,499 patients were included in the study. Patients were divided into two groups: a vancomycin group (VG, n = 818) and a control group (CG, n = 681). No statistically significant group differences were observed for wound healing disorders ($p = 0.775$) or PJI rates (1.0% VG vs. 0.9% CG, $p = 0.846$). However [...]

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Validity and prognostic value of a novel intraoperative assessment tool for reduction in trimalleolar fractures: the SGSC checklist approach.

Gao H, Xu G, Ma J, Wang Y, Wang J, Zhou R, Liu Z, Wang B

BACKGROUND: Trimalleolar fractures present substantial surgical challenges, and the quality of fracture reduction significantly influences patient outcomes. Current intraoperative assessment methods, including fluoroscopy and arthroscopy, each possess inherent limitations when used independently. This study aimed to validate the effectiveness of a novel intraoperative evaluation tool, the SGSC (Step, Gap, Syndesmosis, Congruity) "traffic light" checklist, that integrates arthroscopic and fluoroscopic assessment, and to evaluate its predictive value for clinical prognosis.

METHODS: This single-center retrospective cohort study enrolled 36 patients who underwent surgical treatment for trimalleolar fractures. Validation proceeded in two stages: first, we calculated inter-rater reliability (Kappa statistic) and concordance with postoperative CT as the reference standard to establish the reliability and validity of the SGSC checklist (accuracy validation); second, patients were stratified into "All-Green" and "Non-All-Green" groups based on intraoperative SGSC assessment, and we compared AOFAS scores and early radiographic degenerative changes at final follow-up (prognostic validation).

RESULTS: For accuracy validation, inter-rater agreement between the two assessors demonstrated excellent reliability (Kappa = 0.827; 95% CI: 0.644-1.000). The overall concordance between SGSC check [...]

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Radiographic assessment of femoral, acetabular and global offset following hip spacer implantation in staged total hip arthroplasty".

Cavagnaro L, Ferrari E, Providenti V, Marciante V, Carrega G, Formica M

BACKGROUND: The number of total hip arthroplasties (THAs) performed worldwide is steadily increasing, and, consequently, the demand for revision procedures is expected to rise as well. Two-stage revision procedure continues to be considered the gold standard in many clinical scenarios. During the first step, failure to restore femoral offset has been implicated as a contributor to instability and mechanical failure, although a clear consensus is still lacking.

OBJECTIVES: The primary aim of this study is to evaluate the restoration of biomechanical parameters of the hip—specifically leg length discrepancy (LLD), femoral offset (FO), acetabular offset (AO), and global offset (GO)—following implantation of a specific type of articulating hip spacer during the first stage of a two-stage revision for PJI. A secondary objective is to assess the correlation between variations in offset parameters and the rate of interstage mechanical complications.

MATERIALS AND METHODS: We retrospectively reviewed all patients undergoing staged revision with a specific articulating spacer between 2020 and 2022 at a single institution. Harris Hip Scores, Oxford Hip Scores, and Visual Analogue Scales for pain were obtained at different time points. Radiographic analysis included LLD, FO, AO and GO measurements on the affected (spacer and post-reimplantation) and the contralateral side. Complications [...]

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Simulated tears of antero-posterior rotator cuff force-couple and reduced glenoid concavity decrease anterior glenohumeral stability: a robot-assisted biomechanical analysis of a load and shift sequence.

Karlsfeld MJ, Wermers J, Tänzler S, Sußiek J, Herbst E, Katthagen JC

BACKGROUND: The glenoid concavity and the compression applied by the rotator cuff (RC) are essential for glenohumeral stability (GHS). This study aimed to determine how different simulated rotator cuff tears (RCT) and the glenoid depth influence GHS.

METHODS: A Load and Shift sequence was performed with eight fresh-frozen cadaveric shoulders in a robotic-assisted setup. Differently configured static loading of the reinforced RC and deltoid muscle (DLT) simulated intact RC and anterior, superior, anterosuperior, posterosuperior, mass, and complete RC plus DLT tears. Anterior dislocation forces and their changes were determined as indicators of stability. The glenoid depth and Bony shoulder stability ratio (BSSR) were defined as indicators of concavity. To assess GHS, the maximal force (Favg), the maximal force increase (dFavg), and their mean deviations ($\Delta F_{\max}$, $\Delta dF_{\max}$) to the intact configuration during the sequence were evaluated.

RESULTS: Simulated tears of the subscapularis tendon (SCP) ($\Delta F_{\max} = 7.90$ N, $\Delta dF_{\max} = 1.54$ N/mm) and simulated tears of the infraspinatus (ISP) + teres minor (TM) tendons ($\Delta F_{\max} = 7.19$ N, $\Delta dF_{\max} = 1.48$ N/mm) resulted in greater differences to the intact shoulder than simulated tears of the supraspinatus tendon ($\Delta F_{\max} = 3.82$ N, $\Delta dF_{\max} = 1.16$ N/mm). High correlations were observed between maximal force concerning glenoid depth ($r = 0.81$) and BSSR ($r = 0$ [...])

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Non-tobacco nicotine dependence is associated with increased complications following clavicle open reduction internal fixation.

Adio AA, Goyal RK, Skiena A, Daher M, Horneff JG, Kassam HF, Hill BW, Abboud JA

BACKGROUND: Tobacco dependence is well established to impair bone healing and increase postoperative complications. However, the independent effects of nicotine, separate from combustion-related factors, remain unclear. This study aimed to evaluate the impact of non-tobacco nicotine dependence (NTND) following open reduction and internal fixation (ORIF) of clavicle fractures, including midshaft and lateral fracture subtypes.

METHODS: A retrospective cohort study was performed using a large national database. Patients undergoing ORIF of clavicle fractures were identified using procedural and diagnostic codes. Patients with non-tobacco nicotine dependence (e.g., vaping, patches, or gum) were identified and propensity score matched to (1) patients without nicotine exposure and (2) patients with tobacco nicotine dependence. In a secondary analysis, patients were stratified by fracture type (midshaft and lateral), and outcomes were compared between nicotine exposure and no nicotine exposure within each subgroup. Outcomes were assessed at 90 days and 1 year and included medical complications, number of opioid prescriptions, and implant-related outcomes. Risk ratios (RRs) with 95% confidence intervals (CIs) were calculated, with $p < 0.05$ considered significant.

RESULTS: After matching, 936 non-tobacco nicotine users were identified. Non-tobacco nicotine dependence was associated with [...]

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Association between timing of surgery and postoperative outcomes in older adults with distal femur fractures: a systematic review and meta-analysis.

Hoshika S, Yamamoto N, Saka N, Tsuge T, Sato T, Isaji Y, Nakao S

INTRODUCTION: The association between surgical timing and clinical outcomes in older adults with distal femur fractures remains unclear. We conducted a systematic review and meta-analysis to evaluate whether earlier surgery is associated with improved outcomes in this population.

MATERIALS AND METHODS: In accordance with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) guidelines, two independent reviewers searched MEDLINE, Embase, the Cochrane Central Register of Controlled Trials, ClinicalTrials.gov, and the World Health Organization International Clinical Trials Registry Platform from inception to October 8, 2024. The primary outcome was 30-day mortality, and secondary outcomes included longer-term mortality and postoperative complications. Risk of bias was assessed using the Quality in Prognosis Studies tool, and the certainty of evidence was evaluated using the Grading of Recommendations Assessment, Development and Evaluation approach.

RESULTS: Of 6,101 records screened, 16 retrospective cohort studies comprising 31,213 patients met the inclusion criteria. Early surgery was not associated with reduced 30-day mortality (adjusted odds ratio [OR]: 0.77, 95% confidence interval [CI]: 0.56-1.05; low-certainty evidence) or with longer-term mortality at 90 days (crude OR: 0.91, 95% CI: 0.51-1.60), 180 days (crude OR: 0.46, 95% CI: 0.12-1.77), or 1 [...]

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Is a central cavity necessary for bioactive glass-ceramic spacers in plated ACDF? A retrospective comparison of solid versus cavity designs.

Kang TH, Lee G, Kang B, Han J, Cho M, Lee JH

PURPOSE: Traditionally, interbody cages feature a central cavity for graft packing. However, for bioactive glass-ceramic (BGS-7) spacers achieving fusion via surface-mediated osseointegration, this cavity may be biologically redundant and structurally disadvantageous. We compared radiological and clinical outcomes of plated anterior cervical discectomy and fusion (ACDF) using BGS-7 spacers with or without a central cavity.

METHODS: We retrospectively reviewed patients undergoing plated ACDF using solid non-cavity (17 patients, 24 levels) or central-cavity (17 patients, 33 levels) BGS-7 spacers. Radiographic fusion was evaluated using dynamic interspinous distance (primary, ≤ 2 mm) and segmental angular motion (secondary, $\leq 4^\circ$), alongside CT-based osseointegration (direct contact $\geq 50\%$). Subsidence (≥ 2 mm) and clinical outcomes were evaluated at 12 months.

RESULTS: Both groups demonstrated comparable 12-month clinical improvements ($p > 0.05$). Radiographic fusion rates were equivalent between non-cavity and central-cavity groups based on the primary distance (91.7% vs. 84.8%, $p = 0.687$) and secondary angular criteria (79.2% vs. 63.6%, $p = 0.331$). The non-cavity group showed a trend toward superior segmental stability, with a smaller dynamic gap distance (0.82 ± 0.64 mm vs. 1.53 ± 1.99 mm, $p = 0.064$). CT-based fusion perfectly matched (90.5% vs. 90.9%, $p = 1.000$). Subsidence ra [...]

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Injury

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Medial clavicle resection for acute traumatic posterior sternoclavicular dislocation: Experience from Grenoble Alpes University Hospital.

Datry V, Gagnepain E, Guillot M, Pirvu A, Frey G

BACKGROUND: Posterior sternoclavicular dislocations (PSCDs) are rare but potentially life-threatening injuries. Beyond the acute management of mediastinal complications, restoring optimal shoulder function remains the primary goal of treatment. Although the optimal surgical strategy remains controversial, our institutional protocol consists of systematic medial clavicle resection performed during emergency surgery, with the aim of reducing the risk of residual sternoclavicular instability. The aim of this study was to evaluate the functional outcomes and complications of systematic medial clavicle resection performed as an emergency procedure for acute traumatic PSCDs.

METHODS: A retrospective single-centre study was conducted at Grenoble Alpes University Hospital (2019-2026). All consecutive patients who underwent medial clavicle resection for acute traumatic PSCDs were included. The primary outcome was the QuickDASH score at 12 months. Secondary outcomes included the QuickDASH score at 1-month, postoperative complications according to the Clavien-Dindo classification, and return-to-work and return-to-sport outcomes.

RESULTS: Twenty-five patients (23 males, 2 females; mean age 29 years) were included. Sports injuries accounted for 84% of cases. The median time from injury to surgery was 2 days [IQR, 1-6]. The median operative time was 33 min [IQR, 24-45]. No intraoperative c [...]

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Operative versus nonoperative management of pelvic ring fractures in older adults: A propensity-matched analysis of mortality and complications.

Johnson M, Sabat L, Allam J, Perdue P, Zuelzer D

INTRODUCTION: Pelvic ring fractures are an increasingly common injury in the aging population, carrying mortality comparable to hip fractures and substantial functional decline. Most geriatric fractures are managed nonoperatively, with operative fixation reserved for unstable patterns, but high-quality evidence comparing both treatment strategies is lacking. This study compared mortality and complications between operatively and nonoperatively managed pelvic ring fractures in older adults across multiple time points.

METHODS: This retrospective cohort study used the TriNetX database to identify patients aged 55 years or older with a pelvic ring fracture. Patients were assigned to operative or nonoperative cohorts and matched 1:1. Co-primary outcomes were 30-day and 1-year all-cause mortality. Secondary outcomes were mortality at 90 days and 2 years, and VTE, pneumonia, and MI at each time window. A multivariable Cox model was constructed for 1-year mortality.

RESULTS: Following matching, 5314 patients were retained in each cohort. Mortality did not differ at 30 days ($p = 0.108$) or 90 days, but was significantly lower in the operative cohort at 1 year ($p = 0.034$) and 2 years ($p < .001$). This benefit did not persist in the multivariable Cox model (HR 0.939; $p = 0.296$). VTE and pneumonia were significantly more common in the operative cohort at all time points, while MI did not [...]

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Determinants of road traffic injury preventive behaviors among old pedestrians: An application of the PRECEDE model.

Bashirian S, Ayubi E, Nezafati A, Razi T, Hosseinalipour SA, Karami S, Rezapur-Shahkolai F

OBJECTIVES: Nowadays, road traffic injuries (RTIs), a major public health concern in many countries, have gained even greater significance among elderly, especially among elderly pedestrians who are one of important vulnerable groups of road users. This study aimed to identify factors influencing RTI preventive behaviors in older pedestrians using the PRECEDE model.

METHODS: A cross-sectional study was conducted in 2024 with 417 adults aged 60 years or older attending comprehensive health centers in Hamadan, located in the west part of Iran. Participants were selected using a multi-stage cluster sampling method. Data were collected via a validated, self-administered questionnaire based on the PRECEDE model, covering demographic characteristics, behavioral and environmental factors, as well as predisposing, reinforcing, and enabling factors.

RESULTS: The study population consisted of 240 (57.6%) males and 177 (42.4%) females. Multi-variable analysis showed that environmental factors ($\beta = 0.16$, $p < 0.01$) and reinforcing factors ($\beta = 0.12$, $p = 0.04$) were significant positive predictors of preventive behaviors. Additionally, education level, marital status, hearing aid use, and insurance coverage demonstrated significant associations with RTI preventive practices ($p < 0.05$).

CONCLUSIONS: The PRECEDE model offers a useful framework for understanding RTI preventive behaviors among [...]

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Structures to improve the implementation of mental health practices in adult trauma centers: A Canadian nationwide practice survey.

Bérubé M, Macedo T, O'Donnell M, Laberge-Séguin M, Moore L, Stelfox HT, Rivest M, Kureshi N et al.

BACKGROUND: Up to one third of trauma patients experience mental disorders, placing them at greater risk of morbidity and healthcare utilization. However, few studies have examined knowledge-to-practice gaps when it comes to improving their mental health. This study aims to assess how trauma program leaders perceive the implementation of structures to facilitate the adoption of recommendations aimed at improving mental health outcomes in trauma patients.

METHODS: We conducted a cross-sectional practice survey across Levels I to III Canadian trauma centers ($n = 69$). The survey focused on empirically supported structures for optimizing mental health practices. Responses were analyzed using descriptive statistics and content analysis.

RESULTS: The overall response rate was 62%, with 75% in Level I trauma centers, 91% in Level II centers, and 47% in Level III centers. The structures most frequently perceived as having been implemented were: referral mechanisms to a rehabilitation program to improve psychological outcomes in severely injured patients (58%), referral mechanisms to mental health resources for patients with a psychological condition that contributed to the injury (56%), protocols for screening suicidal ideation (56%) and substance use disorders (51%), protocols for the pharmacological treatment of aggression in patients with traumatic brain injury (TBI) (51%), and fo [...]

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Trends, disparities, and forecasts of osteosarcoma mortality in the United States, 1999-2035.

Gu Y, Wang T, Liu B

BACKGROUND: Osteosarcoma-related mortality in the United States remains a concern, particularly in the context of population aging.

METHODS: We used the CDC WONDER Underlying Cause of Death database (1999-2023) to identify osteosarcoma deaths (ICD-10 C40-C41). Crude mortality rates and age-adjusted mortality rates (AAMRs) per 100,000 (standardized to the 2000 U.S. population) were calculated for individuals aged ≥ 15 years and stratified by sex, age group, race/ethnicity, census region, and urban-rural classification. Temporal trends, annual percent change (APC), and average annual percent change (AAPC) were estimated using Joinpoint regression. Future mortality trends through 2035 were projected using autoregressive integrated moving average (ARIMA) and exponential smoothing state-space (ETS) models.

RESULTS: From 1999-2023, the overall AAMR increased from 0.52 to 0.59 per 100,000 (AAPC 0.67; 95% CI 0.34-0.99). Men consistently had higher mortality than women. Mortality was strongly age-graded, peaking among adults aged ≥ 85 years, whereas rates in those aged 15-24 remained essentially stable. Non-Hispanic White individuals and residents of the South had the highest AAMRs, while the Northeast showed the lowest. Nonmetropolitan areas exhibited higher AAMRs than metropolitan areas, despite faster recent increases in cities. Forecasts based on ARIMA and ETS models indicate that [...]

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Disparities in traumatic brain injury-related deaths - United States, 2023.

Waltzman D, Daugherty JD, Thomas KE

INTRODUCTION: Traumatic brain injury (TBI) is a significant public health problem. Conducting regular surveillance of TBI-related deaths is important in order to determine which groups are at disproportionate risk of dying from a TBI, tracking trends over time, and informing prevention efforts.

METHODS: TBI-related deaths from 2023 were analyzed using multiple-cause-of-death data from the National Vital Statistics System by age, sex, race/ethnicity, injury intent/mechanism of injury, and geography. TBI-related deaths were identified if a death had an injury-related International Classification of Diseases, Tenth Revision underlying cause of death and a TBI-related diagnosis code in one of the multiple-cause-of-death fields.

RESULTS: In 2023, there were 68,663 TBI-related deaths in the United States. Older adults aged ≥ 75 years (82.5 per 100,000 population), males (29.1), non-Hispanic American Indian/Alaska Native (AI/AN) individuals (29.7), and several states in the West US Census region displayed the highest rates of TBI-related deaths compared to their respective groups. Suicide (rate of 7.0 [36.0% of all TBI-related deaths]) was the most common cause of TBI-related deaths in the United States in 2023, followed by unintentional falls (rate of 5.2 [31.6% of all TBI-related deaths]).

CONCLUSIONS: Results show that deaths due to TBI are common but not all groups in the United States [...]

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Understanding the outliers: A qualitative analysis of leaders' perspectives on performance and safety in trauma centers.

Hamad DM, Conn LG, Haas B, Jerath A, Nathens AB

INTRODUCTION: The factors explaining variability in trauma center performance across quality measures are not well understood. We aimed to characterize the influence of social, cultural, and organizational factors on trauma center performance.

METHODS: We used an explanatory sequential design to study ACS TQIP-participating level I and II trauma centers that were ranked as high- and low-performing (first and last decile) across multiple representative quality indicators. The Chief Safety Officers, Directors of Quality and Safety, Trauma Medical Directors, and Trauma Program Managers within these centers were approached for semi-structured interviews guided by the framework of high-reliability organizations (HROs). Qualitative description methodology was used. Salient themes were inductively developed, and themes between centers in the first and last deciles were contrasted.

RESULTS: We interviewed 28 participants across 15 centers. Of these, 8 centers were in the first decile, and 7 centers were in the last decile. All participants expressed a commitment to making care safer and to improving hospital performance. Leaders in first decile centers more strongly expressed having adopted and used theory pertinent to safety and high reliability organizations (HROs). This manifested as an increased preoccupation with failure and with the ongoing and never-ending maintenance of safe [...]

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Epidemiology of hand fractures in children - A population-based study with data from the Danish National Patient Registry.

Raja A, Faergemann C, Gundtoft P, Viberg B

INTRODUCTION: Hand fractures are among the most common pediatric injuries, but despite their prevalence, comprehensive data on incidence and anatomical distribution remain sparse. The aim of this study was to report the incidence and anatomical distribution of hand fractures in children aged 0-15 years.

METHODS: This population-based register study utilized data from the Danish National Patient Registry (DNPR) over a 20-year period from 1999 to 2018. All cases of hand fractures were identified using ICD-10 codes and stratified by age, sex, anatomical site, and treatment type. Fracture incidences were calculated based on annual population data, gathered from Statistic Denmark. Data was analyzed using descriptive statistics and results presented in the form of number of cases and percentages.

RESULTS: A total of 158,070 hand fractures were recorded, with an average annual incidence of 748 per 100,000 children/year and a 17% increase in incidence over the study period. Boys accounted for 59% of cases, and the highest incidence was observed in the 12-15-year age group (51%). Finger fractures were the most common (60%) followed by metacarpal fractures (19%). There were 1% that received surgical treatment.

CONCLUSION: We found a 17% increase in incidence over the 20-year period. The highest incidence was in the 12-15-year-olds. Boys had the majority of fractures, and finger fractu [...]

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Comparison of anterior-only and combined anterior-posterior approaches for complex acetabular fractures.

Kaptan AY, Dolap MA, Göçer E, Demir M, Varol R, Bal T, Durmaz Ü, Yılmaz AE

BACKGROUND: Complex acetabular fractures involving both columns (T-type, both-column, and anterior column with posterior hemitransverse) often require combined anterior-posterior surgical exposures. Advances in anterior intrapelvic fixation raise the possibility that the posterior column can be reduced and stabilized from the intrapelvic side alone in selected cases, avoiding an additional posterior exposure. We compared radiological and clinical outcomes between an anterior-only intrapelvic strategy and a combined anterior-posterior strategy.

METHODS: This retrospective comparative study included 31 patients with T-type, both-column, or ACPHT acetabular fractures: 12 treated through the modified Stoppa approach alone (anterior-only) and 19 treated with combined Kocher-Langenbeck and anterior intrapelvic exposures (combined). Postoperative CT-based reduction quality (Matta-based thresholds), Harris Hip Score (HHS), Majeed Pelvic Score, complications, and perioperative transfusion requirements were compared between groups.

RESULTS: Anatomic reduction was achieved in 50.0% of the anterior-only group and 36.8% of the combined group ($p = 0.710$). Native-hip HHS (83.0 vs. 81.0, $p = 0.229$) and Majeed scores (76.0 vs. 79.0, $p = 0.795$) were comparable between groups. Total cumulative erythrocyte suspension transfusion was higher in the combined group (median 3 vs. 2 units, $p = 0.003$), [...]

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Return to duty after suture anchor repair of thumb metacarpophalangeal ulnar collateral ligament injuries in active duty soldiers.

Lee J, Chung JI, Yun Y, Jeong C, Lee SM, Woo I, Son S, Cheon JH et al.

OBJECTIVE: Thumb metacarpophalangeal (MCP) ulnar collateral ligament (UCL) injury, commonly referred to as "Skier's thumb," is traditionally considered a sports-related injury. However, military personnel are frequently

exposed to activities that may generate similar valgus stress mechanisms. This study aimed to evaluate the clinical characteristics, injury mechanisms, and surgical outcomes of thumb MCP UCL injuries in a South Korean military referral hospital.

METHODS: A retrospective review was conducted on 35 active duty military personnel who underwent surgical treatment for acute or subacute thumb MCP UCL injuries between May 2022 and May 2025. Patients with associated avulsion fractures requiring Kirschner-wire fixation or chronic injuries requiring ligament reconstruction were excluded. Demographic characteristics, injury mechanisms, presence of Stener lesions, clinical outcomes, return-to-duty status, and postoperative complications were analyzed. Surgical repair was performed using a suture anchor technique.

RESULTS: The mean follow-up period was 12.0 months. Rifle-related injuries accounted for 60.0% of all cases and represented the most common injury mechanism. Stener lesions were identified in 18 patients (51.4%). At final follow-up, the mean Visual Analog Scale score was 0.5, the mean Quick Disabilities of the Arm, Shoulder and Hand score was 4.2, and the mean Kap [...]

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Occupational recovery after work-related AO Spine type A1 compression fractures: Association between treatment strategy and sick-leave duration.

Jordá-Gómez P, Saiz-Sapena N, Vanaclocha L, Barrios C, Vanaclocha V

PURPOSE: Return-to-work (RTW) is an important indicator of functional recovery after spinal trauma, yet occupational outcomes following stable thoracolumbar compression fractures remain poorly characterized. This study evaluated factors associated with sick-leave duration and occupational recovery in patients with work-related AO Spine type A1 vertebral compression fractures.

METHODS: This retrospective cohort study included adults with work-related AO Spine type A1 thoracolumbar compression fractures without neurological deficit treated at a tertiary referral center between 2007 and 2025. Patients underwent percutaneous vertebroplasty, orthopedic bracing, or posterior spinal arthrodesis. The primary outcome was sick-leave duration, defined as the interval between injury and resolution of work capacity. Multivariable linear regression using log-transformed sick-leave duration was performed to identify factors independently associated with occupational recovery.

RESULTS: Seventy patients were included: 35 underwent vertebroplasty, 18 orthopedic bracing, and 17 spinal arthrodesis. Sick-leave duration differed significantly among treatment groups ($p < 0.001$). Median sick-leave duration was 62 days (IQR 38-94) after vertebroplasty, 144.5 days (IQR 112-186) after orthopedic bracing, and 178 days (IQR 120-327) after spinal arthrodesis. In multivariable analysis, treatment strategy [...]

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Anterior fix-and-replace strategy using direct anterior and pararectus approaches for acetabular fractures in older patients.

Ivanova S, Keel F, Keel MJ, Siebenrock KA, Tannast M, Bastian JD

BACKGROUND: Most reported fix-and-replace procedures use a posterior or lateral approach for total hip arthroplasty (THA) and a Kocher-Langenbeck, anterior intrapelvic, or ilioinguinal approach for acetabular fixation, which may require separate anterior and posterior exposures. Evidence on fully anterior, single-position workflows combining anterior acetabular fixation with THA remains limited.

AIM: The aim of this study was to assess the outcomes of acute THA through the direct anterior approach (DAA) combined with pararectus in patients considered suitable for a fix-and-replace strategy.

METHODS: This retrospective single-centre case series included 20 patients aged ≥ 60 years (mean age: 82 ± 10 years) selected for a fix-and-replace strategy based on predefined indications - such as acetabular dome destruction, femoral head damage, pre-existing hip degeneration - between 2009 and 2023. All underwent single-stage pararectus fixation and DAA THA in the supine position without posterior exposure or repositioning. Acetabular reconstruction constructs were used in 19 patients and dual-mobility articulation in five patients. The primary outcome was revision surgery within two years. Secondary outcomes included surgical data, 30-day complications, mortality, postoperative weight bearing, radiographic findings, mobility, pain, and patient satisfaction.

RESULTS: Revision surgery w [...]

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Persistent postoperative hypercoagulability and venous thromboembolism risk following pelvic and acetabular fractures: A prospective pilot study.

Clarke A, Tera Y, Skeith L, Othman M, Dodd A, Duffy P, Korley R, Martin CR et al.

BACKGROUND: Pelvic and acetabular (PA) fractures are severe orthopaedic injuries associated with high risk of venous thromboembolism (VTE), a potentially life-threatening complication, despite current thromboprophylaxis strategies. The duration of postoperative hypercoagulability and associated VTE risk following surgical fixation remain unknown. Thromboelastography (TEG) analysis can assess the entire clotting process and can be used to quantify hypercoagulability and increased VTE risk through maximal amplitude (MA) and coagulation index (CI) parameters.

PURPOSE: To characterize the extent and duration of postoperative hypercoagulability and VTE risk following PA fractures using serial TEG analysis.

METHODS: Adults undergoing surgical management of PA fractures were prospectively enrolled. Serial TEG analysis was performed from admission to 3 months postoperatively. Standardized thromboprophylaxis consisted of low-molecular-weight heparin for 28 days postoperatively. Screening compression ultrasound was performed to detect asymptomatic proximal deep vein thrombosis (DVT) on postoperative day 3 (POD3). Mobility was measured using Timed Up and Go (TUG) test. One-sample t-tests compared mean MA and CI values at each time point with predefined hypercoagulable thresholds (MA ≥ 65 mm, CI > 3).

RESULTS: Thirty patients were enrolled, including 22 men (median age: 52.5 years). Most [...]

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Epidemiology of sports- and recreation-related shoulder and elbow injuries in older adults presenting to emergency departments in the United States between 2010 and 2024.

Adio AA, Boufadel P, Ilyas MH, Black EM, Hill BW, Kassam HF, Khan AZ, Horneff JG et al.

BACKGROUND: Participation in sports and recreational activity among adults aged 65 years and older continues to increase, yet contemporary data on shoulder and elbow injury patterns in this population remain limited. This study aimed to characterize national trends in demographics, diagnoses, seasonality, and sport-specific injury risk.

METHODS: The National Electronic Injury Surveillance System was queried from 2010 to 2024 for adults aged ≥ 65 years presenting with sports- or recreation-related shoulder or elbow injuries. Demographics, diagnoses, injury mechanisms, temporal trends, and sport-specific fracture risk were evaluated. Logistic regression assessed sex-specific odds, and narrative text was analyzed using tidytext.

RESULTS: An estimated 248,207 shoulder and 106,031 elbow injuries occurred nationally. Males accounted for 63.8% of shoulder and 60.3% of elbow injuries. Fractures (26.5%) and strains or sprains (24.0%) predominated in shoulder injuries, while fractures represented 32.1% of elbow cases. From 2010-2024, the proportion of these injuries increased, comprising approximately 8% of all emergency visits by 2024. Bicycling was the most common mechanism for both joints, followed by general exercise and weightlifting. Shoulder and elbow injuries peaked in summer months.

CONCLUSION: Sports-related shoulder and elbow injuries in older adults are rising, with fractu [...]

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Reverse-engineering the diagnostic process: An indirect finding-driven deep learning model for pelvic active bleeding detection on CT that achieves specialist-level performance and surpasses residents - a multicenter study.

Okada N, Inoue S, Hirano Y, Liu C, Mitarai S, Honda S, Sato K, Hiraki S et al.

Contrast-enhanced computed tomography (CT) is useful for detecting active bleeding in patients with pelvic trauma. However, early detection of active bleeding from CT images is challenging due to the extensive image volume, which can delay intervention. Although the development of accurate and rapid detection support systems for pelvic active bleeding is highly anticipated, a significant data imbalance between bleeding and non-bleeding

images in real-world hospital settings poses a significant challenge to clinically applicable models. We reverse-engineered the physician's diagnostic process and implemented a curriculum-based deep learning algorithm using indirect findings to prevent overfitting caused by the imbalance. In this multicenter study, we developed and implemented a multistage deep learning algorithm for the detection of active bleeding using enhanced CT of pelvic trauma patients. From April 18, 2008 to December 31, 2023, we collected 258,057 slices of whole-body CT images of 2178 patients for blunt trauma with pelvic injuries from five emergency centers. Two types of classification-based deep learning models were developed and evaluated: anatomical structure extraction (ASE) and active bleeding detection (ABD) models. An algorithm using these models was implemented, and its accuracy in detecting active bleeding in the pelvis and the inference speed were evaluated. F [...]

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Robot-assisted fracture reduction and treatment outcomes in type III and IV fragility fractures of the pelvis: A retrospective cohort study.

Xiao H, Zhao C, Cao Q, Sun Q, Ma Y, Jian Y, Ming Y, Wang Y et al.

INTRODUCTION: This study compared robot-assisted fracture reduction (RAFR) with manual closed reduction followed by navigation-assisted fixation in surgically treated patients with fragility fractures of the pelvis (FFP) type III or IV. Outcomes after operative versus initial non-operative management were also examined.

PATIENTS AND METHODS: This retrospective cohort study included 115 patients aged ≥ 60 years treated between March 2021 and June 2025: 39 underwent RAFR, 34 manual closed reduction with navigation-assisted fixation, and 42 initial non-operative treatment. The primary outcome was postoperative reduction quality according to the Matta criteria. Secondary outcomes included 1-year functional scores, mortality, and surgery-related complications.

RESULTS: RAFR was associated with lower odds of a worse postoperative Matta grade than manual closed reduction with navigation-assisted fixation (adjusted common OR=0.139, 95% CI 0.039-0.440; $P = 0.001$). Among 58 operated survivors with complete 1-year assessments, RAFR was associated with a higher Parker Mobility Score ($\beta=2.030$, 95% CI 1.093-2.966; $P < 0.001$), whereas adjusted differences in Barthel Index, EQ-5D-5L, and NRS were not statistically significant. In the operative versus initial non-operative comparison, 6-month mortality was 4.3% versus 15.8% ($P = 0.064$) and 1-year mortality was 8.6% versus 21.1% ($P = 0.078$), w [...]

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Factors associated with U-type sacral fractures in geriatric patients: A retrospective cohort study.

Wong AW, Musick AN, Wagner RK, Lehle CH, Vise HS, van Weezenbeek MA, Patel R, Zhang BB et al.

OBJECTIVES: To identify demographic and clinical factors associated with geriatric U-type sacral fractures.

METHODS: Design: Retrospective cohort study.

SETTING: Two Academic Level 1 trauma centers. Patient Selection Criteria: Patients ≥ 65 years with sacral fractures between January 2023 and January 2025 who underwent advanced imaging (CT or MRI) were included. Outcome Measures and Comparisons: The primary outcome was U-type sacral fracture morphology (AO/OTA 55C0, 54C3), defined as bilateral sacral ala fractures with a transverse fracture through the sacral body, consistent with spinopelvic dissociation. Factors associated with U-type morphology were evaluated using univariate logistic regression, followed by multivariable logistic regression to identify independent associations.

RESULTS: 263 geriatric patients with sacral fractures were included, of whom 76 (28.9%) had U-type fractures. On multivariable analysis, spondylosis/spondylolisthesis (OR 2.08, 95% CI 1.10-3.98, $p = 0.025$), independent ambulation prior to injury (with assistive device use as the reference group; OR 2.18, 95% CI 1.13-4.29, $p = 0.022$), and an atraumatic mechanism of injury (with low-energy injury mechanism as the reference group; OR 2.34, 95% CI 1.05-5.27, $p = 0.038$) were independently associated with U-type fractures.

CONCLUSIONS: Nearly one-third of geriatric patients with sacral fractures who un [...]

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Traumatic spinal injuries in a rural tourist area: Epidemiology, seasonal trends and implications for healthcare resource allocation.

Peppucci E, Ferlito D, Meletti L, Montalbetti A, Piras G, Bucpapaj R, Giussani CG, Zoia C

PURPOSE: Several epidemiological studies have evaluated trends and epidemiology of Traumatic Spinal Injury (TSI) but often they don't take into account the geographical variability of nations. Having identified a lack of literature on the epidemiology of rural and touristic areas we conducted this retrospective study in our hospital near Como Lake to understand how best to manage TSI patients.

METHODS: This retrospective epidemiological study included all patients with TSI admitted to Emergency Department (ED) of Gravedona ed Uniti Hospital between April 2018 and March 2024. We focused on vertebral fractures. Demographics, trauma mechanisms, regions of the spine involved, patient comorbidities, polytrauma or Traumatic Brain Injury (TBI), any surgical treatment were analysed. We then focused on differences between warm and cold seasons.

RESULTS: 140 patients were included, but only 132 were considered due to missing data. Average age was 60.0 years. TSI involved cervical spine in 23.5% of the cases, thoracic in 27.3%, lumbosacral in 37.1%, and multiple sites in 12.1%. 54.5% of the patients were treated with conservative therapies while 45.5% underwent surgery (78.3% spinal arthrodesis and 21.7% vertebroplasty). Mechanisms of trauma were as follows: accidental falls 56.8%, vehicular accidents 21.2%, sports 20.4% and violence 1.5%. There was a statistically significant increase [...]

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Stress displacement and surgical benefit in Beckmann 7-9 pelvic ring injuries: A retrospective multicenter analysis.

Achebe CC, Thorne T, Mau M, Higgins TF, Rothberg DL, Haller JM, Sato E, Steffenson L et al.

INTRODUCTION: We assessed whether stress-induced displacement identifies stability-indeterminate lateral compression type 1 (LC1) pelvic ring injuries in which surgical treatment is associated with fewer early adverse or ambulatory-decline outcomes.

METHODS: This multicenter retrospective cohort study was performed at eight Level I trauma centers. Skeletally mature patients with OTA/AO 61B2 LC1 pelvic ring injuries, Beckmann scores 7-9, and documented stress examination were included; follow-up duration was modeled as a covariate rather than used as an inclusion restriction. The primary outcome was a composite of all-cause mortality, non-elective hardware failure or revision, or worsening ambulatory category. Outcomes were compared between surgical and nonoperative management within displacement strata of less than 12 mm, 12-16 mm, and greater than 16 mm.

RESULTS: The primary analytic cohort included 187 patients (98 nonoperative, 89 operative); follow-up duration was modeled as a covariate. Surgical patients had greater median displacement than nonoperative patients (14.3 vs 5.8 mm; $P < 0.001$). The composite outcome occurred in 41 nonoperative patients (41.8%) and 21 operative patients (23.6%; $P = 0.009$). In the 12-16 mm stratum, events occurred in 6 of 11 nonoperative patients (54.5%) and 3 of 20 operative patients (15.0%), corresponding to a 39.5% absolute risk reduction, [...]

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Longitudinal radiographic comparison of percutaneous and open pedicle screw fixation for thoracolumbar burst fractures: A two-year follow-up study.

Wung CH, Chang CY, Hsu CC, Wu RW, Yang FS, Lu ML, Chen HT

PURPOSE: While percutaneous pedicle screw fixation (PPSF) and open pedicle screw fixation (OPSF) demonstrate comparable clinical outcomes at 1-year follow-up, the longitudinal evolution of sagittal alignment beyond one year remains poorly characterized. This study compared the longitudinal radiographic outcomes of PPSF and OPSF in patients with thoracolumbar burst fractures (TLBF) over a two-year follow-up period.

METHODS: This retrospective cohort study included 54 patients with TLBF who underwent short-segment posterior fixation using either PPSF ($n = 29$) or OPSF ($n = 25$). The primary radiographic outcome was Cobb angle. Secondary outcomes included lumbar lordosis angle (LLA), operative time, intraoperative blood loss, postoperative hospital stay, and revision surgery. Serial radiographic evaluations were performed preoperatively and at 3 months, 6 months, 1 year, and 2 years postoperatively.

RESULTS: Baseline demographic characteristics and preoperative radiographic parameters were comparable between groups. PPSF was associated with significantly lower intraoperative blood loss than OPSF (61.9 ± 34.8

mL vs. 262.7 ± 205.1 mL, $P < 0.001$), whereas operative time and postoperative hospital stay did not differ significantly. Both groups demonstrated significant reductions in Cobb angle over the two-year follow-up. No significant between-group differences in Cobb angle were observed [...]

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A structured three-step reduction strategy for locked posterior fracture-dislocation of the proximal humerus through a standard deltopectoral approach: A retrospective case series.

Jiang Z, Xiang Y, Lei C, Fu Q, Huang M, Zhi XS, Wang HS

BACKGROUND: Locked posterior fracture-dislocation of the proximal humerus is a rare and technically demanding injury in which the humeral head remains impacted posterior to the glenoid. Although several reduction techniques have been described, the practical details of anterior-only reduction remain incompletely standardized. We evaluated a structured three-step anterior reduction strategy performed through a standard deltopectoral approach.

METHODS: We retrospectively reviewed 16 consecutive patients with acute locked posterior fracture-dislocation of the proximal humerus treated between January 2016 and January 2022. All patients underwent reduction through a single deltopectoral approach followed by locking-plate fixation supplemented with suture fixation. The reduction strategy consisted of (1) establishing an anterior reduction corridor by opening the anterior joint through an L-shaped subscapularis incision or a lesser-tuberosity fracture interval, (2) guiding the humeral head with a periosteal elevator introduced between the humeral head and glenoid, and (3) unlocking and translating the head anteriorly using gentle elevator assistance, controlled arm rotation, and, when head-shaft control was limited, direct digital manipulation. Outcomes included fracture union, avascular necrosis (AVN), recurrent dislocation, implant-related complications, range of motion, Constant-M [...]

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Prehospital transfusion requirements in severe trauma: A multicenter, retrospective estimate of requirements.

Thabouillot O, Narcisse J, Girard A, Raphael V, Rozenberg E, Wolff D

PURPOSE: To estimate the proportion of severely injured trauma patients who would have met criteria for prehospital blood transfusion, assuming a benefit when prehospital management exceeds 20 min, by applying the ABC score to a multicenter French prehospital cohort.

METHODS: We performed a retrospective observational study across four prehospital services in the Paris region over one year. Among all trauma missions, we included patients transported to a major trauma center or undergoing resuscitation for traumatic cardiac arrest. Exclusions were age < 18 , burned patients, and interhospital transfers. We retrospectively calculated the ABC (Assessment of Blood Consumption) score at 10-minute intervals during the intervention (until 20 min before hospital arrival). Patients were considered "eligible" if $ABC \geq 2$ and total prehospital intervention duration > 20 min. Secondary endpoints included: proportion receiving tranexamic acid, time intervals, and prehospital deaths or arrests. Data were double-reviewed by two physicians, with adjudication by a third in case of discrepancy. Confidence intervals were computed using Wilson's method.

RESULTS: Of 5921 missions performed, 139 met inclusion criteria. Twenty-seven patients (19.7%, 95% CI: 13.7-26.8%) met both $ABC \geq 2$ and intervention time > 20 min, thus qualifying for prehospital transfusion under our assumptions. Median time interval [...]

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Using injury prevention priority scores to investigate adult traumatic injuries in Nova Scotia, Canada.

Green D, Jessula S, Opra I, Green RS

BACKGROUND: Traumatic injuries are a primary public health concern and a leading cause of mortality, placing significant strain on healthcare systems. To address this, the objective is to develop province-wide Injury Prevention Priority Scores (IPPS) for major traumatic injuries in Nova Scotia. Allowing for the identification of which mechanisms, if prevented, would yield the greatest impact.

METHODS: We included all adults (≥ 20 years old) major trauma patients in Nova Scotia from April 1, 2016, to March 31, 2023, from a population-based trauma registry. We calculated Injury Prevention Priority Scores (IPPS) for each mechanism of injury. Frequency plus one of the following measures: injury severity, mortality, resource utilization, and societal cost, is included in the IPPS calculation.

RESULTS: A total of 6240 patients were included in the analysis, with falls the leading IPPS scores in two domains (Severity = 66 and Resource Utilization = 65). Motor vehicle collisions scored the highest (65) in the societal cost domain, while intentional self-harm injuries and falls shared the highest score (64) for the mortality domain.

CONCLUSIONS: Based on our investigation, falls and motor vehicle collisions are the top injury priorities for injury prevention in Nova Scotia, while intentional self-harm linked to substance use also demonstrated high mortality. High hospital costs from f [...]

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Therapeutic potential of dermal matrix in orthopaedic trauma.

Christenson MM, Boike SC, Puckett HD, Agoro KS, Sullivan D, Vang S, Cunningham BP, Busel GA

BACKGROUND: The primary aim was to examine the outcomes of dermal matrix (DM) in populations with non-closable traumatic, infectious, or post-surgical wounds managed by an orthopaedic trauma service line. The secondary aim was to identify the success and complication rates of DM, as well as attempting to document time to healing.

METHODS: This was a multi-center retrospective chart review at urban community and academic affiliated trauma centers. Consecutive patients with any traumatic, infectious or post-surgical wounds treated with dermal matrix from January 2020 to August 2025. Patients were excluded if lost to follow-up and/or had ongoing treatment. Data was collected for demographics and comorbidities, alongside origin, treatment, and time to complete healing of the wound.

RESULTS: Seventy-three patients had application of DM on 75 wounds. Four patients were lost to follow-up, 9 had ongoing treatment, and 3 died. The final cohort consisted of 57 patients, with 59 wounds. There were 33 males and 24 females. The average age was 58 ± 19.4 years. Wound origins were postoperative ($n = 17$), traumatic ($n = 30$), and infectious ($n = 12$) etiologies. The average number of washouts prior to application was 2.3 ± 1.2 . Two wounds required salvage plastic surgery involving sural flaps. One wound had a failure of skin graft after DM with subsequent treatment by secondary healing vi [...]

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State-level retrospective analysis of the burden of emergency and operative conditions in India from 1980 to 2023.

Gerardo R, Zadey S, Shetty R, Arora A, Gupta U, Rao S, Afsar AP, Staton CA et al.

INTRODUCTION: Disease burden is a crucial factor in determining healthcare policy and resource allocation. We analyzed the subnational burden of emergency and operative conditions in India from 1980 to 2023.

MATERIALS AND METHODS: We extracted mortality and disability-adjusted life-year (DALY) estimates for 31 Indian states and Union Territories from 1980 to 2023 from the Global Burden of Disease 2023 Study. We used existing expert-consensus-based classifications for defining emergency (30 conditions), operative (57 conditions), and emergency-operative conditions (7 conditions). We investigated rates per 100,000 people and proportions attributable to the above conditions, expressed as percentages of total mortality and DALYs. We conducted a post-hoc uncertainty analysis to calculate the mean and 95% uncertainty interval (UI) for each estimate.

RESULTS: In 2023, emergency (mortality rate: 340 deaths per 100,000, 95% UI [282, 416]; DALY rate: 13,732 DALYs per 100,000 people, 95% UI [11,492, 16,277]), operative (mortality rate: 158 deaths per 100,000, 95% UI [123, 196]; DALY rate: 7284 DALYs per 100,000 people, 95% UI [5765, 9018]), and emergency-operative conditions (mortality rate: 70 deaths per 100,000, 95% UI [58, 83]; DALY rate: 3830 DALYs per 100,000 people, 95% UI [3229, 4396]) accounted for 49.76% (95% UI [43.44%, 58.14%]), 23.12% (95% UI [19.05%, 27.52%]), and 10.31% (9 [...])

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Age-specific TBSA thresholds for pediatric burn complications: A 13-year single-center inpatient retrospective cohort.

Hilewitz D, Har-Shai L, Keller A, Bal M, Hayun Y, Meshulam-Derazon S, Ad-El D, Kalish E et al.

INTRODUCTION: Pediatric burns are common, yet practical tools that stratify presentation features into age-specific risk remain limited.

OBJECTIVE: To characterize pediatric burn presentations and derive clinically interpretable, age-specific total body surface area (TBSA) thresholds, with absolute-risk tables, for fever (≥ 38.5 °C), Intensive Care Unit (ICU) transfer, surgery, and infection, to guide emergency department (ED) triage and inpatient management.

METHODS: A retrospective cohort of children 0-18 years with acute burns admitted to a tertiary pediatric center (December 2012-May 2025). Exposures included age, TBSA, depth, anatomic site, circumferential pattern, etiology, and post-burn days. Outcomes were ICU transfer, surgery, and in-hospital infection. Fever was analyzed as a clinical sign. We estimated absolute risks by age $\times$ TBSA, assessed co-occurrence of adverse events, and derived age-specific TBSA cutoffs using Youden optimizations.

RESULTS: Among 942 children (mean age 4.6 ± 4.6 y; 57.4% male; mean TBSA $5.0 \pm 5.7\%$), scalds predominated (74.5%). Event rates were fever 29.0%, ICU transfer 4.4%, surgery 7.7%, infection 5.2%. Delayed presentation (>24 h) was associated with more fever ($p \approx 0.05$). Operative risk varied by site: highest in lower extremities and buttocks and was more frequent with circumferential burns. ICU transfer and surgery rarely occurred without [...]

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Epidemiology of mountain bike-related injury presentations to emergency departments: A 15-year retrospective analysis.

Course G, Sharman JE, Zhao T, Otahal P, Tran V

OBJECTIVES: To determine the epidemiology of mountain bike (MTB)-related presentations to public hospital emergency departments (ED) in Tasmania, Australia, pre- and post-development of purpose-built MTB trails.

METHODS: Retrospective analysis of presentations to Tasmanian public hospital EDs pre- and post-MTB trail development in 2015 (total period: 1st January 2010-31 st December 2024). MTB-related injuries were determined from hospital medical records. Epidemiological characteristics were prevalence, clinical urgency (Australasian Triage Scale [ATS] categories 1-5, most to least urgent), arrival mode (ambulance vs private transport), mortality, admission rate, seasonal variation, injury profile and patient demographics.

RESULTS: For the total period, there were 6643 MTB-related presentations. Prevalence increased 3-fold post-trail development, 1.0-3.6/1000 ED presentations ($n = 743$ -5900) with seasonal burden highest in summer. Clinical urgency increased 375% for ATS category 1 from 0.8 (0.8) mean (SD) to 3.8 (2.8) presentations/year, and 1343% for category 2 from 8.2 (4.0) to 118.3 (65.5) presentations/year. Ambulance arrivals increased 372% from 21.2 (3.6) to 100 (47.6) /year. Hospital admission rate increased 663% post-trail development, from 17.2 (7.4) to 131.3 (64.4) /year. All four deaths were post-trail development. Over the total period, 41.6% of MTB injuries were o [...]

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Diagnostic accuracy and predictive value of hard signs for operative or endovascular management and major injury in penetrating neck trauma: a single-centre retrospective cohort study.

Barrientos ML, Zapata CAL, Toro DAM, López CAD

PURPOSE: Hard signs guide operative decision-making in penetrating neck trauma (PNT), yet their accuracy and predictive value remain incompletely defined. We quantified the diagnostic accuracy of hard signs for major injury and for operative or endovascular management, and explored multivariable risk estimation.

METHODS: Single-centre retrospective cohort of 310 consecutive patients with PNT (June 2013-July 2025). Hard and soft signs were assigned by the attending surgeon at initial evaluation and abstracted retrospectively from the clinical record. The primary outcome was major injury (confirmed vascular or aerodigestive injury). Operative or endovascular management was analysed as a secondary outcome describing institutional practice, not as an independent diagnostic property, because the same signs informed both the index test and the treatment decision. We estimated sensitivity, specificity, predictive values and likelihood ratios with exact binomial confidence intervals, and fitted exploratory multivariable models (STARD/TRIPOD).

RESULTS: Major injury occurred in 92/310 (29.7%) and operative or endovascular management in 96/310 (31.0%). Hard signs were specific but insensitive for major injury (sensitivity 55.4%, 95% CI 44.7-65.8; specificity 88.1%, 83.0-92.1) and for management (sensitivity 59.4%, 48.9-69.3; specificity 90.7%, 85.9-94.2). Outcome frequency rose across n [...]

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Epidemiology of spinal fractures in polytrauma patients.

Malan SJ, Meakes S, Fischer A, Balogh ZJ

PURPOSE: The epidemiology of spinal fractures in polytrauma, population characteristics and their hospital outcomes are not well defined. This study aimed to describe the epidemiology and outcomes of spinal fractures in a consensus defined polytrauma cohort.

METHODS: A 13-year (2009-2021) retrospective study was conducted on Newcastle Definition based polytrauma patients at a Level-1 centre. Polytrauma patients were categorised based on the presence of vertebral fractures and compared in demographics, injury mechanism, injury severity and pattern, physiological parameters, treatment aspects, and outcomes.

RESULTS: From the 1425 polytrauma patients, 632 (44.4%) sustained spinal fractures. These patients were older (48 vs. 38 years, $p < 0.001$) with identical injury severity (ISS = 29), were more frequently admitted (62.8% vs. 57.4%, $p = 0.037$) and also stayed longer in ICU (6 vs. 3 days, $p < 0.001$) and in hospital (14.5 vs. 12 days, $p = 0.002$). Mortality did not differ significantly (10.4% vs. 13.5%, $p = 0.08$). The lumbar spine was the most frequently fractured segment (7.3%), followed by thoracic (3.3%), cervical (3.3%) and sacral (3.1%) levels. Spinal cord injury occurred in 41 (6.5%) spinal fracture patients and was associated with a high ISS and prolonged hospitalisation [ISS:34(IQR 20.5); ICU LOS:10(IQR21.75); Hospital LOS:22(IQR41)]. Spinal fracture patients that classifi [...]

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Age-dependent injury mechanisms and injury patterns in pediatric window falls.

Badina A, Deygas A, Saliba I, Corbu A, Da Costa EP, Georges S, Wicart P

PURPOSE: Pediatric defenestration is a high-energy trauma mechanism in which age influences injury patterns and clinical severity. While previous studies have focused primarily on epidemiology and mortality, detailed analyses of age-related injury distribution and surgical burden remain limited. This study aimed to characterize age-dependent injury patterns, neurological severity, and hospital resource utilization following pediatric window falls.

METHODS: We conducted a retrospective single-center cohort study including all children aged 0-18 years admitted for defenestration between 2008 and 2023. Demographic data, fall height, intentionality, injury distribution, Glasgow Coma Scale (GCS), fracture patterns, and number of surgical procedures were collected. Multivariable logistic regression identified predictors of severe neurological impairment ($GCS \leq 8$), and Poisson regression assessed determinants of surgical burden.

RESULTS: A total of 333 children were included. Age distribution was bimodal, with predominantly accidental falls in young children (median 4.6 years) and intentional falls in adolescents (median 13.7 years). Younger children more frequently sustained craniofacial injuries and presented with lower admission GCS values, whereas adolescents more often sustained combined lower limb and spinal injuries. Fall height correlated negatively with GCS ($p = -0.32$) and [...]

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Clinical outcomes and risk factors in the surgical treatment of distal femur fractures.

Höller LF, Höller S, Wittur P, Sehmisch S, Lehmann W, Weiser L

PURPOSE: Distal femoral fractures are rare but severe injuries with high complication rates, particularly in elderly or multimorbid patients. This study aimed to analyze complication profiles and identify risk factors for surgery-related and non-surgery-related complications following operative treatment.

METHODS: A retrospective cohort study was conducted including all adult patients undergoing surgical treatment of distal femur fractures at a Level I trauma center between 2012 and 2018. Demographic, pre-operative, operative, and postoperative characteristics were analyzed. Risk factors were assessed using univariate statistics.

RESULTS: A total of 56 patients with 60 distal femur fractures were included. Surgery-related complications occurred in 11.7% of cases, while non-surgery-related complications were observed in 58.3%. In univariate analysis, gender ($p = 0.004$), age ($p = 0.014$), BMI ($p = 0.006$), osteoporosis ($p = 0.047$), open fracture ($p < 0.001$), high-energetic trauma (0.031) and time to surgery ($p = 0.024$) were significant risk factors for surgery-specific complications. For non-surgery-related complications, need of an external fixator for primary surgical care and type of operation were significant risk factors in univariate analysis. In multivariable logistic regression analysis, need of an external fixator was the only significant risk factor for developing non-s [...]

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Hip fracture surgery within 24 hours: no association between time to surgery and mortality in a real-world cohort.

Pankratz C, Wolters M, Cintean R, Gebhard F, Schuetze K, Eickhoff A

PURPOSE: Early surgery is recommended for hip fractures, and in Germany, operative treatment within 24 h has been mandatory since 2021. While delays beyond 24-48 h are associated with worse outcomes, it remains unclear whether further acceleration within a 24-hour timeframe improves survival. This study evaluated mortality and perioperative risk factors within a strictly implemented early-treatment pathway.

METHODS: This retrospective single-centre cohort study included 1,051 consecutive patients with femoral neck and trochanteric fractures treated surgically within 24 h of admission (2016-2020). Time to surgery was grouped into < 6 h, 6-12 h, and > 12 -24 h. The primary outcome was one-year mortality; secondary outcome was two-year mortality. Mortality data were obtained from the national citizen registry. Multivariable logistic and Cox proportional hazards regression analyses were performed for one-year mortality.

RESULTS: Mean time to surgery was 10.2 ± 6.2 h. One-year mortality was 29.5% and increased to 40.1% at two years. Mortality did not differ significantly between time-to-surgery groups (< 6 h: 30.9%, 6-12 h: 26.4%, > 12 -24 h: 31.5%; $p = 0.258$). This finding was consistent across all adjusted analyses. In contrast, non-surgical complications were the strongest independent predictors of mortality, particularly pneumonia (OR 2.1), renal failure (OR 5.2), and cardiac ev [...]

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Small bowel obstruction in older adults: similar nonoperative success but worse outcomes across management strategies - insights from the ESTES SnapSBO study.

Bunino FM, Cimino MM, Martinez Casas I, Bellio G, Porta M, Cayre L, Mohseni S, Kaplan LJ et al.

BACKGROUND: Small bowel obstruction (SBO) is a common surgical emergency. It remains unclear whether adverse outcomes in older patients reflect impaired obstruction resolution, differences in disease phenotype, or reduced physiologic tolerance.

METHODS: We performed a prespecified age-stratified analysis of the multicenter SnapSBO cohort, which consecutively enrolled adults with radiologically-confirmed mechanical SBO, stratified as younger (< 65 years) or older ($\geq$ 65 years). Presentation, etiology, management, and outcomes were compared using risk differences and odds ratios. Exploratory analyses evaluated patients aged 65-79 years and $\geq$ 80 years.

RESULTS: Among 1,731 patients, 729 (42.1%) were aged $\geq$ 65 years. Older adults had more non-adhesive etiologies, including neoplastic and inflammatory obstruction, and more imaging features of severe disease. They were less likely to undergo a trial of nonoperative management (67.9% vs. 72.7%; absolute risk difference, - 4.8%; $P = 0.037$). However, among those managed nonoperatively, resolution rates were similar (68.8% vs. 69.5%). Despite this, older adults experienced higher rates of complications (16.7% vs. 6.7%), aspiration pneumonia (3.8% vs. 1.0%), and in-hospital mortality (6.9% vs. 3.6%). Those aged $\geq$ 80 years showed similar management but substantially higher mortality and complications.

CONCLUSIONS: Older adults achieved s [...]

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Pancreatic stent and transpapillary pseudocyst drainage via endoscopic retrograde cholangiopancreatography for children with blunt pancreatic duct injury.

Kouchi K, Fumita T, Nakata C, Kagaya K

BACKGROUND: Nonoperative management (NOM) in blunt pancreatic injury (BPI) is frequently complicated by pseudocyst formation and pancreatic atrophy. To address these complications, we have utilized a pancreatic stent via endoscopic retrograde cholangiopancreatography (ERCP) for children with PDI since 2019.

AIM: We present our experience with ERCP-based NOM for PDI and provide a literature review of stent therapy in pediatric PDI.

MATERIALS AND METHODS: Ten children with BPI treated between January 2007 and December 2025 were classified according to the 2024 American Association for the Surgery of Trauma pancreatic injury grades: IB ($n = 3$), IIA ($n = 3$), IIIA ($n = 2$), and IIIB ($n = 2$). Pancreatic stents were placed in two IIIA cases and one IIIB case. The IIIB case subsequently underwent transpapillary pseudocyst drainage (TPPD) due to a pseudocyst infection.

RESULTS: NOM was successful in all low-grade injuries (< II) without complications. One IIIB patient treated without stenting developed a pseudocyst requiring cyst-gastrostomy; 1-year follow-up demonstrated severe pancreatic atrophy. In the two IIIA stent cases, the duct injury was successfully bridged: one patient had no pseudocyst formation, and the other developed a pseudocyst that resolved spontaneously; both showed preserved pancreatic parenchyma. The IIIB patient treated with TPPD experienced rapid cyst resolution [...]

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Impact of pre-injury anticoagulation on outcomes in middle-aged trauma patients: a matched-pair analysis of the TraumaRegister DGU®.

Koch DA, Faul P, Jakobi T, Schnetz M, Egert A, Münzberg M, Lefering R, Schweigkofler U et al.

PURPOSE: Pre-injury anticoagulation is a well-known risk factor associated with adverse outcomes in geriatric trauma patients, but its impact in younger populations remains unclear. The study aimed to evaluate the association between pre-injury antithrombotic therapy and clinical outcomes in trauma patients aged 40-60 years.

METHODS: Data were obtained from the TraumaRegister DGU® (2015-2023). Patients aged 40-60 years from European trauma centers with ISS ≥ 9 were included. Patients were categorized according to the presence or absence of pre-injury antithrombotic therapy. A matched-pair analysis was performed based on age group, sex, ASA classification, and injury severity by body region. The primary endpoint was in-hospital mortality; secondary outcomes included coagulopathy, transfusion requirements, emergency surgery, and organ failure. Analyses were primarily descriptive.

RESULTS: A total of 64,215 patients were included, of whom 6.7% ($n = 4,326$) received antithrombotic therapy. Antiplatelet agents were most common (68%), followed by direct oral anticoagulants (DOAC) (18%) and

Vitamin-K-Antagonists (VKA) (11%). Coagulopathy was more frequent in anticoagulated patients, particularly in those receiving VKA and DOAC. In unmatched analyses, mortality was slightly higher in anticoagulated patients (6.7% vs. 5.0%) and showed a non-significant trend towards higher mortality in [...]

[PubMed](#) · [DOI](#)

Correction: Impact of routine use of the GFAP and UCH-L1 TBI blood test for management of mild TBI on patient care in a European emergency department.

Brüning-Wolter F, Wolff N, Schrader M, Musaelyan K, Chandran R, Hoffmann M, Marino JA, McQuiston B et al.

[PubMed](#) · [DOI](#)

From rib fixation to dynamic chest wall reconstruction: historical lessons for modern surgical stabilization of rib fractures.

Sutori D, White TW, Bauman ZM, Whitbeck SS, Spering C

PURPOSE: Surgical stabilization of rib fractures (SSRF) has re-emerged as an important component of chest wall trauma care, yet its history is often presented as a simple timeline rather than as a source of operative and biomechanical lessons. This literature, problem-oriented historical review re-examines earlier rib fixation strategies in order to identify the mechanical and operative problems they were intended to solve, why many disappeared, and which principles remain relevant to contemporary SSRF practice.

METHODS: A literature review was conducted focusing on historically important fixation techniques and conceptual milestones in the management of serious rib fractures and flail chest. PubMed/MEDLINE and Google Scholar were searched from database inception to 19 March 2026 using terms related to rib fracture fixation, flail chest, SSRF, intramedullary fixation, and history. Studies were purposively selected if they described key fixation concepts, major shifts in management strategy, recurrent technical problems still seen in modern SSRF, or contemporary guideline context. Forty-seven articles were included and synthesized using a problem-based rather than purely chronological framework.

RESULTS: Across eras, rib fixation evolved around recurring challenges: (1) restoration of chest wall stability and thoracic continuity, (2) the tension between internal pneumatic stab [...]

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Patient-reported outcome measures within European cohorts of severely injured patients: a systematic review and meta-analysis.

van den Berg L, van Ditschuijzen JC, Leliefeld PHC, den Hartog D, Giannakópoulos GF, Verhofstad MHJ, de Jongh MAC

PURPOSE: Severe injury affects multiple health-related domains, yet comprehensive European data on patient-reported outcomes remain limited. This systematic review and meta-analysis evaluates patient-reported outcome measures (PROMs) use and outcomes in severely injured European cohorts.

METHODS: A systematic search of four databases up to October 14, 2025, identified European studies from 2000 onward reporting PROMs in severely injured patients. Severe injury was defined as an Injury Severity Score ≥ 16 , Glasgow Coma Scale ≤ 8 , intensive care unit admission, spinal cord injury, traumatic amputations, or pelvic fractures. Two reviewers independently screened records, with disagreements resolved by a third reviewer. Meta-analysis was performed when ≥ 3 studies reported comparable PROMs at similar follow-up timepoints.

RESULTS: Of 2,479 studies, 119 were included. Most cohorts originated from the Netherlands (26%), Norway (18%), and Germany (16%). General severely injured cohorts were most frequently studied (61%), followed by traumatic brain injury (17%), and spinal cord injury (15%). In total, 94 PROMs were used across 277 follow-up timepoints. Health-related quality of life was assessed most frequently (63%), after that anxiety/depression (14%), post-traumatic stress (9%), and social functioning (6%). At one year follow-up, the pooled EuroQol-5D-3 L index score was 0.70 (95% [...])

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ABCB5^{hi} mesenchymal stem cells enable high-ratio cellular augmentation of bone grafts without loss of osteogenic function.

Autologous bone grafting remains the standard for reconstruction of bone defects but is constrained by limited graft volume and loss of osteogenic capacity when expanded with acellular materials. We hypothesized that allogeneic ABCB5⁺ mesenchymal stem cells (MSCs) can serve as a biologic cell source to augment bone marrow-derived MSC (BM-MSC) populations without impairing osteogenesis. Primary human BM-MSCs from five donors were combined with GMP-manufactured ABCB5⁺ MSCs at graded replacement ratios (0-95%) and cultured under osteogenic conditions for 21 days. Osteogenic potential was evaluated using complementary assays of mineralization, biochemical activity, and matrix composition. Robust osteogenesis was maintained across all groups, with cultures containing 50% ABCB5⁺ MSCs demonstrating comparable functional osteogenic activity to BM-MSC controls, including similar 99mTc-HDP uptake (2.96 vs. 2.68 MBq) and calcium-phosphate ratios (1.2 vs. 1.1). Higher substitution levels showed a progressive decline in osteogenic output, consistent with dilution of BM-MSC-driven effects. These findings establish a quantitative threshold for biologic augmentation and demonstrate that substantial replacement of BM-MSCs is feasible without loss of function. This approach may enable cell-based expansion of bone grafts and supports further evaluation in translational in vivo models.

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Surgical outcomes in an improvised underground operating room during missile attacks.

Mitchnik IY, Essa A, Heinig O, Silberzweig JG, Ben Sira Y, Rabau O

PURPOSE: Armed conflict disrupts healthcare when hospital infrastructure is vulnerable to missile attacks. During a 12-day missile strike period, our hospital established an improvised operating room (OR) in an underground parking lot. This study evaluated perioperative and short-term outcomes among patients treated in the improvised OR compared with those treated in the hospital's fortified standard OR.

METHODS: This retrospective cohort study included patients undergoing general or orthopaedic surgery during the 12-day conflict. Procedures identified via surgical logs were categorized by OR location (improvised or standard). Demographic, perioperative, and post-discharge data were collected from electronic records. Primary outcomes included operative time, estimated blood loss, transfusions, complications, hospital and ICU length of stay, ED visits, readmissions, and 3-month mortality.

RESULTS: Overall, 74 surgeries took place in the improvised OR and 77 in the standard OR. Patients in each group were comparable in age, sex, comorbidities, and anesthesia. Improvised OR surgery time was longer (mean 68.1 vs. 47.7 minutes, $p < 0.001$). There were no significant differences between groups in terms of blood loss, transfusions, postoperative complications, or hospital length of stay. After 3-month follow-up, there were no significant differences in readmissions or mortality rates [...]

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Repeated scoring with the adult appendicitis score improves the sensitivity and the specificity of appendicitis diagnosis in patients with early equivocal signs of appendicitis: a secondary analysis.

Lastunen KS, Leppäniemi AK, Mentula PJ

PURPOSE: The utilization of computed tomography in the early stage of acute appendicitis may result in overdiagnosis and unnecessarily expose patients to ionising radiation. The Adult Appendicitis Score (AAS) can be used to select patients for imaging. Observation and re-scoring in the DIAMOND trial reduced the need for imaging. Now, we wanted to determine if the change in AAS (Δ AAS) can serve as a diagnostic tool to select patients for imaging even more precisely.

METHODS: Eighty-eight patients with early equivocal appendicitis participated in the observation arm of the DIAMOND trial. The data for these patients were reanalysed, and Δ AAS during the observation was calculated. The baseline AAS, final AAS, and the change in C-reactive protein (Δ CRP) were selected as reference standards.

RESULTS: Eighty-three patients with complete data were included in the analysis. The AUROC (Area Under the Receiver Operating Characteristic) values are as follows: Δ AAS, 0.932 (95% CI 0.868-0.996); baseline AAS, 0.629 (95% CI 0.498-0.760); final AAS, 0.936 (95% CI 0.886-0.987); and Δ CRP, 0.796 (95% CI 0.696-0.897). Using receiver operating characteristic curves, we established the thresholds for low (AAS ≤ -2), intermediate (AAS -1 to 0), and high (AAS ≥ 1) probability of appendicitis. The negative predictive value for the low-probability group and the positive predictive value for the high-pr [...]

Long-term patient-reported outcomes and quality of life after tibiofemoral knee dislocations in patients with open growth plates: a retrospective case series from a Level 1 trauma center.

Hepp P, Youssef Y, Henkelmann R

BACKGROUND: Tibiofemoral dislocations in children and adolescents are rare, and treatment guidelines are primarily based on adult data and algorithms. This case series of four pediatric and adolescent patients aims to add to the limited literature and highlights key aspects of diagnosis, treatment, and patient-reported outcomes.

METHODS: We conducted a retrospective-prospective single-center case series of pediatric and adolescent patients (< 18 years, open growth plates) treated for tibiofemoral knee dislocations at a Level 1 trauma center between 2017 and 2019. Clinical, radiographic, and intraoperative data were reviewed. In addition, long-term patient-reported outcomes covering both functional recovery and health-related quality of life were assessed using KOOS, IKDC, and EQ-55 D-L.

RESULTS: Four patients (2 male, 2 female), aged between 12 and 16 years, were included in this case series. The mean follow-up was 7.3 years (range 6 to 8 years). All patients sustained Schenck Type III-L dislocations. Early MRI and vascular imaging identified frequent associated injuries such as meniscal and chondral lesions, avulsion fractures, in one case, peroneal nerve injury. A staged surgical approach was used in two of the four cases. No growth disturbances or deformities were observed in the patients based on the available clinical and radiological follow-up. Functional outcomes range [...]

The need for, feasibility of, and barriers to implementation of a trauma traveling fellowship: a survey of ESTES members.

Wohlgemut JM, Gupta S, Cioffi SPB, Llaquet-Bayo H, Wohlgemut HS, Hobby J, Mohseni S, Mariani D et al.

PURPOSE: Many European surgeons lack access to high-volume trauma training. A Trauma Traveling Fellowship, supported by the European Society of Trauma and Emergency Surgery (ESTES), could broaden educational and clinical exposure to trauma care, but feasibility and implementation factors are unclear. The aims of this survey are to assess the aspirations, expectations, and perceived challenges of prospective fellows and host centres for an ESTES Trauma Traveling Fellowship.

METHODS: A cross-sectional, web-based survey was distributed to all ESTES members, capturing demographics, fellowship aspirations, perceived barriers, and host centre capacities. The survey incorporated Likert scales, free-text responses, and was guided by the Consolidated Framework for Implementation Research (CFIR) and the Reach, Effectiveness, Adoption, Implementation, and Maintenance / Practical Robust Implementation and Sustainability Model (RE-AIM/PRISM).

RESULTS: Of 903 invited, 210 responses (23.3%) were analysed. The median age was 38 years; 71% were consultants, and 29% were residents. Most (71%) would consider applying for a Fellowship, and 47% stated their centre could host Fellows. Only 23% currently access funded fellowships. Key aspirations included hands-on participation in trauma care, although hosts could more often only guarantee observational roles. Major barriers included funding and in [...]

Antegrade transcystic balloon dilatation in emergency laparoscopic cholecystectomy with MRCP validation.

Lipping E, Saar S, Kirss J, Aaspõllu H, Bahhir A, Gapejeva O, Kangur T, Rauk M et al.

PURPOSE: Endoscopic retrograde cholangiopancreatography (ERCP) remains the predominant treatment of common bile duct stones but single-stage laparoscopic management via antegrade transcystic balloon dilatation (ATBD) offers a compelling alternative. All prior studies validate success solely by intraoperative fluoroscopic cholangiography, with no prospective trial employing postoperative magnetic resonance cholangiopancreatography (MRCP).

METHODS: This single-centre prospective study enrolled 100 consecutive patients with confirmed CBD stones requiring emergency laparoscopic cholecystectomy (LC). ATBD was attempted in all patients. Primary outcome was MRCP-confirmed CBD stone clearance. Secondary outcomes included operative time and postoperative complications.

RESULTS: One hundred consecutive patients were enrolled (median age 69 years; 38% male; 56% ASA III-IV). ATBD was performed in 88 patients (88%). Postoperative MRCP was performed in 86 of 88 patients (98%), with residual stones identified in 24 (28%; 95% CI 19.5-38.2%), corresponding to an MRCP-confirmed CBD clearance rate of 72% (95% CI 61.8-80.5%). The median operative time was 90 min (IQR 75-111.5). Among the 88 patients who underwent ATBD, postoperative complications occurred in 12 (14%), most commonly pancreatitis (4.5%) and deep surgical site infection (4.5%); one patient died following severe post-procedural panc [...]

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Closed reduction and percutaneous fixation versus open reduction and internal fixation for displaced medial malleolar fractures: a randomized controlled trial.

Elkohail A, Elkasaby A, Shaheen AMZ, Teama M, Raza ML, Ibrahim IA

OBJECTIVE: To evaluate the radiographic and functional outcomes of closed reduction and percutaneous fixation (CRPF) compared to open reduction and internal fixation (ORIF) in the treatment of isolated displaced medial malleolar fractures.

METHODS: This randomized controlled trial included 50 adult patients with isolated displaced medial malleolar fractures, Herscovici classification type B and C, across two hospitals. Participants were randomly assigned (1:1) to either CRPF (Group A, n = 25) or ORIF (Group B, n = 25). The principal objectives were radiological union and clinical function evaluated by the American Orthopedic Foot and Ankle Society (AOFAS) score at a 6-month follow-up. Secondary outcomes included the incidence of postoperative complications and correlations with patient demographics and mechanism of injury.

RESULTS: At 6 months, there was no significant difference between groups ($p > 0.05$); in Group A, 13 patients rated as "Good," 11 as "Fair," and 1 as "Poor," while in Group B, 15 were "Good," 9 "Fair," and 1 "Poor." AOFAS outcomes showed 13 excellent, 7 good, 4 fair, and 1 poor in Group A, versus 12 excellent, 9 good, 3 fair, and 1 poor in Group B, with no significant difference ($p > 0.05$). Complication rates were similar: wound infection (CRPF 4% vs. ORIF 12%), malunion/non-union (8% vs. 4%), hardware discomfort (12% vs. 20%), and deep venous thrombosis (4% [...])

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Long-term radiological and clinical outcomes after surgical treatment of non-union: a ten-year follow-up study.

Böpple JC, Berger J, Ferbert T, Bewersdorf TN, Schmidmaier G, Findeisen S

PURPOSE: Non-unions are a major challenge in trauma surgery. They are known to affect patients' lives for years after the initial trauma. Nevertheless, most studies only examine the first two years after treatment. The question what the long-term results look like ten years after surgery was posed in light of observations made in everyday clinical practice. To answer this, our study was designed to examine all patients who underwent non-union surgery in our department ten years ago.

METHODS: All patients who underwent surgery between July 2014 and October 2015 were invited to participate in the study. During a one-site appointment, an X-ray image was taken, which was then used to assess consolidation. The patients underwent a physical examination and answered a questionnaire, covering topics such as pain on a visual analogue scale (VAS) and overall quality of life with the short-form 12 (SF-12).

RESULTS: A total of 54 patients were included in the study. The median follow-up period was 122 months (IQR: 118-124). After 10 years 88.9% patients showed osseous consolidation with a median Lane-Sandhu-Score (LSS) of 4 (IQR: 3-4). The Wilcoxon signed-rank test revealed a significant difference compared to 24 months postoperatively ($Z = 4.716$, $p < 0.001$). The difference in the consolidation showed a marginally significant result in the McNemar test with $p = 0.063$. A total of 51 patient [...]

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Artificial intelligence for predicting 30-day mortality after emergency laparotomy.

Bedrikovetski S, Murshed I, Seow W, Bunjo Z, De Fontgalland JS, Traeger L, Vather R

PURPOSE: Accurate pre-operative risk prediction in emergency laparotomy (EL) is essential for appropriate resource allocation and improve patient outcomes. This study aimed to establish the accuracy of an artificial intelligence (AI) model in predicting 30-day mortality after EL using a national dataset.

METHODS: Data was extracted from the Australian and New Zealand Emergency Laparotomy Audit-Quality Improvement (ANZELA-QI) database from 1 July 2018 to 24 July 2023. AI models were created employing multilayer perceptron (MLP) and radial basis function (RBF) architectures. Extended AI models were developed using all available pre-operative variables and simplified AI models were developed using statistically significant variables identified through univariate analysis. We assessed the performance of AI models with that of a multivariate logistic regression (LR) and the UK-based National Emergency Laparotomy Audit (NELA) models using the area under the receiver operator characteristic curve (AUROC).

RESULTS: Data from 8293 ELs were included in the model and were randomly divided into a training dataset of 6619 (80%) and a testing dataset of 1674 (20%). There were 537 (6.5%) deaths 30 days after EL. In the testing dataset, the NELA model performed the best (AUROC 0.836), followed by the multivariate LR model (0.824), MLP simplified (0.817). MLP extended (0.802), RBF extended (0 [...])

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Predicting postoperative course in Fournier's gangrene surgery: evaluation of the E-PASS score in a single-center retrospective study.

Gercek O, Senkol M, Ulucak MA, Yazar VM

BACKGROUND: The E-PASS scoring system, which incorporates preoperative data and surgical stress factors, may provide a reliable risk classification for predicting mortality risk in patients with Fournier's gangrene. This study aimed to investigate the value of the E-PASS score in predicting mortality in patients with Fournier's gangrene.

METHODS: The study included 41 patients who were diagnosed with FG clinically and underwent surgery. Preoperative demographic and clinical data, laboratory values, FGSI, and E-PASS scores were recorded. The patients were divided into two groups: 8 (19.5%) patients who developed mortality and 33 (80.5%) patients who were discharged without mortality. Demographic, clinical, and laboratory data, as well as FGSI and E-PASS scores, were compared between the groups.

RESULTS: In the mortality group, CRP, serum creatinine levels, FGSI, E-PASS PRS, and E-PASS CRS scores were observed to be significantly higher, while serum albumin and hematocrit levels were lower compared to the other group (respectively, $p = 0.020$, $p = 0.001$, $p < 0.001$, $p < 0.001$, $p < 0.001$, $p = 0.001$, $p < 0.001$). FGSI, E-PASS CRS, CRP, and serum albumin values were found to have diagnostic value in predicting mortality. The cut-off values were determined as 7.5 for FGSI, 0.55 for E-PASS CRS, 186.25 for CRP, and 2.92 for albumin.

CONCLUSION: Our findings show that E-PASS can provide [...]

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Thoracoscopic internal fixation, open surgical internal fixation, and conservative treatment of rib fractures: differences in postoperative rehabilitation and chest volume (a randomized controlled trial).

Li T, Zhao P, Li W, Liu B

BACKGROUND: Currently, the standard treatment for multiple rib fractures is still under debate. This study explores the efficacy of three treatments (thoracoscopic internal fixation, open surgical internal fixation and conservative treatment) for multiple rib fractures and their influence on thoracic cavity volume and postoperative rehabilitation.

METHODS: Patients with multiple rib fractures admitted to Weifang People's Hospital of Shandong Province from March 2019 to March 2021 were enrolled in this study. The thoracic volume changes before and after treatment were measured by three-dimensional computed tomography (CT) reconstruction. Twenty-seven patients in the open operation group underwent internal fixation with the rib cage, and 24 patients in the non-internal fixation group underwent external fixation with the chest band. The recovery of multiple rib fractures was evaluated by

comparing the difference in thoracic volume before and after treatment, the incidence of related complications and the Comprehensive Assessment of Generic Quality of Life Inventory-74 (GQOLI-74) score one month after the operation.

RESULTS: After 12 months of follow-up, thoracic volume was significantly different between the thoracoscopic and open fixation groups ($P < 0.001$). One month after the thoracoscopic internal fixation of rib fractures, the scores of physical function, psychological func [...]

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What is the rate and degree of deformity in femoral diaphyseal nonunion? A CT-based retrospective cohort study.

Nabben J, Wagner RK, Groepenhoff F, Verwest TA, Bont Y, Kloen P, Janssen SJ

BACKGROUND: This study investigated the rate and degree of deformity in femoral diaphyseal nonunion using preoperative bilateral CT-scans.

METHODS: A retrospective cohort study was conducted at a single Level 1 trauma center. Patients aged > 18 years who underwent operative treatment for femoral diaphyseal nonunion between 2015 and 2025 and had preoperative bilateral femur CT-scans were included. Axial, sagittal, coronal, and length differences between the femoral nonunion and the contralateral healthy femur were measured. Torsional deformity $\geq 15^\circ$ and length discrepancy ≥ 20 mm were considered clinically relevant. Thresholds of $\geq 5^\circ$ and $\geq 10^\circ$ were used to identify outliers in coronal and sagittal plane deformity, respectively.

RESULTS: Thirty-two patients met the inclusion criteria (median age 57 years, 63% male). Torsional deformity $\geq 15^\circ$ was present in 10 patients (31%), including internal rotation in 4 (13%) and external rotation in 6 (19%). Sagittal plane deformity $\geq 10^\circ$ was observed in 5 patients (16%), with apex anterior angulation in 1 (3%) and apex posterior angulation in 4 (13%). Mean sagittal plane angulation was $9.7^\circ \pm 6.7^\circ$ in the affected limb and $12.5^\circ \pm 2.6^\circ$ in the unaffected limb ($p = 0.017$). Coronal plane deformity $\geq 5^\circ$ was present in 10 patients (31%), including varus in 8 (25%) and valgus in 2 (6%). Mean coronal angulation was $5.3^\circ \pm 6.0^\circ$ in the affected li [...]

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The impact of pre- versus postoperative geriatric consultation in elderly patients with hip fractures.

Aydin FB, Laane DWPM, van Rossum du Chattel A, DiStefano MT, King AH, Wignakumar T, Nwagbata A, van der Velde D et al.

PURPOSE: To determine whether preoperative geriatric consultation was independently associated with lower incidence of delirium for elderly patients with hip fractures compared with postoperative geriatric consultation.

METHODS: This retrospective cohort study was conducted at two urban Level 1 trauma centers in the United States. Patients aged ≥ 70 years with isolated proximal femur fractures treated surgically between 2018 and 2024 were included. Patients with pathological or periprosthetic fractures, non-orthopedic admissions, and those without geriatric consultation were excluded. Patients were categorized based on timing of initial geriatric consultation: preoperative versus postoperative. Delirium was selected as the primary outcome because geriatric consultation most directly addresses modifiable delirium risk factors. Secondary outcomes included the incidence of one or more in-hospital complications, length of stay, 30-day all-cause readmissions, and mortality. Multivariable logistic and linear regression analyses were used to determine the independent association between timing of initial geriatric consultation and primary and secondary outcomes.

RESULTS: Of 1776 patients included, 1382 (78%) received initial geriatric consultation preoperatively and 394 (22%) postoperatively. Median age was 84 years (IQR 78-90), 70% were female, 73% ASA III, and 87% lived at home pr [...]

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Volume-dependent detection of free intraperitoneal fluid by FAST: a prospective in-vivo study with sequential fluid instillation.

Güsgen C, Nada A, Kornek MT, Willms A, Schaaf S

PURPOSE: To assess the volume-dependent sensitivity of the Focused Assessment with Sonography for Trauma (FAST) and to analyse regional detection performance under controlled in-vivo conditions.

METHODS: In this prospective single-centre study, sequential intraperitoneal instillation of 0.9% NaCl (250-1500 mL) was performed during elective laparoscopic procedures in 35 adults. Standardised FAST examinations were conducted by two experienced sonographers using transverse and longitudinal views of Morison's pouch, the Douglas recess, and Koller's space. Sensitivity was calculated for each instilled volume and anatomical region. Specificity could not be assessed due to the experimental design with mandatory fluid presence. Regional differences were tested using one-way ANOVA, and a Monte Carlo simulation (1000 resamples) evaluated detection stability across body-mass-index (BMI) groups.

RESULTS: FAST sensitivity increased with volume: 14.3% (95% CI 4.8-30.3) at 250 mL, 37.1% (21.5-55.1) at 500 mL, 74.3% (56.7-87.5) at 1000 mL, and 91.4% (76.9-98.2) at 1500 mL. The highest detection rate occurred in Morison's pouch, followed by the Douglas and Koller recesses. Longitudinal scanning of the Douglas pouch improved sensitivity by > 20% compared with the transverse view. ANOVA confirmed significant regional variability ($F(5, 153) = 4.37$, $p = 0.001$, $\eta^2 = 0.125$). Detection rates decline [...]

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When demography challenges policy: the rising burden of proximal femur fractures as a role model and driver for geriatric trauma care in Germany.

Klug A, Münzberg M, Sibbel R

PURPOSE: Germany's hospital reforms will substantially reshape service delivery and capacity planning. Proximal femur fractures (PFFs), the most frequent geriatric fractures, are highly sensitive to demographic aging and therefore serve as a model condition to assess future healthcare system requirements.

METHODS: Nationwide inpatient data from the German Federal Statistical Office (2000-2023) were analyzed for proximal femur fractures (ICD-10-GM S72.0-S72.2). Demographic projections up to 2050 were combined with Poisson regression, exponential smoothing, and autoregressive integrated moving average models to forecast future incidence and case numbers. Model performance was evaluated in a hold-out period (2016-2023) using mean absolute percentage error and root mean square error. Direct costs were estimated using the official German cost-of-illness framework. Staffing and hospital bed requirements were derived from case-based activity times, profession-specific full-time equivalent coefficients, and observed department-specific occupancy rates.

RESULTS: Between 2000 and 2023, annual proximal femur fracture cases in Germany increased from 118,243 to 182,447 (+ 54.3%). Forecasts project a rise to 317,877 cases by 2050 (95% CI 262,436-373,317), corresponding to an increase of approximately 75% compared with 2023. Patients aged ≥ 80 years are projected to account for 74% of all c [...]

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Orbital floor fractures: clinical characteristics, mechanisms of injury, and surgical outcomes in a 25-year single-center study.

Kasap ■, Aydin MS, Özler G, Ni■anc■ M, Özvurmaz O

PURPOSE: Orbital floor fractures are common injuries following blunt midfacial trauma, presenting as isolated defects or as part of complex craniofacial fractures. This study aimed to evaluate clinical characteristics, fracture patterns, management strategies, and postoperative outcomes in surgically treated patients.

METHODS: This retrospective single-center study included 56 patients who underwent surgical treatment for orbital floor fractures between 2000 and 2025. Demographic data, injury mechanisms, imaging findings, associated fractures, clinical presentation, surgical techniques, and outcomes were analyzed. Minimum follow-up was 3 months. Statistical analysis was performed using IBM SPSS Statistics version 25.0, with $p < 0.05$ considered significant.

RESULTS: Among 56 patients, 80.4% were male, with a mean age of 38.6 years (10-70). Assault (44.6%) was the most common cause, followed by in-vehicle accidents (21.4%) and falls (17.9%). Inferior wall fractures were most frequent (55.4%), followed by medial (28.6%) and lateral (16.1%) involvement; no superior fractures were observed. Common findings included periorbital edema/ecchymosis (75.0%), diplopia (26.8%), and gaze limitation (19.6%); infraorbital hypoesthesia occurred in 12.5%. Associated fractures involved the maxilla (60.7%) and

zygoma (46.4%).Surgical treatment included exploration/elevation (46.4%), titanium mes [...]

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Epidemiology and healthcare burden of implant-associated complications in Germany: a nationwide population-based analysis.

Anees H, Heiss C, El Khassawna T

PURPOSE: Implant-associated complications remain a major challenge in orthopaedic and trauma surgery, particularly among elderly patients requiring revision procedures or infection management. This study analysed nationwide trends and healthcare burden of implant-associated complications in Germany.

METHODS: Nationwide German hospital discharge data from 2000 to 2023 were analysed and complemented by Diagnosis-Related Group (DRG) data from 2024. Cases were identified using ICD-10-GM code T84 and its subcategories. Outcomes included temporal trends, age-standardised incidence, demographic characteristics, in-hospital mortality, length of stay (LOS), and DRG-based healthcare resource utilisation.

RESULTS: Hospitalisations for implant-associated complications increased from 45,121 in 2000 to a peak of 95,154 in 2011, followed by a decline to 72,846 in 2023. Age-standardised incidence decreased by approximately 31% between 2010 and 2023. Segmented regression identified a temporal breakpoint in 2010.6, with annual percentage changes of + 6.21% before and - 3.55% after the breakpoint. Annual hospitalisations for periprosthetic joint infection increased from 5,367 to 11,215, while osteosynthesis-associated infections increased from 2,469 to 7,059. Mean LOS decreased from 20.8 to 11.8 days, whereas in-hospital mortality remained stable. Compared with low-complexity cases, infection-r [...]

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Effect of periprosthetic greater trochanter fractures (Vancouver AG) on primary stability of the femoral stem in total hip arthroplasty: a biomechanical micromotion analysis.

Bruder J, Goronzi M, Cavalcanti Kußmaul A, Wulf J, Pachmann F, Holzapfel BM, Böcker W, Kistler M

PURPOSE: Primary stability is a crucial factor for the long-term success of cementless total hip arthroplasty (THA). Periprosthetic fractures represent an important cause of loss of primary implant stability. While Vancouver AG fractures rarely lead to stem loosening, their effect on the metaphyseal anchoring of the stems has not yet been investigated. The present study aimed to evaluate the influence of a Vancouver AG fracture on the primary stability of the CLS Spotorno stem.

METHODS: Six fourth-generation composite femora were implanted with cementless CLS Spotorno stems (Zimmer, Warsaw, USA). Primary stability was determined by three-dimensional digital image correlation under dynamic loading conditions (300-1700 N, 1 Hz) simulating physiological gait. Micromotion was recorded at six standardized measurement points (proximal, mid, distal, medial and ventral) before and after a simulated Vancouver AG fracture. Data was statistically analyzed using paired one-sided t-tests ($\alpha = 0.05$).

RESULTS: The results revealed no significant increase in micromotion at the primary anchorage zone of the stem. However, a non-significant increase in micromotion was observed at the prosthesis tip (DGventral and DGmedial), where the micromotion values increased from 164.4 μm ($\pm 59.6 \mu\text{m}$) and 140.4 μm ($\pm 75.6 \mu\text{m}$) to 193.1 μm ($\pm 48.8 \mu\text{m}$) and 186.9 μm ($\pm 60.5 \mu\text{m}$), respectively.

CONCLUSION: Vanco [...]

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Injury patterns, management and outcomes of hemodynamically unstable pelvic fractures at an Australian level 1 trauma centre: a 10-year registry analysis.

Louw L, Raubenheimer K, Blakeney WG, Young S, Balogh ZJ, Weber DG

INTRODUCTION/BACKGROUND: Pelvic ring injuries (PRI) in traumatic shock are associated with over 30% mortality even in high income countries' mature trauma systems, but recent Australian data showed no haemorrhage related mortality in this group. We aimed to describe the epidemiology, management and outcomes of PRI with haemodynamic instability managed in a state's only Level 1 adult trauma centre.

DESIGN: We conducted a retrospective cohort study of all trauma patients with major PRI (Abbreviated Injury Scale (AIS) > 3) and hemodynamic instability (defined as systolic blood pressure (SBP) <90 mmHg or a shock index (heart rate (HR)/SBP) > 0.7) presenting to the state adult trauma centre between 2013 and 2022. Data on demographics, injury patterns, management strategies, and outcomes were collected from the State Trauma Registry and hospital records.

RESULTS: Of 1369 patients with pelvic or acetabular fractures during the 10-year study period, 152 (11%) had major PRI with hemodynamic instability at presentation. Most were male (72%) with a median age of 35 years. Motor vehicle and motorbike crashes accounted for 52% of injuries. The median Injury Severity Score was 33. Massive transfusion protocols were activated in 45% of cases, while angioembolisation was utilised in 5.9%. Overall in-hospital mortality was 7.2% and mortality due to haemorrhage was 2%.

CONCLUSIONS: In our mat [...]

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Time to surgery and its impact on length of stay and mortality for neck of femur fractures at a major trauma centre.

Tang LY, Ho SYS, Zhou AK, Geetala R, Krkovic M

PURPOSE: Neck of femur (NOF) fractures are a major public health concern. Timely surgical intervention (current guidelines is within 36 h) is associated with improved survival, shorter hospital stays, and better functional outcomes. However, surgical delays remain common in hospitals, especially in major trauma centres, and their impact on length of stay (LOS) and mortality is not well defined across different NOF fracture subtypes.

METHODS: We conducted a retrospective cohort study of 4415 patients with NOF fractures, classified into intracapsular (n = 2507) and extracapsular (n = 1908). The primary outcome data gathered were LOS (from surgery to discharge) and in-hospital mortality. Regression analyses were performed to assess associations between LOS, and mortality, adjusting for age, sex, Charlson Comorbidity Index (CCI), American Society of Anesthesiologists (ASA) score, intra-operative blood transfusion, and surgical methods.

RESULTS: For intracapsular fractures, delays in time to surgery were significantly associated with increased LOS in patients expected to have short to median lengths of stay, after adjusting for other variables ($p < 0.05$). Delayed surgery was associated with increased mortality for both intracapsular and extracapsular fractures ($p < 0.0001$). Survival curves showed significant differences for the surgical delayed group. After controlling for time to [...]

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Pattern of blast injuries. A Systematic Review: Part 1-Improvised, multiple and unspecified explosive devices.

Neubert A, Seidelmann M, Gaeth C, Bieler D

PURPOSE: This systematic review aims to comprehensively identify and categorize the specific injury patterns associated with IEDs and other explosive devices across military and civilian populations to improve medical preparedness, resource planning strategies, and treatment approaches.

METHODS: A search on PubMed und Scopus was conducted on 26. March 2025 for observational studies reporting blast injuries in civilian and military populations of all ages. Studies were clustered by explosion type - Part 1 IED and multiple/unspecified explosives. A narrative data synthesis was performed. Reporting was assessed with the JBI tool for case series.

PROSPERO: CRD42023391736.

RESULTS: 33 studies with 4,373 victims (mainly military personnel) from 2002 - 2022 reported injuries caused by IEDs. The average age was 25.1 years. 97.2% were male. Injuries to the upper and lower extremities were predominant in all study populations followed by head injuries among military personnel, and thorax and abdomen injuries among civilians. 37 studies with 24,600 casualties (mean age 25.7, 85.8% males) were included in multiple/unspecified explosives. Extremities were predominant followed by facial injuries among military personnel, and thorax injuries among civilians.

CONCLUSION: The first part of this systematic review on the pattern of blast injuries shows that injuries to the extremities, includ [...]

Clinical and laboratory predictors of mortality in Fournier's gangrene: prognostic role of the HALP score.

Yavuz A, Ersöz V, Çamöz MS, Yurdakul ZN, ■■■deci Tut G, Demirci BF, Koçsoy NS, Dönmez M

OBJECTIVE: Fournier's gangrene (FG) is a rapidly progressive necrotizing soft-tissue infection associated with substantial mortality. This study evaluated the prognostic performance of the Hemoglobin-Albumin-Lymphocyte-Platelet (HALP) score and routinely available clinical and laboratory parameters in patients with FG.

METHODS: This retrospective cohort study included 236 consecutive patients who underwent surgical treatment for FG between February 2019 and May 2025. The primary outcome was in-hospital mortality. Prognostic factors were evaluated using univariate and multivariable logistic regression analyses, and discriminative performance was assessed by receiver operating characteristic (ROC) curve analysis.

RESULTS: The overall in-hospital mortality rate was 12.3%. Non-survivors had significantly lower HALP scores, hemoglobin, hematocrit, albumin, lymphocyte, and monocyte levels (all $p < 0.05$). In univariate analysis, younger age, disease confined to the perianal region, and higher albumin, monocyte, hemoglobin, hematocrit, and HALP values were associated with survival. In the primary multivariable model, advanced age (OR = 0.966, 95% CI: 0.936-0.997; $p = 0.030$) and monocyte count (OR = 5.243, 95% CI: 1.055-26.049; $p = 0.043$) remained independent predictors of survival. Hemoglobin showed the highest discriminative performance (AUC = 0.781), followed by albumin (0.767), he [...]

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Surgical stabilization of rib fractures is associated with lower adjusted odds of adverse pulmonary outcomes across RibScore-defined injury severity: a single-center cohort study.

Muldiyarov V, Liu JL, Campbell A, Bouldoukian S, Daro A, Kemp SE, Matos M, Cantrell E et al.

PURPOSE: Surgical stabilization of rib fractures (SSRF) is increasingly used for selected patients with severe chest wall injury, but its relationship with pulmonary outcomes after accounting for RibScore-defined injury severity remains unclear. This study evaluated adverse pulmonary outcomes after SSRF versus nonoperative management (NONOP) among adults with traumatic rib fractures.

METHODS: Single-center retrospective cohort study at an ACS-verified Level I trauma center from January 2016 to April 2023. Adults with CT-confirmed blunt traumatic rib fractures were categorized as SSRF or nonoperative management. RibScore was calculated from initial CT imaging. The primary outcome was a composite of pneumonia, invasive mechanical ventilation for more than 48 h, or tracheostomy. Multivariable logistic regression adjusted for demographics, RibScore, thoracic and extra-thoracic injury severity, calendar year, COPD, and current smoking status.

RESULTS: Among 3,066 patients, 444 underwent SSRF and 2,622 were managed nonoperatively. SSRF patients had greater radiographic chest wall injury severity, including a higher median RibScore than NONOP patients (2 vs. 0; $p < 0.001$). Composite adverse pulmonary outcome occurred in 14.6% of SSRF patients and 16.6% of NONOP patients. In adjusted analysis, SSRF was associated with lower odds of composite adverse pulmonary outcome (aOR 0.57, 95% C [...]

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Physician-staffed prehospital units for severely injured patients in the Stockholm trauma system - is there a survival benefit?

Helmersson K, Lapidus O, Alm T, Bäckström D, Rubensson Wahlin R

BACKGROUND: The role of prehospital physicians for patients with major trauma has previously been examined with mixed results. No studies have assessed the survival benefit of physician-staffed prehospital units in the context of the Stockholm trauma system.

AIM: To investigate the association between exposure to prehospital physicians and mortality for severely injured trauma patients in the Stockholm trauma system.

METHODS: A total of 11,063 trauma patients were obtained from the Swedish Trauma Registry and stratified according to physician exposure in the prehospital setting. Logistic regression was used to evaluate the association between prehospital physician exposure and 30-day mortality and Glasgow Outcome Score (GOS).

RESULTS: No significant reduction in 30-day mortality was observed in the P-EMS group compared to the EMS group (OR 1.06, $p = 0.71$). However, transport with helicopter emergency medical services (HEMS) was independently associated with greater odds of survival. The P-EMS group had significantly lower GOS at discharge compared to EMS (OR 1.72, $p < 0.001$).

CONCLUSION: The potential benefit of prehospital physician exposure for major trauma patients remains uncertain. However, HEMS transport was associated with improved survival. Further research is needed to determine which patients benefit from prehospital physicians and to evaluate the optimal allocation [...]

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Clinical impact and economic burden of electric scooter-related facial fractures in Barcelona.

Lugilde-Guerbek A, López-Masramon B, García Sánchez V, Andreu-Sola V, Collado-Delfa JM, Pueyo Morillo C, Arteaga A, Grabosky-Elbaile A et al.

PURPOSE: The landscape of facial trauma has evolved with the integration of micromobility devices into urban environments. This study aimed to characterize the clinical profile of electric scooter (e-scooter) accidents compared with traditional etiologies (traffic, assaults, falls) and evaluate the economic burden imposed on the public health system.

METHODS: A cohort of 119 patients who underwent surgery for facial fractures at a Spanish tertiary hospital between 2020 and 2024 was analyzed. To accurately assess economic impact, a specific subgroup of 51 "pure facial fractures"-those with isolated injuries-was utilized for cost analysis.

RESULTS: E-scooter users (10.1% of the total cohort) were significantly younger (mean age 27.9 years, $p = 0.044$), had shorter hospital stays (7.7 days, $p = 0.007$), and lower ICU requirements (16.7%) than the traditional group. Adverse outcomes accounted for 29.4% of cases overall, with a significantly higher rate in the e-scooter group compared to other etiologies (58.3% vs. 26.2%; $p = 0.039$). The median cost for an e-scooter injury was €8,092; though higher than other causes, this was not statistically significant ($p = 0.33$).

CONCLUSION: E-scooters generate complex facial injuries with clinical severity and complication rates comparable to high-energy trauma. The high economic cost and reconstructive complexity, suggest a need for stricter [...]

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Significant other reported outcome measures within European cohorts of severely injured patients: A systematic review.

Van Ditshuizen JC, Berg LVD, Verhofstad MHJ, Leliefeld PHC, Jongh MAC, Hartog DD

PURPOSE: Major trauma can have a long-term impact, leading to diminished health-related quality of life (HRQoL) for patients and their significant others (SOs). Patient reported outcome measures (PROMs) make this impact visible, providing expectation- and self-management. European studies assessing Significant Other Reported Outcome Measures (SOROMs) in cohorts of severely injured patients were reviewed.

METHODS: A systematic literature search was conducted. European studies from 2000 onwards assessing SOROMs in severely injured patient cohorts were included. Two reviewers independently screened all records, consensus was reached by a third reviewer.

RESULTS: 2,909 studies were screened, resulting in 30 inclusions. Studies originated from Western and Northern Europe, overrepresented by traumatic brain injury ($n = 27$, 90%). Forty-three different SOROMs were identified. Twenty (67%) studies reported subacute and long-term reduction in HRQoL, and high prevalence (28-56%) of depression, anxiety, and stress within SOs. Outcomes of SOs and patients run parallel and are associated with coping strategies. Sixteen (53%) studies reported around 50% of unmet family needs (information and emotional support), and a disproportionate burden of care associated with loneliness and life satisfaction.

CONCLUSION: SOs of severely injured patients reported reduced HRQoL, with high levels of depr [...]

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Early treatment goal uncertainty as a barrier to regional analgesia and respiratory deterioration in traumatic rib fractures: a propensity-weighted analysis.

Yi SB, Wu JH, Zhang XN, Lin DC, Zheng YL

PURPOSE: Patients with multiple rib fractures (≥ 3) admitted to the intensive care unit (ICU) are at high risk of early respiratory deterioration, for which timely and effective analgesia is essential. Although multimodal analgesic strategies are widely recommended, their real-world implementation may be influenced by variability in early clinical decision-making, including treatment goal uncertainty (TGU). We aimed to investigate the association between early TGU, analgesic strategy selection, and respiratory outcomes in critically injured patients with rib fractures.

METHODS: This single-center observational cohort study included 236 adult ICU patients with ≥ 3 radiologically confirmed rib fractures who did not require invasive mechanical ventilation at admission (2020-2024). TGU within the first 48 h was assessed using blinded electronic medical record review as a process-level marker of variability in early care prioritization. Analgesic strategies were categorized as regional-based analgesia (RA), systemic-dominant analgesia (SA), or multimodal conservative management. The primary outcome was respiratory deterioration, defined as a composite of escalation of respiratory support, endotracheal intubation, or pulmonary complications requiring intervention. Associations were evaluated using propensity score overlap weighting and adjusted regression models.

RESULTS: High TGU [...]

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Multifocal necrotizing fasciitis: what makes it different? A German trauma center experience.

Keßler F, Frank J, Störmann P, Wagner N, Saman AE, Marzi I, Leiblein M

BACKGROUND: Multifocal necrotizing fasciitis (MulNF) is a rare presentation of necrotizing fasciitis (NF) characterized by involvement of more than one non-contiguous anatomical site during the same disease episode. Comparative data between monofocal and multifocal NF remain limited.

METHODS: We performed a retrospective single-center cohort study of patients treated for NF between 2015 and 2026. Patients were classified as MNF (single site) or MulNF (multiple non-contiguous sites) based on chart review. Demographics, comorbidities, microbiology, laboratory values, organ failure, ICU course, surgical burden, and outcomes were compared.

RESULTS: Forty-seven patients were included, of whom five (10.6%) had multifocal NF. Four of these showed clearly synchronous non-contiguous involvement, whereas one developed a delayed second non-contiguous focus within the first four days of the same disease episode. Compared with MNF, MulNF was associated with higher ICU burden, more frequent mechanical ventilation, and a greater number of debridements and total operations. Higher rates of septic shock, hemodynamic failure, and renal replacement therapy were observed in MulNF. No deaths occurred in the small multifocal cohort, whereas 11/42 patients with monofocal NF died during hospitalization. Given the small multifocal sample size and multiple comparisons performed, all findings should be [...]

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Trauma-related retrobulbar hematoma: predictive risk factors and clinical outcome in 52 cases.

Bicsák Á, ■ahin H, Brandt L, Hassfeld S, Bonitz L

PURPOSE: Retrobulbar hematoma (RBH) is a vision-threatening emergency. This large-scale study integrates clinical data and 3D analysis to identify prognostic factors, refine diagnosis, and support guideline-based emergency management.

METHODS: This study standardizes terminology, applies AO fracture classification, and uses CT-based 3D segmentation to analyze anatomical patterns, treatment protocols, providing evidence-based guidance for diagnosis, surgical management, and postoperative care.

RESULTS: The Dortmund Maxillofacial Trauma Registry (2018-2025) recorded 52 RBHs among 7,671 trauma patients (0.7%). RBH occurred predominantly in middle-aged males and elderly females, with overproportionate incidence of anticoagulant therapy. Most fractures clustered around the orbital floor, medial wall, zygomatic surface, often near the Frankfurt horizontal plane. RBHs were mainly extraconal; poor outcome was predicted with intraconal location, optic nerve contact, orbital floor fracture, high RBH/orbital volume ratio, and aspirin use. Surgical decompression was performed in 15 cases (28.9%); 65.4% (34 of 52) retained vision. Neural network analysis confirmed fracture site and hematoma position as key prognostic factors over demographic or comorbidity variables.

CONCLUSION: In 7,671 midface fracture cases, RBHs occurred in 0.7% (n = 52), mainly after trauma. Falls predominated in ae [...]

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Patterns of negative trauma laparotomy worldwide: a sub-analysis of a global dataset.

Laird WLA, Bath MF, Amoako J, Wohlgemut J, Nuño-Guzmán CM, Khajanchi M, Smith BG, Hobbs L et al.

PURPOSE: Trauma remains a major contributor to global disability. While the trauma laparotomy can be a life-saving procedure in abdominal injury, a select proportion are performed with no pathology identified intra-operatively, potentially exposing patients to unnecessary morbidity. We aimed to assess the global prevalence of negative laparotomy and identify any influencing factors at both the patient- and system-level.

METHODS: This was a secondary analysis of the GOAL-Trauma study, a multicentre prospective international observational study on trauma laparotomy patients, conducted from April to December 2024. We defined a negative laparotomy as where no intra-abdominal injuries were identified during the index operation. A multivariable logistic regression was performed to identify predictive factors for a negative laparotomy.

RESULTS: Of the 1769 patients included, 128 patients (7.2%) underwent a negative laparotomy. Regression analysis demonstrated that a penetrating mechanism of injury (OR 2.37, CI:1.53-3.69, $p < 0.001$) and a lower grade of surgeon (surgical registrar relative to consultant - OR 2.91, CI:1.90-4.47, $p < 0.001$) were independent predictors for negative laparotomy. Neither human development index nor hospital resource level impacted negative laparotomy rates.

CONCLUSION: One in every fourteen trauma laparotomies performed in our global cohort was negative, [...]

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Preoperative stay under 48 h after admission for proximal femur fractures is linked to fewer complications and better outcomes.

Moskal M, Feulner S, Grimm A, Müller-Heck R, Heep H, Godolias P

PURPOSE: To evaluate the association between admission-to-surgery time (with a focus on < 48 h) and postoperative complications and discharge mobility in patients with proximal femur fractures (PFFs), using a large administrative dataset from routine inpatient care in Germany.

METHODS: In this retrospective database analysis, 128,092 patient records from external inpatient quality assurance (North Rhine-Westphalia, Germany, 2016-2020) were included. All patients underwent surgical treatment for PFFs and were stratified by time to surgery as early surgery (Group A; within 48 h of admission) or delayed surgery (Group B; more than 48 h after admission). Two cohorts were analysed separately: hip arthroplasty (HA: early 57,635; delayed 7,504) and osteosynthesis (OST: early 59,466; delayed 3,487). Associations between admission-to-surgery time, postoperative complications, pre-admission anticoagulant therapy and inability to walk at discharge were assessed using descriptive statistics and binary logistic regression. Patients unable to walk before the fracture were excluded from the mobility analysis. Reporting was guided by the STROBE statement.

RESULTS: In-hospital mortality was 4.7% after early versus 9.7% after delayed surgery (OST) and 5.8% versus 9.3% (HA). Delayed surgery was associated with higher odds of postoperative complications (OST: OR 1.21, 95% CI 1.11-1.34; HA: OR 1. [...])

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Impact of routine use of the GFAP and UCH-L1 TBI blood test for management of mild TBI on patient care in a European emergency department.

Brüning-Wolter F, Wolff N, Schrader M, Musaelyan K, Chandran R, Hoffmann M, Marino JA, McQuiston B et al.

OBJECTIVES: Blood biomarkers glial fibrillary acidic protein (GFAP) and ubiquitin C-terminal hydrolase L1 (UCH-L1) were recently introduced into clinical practice for mild traumatic brain injury (mTBI) assessment. However, real world data demonstrating TBI biomarkers' impact on the emergency department (ED) practice is lacking. This study aimed to describe the experience of a single tertiary hospital ED that introduced TBI biomarkers GFAP and UCH-L1 measurements using Alinity i® TBI test into routine mTBI assessment.

METHODS: Retrospective data were extracted from hospital records of patients who had TBI test ordered in the ED. Test results and subsequent clinical care pathway data (CT order, CT scan result, admission or discharge) were analysed.

RESULTS: Data was available for 460 mTBI patients (median age 43, 54% male). Of the 216 patients (47%) with a negative TBI test, 198 did not have a CT scan, and 18 had a CT scan with no abnormalities detected. Among 244 patients with a positive TBI test, 162 had CT scans, with 12 (6.6%) showing intracranial lesions. Overall, among all 180 scanned patients every patient with intracranial lesion findings had a positive TBI test result. The test could rule out CT scans in 47% of mild TBI patients. In practice, 42% of patients avoided a CT scan based on the TBI test results with additional 12% of patients ruled out based on clinical asse [...]

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Electric bicycle versus conventional bicycle crashes: comparative analysis of injury patterns, severity, and the modifying effect of body mass index.

Bauer D, Maier JP, Holzfurtner L, Pietsch C, Wagner FC, Schmal H, Erdle B, Mühlenfeld N

PURPOSE: E-bike trauma is often presumed to result in greater injury severity than conventional cycling, yet evidence remains inconsistent. Furthermore, the impact of factors such as Body Mass Index (BMI) on injury patterns is poorly understood. This study therefore evaluated whether e-bike trauma is associated with increased overall severity or distinct regional injury patterns.

METHODS: This retrospective single-center cohort study analyzed 261 adult trauma-bay patients (e-bike n = 37; conventional bicycle n = 224). Primary endpoints included Injury Severity Score (ISS) and regional maximum Abbreviated Injury Scale (AIS) scores. Secondary endpoints included physiological burden (SAPS II) and resource utilization (LOS, TISS-10). Multivariate logistic regression was used to identify independent predictors of intracranial hemorrhage, e-bike riding and extremity fractures. Event counts and events-per-variable (EPV) ratios are reported for each model.

RESULTS: E-bike riders had a significantly higher BMI than conventional cyclists (26.1 vs. 24.6 kg/m²; p = 0.029). However, the absolute difference of approximately 1.5 kg/m² is modest, and both group medians fall within the normal-to-overweight range. No statistically significant differences were observed in ISS, physiological burden, or resource utilization. Conventional cyclists had a higher proportion of patients with AIS Pelvi [...]

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Effect of management in intermediate care unit versus conventional hospitalization on long-term mortality and autonomy in patients with rib fractures aged 75 years and older: a retrospective study.

Vallod E, Debord-Peguet S, Mikail N, Termoz A, Tripoz S, Abrard S, Gueret-du-Manoir M

PURPOSE: Rib fractures in older adults are associated with substantial morbidity and mortality. Management in an intermediate care unit (IMCU) may improve outcomes, but its long-term benefit remains uncertain. We compared IMCU versus conventional management regarding 12-month mortality and autonomy in patients aged ≥ 75 years with rib fractures.

METHODS: We conducted a retrospective observational cohort study in two Lyon teaching hospitals (Hôpital Edouard Herriot and Hôpital Lyon Sud, Hospices Civils de Lyon, France) between January 2015 and December 2019. The primary outcome was a composite of death or institutionalization within 12 months of hospital admission. Secondary outcomes included discharge modality, opioid prescription at discharge, hospital length of stay,

unplanned readmission within 6 months, and ADL/IADL scores. Logistic regression was adjusted for hospitalization unit, age, sex, pre-existing institutionalization, and associated injuries.

RESULTS: A total of 196 patients were included: 126 (64%) in the conventional group and 70 (36%) in the IMCU group. Death or institutionalization within 12 months occurred in 31 patients (26%) in the conventional group and 8 (12%) in the IMCU group ($p = 0.026$). However, after adjustment, conventional hospitalization was not significantly associated with the primary outcome (OR 2.25, 95% CI 0.90-6.09; $p = 0.092$). Institutional [...]

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The road to the Pearl-String Technique: lessons learned from early IMT modifications using RIA autograft and cancellous allograft granules.

Hückstädt M, Scholl C, Langwald S, Klauke F, Mendel T, Kobbe P, Fischer C

BACKGROUND: The induced membrane technique (IMT) is widely used for reconstruction of large bone defects; however, reported outcomes vary considerably among technical modifications. One potential mechanism impairing consolidation is graft sedimentation, particularly when low-density cancellous autograft harvested using the reamer-irrigator-aspirator (RIA) is applied. This study evaluates a standardized modification of the IMT and explores mechanical factors potentially affecting graft consolidation.

METHODS: In this retrospective single-center study, 28 patients were treated following eradication of osteomyelitis using a defined modification of the IMT. Mean follow-up was 7.4 years. Defect length ranged from 3.2 to 15.7 cm. During the second stage, RIA-harvested autologous cancellous bone was combined with lyophilized cancellous bone allograft granules. Definitive fixation was achieved using intramedullary nailing or locking plate fixation; one patient was treated with a ring fixator.

RESULTS: Primary consolidation was achieved in 57.1% of cases. Aseptic nonunion occurred in 10.7%, with secondary consolidation achieved after revision in all affected patients. Recurrent infection was observed in 35.7%; infection control was achieved in 90% of these cases following additional procedures, and reconstruction was ultimately completed using alternative strategies. Median time to fu [...]

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Non-tourniquet strategy with high-position low-perfusion in distal humeral fracture surgery: reduces swelling without increasing blood loss.

Wang Z, Ai X, Ren T, Wu B, Zhang K, Li Z, Xue H, Li M et al.

INTRODUCTION: To evaluate the feasibility of non-tourniquet strategy combined with "High-position Low-Perfusion" positioning in distal humeral fractures ORIF (DHF-ORIF), and to investigate its effects on perioperative blood loss, postoperative rehabilitation, and functional outcomes.

METHODS: Prospective inclusion of patients aged 18-45 with distal humeral fractures, randomly divided into an experimental group without tourniquets (Group A) and a control group with traditional tourniquets (Group B). The Group A adopted a lateral position with high and low perfusion of the affected limb, and received intravenous injection of TXA (15 mg/kg) combined with continuous flushing with 4 ■ physiological saline during the operation; The control group received routine use of tourniquets.

PRIMARY OUTCOMES: perioperative blood loss, elbow ROM and degree of limb swelling.

SECONDARY OUTCOMES: Postoperative VAS score, Mayo elbow joint score and DASH score, and incidence of complications.

RESULTS: No significant difference was observed in total perioperative blood loss between the Group A and the Group B (240.78 mL vs. 228.53 mL, $p > 0.05$). However, the Group A demonstrated significantly reduced postoperative drainage volume and hidden blood loss ($p < 0.05$). The Group A exhibited significantly superior outcomes in terms of VAS scores and limb swelling, compared with the control group. Additi [...]

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Impacts of intracranial pressure monitoring on mortality and length of stay in severe traumatic brain injury: a systematic review and meta-analysis.

Merakis M, Kim N, Velli G, Balogh ZJ

PURPOSE: This systematic review and meta-analysis examined the association between intracranial pressure monitoring (ICPm) and mortality in patients with severe traumatic brain injury (sTBI), with additional focus on functional neurological outcomes and intensive care unit (ICU) and hospital length of stay (LOS).

METHODS: This review followed Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) guidelines and was registered with PROSPERO (CRD42025643607). MEDLINE, Embase, and the Cochrane Central Register of Controlled Trials were searched through August 1, 2025, for studies comparing ICP-monitored and non-ICP-monitored patients with traumatic brain injury. Eligible study designs included randomized controlled trials, prospective observational cohort studies, retrospective observational cohort studies, registry-based cohort studies, and case-control studies. Study quality was assessed using the Strengthening the Reporting of Observational Studies in Epidemiology checklist, Risk of Bias 2.0 for randomized trials, and the Oxford Centre for Evidence-Based Medicine Levels of Evidence framework.

RESULTS: Thirty-one studies met inclusion criteria. The mortality meta-analysis included 121,799 patients. Pooled crude analysis showed lower mortality in ICP-monitored patients than in non-monitored patients (OR 0.79, 95% CI 0.68-0.91, $p = 0.002$), with high heteroge [...]

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Prehospital point-of-care lactate for identification of severe injury in trauma team-activated patients: a prospective multicenter cohort study.

Faul P, Hagebusch P, Bertram E, Naujoks F, Störmann P, Schweigkofler U, Koch D

BACKGROUND: Accurate prehospital trauma triage remains challenging despite structured activation criteria, particularly in patients without overt physiologic instability. Prehospital point-of-care lactate has been proposed as a physiologic adjunct to support early identification of severely injured patients. This prospective multicenter cohort study evaluated the association between prehospital lactate and injury severity in trauma team-activated patients within a structured guideline-based trauma system.

METHODS: This prospective observational multicenter cohort study included adult trauma patients transported by emergency medical services to three Level I trauma centers and managed in the trauma room following trauma team activation (TTA). Prehospital lactate was measured by capillary earlobe sampling before or during transport and was not used for clinical decision-making. The primary outcome was severe injury, defined as Injury Severity Score (ISS) ≥ 16 . Secondary outcomes were Abbreviated Injury Scale (AIS) ≥ 4 in at least one body region and intensive care unit (ICU) admission > 24 hours. Diagnostic performance was assessed using receiver operating characteristic (ROC) analysis and predefined lactate thresholds of ≥ 2 mmol/L and ≥ 4 mmol/L.

RESULTS: A total of 108 patients were included, of whom 26 (24.1%) had ISS ≥ 16 . Prehospital lactate levels were higher in severely inj [...]

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Surgical timing and mortality in acute traumatic subdural hematoma: is the four-hour threshold confounded by injury severity?

Rakip U, Yildizhan S, Canbek ■, Boyac ■ MG, Korkmaz S, Öztürk KH, Aslan MZ, Aslan A

PURPOSE: Early surgical evacuation within four hours has long been advocated for acute traumatic subdural hematoma (ASDH), yet the supporting evidence is largely observational and unadjusted for injury severity. We examined whether surgical timing is independently associated with in-hospital mortality after accounting for baseline severity.

METHODS: We retrospectively analyzed 91 consecutive patients who underwent surgical evacuation for ASDH between January 2014 and December 2024. The primary outcome was in-hospital mortality; the secondary outcome was functional status at discharge measured by the Glasgow Outcome Scale (GOS). Surgical timing (≤ 4 vs. > 4 h) was assessed through unadjusted comparison, stratification by Glasgow Coma Scale (GCS) score, multivariable Firth penalized logistic regression with 1000-iteration bootstrap internal validation, and sensitivity analyses including alternative timing thresholds (1-12 h), restricted cubic spline modeling, and timing-by-severity interaction testing.

RESULTS: In-hospital mortality was 33.0% (30/91; 95% Wilson CI 24.2-43.1%). Crude analysis showed higher mortality with early surgery (50.0% vs. 15.6%, $p = 0.001$), but these patients had lower admission GCS and higher

rates of CT-defined traumatic axonal injury (TAI) and cerebral contusion. After stratification, no statistically detectable mortality difference was observed within [...]

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Association between time to transfusion and outcomes in patients with traumatic haemorrhagic shock: an analysis of a nationwide trauma registry in Japan.

Shibahashi K, Inoue K, Kato T, Sugiyama K

PURPOSE: To investigate the association of in-hospital time to transfusion on outcomes of patients with traumatic haemorrhagic shock.

METHODS: Using data from the Japan Trauma Data Bank (January 2019-December 2023), we identified adult patients (aged ≥ 18 years) with haemorrhagic shock following trauma (systolic blood pressure < 90 mmHg; shock index ≥ 1.0 upon hospital arrival) and received ≥ 10 units red blood cells within 24 h of injury. We excluded patients transferred from or to other hospitals after emergency treatment, with burn injuries or cardiac arrest, without transfusion time records, and with first transfusion > 60 min after hospital arrival. Mixed-effect multivariable logistic regression estimated the association between time to transfusion for in-hospital mortality, adjusting for patient characteristics, comorbidities, and injury severity. Heterogeneity in the association between transfusion timing and mortality was assessed through nine subgroup analyses.

RESULTS: Overall, 698 patients (median age 59 years, 66.2% male) from 132 hospitals were analysed, of whom 265 (38.0%) died during hospitalisation. Mean time to transfusion varied from 4 to 58 min across hospitals. A positive association was observed between time to transfusion and in-hospital mortality. Each 1-min delay in transfusion initiation was associated with higher in-hospital mortality (adjusted odds [...])

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Neutrophil phenotyping reflects injury severity when base excess and lactate remain normal.

van Eerten FJC, Duebel LV, de Fraiture EJ, Brands SR, van Wessem KJP, Vrisekoop N, Venhuizen JH, de Groot MCH et al.

INTRODUCTION: Accurate assessment of injury severity is essential for optimal trauma care, as it dictates timing and intensity of interventional strategy. Conventional markers, such as base excess and lactate levels are usually borderline deranged in patients with midrange injury severity. This study explored whether determining circulating neutrophil phenotypes in these patients (ISS 9-35) might aid in the assessment of injury severity.

METHODS: A descriptive single-center cohort study of trauma patients ≥ 16 years between 2013 and 2023 was conducted. Data was collected from the Dutch National Trauma Registry and the Utrecht Patient-Oriented Database ($< 10\%$ missing values). Base excess, lactate levels and neutrophil phenotypes were routinely measured in emergency department blood samples. The dynamics of these parameters was analyzed in the context of increasing Injury Severity Scores (ISS).

RESULTS: 807 patients were included, of whom 591 were classified as midrange severely injured. In these patients, banded neutrophils showed a greater fold increase between ISS 9 and 35 compared with base excess and lactate (6.7, 2.7, and 1.5, respectively). Furthermore, the number of banded neutrophils exceeded the upper interquartile range of controls at lower trauma severities compared to base excess or lactate (ISS 17 vs. 30 and 39, respectively).

DISCUSSION: Although base excess and [...]

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Limb occlusion pressure-based individualized tourniquet pressure management in patients undergoing internal fixation for extremity fractures: a prospective comparative feasibility study.

Yang J, Hu P, Wang L

OBJECTIVE: This study aimed to evaluate the feasibility of limb occlusion pressure (LOP)-based individualized tourniquet pressure management and to quantify the reduction in applied tourniquet pressure compared with a

traditional fixed-pressure protocol in patients undergoing internal fixation for extremity fractures.

METHODS: A prospective comparative study was conducted at Civil Aviation Shanghai Hospital from January to September 2025. Patients undergoing internal fixation for extremity fractures were enrolled and allocated to either the LOP-based group (n = 171) or the traditional fixed-pressure group (n = 235) according to the attending surgical team and operative scheduling, without randomization. The LOP-based group received individualized tourniquet pressure based on preoperative LOP measurement plus a safety margin, while the control group received standardized pressures (40-50 kPa; 300-375 mmHg). The primary outcome was applied tourniquet inflation pressure. Secondary outcomes included demographic characteristics, extremity location, operative duration, and subgroup analyses stratified by extremity location, age, and sex; multivariable linear regression was performed to adjust for potential confounders.

RESULTS: The LOP-based group demonstrated significantly lower mean tourniquet pressure compared with the traditional group (32.82 +/- 3.63 kPa [246 +/- 27 mmHg] vers [...])

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Impairment in activities of daily living and textbook outcome after major emergency abdominal surgery.

Mima K, Nitta H, Shikiji Y, Nakashima R, Uemura S, Morinaga T, Itoyama R, Umezaki N et al.

PURPOSE: Textbook Outcome (TO) has been a useful tool to assess benchmark patient-centred surgical outcomes in elective abdominal surgery. This study aimed to evaluate the association of preoperative activities of daily living (ADL) status with TOs following major emergency abdominal surgery.

METHODS: This retrospective study included 301 consecutive patients who underwent major emergency abdominal surgery for high-risk acute abdominal conditions, including perforated peptic ulcer, intestinal ischemia, bowel obstruction, bowel perforation, and severe intraabdominal bleeding or infection, at Kumamoto Regional Medical Center between April 2019 and March 2025. Preoperative ADL was evaluated using the Barthel Index (range, 0 to 100), with higher scores indicating greater independence. Barthel Index ≤ 85 was defined as ADL impairment. According to a recent international consensus, TO was defined as discharge to "home" without major postoperative complications (Clavien-Dindo grade $\geq$ III). Multivariable logistic regression analysis was conducted to assess the association of preoperative ADL with TO achievement, adjusting for clinical features and surgery-related factors.

RESULTS: Among the 301 patients, 90 (30%) had preoperative ADL impairment (Barthel Index ≤ 85) and 211 (70%) achieved TO. Multivariable analysis revealed that preoperative ADL impairment was independently associated [...]

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Early calcium replacement after minimal blood transfusion is associated with adverse outcomes.

Hewgley WP, Griffin RL, Baird E, Hashmi ZG, Baker SJ, Jansen JO, Kerby J, Holcomb JB et al.

PURPOSE: Calcium disturbances are common in patients with traumatic injuries, exacerbated by shock and blood product resuscitation. Recently, empiric calcium supplementation has been recommended with the first unit of blood transfusion, despite a lack of data supporting this practice. We hypothesize that there are risks associated with calcium administration in patients receiving small volume transfusion.

METHODS: This is a retrospective review of patients admitted to our Level 1 trauma center from 2018 to 2024 who received 1-3 units of red blood cells (RBCs). Patients were stratified by calcium levels and whether they received early calcium supplementation. A logistic regression was used to estimate odds ratios for the association between calcium levels, calcium supplementation, and outcomes. The primary outcome was in-hospital mortality. Secondary outcomes included acute kidney injury, myocardial infarction, acute respiratory distress syndrome, and venous thromboembolism. The model was adjusted for age, sex, ISS and shock index.

RESULTS: A total of 2,680 patients were included. When stratified by calcium levels and supplementation status, patients were similar in terms of age, sex, mechanism and injury severity score, but shock index was higher among patients receiving calcium supplementation. After adjustment, hypocalcemic patients who received calcium supplementation had [...]

Peripheral perfusion index as a predictor of severe shock in trauma patients: a prospective observational study.

Sundar H, Purushothaman V, Mathuram SR, Chandy GM, Sharma SL, Nayak S, James JD

BACKGROUND: Early recognition of haemorrhagic shock is critical in trauma care, particularly in low- and middle-income countries where delayed referral is common. Conventional vital signs may remain normal despite ongoing hypoperfusion. The Perfusion Index (PI), derived from pulse oximetry, is a non-invasive and continuously available marker of peripheral perfusion. This study evaluated the ability of PI to identify severe haemorrhagic shock in trauma patients and compared its performance with the Shock Index (SI) and Modified Shock Index (mSI).

METHODS: This prospective, single-centre study was conducted at a Level 1 trauma centre between January and May 2024. Adult Priority 1 trauma patients were enrolled and classified as having no/mild shock (ATLS grades 1-2) or severe shock (grades 3-4). PI was recorded during initial assessment. SI, mSI, lactate, injury severity, transfusion requirement, ICU admission, and 24-hour mortality were analysed. Receiver operating characteristic (ROC) analysis and multivariable logistic regression were performed.

RESULTS: A total of 120 patients were included (mean age 40.4 ± 17.2 years; 90% male), predominantly with blunt trauma (85% road traffic injuries). Severe shock was present in 35 patients (29.2%). Median PI was significantly lower in severe shock compared with no/mild shock (0.54 [IQR 0.28-1.31] vs. 1.33 [0.71-2.16], $p = 0.008$). Mean [...]

A prospective study on abdominal trauma: patterns of injury, clinical presentation, organ involvement, associated injuries, and outcomes at a tertiary hospital in Mogadishu, Somalia.

Mohamud AA, Hassan AI, Ali AK, Jubur AA, Ahmed MR, Mohamed SS, Mohamed MM, Bashir AM

BACKGROUND: Trauma is a major global public health problem and one of the leading causes of death and disability worldwide, particularly among individuals younger than 44 years of age. Approximately 6 million people die each year as a result of traumatic injuries, with nearly 90% of these deaths occurring in low- and middle-income countries. Previous epidemiological studies have reported postoperative complication rates ranging from 18% to 42% and mortality rates between 8.5% and 13%.

OBJECTIVE: To evaluate the patterns of injury, clinical presentation, organ involvement, associated injuries, management strategies, and outcomes among adult patients with abdominal trauma in Mogadishu, Somalia.

METHODS: A prospective observational study was conducted from August 2023 to July 2024 at Mogadishu Somali-Türkiye Recep Tayyip Erdoğan Training and Research Hospital. A total of 146 adult patients with abdominal trauma were enrolled. Data were collected using a pretested structured questionnaire and analyzed using Stata version 16. Univariable and multivariable logistic regression analyses were performed to identify factors independently associated with in-hospital mortality.

RESULTS: The study population consisted predominantly of young adult males, with a mean age of 30.18 ± 9.2 years, and males accounted for 97.3% of all patients. Penetrating trauma was the predominant mechanism of [...]

Comparison of fluoroscopic versus electromagnetic distal locking in humeral shaft osteosynthesis using intramedullary nail: a retrospective study.

Madeja R, Szeliga J, Voves J, Pometlova J, Konde A, Janosek J, Dousa P

PURPOSE: Intramedullary nailing is a standard method for osteosynthesis of diaphyseal humeral fractures. Distal locking, however, typically requires fluoroscopic guidance, which increases radiation exposure. This study aimed to compare standard fluoroscopy-guided distal locking with the Ezy-Aim system (Austofix, Australia) based on electromagnetic pulse detection from the perspectives of surgical times, radiation exposure, and complications.

METHODS: A retrospective study included 58 patients treated for diaphyseal humeral fractures (AO/OTA 12A, 12B) between 2019 and 2023. Group A ($n = 28$) received osteosynthesis using the Ezy-Aim system; Group B ($n =$

30) underwent fluoroscopy-guided distal locking. Data on operative time, fluoroscopy time, radiation dose, and complications were analysed.

RESULTS: In all, distal locking was not successful at the first attempt in 14.3% (4/28) of screws placed with Ezy-Aim and 20.0% (6/30) when fluoroscopy was used. Time and dose analyses were performed on the remaining 48 patients. The total surgical time was significantly longer with the Ezy-Aim system (median 58 min) compared to fluoroscopy (median 52 min, $p = 0.034$), primarily due to the time required for the device assembly. However, distal screw placement was faster with Ezy-Aim (median 12 min vs. 14 min, $p = 0.019$). Radiation exposure was significantly lower in the Ezy-Aim group (median [...])

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Patterns of patient worry following major emergency abdominal surgery: a 180-day follow-up study.

Parner MK, Jensen LR, Gamst-Jensen H, Burcharth J, Kokotovic D

PURPOSE: Major emergency abdominal surgery is associated with high postoperative morbidity and mortality. Beyond physical complications, patients experience psychological distress. This study aimed to describe the distribution of patient-reported worry across independent cross-sectional samples and its association with Days Alive and out of Hospital (DAOH) over 180 days postoperatively.

METHODS: Prospective cohort at Copenhagen University Hospital Herlev (March 30, 2023-March 31, 2024). Degree of Worry (DOW) was surveyed at discharge and postoperative day (POD) 30, 90, and 180, categorized as physical, mental, and physical-generic domains of worry. DAOH was computed cumulatively and incrementally for 0-30, 30-90, and 90-180 days. Associations between high DOW (hDOW) and subsequent DAOH were tested with Mann-Whitney U.

RESULTS: hDOW was reported by 36.1% at discharge, 22.4% at POD 30, 45.0% at POD 90, and 32.0% at POD 180. Across 180 days, the most frequent worries were relapse (21.3%), disease (14.1%), functional level (12.0%) and daily living (9.1%). The proportion of physical worries was significantly higher at POD 30 than at discharge and significantly lower at POD 180; mental worries varied between time points without statistically significant differences. hDOW at POD 30 was associated with fewer cumulative DAOH by POD 90 ($p = 0.008$).

CONCLUSION: Patient-reported worry f [...]

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Outcomes and complications of metallic vs. bioabsorbable screw fixation for displaced medial epicondylar fractures in pediatric and adolescent patients.

Jhala T, Scherer S, Esser M, Lieber J, Fuchs J, Dietzel M

PURPOSE: The use of bioabsorbable screws in orthopedics and trauma surgery is increasing. The major advantage of not having to remove the implant is of particular interest in pediatric surgery. Due to drawbacks, most notably implant fracture, bioabsorbable screw osteosynthesis has not yet surpassed the gold standard of metallic screw osteosynthesis. This single-centre study compares metallic screw and bioabsorbable screw osteosynthesis of medial humeral epicondyle fractures in children and adolescents over a two-year period.

MATERIALS AND METHODS: Retrospective analysis of surgically treated (screw fixation with either bioabsorbable magnesium-based or metallic screw) patients with a medial humeral epicondyle fracture between 2020 and 2022 at the author's institution.

RESULTS: Seventeen patients with medial epicondyle fractures underwent surgical treatment using either bioabsorbable screws ($n = 8$) or metallic screws ($n = 9$). Baseline characteristics, surgical approach, and postoperative care were similar between groups. Two complications (Clavien-Dindo I) occurred in the bioabsorbable group due to implant failure (screw breakage), though without clinical sequelae. These implant failures led to a significantly increased number of postoperative radiographs in the bioabsorbable group (median 4.8 vs. 2.5; $p = 0.014$). Functional outcomes (LOM, MEPI) and follow-up durations were com [...]

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Characteristics and temporal trends of mobility scooter accidents in the Netherlands: a multicenter cohort study.

Stensen Z, Peters A, Keereweer D, Krul I, van Osch F, de Groot B, Barten D

PURPOSE: Mobility scooters are increasingly used by older individuals. The aim of this study was to assess the characteristics and temporal trends of mobility scooter accidents (MSAs) in the Netherlands over a 10-year period using a representative dataset derived from fourteen Emergency Departments (EDs).

METHODS: In this retrospective multicenter cohort study, we analyzed MSA-related ED visits between January 2014 and December 2023, using the Dutch Injury Surveillance System (DISS). Information about age, sex, hospital admission, type and location of injury, mechanism of injury, and operational errors was analyzed. Injury severity was classified using the maximum abbreviated injury score (MAIS). Trend analyses were only performed for ED visits of patients with MAIS2+ injuries.

RESULTS: In total, 2,702 MSA-related ED visits were identified. Forty-nine percent of patients was male and the median age was 75 years (IQR 64.0-83.0). The most common mechanism of injury was a mobility scooter rollover (34.2%). From 2014 to 2023, the number of MSA-related ED visits of patients with MAIS2+ injuries increased by 38.0%. In the 70-79 age group, this rise was 91%. The total number of traumatic brain injuries (TBI) rose steeply by 103.0%, which includes a modest (+34.0%) increase in moderate-severe TBI. The hospital admission rate was 30.1%, with a median length of stay of 7 days (IQR 3.0 [...])

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"Is the effect of intraoperative 3D imaging beneficial in plate fixation of distal radius fractures: a matched-pair analysis".

Zumsteg JS, Pieringer A, Hinkelmann MA, Isenmann K, Pape HC, Allemann F, Barthel C

BACKGROUND: Distal radius fractures (DRFs) are among the most common adult fractures and are frequently treated with volar plate fixation. While intraoperative 3D imaging has been proposed to improve detection of malreduction and implant malposition, its impact on clinical and radiological outcomes remains unclear. This study aimed to evaluate whether intraoperative 3D imaging provides advantages over conventional 2D fluoroscopy. Outcome criteria were accuracy of fracture reduction, plate positioning relative to the watershed line, complication rates, operative time, and functional outcomes following volar plate fixation of distal radius fractures.

METHODS: This retrospective single-centre study included patients aged ≥ 16 years who underwent volar plate fixation for distal radius fractures between 2016 and 2024. Patients were assigned to a 3D imaging group or a conventional 2D fluoroscopy group. To ensure comparability, patients were matched 1:1 using exact AO/OTA subgroup matching and propensity score matching for age and sex. Outcomes included operative duration, radiographic reduction quality, plate positioning, complications, and wrist range of motion. Matched-pair comparisons were performed using appropriate paired statistical tests.

RESULTS: Of 1,259 patients screened, 418 patients (209 matched pairs) were included after exact AO/OTA subgroup matching and propensity score matching [...]

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High-velocity gunshot chest trauma: tailoring patient care through insights from a mass casualty experience.

Azulay AA, Stein M, Tabo LY, Lebenthal A, Ruderman L, Refaely Y

OBJECTIVES: This study describes the management strategies and clinical outcomes of a cohort of patients sustaining high-velocity military rifle gunshot wounds to the thorax during a mass casualty incident (MCI). The analysis focuses on damage control surgical techniques, the incidence and timeline of venous thromboembolism (VTE), regional pain management, and early psychiatric protocols.

METHODS: A descriptive retrospective analysis was conducted on patients presenting with a single penetrating high-velocity ballistic wound to the thorax on October 7, 2023. Inclusion criteria were limited to patients with a projectile trajectory traversing the lung parenchyma, managed by the cardiothoracic surgery department. Injuries to immediately adjacent structures (subclavian vessels, brachial plexus, clavicle, and scapula) were recorded as associated injuries. Patients presenting with multi-system trauma involving entirely separate, distant anatomical regions were excluded.

RESULTS: The cohort comprised 11 male patients (age range: 19-49 years; 9 soldiers, 2 civilians) injured by 7.62 mm assault rifle rounds. Six patients (54.5%) required emergency thoracotomy, sternotomy, or VATS within

the first 24 h due to ongoing hemorrhage; primary hemorrhage control consisted of deep parenchymal lung suturing (n = 5) and temporary chest packing (n = 1). Four patients (36.4%) underwent delayed, el [...]

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Elevated risk of wound and systemic complications in MRSA-colonized patients undergoing lower extremity orthopedic trauma surgery.

Areti AS, Hafeez H, Sangineni PV, Argao L, Deveza L, Dawson J

PURPOSE: Preoperative colonization with methicillin-resistant *Staphylococcus aureus* (MRSA) is a known risk factor for adverse outcomes in arthroplasty but remains under-investigated in orthopedic trauma. This study evaluates the impact of preoperative MRSA colonization on postoperative complication rates in patients undergoing lower extremity orthopedic trauma surgery.

METHODS: We conducted a retrospective cohort study using the TriNetX Research Network, identifying adult patients who underwent lower extremity orthopedic trauma procedures over a 20-year period. Patients with a documented MRSA diagnosis within three months before surgery were matched 1:1 to MRSA-negative controls based on demographics and comorbidities. Postoperative outcomes were assessed within three months of surgery, including surgical site infections (SSIs), wound dehiscence, organ dysfunction, thromboembolic events, and healthcare utilization. Odds ratios (ORs) and p-values were calculated, with statistical significance set at $p < 0.05$.

RESULTS: A total of 1,868 MRSA+ and 629,342 control patients were identified. After matching, MRSA+ patients remained at significantly increased risk for wound dehiscence (OR = 1.80, $p = 0.005$), pneumonia (OR = 2.33, $p < 0.001$), sepsis (OR = 3.71, $p < 0.001$), acute renal failure (OR = 1.45, $p < 0.001$), blood transfusion (OR = 1.74, $p < 0.001$), deep vein thrombosis (OR = 2 [...])

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A global contrast shortage: a descriptive study exploring IV contrast utilization and outcomes in trauma patients at a level I trauma center.

Zakaria H, Johansen C, Waldrop C, Wan B, Graham C, Fetzer D, Minei J, Bruns B et al.

PURPOSE: Computed Tomography (CT) with intravenous (IV) contrast is critical to the evaluation of the trauma patient. A pandemic-related lockdown occurred in Spring 2022 in the city where iohexol iodinated contrast media is manufactured, resulting in a global shortage. We explored the potential impact of this shortage on contrast utilization and patient outcomes.

METHODS: A retrospective study was performed of all trauma patients who underwent CT between January and December 2022 at a level I trauma center. Pre-contrast conservation (PRE) period was defined as January to April 2022, and post-conservation (POST) as May to December 2022. Demographics, utilization rates (bolus/patient) and outcomes were evaluated. Chi-square tests were performed for categorical variables and Mann-Whitney U tests for continuous variables. Statistical significance was set at $p < 0.05$.

RESULTS: 1,097 patients were included; 509 in the pre-shortage period, 588 post-shortage. 857 CTs with contrast (1.68 contrast bolus/patient) were performed pre-conservation and 979 post-conservation (1.66 contrast bolus/patient) ($p = 0.04$). Maximum Abbreviated Injury Scale (AIS) head, head/neck, and extremity were higher post-shortage ($p < 0.05$); however, mortality was higher in the pre-conservation group ($p < 0.0012$). There were 9 cases of Acute Kidney Injury (AKI, 8 PRE, 1 POST), 22 delayed diagnoses (14 PRE, 8 P [...])

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Trauma stabilization points in armed conflict: operational evolution and strategic reflections from mosul to Gaza.

Marta C, Luca P, Kai-Hsun H, Areej AF, Athanasios G, Wessam EG, Jonas Z, Johan VS et al.

BACKGROUND: Forward trauma care remains a critical gap in emergency responses to armed conflict, particularly where conventional medical evacuation is disrupted. Trauma Stabilization Points (TSPs) were introduced in previous conflicts to bring life-saving care closer to the point of injury. To improve standards and adaptability, the World Health Organization convened a Technical Working Group to develop operational guidance for TSPs, drawing on diverse field experiences, including the ongoing Gaza deployments.

CASE PRESENTATION: Between February and July 2024, three TSPs were established in Gaza, managing over 4,000 consultations. Initially focused on trauma stabilization and referral, these sites quickly adapted to minor injuries and non-traumatic conditions, reflecting population needs and access challenges. Referral rates varied across sites due to hospital proximity, ambulance availability, and shifting frontlines. Security threats limited forward deployment and safe patient access, requiring high mobility and rapid relocation. Experience from Gaza highlighted key operational principles: locating TSPs near the point of injury; integrating within a functioning trauma referral pathway supported by evacuation capacity and hospital readiness; maintaining clear clinical functions and staffing standards; and using standardized documentation for quality assurance and continuity. [...]

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Acute-on-chronic versus isolated acute subdural hematoma in complicated mild traumatic brain injury: association with radiological mass effect markers in a multicenter cohort.

Crombé A, Benhamed A, Seux M, Frassin L, Matichard R, L'Huillier R, Millon D, Gorincour G

PURPOSE: The clinical impact of acute-on-chronic subdural hematoma (acSDH) in mild traumatic brain injury (mTBI) remains incompletely characterized. We assessed whether acSDH, compared with isolated acute SDH (aSDH), is associated with worse neurological presentation and increased radiological markers of mass effect.

METHODS: We conducted a retrospective multicenter cohort study using the IMADIS teleradiology head trauma workflow (103 emergency departments in France, January 2020 to December 2022). Adult patients with complicated mTBI (i.e., GCS 13-15 plus intracranial hemorrhage or skull fracture) and acute SDH on non-contrast head CT were eligible. Exposure was acSDH versus aSDH based on structured radiology reports. Outcomes were (1) GCS < 15 at presentation, (2) SDH maximal thickness ≥ 7 mm, (3) radiological brain herniation, and (4) intermediate or high QueBIC risk category. Univariable and multivariable logistic regression models were used to estimate associations.

RESULTS: Among 935 included patients, 107 (11.4%) had acSDH. Patients with acSDH were older (≥ 75 years: 57.0% vs. 39.7%, $p < .0001$) and more frequently had dementia (8.4% vs. 3.7%, $p = .0465$). Compared with aSDH, acSDH was associated with GCS < 15 (37.4% vs. 21.7%, $P = .0010$) and confusion (41.0% vs. 25.8%, $p = .0016$). On CT, acSDH was associated with SDH thickness ≥ 7 mm (54.2% vs. 20.4%, OR 4.50, 95%CI 2.9 [...]

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Persistent neuropathic pain symptoms after FPV drone-related thoracic blast injury without rib fractures: a pilot observational study from the Ukraine War.

Battle C, Baker E, Giamello JD, Melchio R, Dmytriiev D

PURPOSE: The aim of this exploratory pilot study was to describe First Person View drone (FPV) related, blast-mechanism thoracic trauma with no rib fractures in Ukrainian military patients sustained during the Russia-Ukraine Conflict, and report the prevalence of persistent neuropathic pain symptoms at three months among patients admitted to a rehabilitation and pain medicine service.

METHODS: A retrospective, single-centre observational, exploratory study design was used. Military patients (≥ 16 years old) with blast-mechanism thoracic injuries with no rib fractures, admitted to a hospital's rehabilitation and pain medicine service in Ukraine for ≥ 24 h were included. The PainDETECT survey was completed at three months post-injury. Participant characteristics were summarised using descriptive statistics. Exploratory univariate and multivariate analyses were performed to explore possible associations between variables and the development of persistent neuropathic pain symptoms.

RESULTS: 217 patients were included in the final analysis. Median age was 39 years (IQR: 32-47.5). 131 (60.4%) patients were admitted to critical care with 91 (41.9%) patients requiring mechanical ventilation. The median hospital length of stay was 22 days (IQR: 16-25) and critical care length of stay was 4 days (IQR: 2-5). Persistent neuropathic pain symptoms were reported in 173 out of 217 patients ([...]

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The association between pancreatic enzyme serum concentrations and blunt pancreatic trauma: a systematic review.

Sykes B, Gracias D, Bendinelli C, Balogh ZJ

PURPOSE: Blunt pancreatic trauma is uncommon. When suspected, serum amylase and lipase are measured, but their diagnostic value remains uncertain. This systematic review evaluated the diagnostic performance of serum amylase and serum lipase in detecting blunt pancreatic trauma in adults.

METHODS: MEDLINE, Embase, Scopus and Cochrane Central were searched for studies evaluating serum amylase and/or lipase in adults with blunt pancreatic trauma. Observational studies reporting diagnostic accuracy or clinical outcomes were included. Two reviewers independently screened studies and assessed risk of bias using the Newcastle-Ottawa Scale.

RESULTS: Four observational studies met inclusion criteria. A total of 4937 patients with blunt abdominal trauma, including 150 with pancreatic injury were investigated. Two studies evaluated serum lipase alone and two compared amylase and lipase. Pancreatic injuries were graded according to the American or Japanese Association for the Surgery of Trauma. Early serum amylase measurement (< 90 min post-injury) demonstrated low sensitivity. In contrast, testing performed more than 3 h after injury has shown improved diagnostic accuracy with one study suggesting combined serum amylase and lipase testing can achieve a sensitivity of 85% and specificity of 100%.

CONCLUSION: Early serum pancreatic enzymes measurement does not appear to aid initial management [...]

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Extension of the fluidics NTF-Biobank of polytraumatized patients with extracellular vesicles: improved vesicle preservation through local processing.

Weber B, Leppik L, Han J, Becker N, Wang T, Groven RVM, Balmayor ER, Zhao Q et al.

PURPOSE: Extracellular vesicles (EVs) show diagnostic relevance in trauma, prompting the development of the NTF■EV■Biobank as an extension of the established Network Trauma Research (NTF) plasma/serum biobank. After optimizing the EV isolation and characterization standard operating procedure (SOP) at a single center, this multicenter study evaluates its transferability and examines how sample transport affects EV quality before large■scale implementation in a nationwide biobank setting.

METHODS: Plasma and serum from polytrauma patients and healthy controls (n=10) were processed under the standardized NTF■Biobank protocol and distributed to three centers. EVs were isolated using the unified NTF■EV■Biobank workflow, exchanged between centers, and analyzed for protein markers, particle size and concentration (NTA), total protein content, and morphology (TEM) to compare EV quality across sites.

RESULTS: Implementation of the EV biobank protocol across participating centers, as well as inter-center transport on dry ice, was successfully performed without complications. All samples arrived at the analyzing centers in a frozen state. EVs exhibited typical vesicular morphology as confirmed by transmission electron microscopy and remained positive for CD9 and CD81, indicating that transportation did not affect morphology or surface marker expression. However, transportation did reduce [...]

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Whole blood first resuscitation and association of blood product utilization based on mechanism of injury.

Hadley J, Conner J, Thomas C, Llorente K, Gilani A, White B, Miller Iii P, Nunn A et al.

INTRODUCTION: Studies evaluating the effect of whole blood (WB) vs. component-based resuscitation accounting for mechanism of injury are lacking. We sought to evaluate the effects of a WB first resuscitation strategy regarding mechanism of injury.

METHODS: This is a retrospective cohort study of trauma patients who received any blood product during resuscitation from January 2016 to November 2021. The primary outcomes were total blood volume utilized within 24 h and 30-day mortality. Multivariable gamma regression with a log link was used for blood volume and logistic regression for mortality. A likelihood ratio test evaluated whether injury mechanism modified the treatment effect.

RESULTS: Between January 2016 and November 2021, 1,013 injured patients received blood products (785 WB-first). We did not detect statistically significant effect modification by injury mechanism for either outcome (interaction p = 0.44 for blood volume, p = 0.25 for mortality). In multivariable gamma regression, WB-first resuscitation was associated with a ratio of mean total blood volume of 0.71 (95% CI 0.55 to 0.90, p = 0.005) relative to components, representing approximately 29% lower utilization. WB-first was not associated with 30-day

mortality (OR 0.90, 95% CI 0.48 to 1.71, $p = 0.73$).

CONCLUSIONS: A WB-first resuscitation strategy is associated with lower blood volume utilized within the f [...]

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Reported complications after spinopelvic fixation for U-type sacral fractures in older adults: a systematic review with narrative synthesis.

Giordano V, Pinheiro HRS, Pires RE, Varga P, Freitas A, Giannoudis PV

BACKGROUND: Spinopelvic fixation (SPF), alone or combined with iliosacral or transiliac-transsacral fixation, is often used for highly unstable posterior pelvic ring injuries and some fragility fracture patterns in older adults. However, the evidence specifically describing complications after SPF in older patients with U-type sacral fractures is limited.

METHODS: This systematic review was conducted according to the Cochrane Handbook and PRISMA recommendations. PubMed, Embase, and grey-literature sources were searched from January 1, 2000 to July 31, 2025. Eligible studies included adults aged 60 years or older with U-type sacral fractures or closely related spinopelvic dissociation/FFP IVb patterns treated with SPF, and reporting at least one complication. Because of the small number of eligible studies, probable cohort overlap, heterogeneity in fracture definitions, treatment indications, fixation constructs, and outcome reporting, findings were synthesized narratively rather than pooled quantitatively.

RESULTS: Three studies met the eligibility criteria, comprising 157 patients overall; however, only a subset underwent SPF, and two studies were produced by the same group with probable cohort overlap. The available cohorts mixed bilateral sacral fragility fractures, FFP IVb patterns, and comparative SPF-versus-trans-sacral constructs. Reported adverse events included wound [...]

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Influence of allograft augmentation on surgical outcomes and complications in proximal humerus fractures: a matched-pair analysis.

Barthel C, Rudolph F, Kalbas Y, Russek R, Pape HC, Allemann F

BACKGROUND: Proximal humerus fractures predominantly affect elderly patients and pose a growing clinical burden. This study evaluates the impact of allograft augmentation on operative time, fracture reduction quality, and complication rates compared with standard fixation across different fracture morphologies.

METHODS: This retrospective matched-pair study included patients surgically treated for proximal humerus fractures with locking plate fixation at a single tertiary trauma center between 01.01.2014 and 31.12.2024. Patients treated with allograft augmentation (Group A) were matched 1:1 to patients without allograft (Group NA) using exact matching for Mayo FJD fracture morphology and sex, and propensity score matching for age. Radiological outcomes were assessed using pre- and postoperative neck-shaft angles, with normalization defined as correction into the physiological range (120° - 150°). Complications and operative parameters were recorded. Paired statistical analyses were applied.

RESULTS: A total of 1,012 patients were treated during the study period; after selection and matching, 102 matched patient pairs (204 patients) were analyzed. Operative time was longer with allograft augmentation (141.3 ± 40.3 vs. 126.9 ± 50.0 min; $p = 0.08$), reaching statistical significance only in varus posteromedial fractures (144.4 ± 37.8 vs. 122.2 ± 39.6 min; $p = 0.007$). Although overa [...]

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Impact of a comprehensive trauma pathway on workflow and clinical processes in a level III trauma center.

Neitzert L, Druetto C, Forno D, Molino P, Vitale D, Griio M

PURPOSE: Organized trauma systems improve coordination of care and may enhance the efficiency of trauma management, potentially improving outcomes and reducing hospital length of stay and costs. The aim of the study was to evaluate key process-of-care indicators and clinical outcomes before and after the introduction of a comprehensive trauma pathway.

METHODS: We conducted a single-center retrospective-prospective cohort study at Rivoli Hospital, Italy. Trauma patients admitted to the Emergency Department (ED) between 2024 and 2025 who met the criteria for trauma team activation were prospectively included. Outcomes were compared with a retrospective cohort from the period prior to the implementation of the dedicated trauma pathway (2023-2024).

RESULTS: A total of 465 patients were included. Implementation of a comprehensive trauma pathway was associated with significant improvements in several time-dependent quality indicators, including ED boarding time (888 vs. 1021 min; $p = 0.02$), time to imaging (48 vs. 78 min; $p < 0.01$), time to surgery (183 vs. 237 min; $p = 0.03$), time to ICU admission (226 vs. 343 min; $p = 0.02$), and time to interhospital transfer (238 vs. 305 min; $p = 0.04$). We observed a trend toward a reduction in early mortality (2% vs. 3.6%; $p = 0.41$) and 30-day mortality (3.7% vs. 5.9%; $p = 0.62$).

CONCLUSIONS: The adoption of a structured multidisciplinary trauma pathway [...]

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The DRoPH score: development of a tool for predicting reduction difficulty in proximal humerus fracture osteosynthesis.

Barthel C, Rudolph F, Kraus M, Pape HC, Allemann F

BACKGROUND: Proximal humerus fractures (PHFs) are among the most common and challenging fractures due to complex anatomy and variable fracture patterns. While anatomical reduction is critical for successful outcomes, no tool currently predicts the intraoperative difficulty of achieving it. Despite advances in surgical techniques, complications persist. Existing classification systems, such as Codman's, Neer's, AO/OTA, and Mayo-FJD, focus on fracture morphology and displacement but do not address intraoperative complexity. This study aims to develop a new score for the early prediction of intraoperative reduction difficulty in proximal humerus fracture surgery, the DRoPH score (Difficulty of Reduction of Proximal Humerus Fractures).

METHODS: An online survey was conducted among experienced upper extremity surgeons in Switzerland to identify factors influencing the difficulty of reducing and retaining proximal humerus fractures. Inclusion criteria for surgeon grading comprised experience with more than 100 surgically treated proximal humerus fractures. Eligible surgeons rated the relevance of factors influencing reduction difficulty using a five-point Likert scale. A rank-based weighting system derived from expert ratings was applied, assigning two points to the three highest-ranked factors. The resulting DRoPH-Score ranged from 0 to 10, with higher values indicating greater difficulty [...]

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Interface stability of intramedullary nail locking configurations under combined axial and torsional loading.

Marianne H, Martin W, Claus G, Anja B, Peter A

The biomechanical performance of intramedullary nails in metaphyseal regions strongly depends on distal locking configuration. This study aimed to evaluate screw-bone interface stability under combined axial and torsional cyclic loading for different distal locking configurations of intramedullary nails, focusing on the effects of screw number, spatial distribution, interscrew distance, and multiplanar versus coplanar fixation using a standardized synthetic bone model. Six distal locking configurations ($n = 5$ per group) were tested under stepwise cyclic axial compression, superimposed with a constant torsional moment (± 2 Nm, 2 Hz). Primary endpoints were cycles to failure, cycles to 1 mm axial displacement, and cycles to 0.5° rotation. All constructs failed by progressive screw cut-through within the synthetic bone. The three-screw multiplanar configuration exhibited the highest endurance ($\approx 36,000$ cycles to failure) and lowest micromotion ($\approx 34,000$ cycles to 1 mm displacement), significantly outperforming the two-screw configurations overall ($p = 0.002$). Increasing interscrew distance from 10 mm to 34 mm improved migration resistance by approximately 25%, whereas the short 10 mm spacing led to premature failure. Blocked and free-hand configurations showed comparable behavior to the standard setup, indicating no added benefit from toggle elimination alone. Locking configuration [...]

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Three-dimensional finite element analysis for internal fixation of mandibular condylar fractures: a scoping review.

Kakei Y, Sader R, Ebener P, Tanaka A, Akashi M, Kuroda R, Thoenissen P

PURPOSE: Three-dimensional finite element analysis (3D-FEA) has emerged as an essential computational tool for evaluating the biomechanical behaviors of internal fixation systems for mandibular condylar fractures. This scoping review maps and synthesizes the existing literature on 3D-FEA studies investigating the internal fixation of mandibular condylar fractures and explores, in a descriptive manner, differences in methodological focus between European and non-European research centers.

METHODS: PubMed and Cochrane Library databases were systematically searched following PRISMA-ScR guidelines. Original English-language research articles using 3D-FEA to evaluate internal fixation of mandibular condylar fractures were included. Data were extracted using a standardized charting form.

RESULTS: Thirty-six studies met the inclusion criteria. European studies (n = 12) predominantly focused on development of novel plate designs and mechanical testing protocols, whereas Asian and other nationality studies (n = 24) emphasized material comparisons between titanium and bioabsorbable systems, patient-specific modeling, and loading condition analysis. Consensus findings include: (1) double-plate fixation provides superior stability; (2) among 3D plate designs, trapezoid plates demonstrate the best performance for subcondylar and condylar base fractures, whereas alpha and lambda plates may [...]

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Initial surgical management of injuries to the upper extremities for patients with multiple and/or severe injuries - a systematic review and clinical practice guideline update.

Berk T, Könsgen N, Lechler P, Imach S, Schär M, Bieler D, Horst K, Hildebrand F et al.

PURPOSE: Our objective was to update the evidence-based and consensus-based recommendations for the initial surgical management of upper extremity injuries in patients with suspected multiple and/or severe injuries based on current evidence. This guideline topic is part of the 2025 update of the German Guideline on the Treatment of Patients with Multiple and/or Severe Injuries.

METHODS: MEDLINE and Embase were systematically searched to September 2024. Further literature reports were obtained from clinical experts. Randomised controlled trials (RCTs) or observational studies reporting risk-adjusted outcomes were included if they compared early versus delayed surgical treatment for fractures, vascular injuries, or nerve injuries affecting the upper extremities in patients with multiple and/or severe injuries. Studies comparing limb salvage versus amputation or comparing amputation criteria for the upper extremities were also included. We considered patient-relevant outcomes such as mortality and limb salvage, as well as the sensitivity and specificity of scores for predicting upper extremity amputation. Risk of bias was assessed at the outcome level using ROBINS-I for observational studies and AMSTAR-2 for systematic reviews. We used available meta-analyses if possible; alternatively, we synthesised the evidence narratively. We used GRADE to rate the certainty of evidence. Expe [...]

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Transosseous sutures versus vertical cerclage wire for distal pole patellar fractures: a retrospective comparative cohort study.

Laso JI, Muñoz JT, Bianchi S, Martínez D, Rojas T, Franulic N, Olivieri R

PURPOSE: To compare transosseous sutures (TOS) and vertical wire cerclage (VCW) for treating distal pole patellar fractures, evaluating clinical outcomes, patient-reported outcome measures (PROMs), radiographic outcomes, and complications.

METHODS: We conducted a retrospective comparative cohort study of patients surgically treated for patellar distal pole fractures (AO/OTA 34A1b) with either TOS or VCW between 2015 and 2023. Patient characteristics, surgical technique, postoperative data, patellar height, and complications were recorded. Functional outcomes were evaluated at a minimum of 12 months postoperatively using PROMs (Kujala, Lysholm, and Bostman scores). The primary outcome of this study was functional recovery, assessed via the Bostman score at 12 months.

RESULTS: A total of 46 patellar distal pole fractures were included (30 TOS and 16 VCW). Baseline characteristics showed no statistically significant differences between groups. No significant differences were found regarding clinical or functional outcomes. Regarding radiographic outcomes, patellar height remained stable; however, both groups showed significant patellar length variation, with elongation being significantly more pronounced in the TOS group (mean 3.77 mm) compared to the VCW group (mean 0.44 mm; p = 0.001). While overall complication rates

showed no statistically significant difference, all five ca [...]

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Refracture risk following implant removal in consolidated hip fractures: a multicenter retrospective cohort study of 575 patients.

Mohammed J, Mili-Schmidt V, Wadsten M, Juto H, Wolf O, Crnalic S, Sköldenberg O, Mukka S et al.

PURPOSE: The risk of re-fracture after implant removal in healed hip fractures remains a clinical concern. This study aimed to determine the incidence of re-fracture following implant removal after osteosynthesis of a hip fracture.

METHODS: We conducted a retrospective multicenter cohort study including patients aged ≥ 50 years who underwent implant removal between 2003 and 2023 after radiologically confirmed consolidation of a hip fracture. Patients were identified using procedural codes. Baseline variables included age, sex, ASA classification, fracture type (femoral neck or trochanteric), and implant type (pins/screws, sliding hip device [SHD], or short/long cephalomedullary nail [CMN]). Patients were followed from implant removal until re-fracture, conversion to arthroplasty, death, or end of follow-up, with a minimum follow-up of 1 year. Cox proportional hazards regression was used to assess associations between implant type and re-fracture risk, adjusting for age and sex. Because the proportional hazards assumption was violated, a time-stratified Cox regression and restricted mean survival time analyses were applied.

RESULTS: A total of 575 patients (median age 73 years, IQR 65-81) were included, with a median follow-up of 53 months (IQR 18-100). Lateral hip pain was the most common indication for implant removal (72.5%). The overall re-fracture incidence was 10.4%. Ris [...]

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Physical functionality two years after major trauma: predictors from a single center cohort study.

Fetz K, Koetsenruijter J, Rodrigues Recchia D, Rutetzki J, Lefering R, Kaske S

PURPOSE: Traumatic injuries are a major global public health concern, often leading to impaired physical function and disability. Long-term physical function impairments can greatly affect a patient's health related quality of life, influencing their emotional state and social interactions. This study examines patient-reported physical functionality before and after major trauma, identifying key predictors of poor functional recovery.

METHODS: This retrospective single-center cohort study investigates physical functionality 23 months after a traumatic injury using standardized questionnaires combined with clinical data. Inclusion criteria were Injury Severity Score (ISS) ≥ 9 , requiring ICU treatment, and age ≥ 18 resulting in an eligible sample of 515 patients. Health outcomes were assessed using the Trauma Outcome Profile (TOP), the Abbreviated Injury Scale (AIS), and the ISS.

RESULTS: Physical functionality significantly decreased from 94.8 (SD 11.5) to 72.9 (SD 26.7) 23 months post-trauma with almost half of participants still scoring below the critical cutoff of 80 points. The strongest predictor for functional physical impairment was pre-trauma physical functionality (OR 8.21, $p < 0.001$). Other determinants included age (age 50-59 versus 18-29 OR 2.75, $p = 0.001$), as well as an impacted lower extremity including pelvic (OR 1.60, $p = 0.018$).

CONCLUSION: The most importan [...]

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Association between fresh frozen plasma transfusion and mortality stratified by Glasgow Coma Scale scores in isolated traumatic brain injury: a nationwide cohort study in Japan.

Nagamura T, Aoki M, Yamada K, Terayama T, Seno S, Tomura S, Kiyozumi T

PURPOSE: Fresh frozen plasma (FFP) is widely used to manage trauma-induced coagulopathy. However, its effectiveness in isolated traumatic brain injury (TBI) management remains controversial, particularly when stratified by severity based on the Glasgow Coma Scale (GCS) scores. This study aimed to examine the association between FFP administration within 24 h and mortality in patients with isolated TBI, overall and stratified by GCS on

arrival.

METHODS: We conducted a multicenter retrospective cohort study using the Japan Trauma Data Bank (2019-2023). Adults with isolated TBI (intracranial AIS ≥ 3 and extracranial AIS < 3) were included. Patients were classified into FFP and non-FFP groups based on transfusion within 24 h of admission. Subgroup analyses were performed by TBI severity (severe: GCS ≤ 8 ; mild-to-moderate: GCS > 8), craniotomy status, and FFP dose. The primary outcome was 28-day mortality. Propensity score matching was applied to adjust for confounding.

RESULTS: Among 12,480 patients (median age 72 years [IQR: 55, 82]; 66.6% male), 513 received FFP. After matching, FFP was not significantly associated with 28-day mortality (OR 0.80; 95% CI 0.59-1.08). In subgroup analyses, FFP was associated with higher mortality in mild-to-moderate TBI (OR 2.38; 95% CI 1.07-5.31) and lower mortality in severe TBI (OR 0.61; 95% CI 0.42-0.88). No significant differences were observed [...]

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Percutaneous fixation of posterior pelvic ring lesions in the elderly: current evidence and recent advances.

Liodakis E, Schreiber S, Giannoudis VP, Fritz T, Giannoudis PV

INTRODUCTION: Percutaneous fixation techniques are increasingly used in the management of posterior pelvic ring lesions in the elderly. As the incidence of fragility fractures continues to rise, a growing number of implants and surgical techniques have been developed to address the specific challenges of this vulnerable population. The goal of this scoping review is to address the following questions: 1) What are the latest advances in percutaneous fixation of posterior pelvic ring fractures? 2) What are the clinical results and current level of evidence supporting these techniques?

METHODS: A systematic search using the keywords “((Percutaneous) OR (Minimal* invasive) OR (Transsacral) OR (Innovative implant)) AND ((Pelvic fragility fracture) OR (Sacral fracture))” was performed in PubMed and Cochrane Library using PRISMA-ScR guidelines. Studies published from the 11th of August 2020 up to the 11th of August 2025 were included and assessed independently by two reviewers.

RESULTS: A total of 334 studies were found, of which 107 were included in this review following the screening process. Of these, 22 studies pursued a biomechanical approach. Various new implants (e.g. ISG-Rod System, SI-Plate, TriFix, SACRONAIL®) and technologies (e.g. 3D computer-assisted navigation/O-arm/Robot) are described in the recent literature. Using navigation significantly reduces the rate of screw [...]

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Infrared thermography and dual-layer spectral CT for the detection of femoral vascular compromise: an experimental large animal study.

Hübner CT, Ricklin J, Müller AL, Klingebiel FK, Stoeck CT, Weisskopf M, Carneiro Gomes A, Cinelli P et al.

BACKGROUND: Rapid identification of vascular compromise remains a clinical challenge in the acute trauma setting. Conventional diagnostic tools such as CT angiography (CTA) and Doppler-ultrasound might provide decent anatomical and functional assessment. However, they are limited by ionizing radiation, contrast exposure or time requirements. Although not validated, infrared thermography (IRT) might offer a non-invasive alternative capable of detecting surface temperature asymmetries that may reflect underlying perfusion deficits. This study aimed to evaluate the feasibility of thermographic imaging in detecting vascular lesions.

METHODS: As part of a porcine polytrauma model (n = 28), vascular lesion of the right femoral artery was defined as a $\geq 50\%$ diameter reduction on CTA. Temperature differences between both extremities were measured using infrared thermography. SDCT based iodine concentrations distal to the stenosis were quantified to assess perfusion changes. Correlation and regression analyses were performed between stenosis degree, temperature difference, and tissue iodine uptake.

RESULTS: Six animals fulfilled the criteria for relevant vascular compromise. This resulted in a mean vessel diameter reduction behind the lesion of $68\% \pm 18\%$. The vascular lesion group demonstrated a mean inter-limb temperature difference of $6.93\text{ }^{\circ}\text{C} \pm 1.78\text{ }^{\circ}\text{C}$ compared to $0.71\text{ }^{\circ}\text{C} \pm 1.27\text{ }^{\circ}\text{C}$ i [...]

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Fixation strategies for periprosthetic femur fractures around knee replacements: a comprehensive single-center analysis of 102 consecutive cases.

Nomdedéu J, González-Morgado D, Minguell-Monyart J, Joshi-Jubert N, Teixidor-Serra J, Tomàs-Hernández J, Andrés-Peiró JV

PURPOSE: To evaluate 1 year mortality, complication rates, and functional outcomes after internal fixation of femoral periprosthetic knee fractures following total knee arthroplasty, and to identify predictors of adverse outcomes and increased healthcare resource utilization.

METHODS: This retrospective cohort study included 102 consecutive patients with femoral periprosthetic knee fractures classified as UCPF VB3 and VC3 treated with internal fixation at a single tertiary center between 2010 and 2023. Interprosthetic fractures and combined fixation techniques were excluded. Primary outcomes were 1 year mortality and postoperative complications. Secondary outcomes included length of stay, discharge destination, and ambulatory status at follow up.

RESULTS: Patients were markedly frail, with a mean age of 82.4 ± 10.8 years and mean Charlson Comorbidity Index of 5.6. One year mortality was 16.7% and was associated with older age, higher ASA class, higher Charlson index, impaired pre fracture ambulation, and fracture related infection. Overall complication rate was 29.4%, including 15.7% infections and 13.7% mechanical failures. Open reduction was more frequent in plate fixation and resulted in longer operative times, without increasing complication rates. Median hospital stay was 18 days, 66.3% required discharge to nursing facilities, and only 7.8% regained independent ambulation [...]

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Epidemiology and management of pediatric femoral fractures at a level I trauma center: a 10-year retrospective monocentric study.

Dietz SO, Traub F, Schmidt E, Schippers P, Wegner E, Schierjott L, Gercek E, Nowak TE

PURPOSE: Pediatric femoral fractures are relatively uncommon but represent severe injuries frequently requiring inpatient trauma care. Epidemiological characteristics, fracture patterns, and treatment strategies vary between institutions and regions. This study aimed to analyze the epidemiology, fracture distribution, treatment modalities, complications, and fracture-predisposing comorbidities of pediatric femoral fractures treated at a German level I pediatric trauma center over a 10-year period.

METHODS: All patients aged 0-14 years treated for femoral fractures between January 2012 and December 2021 were retrospectively reviewed. Fractures were categorized using the AO Pediatric Comprehensive Classification of Long-Bone Fractures (PCCF). Demographic variables, fracture characteristics, management approaches, time to consolidation, complications, and relevant comorbidities predisposing to fracture were analysed descriptively in accordance with STROBE recommendations.

RESULTS: A total of 183 patients met the inclusion criteria (67.2% male; mean age 5.2 years). Children aged 2-5 years constituted the largest group (42.1%). Femoral shaft fractures were the most frequent femoral fracture type (70%). Surgical management, most frequently elastic stable intramedullary nailing (ESIN), was increasingly utilized with advancing age. The overall complication rate was 14.2%, with the majority [...]

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Blood-based biomarkers GFAP/UCH-L1 for the diagnosis of mild traumatic brain injury (mTBI): a single-center implementation experience.

Maegele M, Breidenbach L, Kaufmann J, Keles Y, Uhl E, Schroeer P

INTRODUCTION: Mild traumatic brain injuries (mTBI) affect millions of people worldwide every year as one of the most common clinical presentations in the emergency department. Diagnosis is mainly based on clinical criteria and computed tomography scans. The use of computed tomography causes high costs, long waiting times in daily clinical practice and radiation exposure. GFAP (glial fibrillary acidic protein) and UCH-L1 (ubiquitin carboxyl-terminal hydrolase-L1) turned out to be potential biomarkers for the diagnosis of mTBI. This study retrospectively evaluates the possible use of these biomarkers combined as negative predictors for excluding brain injuries in patients with suspected mTBI in the emergency department.

METHODS: Adult patients (n = 320) registered in the emergency department at a level 1 trauma emergency center in Germany (Cologne Merheim Medical Center/CMMC) between 11/2023 and 04/2024, with suspected mTBI,

Glasgow Coma Scale (GCS) score 13-15 and within 12 h after trauma were considered. All evaluable patients underwent cranial CT (cCT) scans and blood tests for GFAP and UCH-L1 serum concentrations.

RESULTS: Biomarkers GFAP and UCH-L1 were tested positive in 261 patients (82%) while CT detected intracranial injuries in only 29 patients (9%). Biomarkers combined had a sensitivity of 97% and a negative predictive value (NPV) of 98% in mTBI diagnosis with a nega [...]

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Resuscitative endovascular occlusion of the aorta restores cerebral metabolic markers of ischaemia induced by haemorrhagic shock.

Bader SE, Magnuson A, Brorsson C, Wallin G, Löfgren N, Löfgren F, Blind PJ, Öman M et al.

BACKGROUND: Resuscitative endovascular balloon occlusion of the aorta (REBOA) is used as an adjunct in haemorrhagic shock (HS) to restore proximal perfusion. Its cerebral metabolic effects during haemorrhagic shock, particularly in the presence of elevated intracranial pressure (ICP), remain incompletely characterised.

METHODS: In a controlled porcine model, HS was induced by controlled bleeding to a mean arterial pressure of approximately 40 mmHg. Animals were allocated to a normal ICP group or an experimentally elevated ICP group. Total REBOA (zone 1) was applied for 90 min. Cerebral metabolism was assessed using intracerebral microdialysis with measurements of lactate, pyruvate, and lactate-pyruvate ratio (LPR). Cerebral haemodynamics and ICP were continuously monitored.

RESULTS: HS was associated with a statistically significant increase in LPR in both groups, indicating cerebral metabolic disturbance. Following aortic occlusion, LPR gradually decreased toward baseline in both groups. Animals with elevated ICP demonstrated a transient delay in metabolic normalisation during the early post-occlusion phase. Statistically significant differences between groups were limited to the first 10 min following occlusion. Overall metabolic trajectories were similar thereafter.

CONCLUSIONS: Total REBOA restored cerebral metabolic markers of ischaemia during haemorrhagic shock, as ref [...]

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Delayed single-stage surgical management using a peroneus brevis muscle flap for fracture-related infection after lateral malleolus fixation: a retrospective case series.

Dyregborg K, Jørgensen AI, Peterlin AAN, Gottlieb H

BACKGROUND: Fracture-related infection (FRI) following lateral malleolus fracture fixation complicated by soft tissue breakdown represents a challenging clinical problem with no established standard of care. This study evaluates a treatment strategy consisting of prolonged antibiotic suppression until fracture union, followed by delayed single-stage surgical management.

METHODS: This retrospective consecutive case series included 11 patients treated between 2020 and 2023 at a multidisciplinary orthopaedic infection unit. All patients were managed according to a standardized protocol involving oral antibiotic suppression until radiologically confirmed fracture consolidation. This was followed by a single-stage surgical procedure including implant removal, radical debridement, deep tissue sampling, local antibiotic therapy, and soft tissue reconstruction using a distally based peroneus brevis muscle flap with split-thickness skin grafting.

RESULTS: Eleven consecutive patients (median age 58 years) were treated according to the protocol. Six patients underwent antibiotic suppression for a median duration of 147 days prior to surgery. Among patients available for outcome assessment, infection control was achieved in 10 of 10 surviving patients. One patient died of unrelated causes within 14 days postoperatively and was not classified as treatment failure. Soft tissue complication [...]

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Mixed reality improves agreement on surgical approach selection and patient positioning in tibial plateau fracture planning compared to CT, 3DCT and 3D printing.

Dust T, Henneberg JE, Hartel M, Reiter A, Kylies J, Korthaus A, Streckenbach A, Keller J et al.

PURPOSE: Tibial plateau fractures require precise preoperative planning, particularly regarding treatment concept, patient positioning, and surgical approach. Mixed reality (MR) may enhance three-dimensional understanding of fracture morphology beyond conventional imaging and physical 3D models. This study investigated whether MR improves agreement on key preoperative planning decisions compared to computed tomography (CT), 3D computed tomography (3DCT), and 3D-printed fracture models.

METHODS: Twelve orthopaedic trauma surgeons (6 junior, 6 senior) evaluated 22 surgically treated tibial plateau fractures (AO/OTA type B or C). Each case was assessed in four steps using (1) CT, (2) 3DCT reconstructions, (3) physical 3D-printed models, and (4) MR visualization using Microsoft HoloLens 2. Raters selected (i) treatment concept, (ii) patient positioning, and (iii) surgical approach. Interobserver agreement was analysed using Fleiss' kappa (κ) and percentage match (PM).

RESULTS: Mixed Reality (MR) yielded the highest interobserver agreement for both surgical approach (PM 32%, $\kappa = 0.30$) and patient positioning (PM 57%, $\kappa = 0.35$), particularly among junior surgeons (approach PM 39%, $\kappa = 0.28$; positioning PM 57%, $\kappa = 0.39$). Compared to CT, 3DCT, and 3D printing, MR demonstrated consistent improvements, while agreement on treatment concepts remained high across all modalities (PM > 82% [...])

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Timing of fracture fixation for femur and pelvis fractures in patients with severe traumatic brain injury - an analysis of the TraumaRegister DGU®.

Bayer TP, Teuben MPJ, Halvachizadeh S, Shehu A, Lefering R, Pfeifer R, Pape HC, Jensen KO

PURPOSE: Fracture fixation timing and strategy in polytrauma patients with traumatic brain injury (TBI) remain controversial. This study investigates treatment patterns and outcomes for femoral and/or pelvic fractures stratified by TBI severity.

METHODS: Patients in the TraumaRegister DGU® (2016-2022) with pelvic and/or femoral fractures (AIS ≥ 3) and TBI (head AIS ≥ 3) were included. Strategies were non-operative management (NOM), early total care (ETC), and damage-control orthopedics (DCO). Outcomes included treatment allocation, fixation timing, and in-hospital mortality.

RESULTS: 985 patients were included (mean age 52.5, SD 26.3 years; ISS 27.8, SD 8.1). Allocation was NOM in 320 (32.5%), ETC in 336 (34.1%), and DCO in 329 (33.4%) patients. Head AIS was 3 in 48.5%, 4 in 31.1%, and 5 in 20.3%. NOM patients were older, had the highest ISS and estimated mortality, and showed the largest proportion of critical TBI (AIS 5: NOM 30.9%, ETC 14.3%, DCO 16.1%). Femoral ETC was mainly performed within the first day (median 0, IQR 0-1 days), whereas pelvic ETC was delayed with increasing TBI severity (median 3, IQR 0-5 days for head AIS 3; 5, IQR 0-7 days for AIS 4). Observed mortality was 37.2% after NOM, 9.2% after ETC, and 10.3% after DCO.

CONCLUSION: ETC in patients with moderate TBI (AIS 3) was associated with reduced observed mortality relative to NOM and matching DCO. Increase [...]

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From single to dual platelet inhibition FIBTEM assay: fibrinogen replacement thresholds for critical bleeding after trauma.

Latona A, Winearls J, Ho A, Mitra B

PURPOSE: The FIBTEM assay in Rotational Thromboelastometry (ROTEM®) measures fibrin-based clot formation. The reagent transitioned from single to dual-platelet inhibition with addition of tirofiban to cytochalasin D, offering more complete platelet suppression. The aim of this study was to assess the impact of this change on transfusion thresholds in trauma.

METHODS: In this retrospective cohort study, we included adult patients with major trauma across Queensland who presented between 2022 and 2025 and had results available for paired ROTEM and conventional coagulation tests on arrival. Patients were sub-grouped into single and dual-platelet inhibition assay groups. Correlations between FIBTEM-A5 and Clauss fibrinogen were analysed, comparing correlation strength and diagnostic accuracy for fibrinogen levels of < 2.0 g/L.

RESULTS: The relationship between FIBTEM-A5 and Clauss fibrinogen was similar using single platelet inhibition assays (R^2 0.61; 95%CI: 0.51-0.70) and dual-platelet inhibition assays (R^2 0.70; 95%CI: 0.62-0.77; $p = 0.94$).

Predicted FIBTEM-A5 amplitudes at 2 g/L fibrinogen were 7.83 mm (95%CI: 7.42-8.23) and 7.54 mm (95%CI: 7.22-7.87). Diagnostic performance for detecting fibrinogen < 2.0 g/L was comparable between assays, for both FIBTEM-A5 ≤ 10 mm and FIBTEM-A5 ≤ 8 mm (all p > 0.05). For the dual-platelet inhibition assay, sensitivity and specificity were 97. [...]

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Pattern of blast injuries. A systematic review: Part 2 - Landmines, unexploded ordnance and terrorism.

Neubert A, Gaeth C, Seidelmann M, Bieler D

PURPOSE: This systematic review analyzes the patterns and distribution of blast injuries from landmine explosions, UXOs and terror attacks including suicide bombing, in civilians and military personnel.

METHODS: A search was conducted in two databases on 26. March 2025 for observational studies reporting blast injuries in civilian and military populations of all ages. Studies were clustered by explosion typ. A narrative data synthesis was performed. The JBI tool for case series was used.

PROSPERO: CRD42023391736.

RESULTS: 126 studies were included in the overall systematic review. 45 studies on terror & suicide bombings and 12 studies on landmines & UXOs are analyzed in the present paper. Studies on landmines from 1980 to 2006 included 13,270 individuals (age: 24.0 years (Range 8.2 years - 32.2 years); mainly male (88.9%)) in post-conflict low- and middle-income countries. Lower extremity injuries including amputations were most frequent, followed by eye injuries. Studies on terrorist attacks with a total of 12,518 mainly civilian victims (average age: 33,1 years; 61.6% males) reported on incidents in post-conflict regions and high-income cities mainly from the early 2000s. The injury pattern was multidimensional, with extremity injuries most frequent (28%-30%), including fractures, lacerations, and traumatic amputations (18%-20%).

CONCLUSION: This Systematic Review Part 2 [...]

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Is nail plate fixation the way forward for distal femur fractures with long segmental metaphyseal comminution? - A study of 56 patients.

Mahesan H, Sundaram VP, Devendra A, Ramesh P, Dheenadhayalan J, Rajasekaran S

BACKGROUND: Mechanical instability due to long metaphyseal comminution and medial bone gap after lateral locking plate results in varus collapse and implant failure. We believe that fixation in the form of Nail and plate combination provides better stability and prevents complications.

MATERIALS: We performed a retrospective analysis of 56 distal femur fractures with long metaphyseal comminution treated in our institution with nail and plate construct from 2017 to 2023. The mean age was 43.1 years and the mean follow up was 24 Months (4-36 months). Of these 56 patients, 16 were closed injuries underwent primary fixation, and 40 were open injuries requiring staged reconstruction. At the final follow up, patients were assessed for shortening, knee range of movements, Sander et al. functional score, radiological union and loss in alignment.

RESULTS: The mean length of metaphyseal comminution was 10.15 cm(5-18 cm). Primary Iliac crest bone grafting was done in 25 patients. Of the 56 patients, 51 achieved primary union and the mean time to union was 6 months. Of the remaining, 3 patients united after secondary bone grafting and 2 patients had nonunion. The mean loss in aLDFA at the final follow up was 1.75° in 7 patients. Shortening of the limb was noted in 16 patients, and 4 patients had significant shortening of 2 cm. Three patients developed deep SSI, of which one patient requi [...]

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Impact of modified ERAS protocol implementation on length of stay and complications in elderly hip fracture patients: a before-and-after study.

Liu Y, Ou G, Xie M, Wu F, Wang X, Yang B

AIM: Hip fractures in elderly patients pose significant healthcare challenges with high complication rates and prolonged hospital stays. Enhanced Recovery After Surgery (ERAS) protocols have shown promise in various

surgical specialties, but their application in elderly hip fracture patients, particularly in Traditional Chinese Medicine (TCM) hospital settings, remains limited.

METHODS: This retrospective before-and-after study was conducted at a TCM hospital from July 2022 to July 2025. A total of 300 elderly hip fracture patients (≥ 65 years) were included: 146 in the control group (July 2022 - October 2023) receiving conventional care and 154 in the ERAS group (February 2024 - July 2025) receiving modified ERAS protocol incorporating TCM elements. Primary outcomes were length of hospital stay and perioperative complications within 30 days. Secondary outcomes included functional recovery [Harris Hip Score (HHS), Modified Barthel Index (MBI)], time to first mobilization, pain scores, and healthcare costs.

RESULTS: The ERAS group demonstrated significantly shorter length of stay compared with controls (12.38 ± 3.75 vs. 16.24 ± 5.10 days; mean difference 3.86 days, 95% CI: 2.84-4.88; $P < 0.001$). Total perioperative complications were reduced (7.79% vs. 15.07%; risk difference: -7.28%, 95% CI: -14.5% to -0.1%; $P = 0.047$). The ERAS group showed superior functional outcomes with [...]

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Prehospital endotracheal intubation and 30-day survival in severe trauma: a matched cohort study from Navarra, Spain.

Zulet Murillo D, Ferraz Torres M, Fortún Moral M, Belzunegui Otano T, Tamayo Rodríguez I

PURPOSE: Prehospital endotracheal intubation (ETI) is frequently used in major trauma, but its association with 30-day survival remains debated. We evaluated the association between prehospital ETI and 30-day survival and assessed whether sex modifies the age-related increase in mortality risk.

METHODS: We conducted a retrospective cohort study using the Navarra Major Trauma Registry (2010-2019). Patients aged ≥ 15 years with New Injury Severity Score (NISS) ≥ 15 were included; patients who died before hospital admission or had incomplete records were excluded. To mitigate confounding by indication, intubated and non-intubated patients were matched 1:1 using prehospital Glasgow Coma Scale (GCS), respiratory distress, and prehospital cardiac arrest as matching variables. Because the proportional hazards assumption for Cox regression was not met, parametric Weibull survival models were fitted, including a sex-by-age interaction term.

RESULTS: Overall, 1,909 patients were included; 212 (11.1%) underwent prehospital ETI. Matching yielded 190 pairs ($n=380$). In the matched cohort, prehospital ETI was not independently associated with 30-day survival (adjusted HR 0.91, 95% CI 0.65-1.27). Mortality risk was independently associated with lower GCS (per 1-point increase: HR 0.86, 95% CI 0.82-0.91) and respiratory distress (HR 7.63, 95% CI 4.34-13.4). A significant sex-by-age interaction [...]

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Mechanical and manual cardiopulmonary resuscitation in traumatic cardiac arrest: a retrospective observational study.

El-Menyar A, Ahmed K, Howland IR, Mollazehi M, Mekkodathil A, Rizoli S, Cameron P, de Oliveira Manoel AL et al.

BACKGROUND: Prehospital Traumatic cardiac arrest has a high fatality rate despite advances in trauma systems and resuscitation strategies. Mechanical chest compression devices have been increasingly adopted to optimize cardiopulmonary resuscitation (CPR) during Emergency Medical Services (EMS) transportation. We aimed to evaluate the impact of CPR (mechanical during transportation versus manual CPR only at the scene) on survival among trauma patients.

METHODS: A retrospective analysis was conducted for patients who received CPR at the scene and during transportation to the hospital between 2016 and 2024.

RESULTS: A total of 610 patients sustaining blunt traumatic cardiac arrest were included. The mean age was 34 ± 12.6 years; 94.6% were male, and 5.4% were female patients. Two hundred twenty (36.1%) received mechanical CPR during transportation to the hospital, and 390 (63.9%) received manual CPR only at the scene. Compared with the manual CPR group, the mechanical CPR group had higher rates of primary chest injury (30.5% vs. 20.5%; $p = 0.006$), bystander CPR (15.9% vs. 6.7%; $p = 0.001$), adrenaline administrations (95% vs. 26.4%; $p = 0.001$), and initial non-shockable rhythm (80% vs. 27.2%; $p = 0.001$). The two groups were comparable in median Injury Severity Score (ISS), head Abbreviated Injury Score (AIS), and total transport time. Mechanical CPR was associated with a markedly [...]

Cervical spine clearance in pediatric trauma patients - Consensus algorithm of the Pediatric Spinal Trauma Group of the Spine Section of the German Society for Orthopedic and Trauma Surgery (DGOU).

Bolte J, Rüther H, Brecht P, Wiersbicki DW, Youssef Y, Jarvers JS, Disch AC

PURPOSE: Although cervical spine injuries in pediatric patients are rare, clinical and radiological examinations are clinical routine. Currently, no standardized decision algorithm does exist for detection of these injuries in the European region. The proposed algorithm is aiming to safely identify significant injuries through consistent diagnostics while minimizing immobilization and radiation exposure at the same time.

METHODS: The Pediatric Spinal Trauma Group of the Spine Section of the German Society for Orthopedic and Trauma Surgery (DGOU) developed an algorithm based on own scientific investigations and considering primary and secondary literature. The algorithm was developed through a formal consensus process and thereby aligns with international recommendations.

RESULTS: The algorithm uses the initial pediatric Glasgow Coma Scale (pGCS) to group patients to three treatment pathways. Resulting recommendations for imaging, clinical re-assessment, and further management include the child's age, trauma mechanism, and clinical evaluations.

CONCLUSION: Despite their rarity, potential cervical spine injury in children requires accurate diagnosis. Clinical examinations can be challenging, especially in children with compromised consciousness. Increased radiation exposure from X-rays and computer tomography (CT) scans must be considered, particularly as magnetic resonance im [...]

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Development of a low-cost external fixation system: an off-label solution for resource-limited settings.

Seiser M, Deininger C, Fierlbeck J, Hollensteiner M, Coles P, Reuter C, Tempfer H, Traweger A et al.

PURPOSE: External fixators are essential for the surgical management of long-bone fractures, particularly in resource-limited settings where access to commercial systems is often restricted by cost and logistics. This study aimed to develop a low-cost external fixation concept using widely available construction materials and evaluate its biomechanical performance under axial loading.

METHODS: An external fixator composed of M10 threaded rods, washers, and nuts was developed. Five configurations were tested: SP (Steinmann pins), KT (threaded K-wires), KS (smooth K-wires), and BKS (bilaterally applied, smooth K-wires). A commercial external fixator served as control (CG). Juvenile bovine ulnae with a standardized fracture gap were subjected to cyclic axial loading. Construct stiffness, elastic deformation, and plastic deformation were assessed.

RESULTS: CG showed the highest stiffness (107.80 ± 14.12 N/mm), followed by SP (87.73 ± 17.92 N/mm), BKS (83.81 ± 10.01 N/mm), KS (49.84 ± 3.80 N/mm), and KT (46.66 ± 2.39 N/mm). Post-hoc analyses showed no significant differences between CG vs. SP and CG vs. BKS, whereas KT and KS were significantly less stiff. Elastic deformation followed a similar pattern. Plastic deformation was greatest in CG, while SP and BKS showed significantly lower values.

CONCLUSIONS: The threaded-rod fixator provides relevant mechanical stability at extreme [...]

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Supplementary fixation in multifragmentary midshaft clavicle fractures: do interfragmentary screws or suture cerclage improve outcomes compared with plate-only fixation?

Barthel C, Hinkelmann MA, Schibler D, Frei F, Pape HC, Allemann F

BACKGROUND: Clavicle fractures are common injuries. As displaced or multifragmentary midshaft patterns carry a substantial risk of malunion and nonunion with nonoperative treatment, they have contributed to the increasing use of surgical fixation. Modern operative techniques include plate fixation, intramedullary devices, and hybrid strategies to address comminution and improve fragment stability. Supplementary interfragmentary screws and

cerclage constructs have been proposed to enhance fragment control and optimize load transfer in multifragmentary fracture patterns. This study aimed to compare three fixation strategies for operatively treated midshaft clavicle fractures managed with plate osteosynthesis: Group IS (interfragmentary screw), Group CA (cerclage augmentation), and Group POF (plate-only fixation), with respect to operative time, complication rates, and potential biological and mechanical differences.

METHODS: This retrospective study included patients aged ≥ 16 years who underwent plate fixation for multifragmentary clavicle fractures between 2016 and 2024. Patients were categorized into three groups: Group IS (supplementary interfragmentary screw), Group CA (suture cerclage augmentation), and Group POF (plate-only fixation). Demographic, surgical, and radiographic data were collected. Callus formation was assessed by three independent observers. Statistical anal [...]

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Cerebral glycerol during haemorrhagic shock in normal and raised intracranial pressure resuscitated with total REBOA: an experimental porcine study.

Bader S, Magnuson A, Brorsson C, Wallin G, Löfgren N, Löfgren F, Blind PJ, Öman M et al.

BACKGROUND: Haemorrhagic shock (HS) is frequently associated with secondary cerebral injury due to compromised cerebral perfusion. Resuscitative endovascular balloon occlusion of the aorta (REBOA) is an effective haemorrhage control strategy; however, concerns persist regarding its cerebral effects, particularly in the presence of elevated intracranial pressure (ICP). Cerebral microdialysis (CMD) derived glycerol (CGly) is a recognised marker of cellular membrane stress and injury.

OBJECTIVE: To investigate CGly dynamics during prolonged total REBOA (tREBOA) in HS and to compare cerebral cellular responses between animals with normal and elevated ICP.

METHODS: In this experimental porcine study, eighteen pigs were subjected to controlled HS and resuscitated with tREBOA for 90 min. Animals were allocated to either a normal ICP group (NICPG, $n = 9$) or an elevated ICP group (EICPG, $n = 9$). Continuous monitoring of proximal arterial pressure, ICP, and cerebral perfusion pressure was performed. CGly concentrations were measured using CMD throughout the experimental protocol.

RESULTS: tREBOA effectively restored proximal arterial pressure and cerebral perfusion pressure in both groups. CGly concentrations remained relatively stable during the haemorrhage phase and increased progressively during prolonged aortic occlusion. Importantly, CGly responses did not differ significantly be [...]

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Transport and destination hospital for patients with suspected multiple and/or severe injuries - a systematic review and clinical practice guideline update.

Pavlu F, Könsgen N, Bieler D, Goossen K, Gliwitzky B, Häske D, Faul P, Wöfl C et al.

PURPOSE: Our aim was to update the evidence-based and consensus-based recommendations on criteria for transport and destination hospital for patients with suspected multiple and/or severe injuries based on available evidence. This guideline topic is part of the 2025 update of the German Guideline on the Treatment of Patients with Multiple and/or Severe Injuries.

METHODS: MEDLINE and Embase were systematically searched to January/February 2024. Further literature reports were obtained from clinical experts. Randomised controlled trials (RCTs) or observational studies reporting risk-adjusted outcomes were included if they compared different transport strategies or destination hospitals for patients with (suspected) multiple and/or severe injuries. We considered patient-relevant outcomes such as mortality and neurological outcomes. Risk of bias was assessed at outcome level using ROBINS-I for observational studies and RoB 2 for RCTs. We conducted meta-analyses if possible; alternatively, we synthesised the evidence narratively. We used GRADE to rate the certainty of evidence. Expert consensus was used to develop recommendations and determine their strength.

RESULTS: A total of 127 studies were identified. They addressed helicopter vs. ground emergency medical services (EMS) transport, ground EMS with vs. without a physician, on-scene time, and handover on arrival at the hospital [...]

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Does the addition of peripheral nerve blocks improve analgesia during distal radius fracture reduction? A prospective, randomized, multicenter clinical trial.

Guerrero-Serrano JA, Santamaría-López A, García-Lamas L, Jiménez-Díaz V, González-Fernández Á, Cecilia D

PURPOSE: This study aimed to evaluate whether adding a median nerve and a superficial branch of the radial nerve block to hematoma block improves analgesia during distal radius fracture reduction and to identify factors associated with increased pain perception.

METHODS: A prospective, randomized, controlled study was conducted in two tertiary hospitals, including 180 adult patients with displaced distal radius fractures requiring closed reduction. Patients were allocated either to an isolated hematoma block group or to a combined block group receiving hematoma block with median nerve and superficial branch of the radial nerve blocks. Within the combined group, half of the median nerve blocks were performed under ultrasound guidance and half using anatomical landmarks. The primary endpoint was digital pain during traction and fracture reduction, assessed using a visual analog scale (VAS, 0-10). Secondary outcomes included wrist pain at different procedural stages and the influence of demographic and clinical variables including age, sex, laterality, participating hospital, body mass index (BMI), fracture type (AO/OTA), and baseline psychiatric or chronic analgesic medication use.

RESULTS: Digital pain was significantly lower in combined group compared with isolated hematoma block during traction (4.54 ± 2.53 vs. 7.39 ± 1.76 ; $p < 0.001$) and reduction (5.36 ± 2.56 vs. 8.33 ± 1.1). [...]

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Under pressure - a sensor-based analysis of simulated prehospital pressure dressing application for hemorrhage management.

Brauckmann V, Menzel J, Welke B, Decker S, Macke C

BACKGROUND: The concept of "x-ABCDE", with "X" representing exsanguinating hemorrhage, highlights the priority of immediate bleeding control in trauma care. Despite being a fundamental intervention, pressure dressing techniques lack standardization. Studies show high variability in achieved pressures, often below therapeutic targets or exceeding harmful levels, with no correlation between subjective estimates and objective measurements.

OBJECTIVES: To objectively assess current practices of pressure bandage application and identify influencing factors including material type, technique, and provider experience.

METHODS: This prospective, single-center, simulation-based observational study was conducted from August 2024 to January 2025 at a Level I Trauma Center. Emergency medical providers applied pressure dressings according to personal preference using available materials. Pressure distribution was measured via two calibrated capacitive pressure sensors (61 measurement fields). Analysis included correlation tests, group comparisons, and multiple regression.

RESULTS: A total of 124 emergency medical providers (75% male; 36% paramedics, 44% emergency medical technicians, 13% trainees, 7% emergency physicians) completed pressure dressing applications, with 116 datasets included in final analysis. Mean maximum pressure was 169.35 ± 84.33 mmHg (range 50-412 mmHg) with applicati [...]

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Clinical impact of early postoperative routine radiology in isolated pelvic injuries: a retrospective study.

Bolierakis E, Ioannou P, Schubert SL, Groven RVM, Bläsius FM, Bode M, Alabdulrahman H, Hildebrand F et al.

PURPOSE: The clinical value of routine immediate postoperative conventional radiography following surgical fixation of pelvic fractures remains debated. This study assessed the impact of early postoperative X-rays on management decisions and revision surgery in patients with isolated pelvic ring or acetabular fractures.

METHODS: We conducted a retrospective cohort study at a Level 1 trauma center, including adults undergoing surgical treatment for isolated pelvic ring or acetabular fractures between 2020 and 2023. All patients received high-quality intraoperative fluoroscopy and postoperative conventional radiographs. The primary outcome was any change in standard postoperative care based on radiographic findings. The secondary outcome was revision

surgery prompted by conventional postoperative imaging.

RESULTS: A total of 216 patients were included (median age 63 years), with 156 pelvic ring injuries and 60 acetabular fractures. Postoperative radiographs were obtained at a median of 3 days post-surgery. Changes in postoperative care based on radiographs occurred in 2 patients (3.3%) within the acetabular fracture group. Revision surgery triggered by conventional imaging was required in 1 patient (0.6%) among pelvic ring injuries. In all cases, additional CT imaging preceded definitive management changes.

CONCLUSION: Routine immediate postoperative conventional radiography a [...]

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Older age and post-traumatic organ failure a TraumaRegister DGU® analysis of 34,469 elderly major trauma patients.

Bewersdorf TN, Lefering R, Stein S, Streblow J, Findeisen S, Schmidmaier G, Grossner T

PURPOSE: As the number of elderly major trauma patients (EMTP) increases continuously, this study sought to determine the incidence of organ failure (OF) and multiple organ failure (MOF) and their impact on mortality and recovery status in this cohort.

METHODS: Based on the German TraumaRegister DGU® database this study included EMTP aged between 65 and 99 years with an Injury Severity Score (ISS) of ≥ 16 between 2014 and 2023. OF was defined as Sequential Organ Failure Assessment (SOFA) score of ≥ 3 and MOF as failure of ≥ 2 organs.

RESULTS: The database revealed 34,469 EMTP, here 37% suffered from OF (16.0%) or MOF (21.0%). Mortality was significantly higher in OF and MOF patients compared to non-OF patients (non-OF: 7.4%; OF: 45.0%; MOF: 59.0%; $p < 0.001$). 80.9% of all deaths were associated with failure of at least one organ and 51.1% of non-survivors suffered from MOF. Highest odds ratios (OR) for mortality were found for liver (OR: 5.13, $p < 0.001$), central nervous system (OR: 3.63, $p < 0.001$) and renal failure (OR: 3.25, $p < 0.001$), followed by cardiovascular (OR: 1.95, $p < 0.001$) and lung failure (OR: 1.16, $p = 0.010$). Haemostatic failure had no impact on mortality.

CONCLUSION: OR and mortality differ significantly between distinct OFs due to varying degrees of success in treatment options and systemic impact of these OFs. Clinicians should be aware that OF and MOF o [...]

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Denis pattern-dependent biomechanical behavior of posterior trans-iliac plate fixation compared with bilateral triangular osteosynthesis in unilateral sacral fractures: a finite element study.

Yang J, Böker KO, Li C, Schilling AF, Ritter Z, Acharya M, Lehmann W

OBJECTIVE: Posterior trans-iliac plate osteosynthesis (TPO) is commonly used for the stabilization of unilateral vertically unstable sacral fractures. However, its biomechanical performance across different Denis fracture patterns remains unclear. This study aimed to investigate the fracture pattern-dependent biomechanical behavior of posterior trans-iliac plate osteosynthesis (TPO) in unilateral vertically unstable sacral fractures, using bilateral triangular osteosynthesis (BTO) as a high-stability reference construct.

METHODS: A validated three-dimensional finite element model of the lumbopelvic complex was developed to simulate unilateral vertically unstable sacral fractures corresponding to Denis type I, II, and III patterns. Two posterior fixation strategies, TPO and BTO, were constructed for each fracture type. Seven physiological loading conditions were applied, including standing, flexion, extension, axial rotation, and lateral bending. Von Mises stress distribution and fracture displacement were evaluated. Fracture micromotion was quantified by calculating relative displacement changes between paired points along the fracture gap.

RESULTS: When interpreted relative to the high-stability BTO reference construct, TPO showed fracture pattern-dependent variation in fracture displacement. The difference was most pronounced in Denis type I fractures and progressively decr [...]

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Factors associated with in-hospital mortality in necrotising soft tissue infections. a multicentre retrospective cohort study.

Podda M, Ceresoli M, Viridis F, Rosa F, Pilia T, Murzi V, Dessì A, Tedesco S et al.

PURPOSE: Necrotising soft tissue infections (NSTIs) are rare but life-threatening conditions associated with high mortality rates. This multicentre study aimed to identify admission variables associated with in-hospital mortality.

METHODS: This retrospective study included adult patients with surgically confirmed NSTIs treated at four high-volume academic referral centres in Italy between 2010 and 2024. Demographic, clinical, physiological, and laboratory variables available at hospital admission were analysed. Categorical variables were compared between survivors and non-survivors using the chi-square test or Fisher's exact test, while quantitative variables were compared using the Student's t test or Mann-Whitney U test. Multivariable logistic regression analyses were performed to identify independent predictors of in-hospital mortality. Results were reported as ORs with 95%CI. A prognostic nomogram was developed from the final multivariable model, including variables independently associated with mortality.

RESULTS: A total of 379 patients were included. In-hospital mortality was 16.7%. In subgroup comparisons, mortality was 17.0% in necrotising fasciitis and 22.4% in Fournier's gangrene ($P = 0.275$). In the overall NSTI population, age (aOR 1.061, 95%CI 1.037-1.087) and serum creatinine at admission (aOR 1.301, 95%CI 1.040-1.629) were independently associated with mortality [...]

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Influence of frailty on short-term mortality in older patients with multiple trauma in the emergency department.

Çelik ■, Tokar A, Biçer S, Cebecio■lu ■K

PURPOSE: To investigate the association between frailty and 28-day mortality in older patients presenting to the emergency department with multiple trauma and to assess the discriminative performance of PRISMA-7 and the Trauma-Specific Frailty Index (TSFI) in frailty assessment in relation to commonly used trauma severity scores.

METHODS: This prospective observational study included patients aged ≥ 65 years with multiple trauma admitted to the emergency department between July 1 and August 31, 2024. Frailty was assessed using PRISMA-7 and TSFI. Patients were stratified into frail and non-frail groups and further categorized according to age groups (65-74, 75-84, and ≥ 85 years). Clinical characteristics, trauma severity scores, and outcomes, including 28-day mortality, were compared. Receiver operating characteristic (ROC) analysis was used to evaluate discriminative performance, and logistic regression analysis was performed to assess the association between scores and mortality.

RESULTS: A total of 144 patients were included. Frailty was significantly associated with increased 28-day mortality, higher ICU admission rates, and longer hospital stays ($p < 0.001$). Mortality increased with age, reaching 30.0% in patients aged ≥ 85 years ($p = 0.006$). Frailty scores (PRISMA-7 and TSFI) increased significantly with age, whereas ISS did not differ significantly across age groups. R [...]

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Mortality prediction in older adults with earthquake-related trauma: a model combining MEWS and laboratory biomarkers.

Ozen A, Acehan S, Satar S, Bulut EA, Gulen M, N■g■z K, Sevd■mbas S, Sonmez GO et al.

PURPOSE: This study aimed to identify laboratory parameters and clinical scores that can predict in-hospital mortality among patients aged 65 years and older who presented to the emergency department (ED) with earthquake-related trauma following the February 6 and 20, 2023 earthquakes.

METHODS: In this retrospective observational study, 362 patients with earthquake-related trauma between February 6-21, 2023, were included. Demographic characteristics, laboratory results, and clinical scores such as the Modified Early Warning Score (MEWS), Injury Severity Score (ISS), and Shock Index (SI) were analyzed.

RESULTS: Of the included patients, 53.9% ($n = 195$) were female, and the median age was 73 years (IQR: 69-80). The overall mortality rate was 9.4%. Multivariable logistic regression analysis identified MEWS (OR: 3.535; 95% CI: 2.397-5.212; $p < 0.001$), urea (OR: 1.009; 95% CI: 1.002-1.015; $p = 0.008$), and hs-Troponin I (OR: 1.001; 95% CI: 1.000-1.002; $p = 0.016$) as independent predictors of mortality. Receiver operating characteristic (ROC) analysis demonstrated an AUC of 0.936 for the combined model including MEWS, urea, and hs-Troponin I (95% CI: 0.875-0.996; $p < 0.001$). Among the variables, MEWS showed the highest AUC (0.903; 95% CI: 0.829-0.976).

The optimal cutoff value for MEWS was determined as 1.5, with a sensitivity of 70.6% and a specificity of 97.6%.

CONCLUSIONS: In o [...]

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The economic burden in terms of cost of illness and generic health-related quality of life of posttraumatic long bone non-unions among the adult population of the Netherlands from a societal perspective.

van der Broeck LCA, Shchurov DG, Gabrio A, Lodewijks AJL, Poeze M, Blokhuis TJ, Evers S

PURPOSE: This study assessed the societal economic burden in terms of cost of illness and health-related quality of life (HRQoL) of posttraumatic long bone non-unions in the Netherlands.

METHODS: An incidence-based bottom-up approach was used, focusing on adult patients with posttraumatic long bone non-unions. The analysis included healthcare costs, patient and family costs, and productivity losses, measured using the IMCQ and IPCQ questionnaires. Cost evaluations followed Dutch costing guidelines, and productivity losses were calculated using the friction cost method. HRQoL was assessed with the EQ-5D-5L. A deterministic one-way sensitivity analysis varied baseline characteristics by $\pm 10\%$. Scenario analyses were conducted from healthcare and patient perspectives, as well as for patient subgroups.

RESULTS: Average costs per patient during the three months before the initial visit at a non-union clinic were €8,928 (healthcare, N = 78), €1,360 (patient and family, N = 58), and €4,313 (productivity losses, N = 59), the average of the observed data calculates to 10,831 (N = 44). The mean EQ-5D-5L utility score was 0.390 (± 0.29 SD). Subgroup analysis showed no significant cost increase for patients with infections or open fractures. However, a higher Non-Union Severity Score and a lower HRQoL were significantly associated with higher total costs.

CONCLUSION: Posttraumatic long [...]

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Nonoperative treatment of Maisonneuve fractures.

Frühauf J, Bartonířek J, Fojtík P, Horáková A, Tuřek M

PURPOSE: The aim of this study is to describe the basic pathoanatomical characteristics of a stable Maisonneuve fracture and mid-term results of its nonoperative treatment.

METHODS: The study included 17 prospectively collected patients with a mean age of 59 years. The postinjury ankle CT had to meet the following criteria: nondisplaced or minimally displaced (up to 1 mm) fracture of medial malleolus, medial clear space less than 3 mm, nondisplaced or minimally displaced (up to 2 mm) fracture of posterior malleolus, anatomical position or minimal malposition of the distal fibula in the fibular notch (widening of the tibiofibular space up to 2 mm or external rotation of the distal fibula up to 10°). The average follow-up period was 34 months, the final follow-up included CT examination and functional evaluation based on AOFAS and FAOS scores.

RESULTS: A medial malleolus fracture was recorded in 12% cases, a posterior malleolus fracture in 29% patients and a Tillaux-Chaput tubercle fracture in 18% cases. All fractures of the proximal fibula, medial and posterior malleolus healed within 3 months. The position of the distal fibula in the fibular notch did not change in 11 cases compared to the post-injury CT scan, improved slightly in 5 cases, and worsened slightly in 1 case. The average final AOFAS hindfoot score was 96.9 points and the average final FAOS score was 98.7%.

CONCL [...]

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Age-related differences in prehospital triage and critical interventions: a multicenter TraumaRegister DGU® analysis.

Koch DA, Hagebusch P, Hackenberg L, Pavlu F, Jakobi T, Münzberg M, Lefering R, Schweigkofler U et al.

PURPOSE: Older trauma patients are known to be undertriaged in the prehospital setting. However, it remains unclear whether these disparities translate into differences in the delivery of critical prehospital interventions. This

study aimed to evaluate age-related differences in prehospital triage, allocation, and treatment within a physician-staffed emergency medical service system.

METHODS: A retrospective multicenter analysis of the TraumaRegister DGU® was conducted, including trauma patients aged ≥ 20 years between 2020 and 2023 with trauma team activation and subsequent intensive care unit admission. Patients were stratified into two age groups, 20-59 vs. ≥ 60 years, with additional subgroups up to 90-99 years. Prehospital interventions, transport modality, and trauma center allocation were analyzed. Furthermore, predefined high-risk scenarios (severe motor vehicle trauma, traumatic brain injury with $GCS \leq 8$, and pneumothorax) were evaluated.

RESULTS: A total of 67,069 patients were included, with comparable injury severity across age groups (mean ISS 20.2). Older patients were less frequently transported by air and less often allocated to supra-regional trauma centers. While overall intervention rates were similar, differences emerged in high-risk scenarios. Older patients received endotracheal intubation and tranexamic acid less frequently despite comparable injury severity [...]

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Prediction of risk of death in severely injured patients: the revised injury severity classification score, version 3 (RISC III).

Lefering R, Imach S, Bieler D

PURPOSE: Trauma Registries with a focus on severely injured patients use survival as their primary outcome. In order to compare hospitals, interventions, and changes over time, a precise risk of death prediction is mandatory. The German TraumaRegister DGU® uses the RISC II model (Revised Injury Severity Classification, version II) since 2013. The aging trauma population and an improved handling of missing data required the present revision.

METHODS: A total of 53,738 seriously injured trauma patients documented in 2022-2023 served as basis for development and validation (3:1 ratio). Missing values should now be considered to be within normal physiological range except in patients with specific findings indicative for an altered physiology. These findings were suggested by clinical experts and validated by registry data. The increasing age of trauma patients was addressed by additional categories and higher point weights. A logistic regression analysis provided new point weights for all predictors. Precision (observed versus predicted mortality) and discrimination (area under the receiver operating characteristic curve, AUROC) were calculated in the development and validation dataset.

RESULTS: Patients in both datasets were well comparable, with a mean age of 55 years, 69% males, and an average Injury Severity Score (ISS) of 18 points. The rate of missing values ranged from 0% [...]

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Dislodgement forces and cost effectiveness of prehospital securement of peripheral intravenous catheters on moist skin: a randomized controlled clinical trial in healthy volunteers.

Vogeler J, Schumann S, Heinrich S, Hell J, Schmutz A

OBJECTIVES: Peripheral intravenous catheters (PIVC) are widely used clinical devices for administering fluids or essential drugs to patients. The failure rate of PIVC remains high and Emergency Medicine has been shown to be a risk factor for dislodgement. When the skin is moist from sweat or fluids, standard intra-hospital dressings and securements fail. In emergency situations, a failed catheter can then critically delay intravenous therapies. The most effective dressing to prevent accidental removal of a prehospital PIVC remains unclear. It was the aim of this study to compare the force required to dislodge a PIVC with four different methods of securing PIVCs used in emergency medicine. In addition, the costs were calculated.

METHODS: Artificial sweat was applied to the skin of 180 volunteers. PIVCs were attached onto the forearm using four different securements (elastic gauze, cohesive gauze, clingfilm and a velcro securing device). Continuously increasing traction force was applied until dislodgement of the respective securement. For statistical tests, either Friedman's test or repeated measures ANOVA was used.

RESULTS: Clingfilm showed the greatest resistance to increasing pulling force with cohesive bandages as close second. The velcro securing device was strongest at resisting low level of forces but fell off sharply at higher force. Elastic bandages were the weakest i [...]

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Injury to the thorax is a dominant contributor to post-traumatic neutrophil mediated inflammatory response.

van Eerten FJC, Duebel LV, de Fraiture EJ, Hanlo MF, Breuking EA, Brands SR, van den Bosch TD, van Wessem KJP et al.

PURPOSE: Traumatic injury induces a neutrophil associated inflammatory response mediated by the release of damage-associated molecular patterns (DAMPs). This response can be maladaptive, increasing risk of infection. Since blunt thoracic injuries are associated with high infection rates, we hypothesized that these injuries elicit a stronger neutrophil associated inflammatory response than injuries to other body regions.

METHODS: A cohort study was conducted in adult patients presenting at the trauma bay of a level 1 trauma center between March 2020 and November 2022. These patients underwent routine analysis of their neutrophil associated inflammatory response by phenotyping blood neutrophils using automated flow cytometry. Injury per body region was counted when the Abbreviated Injury Severity (AIS) scores were ≥ 3 to focus on substantial injuries. The primary outcome was an extensive post-traumatic inflammatory neutrophil response.

RESULTS: 861 patients were included. Blunt internal thoracic injuries showed the strongest association with severe neutrophil associated inflammation (OR 5.7), followed by abdominal (OR 2.6), pelvis (OR 2.5) and lower extremities (OR 2.3), even when corrected for ISS, age and hemodynamic instability. In multivariable analysis, only thoracic internal injuries remained a statistically significant contributor to excessive neutrophil associated inflammation [...]

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Major improvement using peroperative 3D compared to 2D imaging for SI screw fixation in traumatic pelvic fractures.

van Eerten FJC, Benders KEM, van Baal MCPM, Hietbrink F

INTRODUCTION: Pelvic fractures, though accounting for only 3% of skeletal injuries, carry a high mortality rate, primarily due to the associated risk of hemodynamic instability. Sacroiliac (SI) screw fixation is a common surgical intervention, which can be technically challenging due to the complex pelvic anatomy. Imaging techniques have evolved from 2D to 3D imaging for its improved accuracy in screw placement. The impact of this transition for SI screw placement was evaluated.

METHOD: A single-center observational retrospective study was performed between 2013 and 2023. Patients ≥ 18 years who underwent SI screw placement following pelvic injuries were analyzed. 2D imaging was used until 2021, after which 3D imaging was implemented. Outcomes included the need for revision surgery during initial admission and < 18 months after trauma.

RESULTS: 2D imaging was used in 42 patients and 3D imaging in 44 patients. Baseline characteristics and injury severity of both groups were comparable. In the 2D group, 18 patients (42.9%) had suboptimal placement of the SI screw. Of these, 3 (7.1%) required revision surgery due to malpositioned screws. In contrast, no patients (0.0%) in the 3D group had suboptimally positioned screws or underwent revision surgery, ($p < 0.001$, $p = 0.22$).

DISCUSSION: Intraoperative 3D imaging enhanced the accuracy of SI screw placement, leading to an encouraging [...]

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Minimally invasive versus open fixation for flail chest in elderly patients: a prospective cohort study on perioperative safety and long-term outcomes.

Tian Y, Zheng Z, Ji Z, Lu W, Song N, Wang J

OBJECTIVE: To systematically evaluate the perioperative safety and short- and long-term efficacy of minimally invasive fixation using a reversed shape-memory alloy plate versus conventional open fixation for the treatment of flail chest in elderly patients.

METHODS: This prospective cohort study enrolled 130 elderly patients (≥ 60 years) with flail chest between January 2022 and January 2026. After 1:1 propensity score matching, 100 patients were included (minimally invasive group, $n = 50$; open group, $n = 50$). Perioperative parameters (operative time, blood loss, C-reactive protein(CRP) at 24 h/48 h/72 h, complications), short-term outcomes (resting/cough-evoked Numerical Rating

Scale(NRS) pain scores, non-steroidal analgesic drug duration, rescue analgesic frequency, Patient-Controlled Analgesia Pump(PCA) consumption, time to ambulation, hospital stay), and long-term outcomes (implant dislodgement, 6-month pulmonary function, Douleur Neuropathique 4 Questions Naïve(DN4), Medical Outcomes Study 36- Item Short Form Health Survey(SF-36)) were compared.

RESULTS: The minimally invasive group had significantly fewer incision-related complications ($P = 0.046$), lower resting NRS scores on postoperative days 1 and 3 ($P < 0.01$; $P = 0.041$), lower cough-evoked NRS score on day 1 ($P = 0.011$), shorter non-steroidal analgesic duration ($P < 0.01$), and lower PCA consumption ($P = 0.027$). At 6 [...]

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Epidemiology and outcomes of traumatic sternal fractures and associated blunt cardiac injury: a nationwide cohort study in the Netherlands.

Klei DS, Benders KEM, Leenen LPH, van Wessem KJP

PURPOSE: Comprehensive data on epidemiology, trauma mechanisms, associated injuries, and outcomes of traumatic sternal fractures are scarce. This study analysed nationwide data to improve diagnosis and management within the Dutch healthcare system.

METHODS: This nationwide retrospective cohort study using the Dutch National Trauma Registry included adult patients admitted with traumatic sternal fractures between 2015 and 2023. Patients with prehospital cardiopulmonary resuscitation or penetrating trauma were excluded. Incidence, patient characteristics, trauma mechanisms, associated injuries, and in-hospital outcomes were analysed. Subgroup analyses evaluated patients with concomitant blunt cardiac injury (BCI).

RESULTS: Of 568,399 adult trauma admissions, 4,765 patients (0.84%) sustained traumatic sternal fractures. Median age was 62 years; 60% were male. Motor vehicle accidents (48%) and falls (28%) were the leading mechanisms. 35% were severely injured ($ISS \geq 16$). Associated injuries included rib fractures (51%), spinal fractures (36%), and lung contusions (18%). Critical care unit admission was 40%, with median mechanical ventilation duration of 4 days; median hospital stay was 5 days. In-hospital mortality was 5.7%, and 30-day mortality 6.0%. BCI occurred in 9.5% of patients and was associated with a higher number of injuries and increased injury severity, emergency inte [...]

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Trauma patients ≥ 65 years old constitute 40% of severely injured adult trauma patients in Sweden - a 10-year analysis of geriatric trauma prevalence.

Bredberg S, Modalen E, Jöneby C, Nilsson D, Lapidus O

BACKGROUND: The elderly population in Sweden is steadily increasing. Age is an independent predictor of mortality in trauma patients. There is growing evidence that geriatric patients constitute an increasing proportion of the Swedish trauma population. No studies have analyzed temporal trends in the proportion of geriatric trauma patients in the context of the national trauma population in Sweden.

AIM: To determine the proportion of geriatric patients in the Swedish trauma population and analyze temporal trends in geriatric trauma prevalence.

METHODS: The study cohort was 11,969 severely injured trauma patients ($NISS > 15$) treated in Sweden during 2013-2022, obtained from the Swedish Trauma Registry. The proportion of geriatric patients was compiled annually. Temporal trends were analyzed using weighted linear regression models. Two subgroups were defined based on injury severity ($NISS \geq 25$ and $NISS \geq 40$).

RESULTS: The proportion of patients ≥ 65 years in the adult trauma population increased from 30% to 40% during 2013-2022 ($p = 0.003$), while the proportion of patient ≥ 80 years also increased, from 8.6% to 17% ($p = 0.009$). Similar increases were also seen in the $NISS \geq 25$ and $NISS \geq 40$ subgroups.

CONCLUSION: There is likely an increase in geriatric trauma prevalence in Sweden, but the full extent of this temporal trend remains uncertain. Trauma patients ≥ 65 years old co [...]

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